



InterQual® Acute Adult Criteria

InterQual® 2024 March Release Clinical Revisions

Release date: March 22, 2024

Global revisions



Revision
Summary

This release includes updates to Decision Reasons in all subsets. Global changes were made to IV fluid and Saline hydration criteria to streamline the review process. Hyperosmolar therapy criteria rule was changed to clarify the intent of the criteria. Hyperkalemia criteria were updated to be more inclusive of additional interventions at the Acute level of care.

Disclaimer: The InterQual® criteria changes in the global revisions listed below may not appear as changed in the individual subset revisions due to the timing of finalizing the documents and locking down the final content. However, the global revisions listed below will be reflected in the final product for the March 2024 release.

Acute Adult	
Global	
Description of Changes	Subsets
Updates to Decision Reason content	All subsets

Acute Adult		
Global		
Previously Available Content	2024 March Release	Revision Description
♦IV fluid, One: ▶ ≥ 75 mL/h and, ≥ One: ♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ▶ ≥ 100 mL/h and weight ≥ 60 kg	♦IV fluid	Rationale: Rates were removed from the criteria to streamline the review process and allow for clinical judgment by the provider to determine an appropriate IV fluid rate excluding KVO

Acute Adult		
Global		
Previously Available Content	2024 March Release	Revision Description
▶Saline hydration, One: ♦≥ 75 mL/h and, ≥ One: ▶Weight < 60 kg ▶Age ≥ 65 ▶Renal insufficiency ▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg	▶Saline hydration	Rationale: Rates were removed from the criteria to streamline the review process and allow for clinical judgment by the provider to determine an appropriate IV fluid rate excluding KVO

Acute Adult		
Global (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
▶Hyperkalemia: ♦Intervention, ≥ One: ▶Calcium chloride ▶Calcium gluconate ▶Diuretic ▶IV fluid, One: ♦≥ 75 mL/h and, ≥ One: ▶Weight < 60 kg ▶Age ≥ 65 ▶Renal insufficiency ▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ▶Sodium polystyrene (Kayexalate) (PO or PR)	▶Hyperkalemia: ♦Intervention, ≥ One: ▶Calcium chloride or gluconate ▶Diuretic ▶Glucose 50%(0.50) with insulin ▶IV fluid ▶Potassium binding agent (includes PO or PR)	Rationale: - Hyperkalemia intervention criteria were updated to include Glucose 50% (0.50) with insulin as this is an appropriate intervention at the Acute level of care “Sodium polystyrene (Kayexalate)” criteria point was removed and “Potassium binding agent” criteria point was added to be more inclusive of additional potassium-binding medications

Acute Adult		
Global (CRITICAL)		
Previously Available Content	YEAR MONTH Release	Revision Description
•CRITICAL, ≥ One: ▸Cerebral edema and, ≥ One: ◆ICP monitoring (includes OR placement) ◆Corticosteroid ◆Hyperosmolar therapy, One: ▸Mannitol ▸Hypertonic saline ≥ 3%(0.03) solution	•CRITICAL, ≥ One: ▸Cerebral edema and, ≥ One: ◆ICP monitoring (includes OR placement) ◆Corticosteroid ◆Hyperosmolar therapy, ≥ One: ▸Mannitol ▸Hypertonic saline ≥ 3%(0.03) solution	Rationale: • Hyperosmolar therapy rule was changed to clarify the intent of the criteria

Acute Coronary Syndrome (ACS)



Revision Summary

Low risk criteria were restructured to streamline the review process at the Observation level of care.

Acute Coronary Syndrome (ACS)

Initial Review (OBSERVATION)

Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ▶ACS suspected and, All: ◆Need for diagnostic cardiac testing based on risk, One: ▶Low risk and, One: ◆ACS related symptoms and age ≥ 40 years ◆Dyspnea and, One: ▶Male and age ≥ 40 years ▶Female and age ≥ 50 years ▶Intermediate risk ◆ECG normal, unchanged, or non-diagnostic ◆Initial troponin negative or indeterminate and requiring serial troponins ◆Intervention scheduled or performed within 24h, One: ▶Cardiac catheterization ▶CCTA ▶Stress test	•OBSERVATION, One: ▶ACS suspected and, All: ◆Need for diagnostic cardiac testing based on risk, One: ▶Low risk and ACS related symptoms and age ≥ 40 years ▶Low risk and dyspnea and, One: ◆Male and age ≥ 40 years ◆Female and age ≥ 50 years ▶Intermediate risk ◆ECG normal, unchanged, or non-diagnostic ◆Initial troponin negative or indeterminate and requiring serial troponins ◆Intervention scheduled or performed within 24h, One: ▶Cardiac catheterization ▶CCTA ▶Stress test	Rationale: • Low risk criteria were restructured to streamline the review process

Acute Coronary Syndrome (ACS)

Episode Day 1 (OBSERVATION)

Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ▶ACS suspected and, All: ◆Need for diagnostic cardiac testing based on risk, One: ▶Low risk and, One: ◆ACS related symptoms and age ≥ 40 years ◆Dyspnea and, One: ▶Male and age ≥ 40 years ▶Female and age ≥ 50 years	•OBSERVATION, One: ▶ACS suspected and, All: ◆Need for diagnostic cardiac testing based on risk, One: ▶Low risk and ACS related symptoms and age ≥ 40 years ▶Low risk and dyspnea and, One: ◆Male and age ≥ 40 years ◆Female and age ≥ 50 years	Rationale: • Low risk criteria were updated to streamline the review process

Acute Coronary Syndrome (ACS)

Episode Day 1 (OBSERVATION)

Previously Available Content	2024 March Release	Revision Description
►Intermediate risk ◆ECG normal, unchanged, or non-diagnostic ◆Initial troponin negative or indeterminate and requiring serial troponins ◆Intervention scheduled or performed within 24h, One: ►Cardiac catheterization ►CCTA ►Stress test	►Intermediate risk ◆ECG normal, unchanged, or non-diagnostic ◆Initial troponin negative or indeterminate and requiring serial troponins ◆Intervention scheduled or performed within 24h, One: ►Cardiac catheterization ►CCTA ►Stress test	

Anemia



Revision Summary

Changes were made to the Anemia content to better describe the patient who requires the Observation, Acute, or Critical levels of care. Responder criteria were updated on continued stay to reflect the patient appropriate for discharge at the Observation and the Acute levels of care. AKI and CRRT criteria were removed on continued stay as this patient would be appropriate to be reviewed in the AKI subset. On Episode Day 3, Non-responder criteria were updated to address the patient with newly diagnosed hemolytic anemia who requires continued stay at a higher level of care.

Anemia

Initial Review (OBSERVATION)

Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ▶Anemia and, ≥ One: ♦Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦Exertional dyspnea ♦Presyncope or syncope	•OBSERVATION, One: ▶Anemia and, ≥ One: ♦Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦Exertional dyspnea worse than baseline ♦Orthostatic hypotension, Both: ▶Sustained drop in blood pressure within 3 min of sitting or standing ▶Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ♦Syncope	Rationale: • “Presyncope” was removed as literature does not speak to presyncope as a finding that requires hospitalization in patients with anemia • “Worse than baseline” was added to “Exertional dyspnea” to clarify that the intent of the criteria point is not appropriate for patients who have chronic dyspnea related to a comorbid condition and describing the patient who has exertional dyspnea over and above their chronic state • Orthostatic hypotension criteria were added to accurately capture symptomatic patients with anemia

Anemia

Initial Review (ACUTE)

Previously Available Content	2024 March Release	Revision Description
•ACUTE, One: ▶Hemolytic anemia and, ≥ One: ♦Hct < 30%(0.30) or Hb < 10.0 g/dL(100 g/L) ♦Exertional dyspnea ♦Presyncope or syncope	•ACUTE, One: ▶Hemolytic anemia and, ≥ One: ♦Hct < 30%(0.30) or Hb < 10.0 g/dL(100 g/L) ♦Exertional dyspnea worse than baseline ♦Syncope	Rationale: • “Presyncope” was removed as literature does not speak to presyncope as a finding that requires hospitalization in patients with anemia

Anemia		
Initial Review (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
		"Worse than baseline" was added to "Exertional dyspnea" to clarify that the intent of the criteria point is not appropriate for patients who have chronic dyspnea related to a comorbid condition and describing the patient who has exertional dyspnea over and above their chronic state

Anemia		
Initial Review (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
•CRITICAL, ≥ One: ▶Mechanical ventilation ▶NIPPV and, ≥ One: ♦Arterial $P_{O_2} \leq 39$ mmHg(5.2 kPa) ♦Arterial or venous $P_{CO_2} \geq 60$ mmHg(8.0 kPa) and, One: ▶COPD and arterial or venous pH ≤ 7.24 ▶No history of COPD ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	•CRITICAL, ≥ One: ▶Hemodynamic monitoring, invasive ▶Mechanical ventilation ▶NIPPV ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	Rationale: "Hemodynamic monitoring, invasive" was added as an appropriate intervention at the Critical level of care - NIPPV subcriteria were removed because any patient who is newly requiring noninvasive ventilator support is appropriate for the Critical level of care on Initial Review

Anemia		
Episode Day 1 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ▶Anemia and, Both: ♦Finding, ≥ One: ▶Hct $< 21\%$(0.21) or Hb < 7.0 g/dL(70 g/L) ▶Exertional dyspnea ▶Presyncope or syncope ▶Intervention, Both: ▶Blood product transfusion	•OBSERVATION, One: ▶Anemia and, Both: ♦Finding, ≥ One: ▶Hct $< 21\%$(0.21) or Hb < 7.0 g/dL(70 g/L) ▶Exertional dyspnea worse than baseline ▶Orthostatic hypotension, Both: ♦Sustained drop in blood pressure within 3 min	Rationale: "Presyncope" was removed as literature does not speak to presyncope as a finding that requires hospitalization in patients with anemia "Worse than baseline" was added to "Exertional dyspnea" to clarify that the intent of the criteria

Anemia		
Episode Day 1 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
►Hct or Hb monitoring at least daily	- of sitting or standing ♦Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ►Syncope ♦Intervention, Both: ►Blood product transfusion ►Hct or Hb monitoring at least daily	point is not appropriate for patients who have chronic dyspnea related to a comorbid condition and describing the patient who has exertional dyspnea over and above their chronic state • Orthostatic hypotension criteria were added to accurately capture symptomatic patients with anemia

Anemia		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
•ACUTE, One: ►Hemolytic anemia and, Both: ♦Finding, $\geq$ One: ►Hct $< 30\%$(0.30) or Hb < 10.0 g/dL(100 g/L) ►Exertional dyspnea ►Presyncope or syncope ♦Hct or Hb monitoring at least 2x/24h and, $\geq$ One: ►Blood product transfusion ►Corticosteroid ►Immunotherapy (includes monoclonal antibody treatment) ≤ 24h ►Immunoglobulin	•ACUTE, One: ►Hemolytic anemia and, Both: ♦Finding, $\geq$ One: ►Hct $< 30\%$(0.30) or Hb < 10.0 g/dL(100 g/L) ►Exertional dyspnea worse than baseline ►Syncope ♦Hct or Hb monitoring at least 2x/24h and, $\geq$ One: ►Blood product transfusion ►Corticosteroid ►Immunotherapy (includes monoclonal antibody treatment) ≤ 24h ►Immunoglobulin	Rationale: • "Presyncope" was removed as literature does not speak to presyncope as a finding that requires hospitalization in patients with anemia • "Worse than baseline" was added to "Exertional dyspnea" to clarify that the intent of the criteria point is not appropriate for patients who have chronic dyspnea related to a comorbid condition and describing the patient who has exertional dyspnea over and above their chronic state

Anemia		
Episode Day 1 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
•CRITICAL, $\geq$ One: ►Hemodynamic monitoring, invasive ►Mechanical ventilation ►NIPPV and, $\geq$ One:	•CRITICAL, $\geq$ One: ►Hemodynamic monitoring, invasive ►Mechanical ventilation ►NIPPV	Rationale: • NIPPV subcriteria were removed because any patient who is newly requiring noninvasive

Anemia		
Episode Day 1 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
♦Arterial $P_{O_2} \leq 39$ mmHg(5.2 kPa) ♦Arterial or venous $P_{CO_2} \geq 60$ mmHg(8.0 kPa) and, One: ▶COPD and arterial or venous pH ≤ 7.24 ▶No history of COPD ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	ventilator support is appropriate for the Critical level of care on Initial Review

Anemia		
Episode Day 2 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Anemia controlled or resolved ♦Hct or Hb within acceptable limits ♦Vital signs within acceptable limits ▶Partial responder, not clinically stable for discharge and requires continued stay, One: ♦Anemia and, Both: ▶Finding, ≥ One: ♦Hct $< 21\%$(0.21) or Hb < 7.0 g/dL(70 g/L) ♦Exertional dyspnea ♦Presyncope or syncope ▶Hct or Hb monitoring at least 2x/24h and, Both: ♦Blood product transfusion ♦Colonoscopy or specialist evaluation ≤ 24h	•OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, Both: ♦Hct or Hb within acceptable limits ♦Vital signs within acceptable limits ▶Partial responder, not clinically stable for discharge and requires continued stay, One: ♦Anemia and, Both: ▶Finding, ≥ One: ♦Hct $< 21\%$(0.21) or Hb < 7.0 g/dL(70 g/L) ♦Exertional dyspnea worse than baseline ♦Orthostatic hypotension, Both: ▶Sustained drop in blood pressure within 3 min of sitting or standing ▶Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ♦Syncope ▶Hct or Hb monitoring at least 2x/24h and, Both: ♦Blood product transfusion ♦Colonoscopy or specialist evaluation ≤ 24h	Rationale: • Responder criteria were updated to reflect the patient appropriate for discharge • "Presyncope" was removed as literature does not speak to presyncope as a finding that requires hospitalization in patients with anemia • "Worse than baseline" was added to "Exertional dyspnea" to clarify that the intent of the criteria point is not appropriate for patients who have chronic dyspnea related to a comorbid condition and describing the patient who has exertional dyspnea over and above their chronic state • Orthostatic hypotension criteria were added to capture continually symptomatic patients with anemia now require Hct or Hb monitoring, blood product transfusion and colonoscopy

Anemia		
Episode Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
•ACUTE, One: ►Responder, discharge expected today if clinically stable last 12h, All: ♦Anemia controlled or resolved ♦Hct or Hb within acceptable limits ♦No complication or active comorbidity relevant to this episode of care ►Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦Hemolytic anemia and, Both: ►Finding, ≥ One: ♦Hct < 30%(0.30) or Hb < 10.0 g/dL(100 g/L) ♦Exertional dyspnea ♦Presyncope or syncope ►Hct or Hb monitoring at least 2x/24h and, ≥ One: ♦Blood product transfusion ♦Corticosteroid ♦Immunotherapy (includes monoclonal antibody treatment) ≤ 24h ♦Immunoglobulin ♦Plasmapheresis ♦Post Critical care ≤ 24h	•ACUTE, One: ►Responder, discharge expected today if clinically stable last 12h, Both: ♦Hct or Hb within acceptable limits ♦No complication or active comorbidity relevant to this episode of care ►Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦Hemolytic anemia and, Both: ►Finding, ≥ One: ♦Hct < 30%(0.30) or Hb < 10.0 g/dL(100 g/L) ♦Exertional dyspnea worse than baseline ♦Syncope ►Hct or Hb monitoring at least 2x/24h and, ≥ One: ♦Blood product transfusion ♦Corticosteroid ♦Immunotherapy (includes monoclonal antibody treatment) ≤ 24h ♦Immunoglobulin ♦Plasmapheresis ♦Post Critical care ≤ 24h	Rationale: • Responder criteria were updated to reflect the patient appropriate for discharge • "Presyncope" was removed as literature does not speak to presyncope as a finding that requires hospitalization in patients with anemia • "Worse than baseline" was added to "Exertional dyspnea" to clarify that the intent of the criteria point is not appropriate for patients who have chronic dyspnea related to a comorbid condition and describing the patient who has exertional dyspnea over and above their chronic state

Anemia		
Episode Day 2 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
•CRITICAL, ≥ One: ►AKI and, Both: ♦Finding, ≥ One: ►Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: ♦Loss of P wave ♦Multifocal PVCs ♦Neuromuscular deficit ♦Widening QRS ►Sodium, One: ♦< 120 mEq/L(120 mmol/L) and volume overload ♦> 160 mEq/L(160 mmol/L) ♦Intervention, ≥ One: ►Calcium chloride ►Calcium gluconate	•CRITICAL, ≥ One: ►Hemodynamic monitoring, invasive ►Mechanical ventilation ►NIPPV and, ≥ One: ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ►Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	Rationale: • AKI and CRRT criteria were removed as the patient with AKI or requiring CRRT would be appropriate to be reviewed in the AKI subset

Anemia		
Episode Day 2 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶Dialysis, One: ♦Hemodialysis ≤ 7d since initiation ♦Peritoneal dialysis ≤ 7d since initiation ▶IV fluid resuscitation ≤ 2d ▶CRRT ▶Hemodynamic monitoring, invasive ▶Mechanical ventilation ▶NIPPV and, ≥ One: ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)		
Anemia		
Episode Day 3 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Anemia controlled or resolved ♦Hct or Hb within acceptable limits ♦Vital signs within acceptable limits ▶Non-responder, not clinically stable for discharge and exceeds episode days at the Observation level of care for this condition, One: ♦For a condition other than anemia use appropriate subset (Episode Day 1) based on clinical findings ♦Refer for secondary review	•OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, Both: ♦Hct or Hb within acceptable limits ♦Vital signs within acceptable limits ▶Non-responder, not clinically stable for discharge and exceeds episode days at the Observation level of care for this condition, One: ♦For hemolytic anemia use Episode Day 3 at a higher level of care ♦For a condition other than anemia use appropriate subset (Episode Day 1) based on clinical findings ♦Refer for secondary review	Rationale: • Responder criteria were updated to reflect the patient appropriate for discharge • Non-responder criteria were updated to address the patient with newly diagnosed hemolytic anemia who requires a higher level of care on Episode Day 3

Anemia		
Episode Day 3 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
• ACUTE, One: ► Responder, discharge expected today if clinically stable last 12h, All: ♦ Anemia controlled or resolved ♦ Hct or Hb within acceptable limits ♦ Complication or comorbidity, One: ► No complication or active comorbidity relevant to this episode of care ► Complication or comorbidity and clinically stable for discharge, One: ♦ AKI, ≥ One: ► Kidney function improving or within acceptable limits ► Outpatient dialysis regimen established ► Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦ Hemolytic anemia and, Both: ► Finding, ≥ One: ♦ Hct < 30%(0.30) or Hb < 10.0 g/dL(100 g/L) ♦ Exertional dyspnea ♦ Presyncope or syncope ► Hct or Hb monitoring at least 2x/24h and, ≥ One: ♦ Blood product transfusion ♦ Corticosteroid ♦ Immunotherapy (includes monoclonal antibody treatment) ≤ 24h ♦ Immunoglobulin ♦ Plasmapheresis ♦ AKI, Both: ► Finding, ≥ One: ♦ Oliguria ♦ Creatinine, One: ► ≥ 1.5x baseline and chronic kidney disease ► ≥ 1.5x ULN ♦ GFR > 25%(0.25) decrease from baseline ♦ Dialysis initiated ► Intervention, ≥ One: ♦ Volume expander ≤ 2d ♦ Diuretic ≥ 2x/24h, ≤ 3d ♦ Dialysis (initial) ≤ 7d since initiation ♦ Dialysis discontinued requiring lab monitoring ≤ 2d since discontinuation ♦ Immunosuppressant therapy ≤ 3d ♦ Post Critical care ≤ 24h	• ACUTE, One: ► Responder, discharge expected today if clinically stable last 12h, Both: ♦ Hct or Hb within acceptable limits ♦ No complication or active comorbidity relevant to this episode of care ► Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦ Hemolytic anemia and, Both: ► Hct < 30%(0.30) or Hb < 10.0 g/dL(100 g/L) ► Hct or Hb monitoring at least 2x/24h and, ≥ One: ♦ Blood product transfusion ♦ Corticosteroid ♦ Immunotherapy (includes monoclonal antibody treatment) ≤ 24h ♦ Immunoglobulin ♦ Plasmapheresis ♦ Post Critical care ≤ 24h	Rationale: • Responder criteria were updated to reflect the patient appropriate for discharge • "Exertional dyspnea" and "presyncope or syncope" were removed as those findings should be managed and resolved by Episode Day 3 • AKI criteria were removed as the patient who develops AKI would be appropriate to be reviewed in the AKI subset

Anemia		
Episode Day 3 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
•CRITICAL, ≥ One: ▶AKI and, Both: ♦Finding, ≥ One: ▶Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: ♦Loss of P wave ♦Multifocal PVCs ♦Neuromuscular deficit ♦Widening QRS ▶Sodium, One: ♦< 120 mEq/L(120 mmol/L) and volume overload ♦> 160 mEq/L(160 mmol/L) ♦Intervention, ≥ One: ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶Dialysis, One: ♦Hemodialysis ≤ 7d since initiation ♦Peritoneal dialysis ≤ 7d since initiation ▶IV fluid resuscitation ≤ 2d ▶CRRT ▶Hemodynamic monitoring, invasive ▶Mechanical ventilation ▶NIPPV and, ≥ One: ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	•CRITICAL, ≥ One: ▶Hemodynamic monitoring, invasive ▶Mechanical ventilation ▶NIPPV and, ≥ One: ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	Rationale: • AKI and CRRT criteria were removed as the patient with AKI or requiring CRRT would be appropriate to be reviewed in the AKI subset

Anemia		
Episode Day 4 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
•ACUTE, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Anemia controlled or resolved ♦Hct or Hb within acceptable limits ♦Complication or comorbidity, One: ▶No complication or active comorbidity relevant to this episode of care ▶Complication or comorbidity and clinically stable for discharge, One: ♦AKI, ≥ One:	•ACUTE, One: ▶Responder, discharge expected today if clinically stable last 12h, Both: ♦Hct or Hb within acceptable limits ♦No complication or active comorbidity relevant to this episode of care ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦Hemolytic anemia and, Both: ▶Hct < 30%(0.30) or Hb < 10.0 g/dL(100 g/L) ▶Hct or Hb monitoring at least 2x/24h and, ≥	Rationale: • Responder criteria were updated to reflect the patient appropriate for discharge • "Exertional dyspnea" and "presyncope or syncope" were removed as those findings should be managed and resolved by Episode Day 3

Anemia		
Episode Day 4 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
►Kidney function improving or within acceptable limits ►Outpatient dialysis regimen established ►Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦Hemolytic anemia and, Both: ►Finding, ≥ One: ♦Hct < 30%(0.30) or Hb < 10.0 g/dL(100 g/L) ♦Exertional dyspnea ♦Presyncope or syncope ►Hct or Hb monitoring at least 2x/24h and, ≥ One: ♦Blood product transfusion ♦Corticosteroid ♦Immunotherapy (includes monoclonal antibody treatment) ≤ 24h ♦Immunoglobulin ♦Plasmapheresis ♦AKI, Both: ►Finding, ≥ One: ♦Oliguria ♦Creatinine, One: ►≥ 1.5x baseline and chronic kidney disease ►≥ 1.5x ULN ♦GFR > 25%(0.25) decrease from baseline ♦Dialysis initiated ►Intervention, ≥ One: ♦Volume expander ≤ 2d ♦Diuretic ≥ 2x/24h, ≤ 3d ♦Dialysis (initial) ≤ 7d since initiation ♦Dialysis discontinued requiring lab monitoring ≤ 2d since discontinuation ♦Immunosuppressant therapy ≤ 3d ♦Post Critical care ≤ 24h	One: ♦Blood product transfusion ♦Corticosteroid ♦Immunotherapy (includes monoclonal antibody treatment) ≤ 24h ♦Immunoglobulin ♦Plasmapheresis ♦Post Critical care ≤ 24h	AKI criteria were removed as the patient who develops AKI would be appropriate to be reviewed in the AKI subset

Anemia		
Episode Day 4 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
♦CRITICAL, ≥ One: ►AKI and, Both: ♦Finding, ≥ One: ►Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One:	♦CRITICAL, ≥ One: ►Hemodynamic monitoring, invasive ►Mechanical ventilation ►NIPPV and, ≥ One: ♦FiO₂ > 50%(0.50)	Rationale: AKI and CRRT criteria were removed as the patient who develops AKI or requiring CRRT would be appropriate to be reviewed in the AKI subset

Anemia		
Episode Day 4 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
♦Loss of P wave ♦Multifocal PVCs ♦Neuromuscular deficit ♦Widening QRS ▶Sodium, One: ♦< 120 mEq/L(120 mmol/L) and volume overload ♦> 160 mEq/L(160 mmol/L) ♦Intervention, ≥ One: ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶Dialysis, One: ♦Hemodialysis ≤ 7d since initiation ♦Peritoneal dialysis ≤ 7d since initiation ▶IV fluid resuscitation ≤ 2d ▶CRRT ▶Hemodynamic monitoring, invasive ▶Mechanical ventilation ▶NIPPV and, ≥ One: ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	♦Respiratory intervention every 1-2h ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	

Anemia		
Episode Day 5 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
•ACUTE, One: ▶Non-responder, not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, One: ♦Use Extended Stay subset ♦Refer for secondary review	•ACUTE, One: ▶Responder, discharge expected today if clinically stable last 12h, Both: ♦Hct or Hb within acceptable limits ♦No complication or active comorbidity relevant to this episode of care ▶Non-responder, not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, One: ♦Use Extended Stay subset ♦Refer for secondary review	Rationale: • Responder criteria were updated to reflect the patient appropriate for discharge

Antepartum



Revision Summary

Antepartum content was updated and restructured to streamline the review process, remove redundancy, and align with current guidelines. Pregestational diabetes mellitus criteria were added to the Antepartum content. High risk of fetal compromise criteria were updated with additional complications and extended to Episode Day 3-X to provide a pathway for continued stay for individuals who experience these complications beyond Episode Day 2.

Antepartum		
Episode Day 1 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, ≥ One: ▶Decreased fetal movement and, Both: ♦Fetal heart rate monitoring ♦Ultrasound assessment ▶Labor or preterm labor rule out and, Both: ♦Fetal heart rate monitoring ♦Uterine monitoring ▶Nausea and vomiting of pregnancy or hyperemesis gravidarum and, Both: ♦Persistent vomiting and unresponsive to IV hydration ♦Intervention, ≥ One: ▶Antiemetic ≥ 3x/24h or serotonin antagonist ≥ 2x/24h ▶IV fluid, One: ♦≥ 75 mL/h and, ≥ One: ▶Weight < 60 kg ▶Renal insufficiency ▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ▶Post cerclage and, Both: ♦Fetal heart rate monitoring ♦Uterine monitoring ▶Prelabor rupture of membranes (PROM), All: ♦≥ 37 wks gestation ♦Finding, ≥ One: ▶Fern (+) ▶Nitrazine (+) ▶Placental immunoassay (+) ▶Pooling of amniotic fluid ♦Fetal heart rate monitoring ♦Uterine monitoring ▶Vaginal bleeding ≤ 1 pad/h without fetal compromise and, Both: ♦Hct or Hb monitoring ♦Transvaginal or transabdominal ultrasound	•OBSERVATION, ≥ One: ▶Decreased fetal movement and, Both: ♦Fetal heart rate monitoring ♦Ultrasound assessment ▶Labor or preterm labor rule out and, Both: ♦Fetal heart rate monitoring ♦Uterine monitoring ▶Persistent nausea and vomiting of pregnancy or hyperemesis gravidarum and, ≥ One: ♦Antiemetic ≥ 2x/24h ♦IV fluid ▶Post cerclage and, Both: ♦Fetal heart rate monitoring ♦Uterine monitoring ▶Pregestational or gestational diabetes mellitus and, All: ♦Blood glucose, ≥ One: ▶Fasting ≥ 95 mg/dL(5.3 mmol/L) ▶1h postprandial ≥ 140 mg/dL(7.8 mmol/L) or 2h postprandial ≥ 120 mg/dL(6.7 mmol/L) ♦Failed outpatient insulin or oral glucose-lowering medication management ≥ 5d ♦Blood glucose monitoring ♦Insulin or oral glucose-lowering medication initiation or adjustment ▶Vaginal bleeding ≤ 1 pad/h without fetal compromise and, Both: ♦Hct or Hb monitoring ♦Ultrasound assessment	Rationale: • “Nausea and vomiting of pregnancy or hyperemesis gravidarum” criteria were updated to remove redundancy and streamline the review process • “Prelabor rupture of membranes” at ≥ 37 weeks gestation was removed from the Antepartum subset and added to the Labor and Delivery subset as literature notes that delivery is recommended and/or imminent within 12 to 24 hours • “Pregestational diabetes mellitus” was moved from the Diabetes Mellitus subset at the Observation level of care and added to the Antepartum subset with gestational diabetes mellitus to streamline the review process and improve organization • “Gestational diabetes mellitus” was removed from the Acute level of care and added to the Observation level of care. • “Hypoglycemic agent” was changed to “glucose-lowering medication,” and “oral glucose-lowering medication” was added to align with current guidelines and to be more inclusive of all treatment options • Vaginal bleeding “transvaginal or transabdominal ultrasound” criteria verbiage has been updated to “ultrasound assessment” to streamline the criteria

Antepartum		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
• ACUTE, ≥ One: ▶ High risk of fetal compromise, Both: ♦ Finding, ≥ One: ▶ Non-reassuring fetal testing on two or more tests prior to decision to admit and, ≥ One: ♦ Amniotic fluid volume, ≥ One: ▶ Fluid pocket ≤ 2 cm/quadrant ▶ Index ≤ 5 cm ♦ Biophysical profile ≤ 6 ♦ Nonreactive nonstress test ♦ Positive or nonreactive stress test ▶ Risk factor, ≥ One: ♦ Chorioamniotic separation (CAS) ♦ Monoamniotic twins ♦ Multiple gestation and complication, ≥ One: ▶ Discordance and fetal growth restriction ▶ Twin anemia polycythemia sequence (TAPS) ▶ Twin-to-twin transfusion syndrome (TTTS) ♦ Prolapse of amniotic sac ♦ Fetal anemia and, ≥ One: ▶ Complication of intrauterine transfusion ▶ Fetal hydrops (e.g. fetal ascites, pleural effusion) ♦ Intervention, ≥ One: ▶ Fetal blood product transfusion ▶ Monitoring, All: ♦ Fetal heart rate monitoring ♦ Ultrasound assessment ♦ Uterine monitoring ▶ Gestational diabetes mellitus and, Both: ♦ Finding, Both: ▶ Blood glucose, ≥ Two: ♦ Fasting > 95 mg/dL(5.3 mmol/L) ♦ 1h postprandial ≥ 140 mg/dL(7.8 mmol/L) ♦ 2h postprandial ≥ 120 mg/dL(6.7 mmol/L) ▶ Failed outpatient hyperglycemic management (includes PO) ≥ 5d ♦ Intervention, Both: ▶ Blood glucose monitoring ▶ Hypoglycemic medication adjustment at least daily (includes PO) ▶ Placenta abruptio and, One: ♦ Mild-moderate abruptio and, Both: ▶ Fetal heart rate monitoring ▶ Uterine monitoring ♦ Severe hemorrhage, uterine spasm, or fetal distress and, Both: ▶ Fetal heart rate monitoring, continuous ▶ Uterine monitoring ▶ Placenta previa, placenta accreta syndrome, or vasa previa, Both: ♦ Finding, ≥ One:	• ACUTE, ≥ One: ▶ High risk of fetal compromise, Both: ♦ Finding, ≥ One: ▶ Biophysical profile ≤ 4 ▶ Biophysical profile 6 and ≥ 37 wks gestation ▶ Non-reassuring fetal testing on two or more tests prior to decision to admit and, ≥ One: ♦ Amniotic fluid index ≤ 5 cm ♦ Biophysical profile 6 and < 37 wks gestation ♦ Nonreactive nonstress test ♦ Single deepest vertical pocket of amniotic fluid ≤ 2 cm ♦ Positive contraction stress test ▶ Risk factor, ≥ One: ♦ Chorioamniotic separation (CAS) ♦ Fetal growth restriction and, ≥ One: ▶ Absent end-diastolic flow ▶ Reverse end-diastolic flow ▶ Oligohydramnios ♦ Monochorionic-monoamniotic twins and expectant management ♦ Multiple gestation and complication ♦ Prolapse of amniotic sac ♦ Fetal anemia and, ≥ One: ▶ Complication of intrauterine transfusion ▶ Fetal hydrops ♦ Intervention, ≥ One: ▶ Fetal blood product transfusion ▶ Monitoring, All: ♦ Fetal heart rate monitoring ♦ Ultrasound assessment ♦ Uterine monitoring ▶ Placental complications and, Both: ♦ Finding, One: ▶ Bleeding, contractions, or pain ▶ Asymptomatic and expectant management, ≥ One: ♦ Limited access to prompt medical care and arrangements for outpatient services pending ≤ 24h ♦ Outpatient services unavailable ♦ Patient or caregiver unreliable and unable to adhere to outpatient instructions or treatment ♦ Severe and persistent mental illness, cognitive impairment, or substance use disorder ♦ Intervention, ≥ One: ▶ Fetal heart rate monitoring or uterine monitoring ▶ IV fluid or blood product transfusion ▶ Preterm labor, All: ♦ Gestation < 37 wks ♦ Contractions ≤ every 10 min for ≥ 30 sec	Rationale: • “Biophysical profile of ≤ 4” and “biophysical profile 6 and ≥ 37 weeks” were added to criteria without a prerequisite requirement of non-reassuring fetal testing on two or more tests prior to decision to admit as literature notes that a biophysical profile (BPP) of 4 or less should result in delivery in most cases and a BPP of 6 should prompt delivery based on gestational age • Amniotic fluid volume “fluid pocket” verbiage was updated to “single deepest vertical pocket” to align with terminology used in literature and guidelines • “Biophysical profile 6” was updated to include a gestational age to differentiate between those who may not necessarily deliver based on gestational age but will require further evaluation • The verbiage “contraction” has been added to “positive stress test” to align with terminology used in literature and guidelines • “Fetal growth restriction” and “absent end-diastolic flow”, “reverse end-diastolic flow”, and/or “oligohydramnios” were added to content as literature notes that absent or reversed end-diastolic flow in the umbilical artery and/or concurrent condition such as oligohydramnios is associated with an increased frequency of perinatal mortality in the context of fetal growth restriction and inpatient management should be considered in most cases • “Monochorionic” and “expectant management” verbiage added to “monoamniotic twins” to align with literature and to provide more detail in specifying that these high risk pregnancies have an increased risk of perinatal mortality and, therefore, are often admitted for expectant management • Placenta abruptio, placenta accreta syndrome (i.e., placenta accreta, increta, percreta), placenta previa, and vasa previa criteria were streamlined

Antepartum		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
▶Bleeding > 1 pad/h ▶Contractions ≥ 4/h ▶Nonreactive nonstress test ♦Intervention, Both: ▶Fetal heart rate monitoring ▶Uterine monitoring ▶Preterm labor and, All: ♦Cervical dilation or effacement ♦Contractions ≤ every 10 min for ≥ 30 sec ♦Fetal heart rate monitoring ♦Gestation < 37 wks ♦Uterine monitoring ▶Preterm prelabor rupture of membranes (PPROM), All: ♦Finding, ≥ One: ▶Fern (+) ▶Nitrazine (+) ▶Placental immunoassay (+) ▶Pooling of amniotic fluid ♦Fetal heart rate monitoring ♦Gestation at point of viability and < 37 wks ♦Uterine monitoring ▶Vaginal bleeding > 1 pad/h and, All: ♦Hct or Hb monitoring ♦IV fluid or blood product transfusion ♦Transvaginal or transabdominal ultrasound	♦Cervical dilation or effacement ♦Fetal heart rate monitoring ♦Uterine monitoring ▶Preterm prelabor rupture of membranes (PPROM) confirmed and, All: ♦Gestation < 37 wks ♦Fetal heart rate monitoring ♦Uterine monitoring ▶Vaginal bleeding > 1 pad/h and, All: ♦Hct or Hb monitoring ♦IV fluid or blood product transfusion ♦Ultrasound assessment	and are now included in a hover note linked to “Placental complications” • “Placental complications” criteria now have pathways for those who are symptomatic or asymptomatic, requiring expectant management with the presence of barriers to discharge and/or social determinants of health, and IV fluid or blood product transfusion criteria line has been added as an intervention • “Preterm prelabor rupture of membranes (PPROM)” criteria were updated and streamlined to better define the individual that is appropriate for management at the Acute level of care with a hover note attached to “confirmed” that includes the required findings and the gestational age was updated to < 37 weeks to account for pregnant individuals at a perivable gestational age • In vaginal bleeding, “transvaginal or transabdominal” ultrasound criteria line was replaced with “ultrasound assessment” to streamline the review process

Antepartum		
Episode Day 1 (INTERMEDIATE)		
Previously Available Content	2024 March Release	Revision Description
♦INTERMEDIATE, ≥ One: ▶Arrhythmia, ≥ One: ♦IV antiarrhythmic and hemodynamically stable ♦Antiarrhythmic conversion from IV to PO, One: ▶No proarrhythmic risk ≤ 24h ▶Proarrhythmic risk and continuous cardiac monitoring (excludes Holter) ≤ 3d ▶Seizure (excludes eclampsia and preeclampsia) and, Both: ♦Activity resolved ♦Anticonvulsant continuous	N/A	Rationale: • “Arrhythmia” content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • “Seizure (excludes eclampsia and preeclampsia)” criteria were removed as an individual with eclampsia can be reviewed in the Hypertensive Disorders of Pregnancy subset, and seizures that

Antepartum		
Episode Day 1 (INTERMEDIATE)		
Previously Available Content	2024 March Release	Revision Description
		are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology

Antepartum		
Episode Day 1 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
•CRITICAL, ≥ One: ▶Arrhythmia, ≥ One: ♦Urgent electrical cardioversion performed within last 24h ♦Defibrillation performed within last 24h ♦Temporary pacemaker insertion performed within last 2d ♦Symptomatic recurrent or refractory arrhythmia requiring antiarrhythmic and, ≥ One: ▶Chest pain ▶Heart failure and, ≥ One: ♦Arterial Po₂ < 56 mmHg(7.4 kPa) ♦O₂ sat < 89%(0.89) and < baseline ▶Hemodynamic instability, ≥ One: ♦Heart rate > 120/min, sustained ♦Systolic blood pressure, ≥ One: ▶< 90 mmHg ▶< 110 mmHg with chronic hypertension ▶> 30 mmHg decrease from baseline ▶Labile ♦MAP < 65 mmHg ▶DIC and blood product transfusion ▶ECMO or ECLS ▶Hemodynamic instability, All: ♦ED treatment, ≥ One: ▶≥ 2 units blood product transfusion ▶≥ 2 liters IV fluid ▶≥ 1 unit blood product transfusion and ≥ 1 liter IV fluid ♦Findings after ED treatment, ≥ One: ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Requiring ongoing resuscitation ▶Hemodynamic monitoring, invasive ▶Mechanical ventilation	•CRITICAL, ≥ One: ▶DIC and blood product transfusion ▶ECMO or ECLS ▶Hemodynamic instability, All: ♦ED treatment, ≥ One: ▶≥ 2 units blood product transfusion ▶≥ 2 liters IV fluid ▶≥ 1 unit blood product transfusion and ≥ 1 liter IV fluid ♦Findings after ED treatment, ≥ One: ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Requiring ongoing resuscitation ▶Hemodynamic monitoring, invasive ▶HFNC ▶Mechanical ventilation ▶NIPPV ▶Oxygen ≥ 40%(0.40) ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	Rationale: • “Arrhythmia” content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • “HFNC” criterion was added to reflect the patient who is appropriate at the Critical level of care • NIPPV subcriteria were removed because any patient who is newly requiring noninvasive ventilator support is appropriate for the Critical level of care on Episode Day 1 • “Oxygen ≥ 40%” criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Critical Level of Care for the patient who is pregnant • “Seizure (excludes eclampsia and preeclampsia)” criteria were removed as an individual with eclampsia can be reviewed in the Hypertensive Disorders of Pregnancy subset, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology

Antepartum		
Episode Day 1 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
▶NIPPV and, ≥ One: ♦Arterial Po₂ ≤ 39 mmHg(5.2 kPa) ♦Arterial or venous Pco₂ ≥ 60 mmHg(8.0 kPa) and, One: ▶COPD and arterial or venous pH ≤ 7.24 ▶No hx of COPD ▶Oxygen ≥ 40%(0.40), ≤ 2d ▶Seizure (excludes eclampsia or preeclampsia), recurrent or refractory requiring anticonvulsant ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure)		

Antepartum		
Episode Day 2 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
●OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, Both: ♦Undelivered ♦Clinical stability, ≥ One: ▶Reassuring fetal testing ▶Tolerating PO ▶Post cerclage and Category I fetal heart rate tracing ▶Labor ruled out ▶Vaginal bleeding controlled or resolved ▶Partial responder, not clinically stable for discharge and requires continued stay, One: ♦Persistent nausea and vomiting of pregnancy or hyperemesis gravidarum and, Both: ▶Antiemetic ≥ 3x/24h or serotonin antagonist ≥ 2x/24h ♦IV fluid, One: ♦≥ 75 mL/h and, ≥ One: ▶Weight < 60 kg ▶Renal insufficiency ▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg	●OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Undelivered ♦Tolerating PO ♦Clinical stability, ≥ One: ▶Reassuring fetal testing ▶Blood glucose within acceptable limits ▶Vomiting controlled ▶Post cerclage and Category I fetal heart rate tracing ▶Labor ruled out ▶Vaginal bleeding controlled or resolved ▶Partial responder, not clinically stable for discharge and requires continued stay, One: ♦Persistent nausea and vomiting of pregnancy or hyperemesis gravidarum and, Both: ▶Antiemetic ≥ 2x/24h ♦IV fluid ♦Pregestational or gestational diabetes mellitus and, All: ▶Blood glucose monitoring ▶Insulin or oral glucose-lowering medication initiation, adjustment, or dose held ▶Patient or caregiver knowledge deficit and diabetes education	Rationale: • Responder criteria were updated to reflect the patient appropriate for discharge • “Nausea and vomiting of pregnancy or hyperemesis gravidarum” criteria have been updated to remove redundancy and streamline the review process • “Pregestational diabetes mellitus” was moved from the Diabetes Mellitus subset at the Observation level of care and added to the Antepartum subset with gestational diabetes mellitus to streamline the review process and improve organization • “Gestational diabetes mellitus” was removed from the Acute level of care and added to the Observation level of care. • “Hypoglycemic agent” was changed to “glucose-lowering medication,” and “oral glucose-lowering medication” was added to align with current guidelines and to be more inclusive of all treatment options

Antepartum		
Episode Day 2 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
		Diabetes education criterion was added to be inclusive of patient or caregiver knowledge deficit and educational needs in addition to blood glucose monitoring, insulin or oral glucose-lowering medication initiation, adjustment, or doses being held on continued stay

Antepartum		
Episode Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
• ACUTE, ≥ One: - High risk of fetal compromise and, ≥ One: - Fetal blood product transfusion - Fetal heart rate and uterine monitoring - Fever and, Both: - Temperature ≥ 100.4°F(38.0°C) - Intervention, One: - Known source and anti-infective - New onset and, ≥ One: - Culture pending ≤ 2d - Drug-induced fever suspected and precipitating drug discontinued ≤ 3d - Imaging study ≤ 24h - Gestational diabetes mellitus and, Both: - Blood glucose monitoring - Hypoglycemic medication adjustment at least daily (includes PO) - Placenta abruptio and, One: - Mild-moderate abruptio and fetal heart rate and uterine monitoring - Placenta previa, placenta accreta syndrome, or vasa previa, requiring fetal heart rate and uterine monitoring - Preterm labor and fetal heart rate and uterine monitoring - Preterm prelabor rupture of membranes (PPROM) requiring fetal heart rate and uterine monitoring - Vaginal bleeding and, Both: - Hct or Hb monitoring - IV fluid or blood product transfusion	• ACUTE, ≥ One: - Delivery, One: - Planned within 48h and, Both: ≥ 22 and < 37 wks gestation - Corticosteroid administered within last 48h - Fever, Both: - Temperature ≥ 100.4°F(38.0°C) - Intervention, One: - Known source and anti-infective - New onset and, ≥ One: - Culture pending ≤ 2d - Imaging study ≤ 24h - High risk of fetal compromise and, ≥ One: - Fetal blood product transfusion - Fetal heart rate or uterine monitoring - Placental complications and, ≥ One: - Fetal heart rate monitoring or uterine monitoring - IV fluid or blood product transfusion - Preterm labor and fetal heart rate and uterine monitoring - Preterm prelabor rupture of membranes (PPROM) requiring fetal heart rate or uterine monitoring - Vaginal bleeding and, Both: - Hct or Hb monitoring - IV fluid or blood product transfusion - Post Critical care ≤ 24h	Rationale: “High risk of fetal compromise” criteria were updated to include a hover-over note displaying high risk conditions that are appropriate and relevant for application of this criteria on continued stay with a requirement of fetal heart rate or uterine monitoring to be inclusive of those individuals that may not receive both fetal heart rate and uterine monitoring “Planned delivery within 48h” criteria were added to be inclusive of those individuals who require antenatal corticosteroids prior to a planned delivery “Fever” criteria were streamlined to focus on the drivers of care and better define the individual who develops fever during hospitalization “Gestational diabetes mellitus” criteria were removed from the Acute level of care and are now present at the Observation level of care - Placenta abruptio, placenta accreta syndrome (i.e., placenta accreta, increta, percreta), placenta previa, and vasa previa criteria were streamlined and are now included in a hover note linked to “Placental complications”

Antepartum		
Episode Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
		• “Preterm prelabor rupture of membranes (PPROM)” criteria were updated and there is now a requirement of fetal heart rate or uterine monitoring to be inclusive of those individuals that may not receive both fetal heart rate and uterine monitoring • “Post Critical care” criterion has been added as a pathway to step-down from the Critical level of care on Episode Day 2

Antepartum		
Episode Day 2 (INTERMEDIATE)		
Previously Available Content	2024 March Release	Revision Description
• INTERMEDIATE, ≥ One: ▶ Arrhythmia, ≥ One: ♦ IV antiarrhythmic and hemodynamically stable ♦ Antiarrhythmic conversion from IV to PO, One: ▶ No proarrhythmic risk ≤ 24h ▶ Proarrhythmic risk and continuous cardiac monitoring (excludes Holter) ≤ 3d ▶ Seizure (excludes eclampsia and preeclampsia) and, Both: ♦ Activity resolved ♦ Anticonvulsant continuous	• INTERMEDIATE, ≥ One: ▶ HFNC and respiratory monitoring every 3-4h ▶ NIPPV	Rationale: • “Arrhythmia” content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • “HFNC” and “NIPPV” criteria were added to provide a pathway for individuals who require one of these interventions at the Intermediate level of care • “Seizure (excludes eclampsia and preeclampsia)” criteria were removed as an individual with eclampsia can be reviewed in the Hypertensive Disorders of Pregnancy subset, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology

Antepartum		
Episode Day 2 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
●CRITICAL, ≥ One: ▶Arrhythmia, ≥ One: ♦Urgent electrical cardioversion performed within last 24h ♦Defibrillation performed within last 24h ♦Temporary pacemaker insertion performed within last 2d ♦Symptomatic recurrent or refractory arrhythmia requiring antiarrhythmic and, ≥ One: ▶Chest pain ▶Heart failure and, ≥ One: ♦Arterial Po₂ < 56 mmHg(7.4 kPa) ♦O₂ sat < 89%(0.89) and < baseline ▶Hemodynamic instability, ≥ One: ♦Heart rate > 120/min, sustained ♦Systolic blood pressure, ≥ One: ▶< 90 mmHg ▶< 110 mmHg with chronic hypertension ▶> 30 mmHg decrease from baseline ▶Labile ♦MAP < 65 mmHg ▶DIC and blood product transfusion ▶ECMO or ECLS ▶Hemodynamic instability persisting after ≥1L of IVF resuscitation or ≥1 unit blood product transfusion and, Both: ♦Finding, ≥ One: ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: ▶Blood product transfusion ▶IV fluid resuscitation ≥1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Oxygen ≥ 40%(0.40), ≤ 2d ▶Seizure (excludes eclampsia or preeclampsia), recurrent or refractory requiring anticonvulsant ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	●CRITICAL, ≥ One: ▶DIC and blood product transfusion ▶ECMO or ECLS ▶Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion and, Both: ♦Finding, ≥ One: ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: ▶Blood product transfusion ▶IV fluid resuscitation ≥ 1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Oxygen ≥ 40%(0.40) ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	Rationale: • “Arrhythmia” content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • “Oxygen ≥ 40%” criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Critical Level of Care for the patient who is pregnant • "Seizure (excludes eclampsia and preeclampsia)" criteria were removed as an individual with eclampsia can be reviewed in the Hypertensive Disorders of Pregnancy subset, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology

Antepartum		
Episode Day 3-X (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
●OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Undelivered ♦Vomiting controlled ♦Tolerating PO ▶Non-responder, not clinically stable for discharge and exceeds episode days at the Observation level of care for this condition, One: ♦For antepartum continued stay use appropriate criteria based on clinical findings ♦For hyperemesis gravidarum use Episode Day 3 at a higher level of care ♦For a condition other than antepartum use appropriate subset (Episode Day 1) based on clinical findings ♦Refer for secondary review	●OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Undelivered ♦Blood glucose within acceptable limits ♦Tolerating PO ▶Non-responder, not clinically stable for discharge and exceeds episode days at the Observation level of care for this condition, One: ♦For hyperemesis gravidarum continued stay use Episode Day 3 at a higher level of care ♦For pregestational or gestational diabetes mellitus continued stay use Episode Day 3 at a higher level of care ♦For a condition not included in the Antepartum subset use appropriate subset (Episode Day 1) based on clinical findings ♦Refer for secondary review	Rationale: • Responder criteria were updated to reflect the patient appropriate for discharge • Non-responder criteria verbiage updated to provide additional detail, clarity, and direction for the Non-responder workflow

Antepartum		
Episode Day 3-X (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
●ACUTE, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Afebrile ♦Reassuring fetal testing ♦Tolerating PO ♦Undelivered ♦Clinical stability, ≥ One: ▶Gestational diabetes mellitus and blood glucose within acceptable limits ▶Placenta abruptio and no active bleeding ▶Placenta previa, placenta accreta syndrome, or vasa previa, and no active bleeding ▶Preterm labor and no cervical dilation or effacement ▶Vaginal bleeding controlled or resolved ▶Vomiting controlled ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦Fever and, Both: ▶Temperature $\geq 100.4^{\circ}\text{F}$ (38.0°C) ♦Intervention, One: ▶Known source and anti-infective	●ACUTE, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Afebrile ♦Reassuring fetal testing ♦Tolerating PO ♦Undelivered ♦Clinical stability, ≥ One: ▶Blood glucose within acceptable limits ▶Preterm labor and no cervical dilation or effacement ▶Vaginal bleeding controlled or resolved ▶Vomiting controlled ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦Delivery, One: ▶Planned within 48h and, Both: ♦≥ 22 and < 37 wks gestation ♦Corticosteroid administered within last 48h ♦Fever, Both: ▶Temperature $\geq 100.4^{\circ}\text{F}$ (38.0°C) ♦Intervention, One: ▶Known source and anti-infective	Rationale: • Responder criteria were updated and streamlined to align with criteria changes • Planned delivery within 48h criteria were added to be inclusive of those individuals that require antenatal corticosteroids prior to a planned delivery • “Fever” criteria were streamlined to focus on the drivers of care and better define the individual who develops fever during hospitalization • “Gestational diabetes mellitus” criteria were removed from the Acute level of care and are now present at the Observation level of care • High risk of fetal compromise criteria were added to Episode Day 3-X and are included in a hover note with a requirement of fetal blood product transfusion and/or fetal heart rate or uterine

Antepartum		
Episode Day 3-X (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦New onset and, ≥ One: ▶Culture pending ≤ 2d ▶Drug-induced fever suspected and precipitating drug discontinued ≤ 3d ▶Imaging study ≤ 24h ♦Gestational diabetes mellitus, uncontrolled blood glucose ≤ 24h and, Both: ▶Finding, ≥ One: ♦Blood glucose < 70 mg/dL(3.9 mmol/L) ♦Blood glucose > 200 mg/dL(11.1 mmol/L) ▶Intervention, Both: ♦Blood glucose monitoring ♦Hypoglycemic medication adjustment at least daily (includes PO) ♦Hyperemesis gravidarum and, All: ▶Finding, ≥ One: ♦Inadequate oral intake ♦Vomiting uncontrolled ▶Antiemetic 3x/24h or serotonin antagonist 2x/24h ▶IV fluid ≤ 4d since initiation, One: ♦≥ 75 mL/h and, ≥ One: ▶Weight < 60 kg ▶Renal insufficiency ▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ♦Placenta abruptio and, One: ▶Maternal or fetal instability, Both: ♦Finding, ≥ One: ▶Active bleeding within last 24h ▶Cervical dilation or effacement ▶Hct or Hb decreased last 24h ▶Non-reassuring fetal testing (e.g., positive or nonreactive stress test) ▶Orthostatic hypotension, Both: ♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Blood pressure, ≥ One: ▶Systolic drop ≥ 20 mmHg ▶Diastolic drop ≥ 10 mmHg ♦Intervention, All: ▶Hct or Hb monitoring ▶Fetal heart rate and uterine monitoring ▶IV fluid or blood product transfusion ▶Expectant management, fetal heart rate and uterine monitoring and, One: ♦Barrier to discharge and arrangements for outpatient services pending ≤ 24h, ≥ One: ▶Limited access to prompt medical care ▶Patient unreliable ♦Outpatient services unavailable ♦Placenta previa, placenta accreta syndrome, or vasa previa, and, One: ▶Maternal or fetal instability, Both:	♦New onset and, ≥ One: ▶Culture pending ≤ 2d ▶Imaging study ≤ 24h ♦High risk of fetal compromise and, ≥ One: ▶Fetal blood product transfusion ▶Fetal heart rate monitoring or uterine monitoring ♦Hyperemesis gravidarum and, All: ▶Finding, ≥ One: ♦Inadequate oral intake ♦Vomiting uncontrolled ▶Antiemetic ≥ 2x/24h ▶IV fluid ≤ 4d since initiation ♦Placental complications and, Both: ▶Finding, ≥ One: ♦Bleeding, contractions, or pain ♦Expectant management, ≥ One: ▶Limited access to prompt medical care and arrangements for outpatient services pending ≤ 24h ▶Outpatient services unavailable ▶Patient or caregiver unreliable and unable to adhere to outpatient instructions or treatment ▶Severe and persistent mental illness, cognitive impairment, or substance use disorder ▶Intervention, ≥ One: ♦Fetal heart rate monitoring or uterine monitoring ♦IV fluid or blood product transfusion ♦Pregestational or gestational diabetes mellitus, uncontrolled blood glucose ≤ 24h and, Both: ▶Finding, ≥ One: ♦Blood glucose < 70 mg/dL(3.9 mmol/L) ♦Blood glucose > 200 mg/dL(11.1 mmol/L) ▶Intervention, All: ♦Blood glucose monitoring ♦Insulin or oral glucose-lowering medication initiation, adjustment, or dose held ♦Patient or caregiver knowledge deficit and diabetes education ♦Preterm labor, All: ▶Gestation < 37 wks ▶Contractions ≤ every 10 min for ≥ 30 sec ▶Cervical dilation or effacement ▶Fetal heart rate monitoring ▶Uterine monitoring ♦Preterm prelabor rupture of membranes (PPROM) and, All: ▶Gestation < 37 wks ▶Finding, One: ♦New onset confirmed ♦Expectant management	monitoring to be inclusive of those individuals that may not receive both fetal heart rate and uterine monitoring • “Hyperemesis gravidarum” criteria were updated to remove redundancy and streamline the review process • Placenta abruptio, placenta accreta syndrome (i.e., placenta accreta, increta, percreta), placenta previa, and vasa previa criteria were streamlined and are now included in a hover note linked to “Placental complications” • “Placental complications” criteria now have pathways for those who are symptomatic or asymptomatic, requiring expectant management with the presence of barriers to discharge and/or social determinants of health, and IV fluid or blood product transfusion criteria line has been added as an intervention • “Pregestational diabetes mellitus” was added to the Acute level of care on Episode Day 3-X with gestational diabetes mellitus to include a pathway for individuals with these conditions who develop uncontrolled blood glucose during hospitalization • “Hypoglycemic agent” was changed to “glucose-lowering medication,” and “oral glucose-lowering medication” was added to align with current guidelines and to be more inclusive of all treatment options • “Preterm labor” criteria were updated to better define the individual with preterm labor that requires evaluation and management at the Acute level of care • “Preterm prelabor rupture of membranes (PPROM)” criteria were updated with the gestational age, an additional pathway for new onset confirmed with a hover note that includes the required findings, and uterine monitoring has been added as an intervention to better define the individual that requires management at the Acute level of care

Antepartum		
Episode Day 3-X (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦Finding, ≥ One: ▶Active bleeding within last 24h ▶Hct or Hb decreased last 24h ▶Non-reassuring fetal testing (e.g., positive or nonreactive stress test) ▶Orthostatic hypotension, Both: ♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Blood pressure, ≥ One: ▶Systolic drop ≥ 20 mmHg ▶Diastolic drop ≥ 10 mmHg ♦Intervention, All: ▶Hct or Hb monitoring ▶Fetal heart rate and uterine monitoring ▶IV fluid or blood product transfusion ▶Expectant management, fetal heart rate and uterine monitoring and, One: ♦Barrier to discharge and arrangements for outpatient services pending ≤ 24h, ≥ One: ▶Limited access to prompt medical care ▶Patient unreliable ♦Outpatient services unavailable ♦Preterm labor and, Both: ▶Maternal or fetal instability ≥ One: ♦Cervical dilation or effacement within last 3d ♦Contractions ≥ 4/h ♦Non-reassuring fetal testing (e.g., positive or nonreactive stress test) ▶Fetal heart rate and uterine monitoring ♦Preterm prelabor rupture of membranes (PPROM), expectant management and, ≥ One: ▶Anti-infective (includes PO) ▶Fetal heart rate monitoring ♦Post Critical care ≤ 24h ▶Non-responder, not clinically stable for discharge and does not meet partial responder criteria, One: ♦Use Extended Stay subset ♦Refer for secondary review	▶Intervention, ≥ One: ♦Anti-infective (includes PO) ♦Fetal heart rate monitoring or uterine monitoring ♦Post Critical care ≤ 24h ▶Non-responder, not clinically stable for discharge and does not meet partial responder criteria, One: ♦Use Extended Stay subset ♦Refer for secondary review	• “Post Critical care” criterion has been added as a pathway to step-down to the Acute level of care on Episode Day 2

Antepartum		
Episode Day 3-X (INTERMEDIATE)		
Previously Available Content	2024 March Release	Revision Description
• INTERMEDIATE, One: ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦ Arrhythmia, ≥ One: ▶ IV antiarrhythmic and hemodynamically stable ▶ Antiarrhythmic conversion from IV to PO, One: ♦ No proarrhythmic risk ≤ 24h ♦ Proarrhythmic risk and continuous cardiac monitoring (excludes Holter) ≤ 3d ♦ Seizure and, Both: ▶ Activity resolved ▶ Anticonvulsant continuous ▶ Non-responder, not clinically stable for discharge, One: ♦ Does not meet partial responder criteria or at the Acute level of care, One: ▶ Use Extended Stay subset ▶ Refer for secondary review	• INTERMEDIATE, ≥ One: ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦ HFNC and respiratory monitoring every 3-4h ♦ NIPPV ▶ Non-responder, not clinically stable for discharge, One: ♦ Does not meet partial responder criteria or at the Acute level of care, One: ▶ Use Extended Stay subset ▶ Refer for secondary review	Rationale: • “Arrhythmia” content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • “HFNC” and “NIPPV” criteria were added to provide a pathway for individuals who require this intervention at the Intermediate level of care • “Seizure (excludes eclampsia and preeclampsia)” criteria were removed as an individual with eclampsia can be reviewed in the Hypertensive Disorders of Pregnancy subset, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology

Antepartum		
Episode Day 3-X (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
• CRITICAL, One: ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦ Arrhythmia, ≥ One: ▶ Urgent electrical cardioversion performed within last 24h ▶ Defibrillation performed within last 24h ▶ Temporary pacemaker insertion performed within last 2d ▶ Symptomatic recurrent or refractory arrhythmia requiring antiarrhythmic and, ≥ One: ♦ Chest pain ♦ Heart failure and, ≥ One: ▶ Arterial Po₂ < 56 mmHg(7.4 kPa) ▶ O₂ sat < 89%(0.89) and < baseline ♦ Hemodynamic instability, ≥ One: ▶ Heart rate > 120/min, sustained ▶ Systolic blood pressure, ≥ One: ♦ < 90 mmHg	• CRITICAL, One: ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦ DIC and blood product transfusion ♦ ECMO or ECLS ♦ Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion and, Both: ▶ Finding, ≥ One: ♦ Heart rate > 120/min ♦ Systolic blood pressure < 100 mmHg and < baseline ♦ MAP < 65 mmHg ▶ Intervention, ≥ One: ♦ Blood product transfusion ♦ IV fluid resuscitation ≥ 1L ≤ 24h ♦ Hemodynamic monitoring, invasive ♦ HFNC and respiratory monitoring every 1-2h ♦ Mechanical ventilation	Rationale: • “Arrhythmia” content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • “Oxygen ≥ 40%” criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Critical Level of Care for the patient who is pregnant

Antepartum		
Episode Day 3-X (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
♦ < 110 mmHg with chronic hypertension ♦ > 30 mmHg decrease from baseline ♦ Labile ▶ MAP < 65 mmHg ♦ DIC and blood product transfusion ♦ ECMO or ECLS ♦ Hemodynamic instability persisting after ≥1L of IVF resuscitation or ≥1 unit blood product transfusion and, Both: ▶ Finding, ≥ One: ♦ Heart rate > 120/min ♦ Systolic blood pressure < 100 mmHg and < baseline ♦ MAP < 65 mmHg ▶ Intervention, ≥ One: ♦ Blood product transfusion ♦ IV fluid resuscitation ≥1L ≤ 24h ♦ Hemodynamic monitoring, invasive ♦ HFNC and respiratory monitoring every 1-2h ♦ Mechanical ventilation ♦ NIPPV and, ≥ One: ▶ FiO₂ > 50%(0.50) ▶ Respiratory intervention every 1-2h ♦ Oxygen ≥ 40%(0.40), ≤ 2d ♦ Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) ▶ Non-responder, not clinically stable for discharge, One: ♦ Does not meet partial responder criteria or at the Acute or Intermediate level of care, One: ▶ Use Extended Stay subset ▶ Refer for secondary review	♦ NIPPV and, ≥ One: ▶ FiO₂ > 50%(0.50) ▶ Respiratory intervention every 1-2h ♦ Oxygen ≥ 40%(0.40) ♦ Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) ▶ Non-responder, not clinically stable for discharge, One: ♦ Does not meet partial responder criteria or at the Acute or Intermediate level of care, One: ▶ Use Extended Stay subset ▶ Refer for secondary review	

Asthma

 Revision Summary	Respiratory rate criteria were updated at the Observation and Acute levels of care to better align with the current Global Initiative for Asthma (GINA) guidelines.
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Asthma		
Initial Review (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ▶High risk for decompensation, ≥ One: ◆Comorbidity ◆History of asthma requiring intubation and mechanical ventilation ◆Hospitalization or ED visit for asthma within the past year ◆Oral steroid use, current or within the past month ◆Poor adherence with asthma medications and/or poor adherence with an asthma action plan ◆Pregnancy ◆Use of > 1 canister of short-acting beta-agonist per month ▶Partial response to at least 2h of initial treatment and, One: ◆Wheezing, persistent and, ≥ One: ▶O₂ sat < 95%(0.95) and < baseline ▶PEF or forced expiratory volume at one second (FEV₁) 40-69%(0.40-0.69) or symptoms of airway obstruction ▶Respiratory rate > 20 - 30/min, sustained	•OBSERVATION, One: ▶High risk for decompensation, ≥ One: ◆Comorbidity ◆History of asthma requiring intubation and mechanical ventilation ◆Hospitalization or ED visit for asthma within the past year ◆Oral steroid use, current or within the past month ◆Poor adherence with asthma medications and/or poor adherence with an asthma action plan ◆Pregnancy ◆Use of > 1 canister of short-acting beta-agonist per month ▶Partial response to at least 2h of initial treatment and, One: ◆Wheezing, persistent and, ≥ One: ▶O₂ sat < 95%(0.95) and < baseline ▶PEF or forced expiratory volume at one second (FEV₁) 40-69%(0.40-0.69) or symptoms of airway obstruction ▶Respiratory rate 22-30/min, sustained	Rationale: • Respiratory rate criterion was updated to align with current Global Initiative for Asthma (GINA) guidelines

Asthma		
Episode Day 1 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ▶High risk for decompensation, Both: ♦Risk factor, ≥ One: ▶Comorbidity ▶History of asthma requiring intubation and mechanical ventilation ▶Hospitalization or ED visit for asthma within the past year ▶Oral steroid use, current or within the past month ▶Poor adherence with asthma medications and/or poor adherence with an asthma action plan ▶Pregnancy ▶Use of > 1 canister of short-acting beta-agonist per month ♦Intervention, Both: ▶Corticosteroid (includes PO) ▶Short-acting beta-agonist ≥ 4x/24h ▶Partial response to at least 2h of initial treatment and, Both: ♦Wheezing, persistent and, ≥ One: ▶O₂ sat < 95%(0.95) and < baseline ▶PEF or forced expiratory volume at one second (FEV₁) 40-69%(0.40-0.69) or symptoms of airway obstruction ▶Respiratory rate > 20 - 30/min, sustained ♦Intervention, Both: ▶Corticosteroid (includes PO) ▶Short-acting beta-agonist ≥ 4x/24h	•OBSERVATION, One: ▶High risk for decompensation, Both: ♦Risk factor, ≥ One: ▶Comorbidity ▶History of asthma requiring intubation and mechanical ventilation ▶Hospitalization or ED visit for asthma within the past year ▶Oral steroid use, current or within the past month ▶Poor adherence with asthma medications and/or poor adherence with an asthma action plan ▶Pregnancy ▶Use of > 1 canister of short-acting beta-agonist per month ♦Intervention, Both: ▶Corticosteroid (includes PO) ▶Short-acting beta-agonist ≥ 4x/24h ▶Partial response to at least 2h of initial treatment and, Both: ♦Wheezing, persistent and, ≥ One: ▶O₂ sat < 95%(0.95) and < baseline ▶PEF or forced expiratory volume at one second (FEV₁) 40-69%(0.40-0.69) or symptoms of airway obstruction ▶Respiratory rate 22-30/min, sustained ♦Intervention, Both: ▶Corticosteroid (includes PO) ▶Short-acting beta-agonist ≥ 4x/24h	Rationale: • Respiratory rate criterion was updated to align with current Global Initiative for Asthma (GINA) guidelines

Asthma		
Episode Day 2 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Wheezing, resolved or resolving ♦O₂ sat within acceptable limits ♦PEF or forced expiratory volume at one second (FEV₁) within acceptable limits or no symptoms of airway obstruction ♦Vital signs stable or within acceptable limits ♦No complication or active comorbidity relevant to this episode of care	•OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Wheezing, resolved or resolving ♦O₂ sat within acceptable limits ♦PEF or forced expiratory volume at one second (FEV₁) within acceptable limits or no symptoms of airway obstruction ♦Vital signs stable or within acceptable limits ♦No complication or active comorbidity relevant to this episode of care	Rationale: • Respiratory rate criterion was updated to align with current Global Initiative for Asthma (GINA) guidelines

Asthma		
Episode Day 2 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
► Partial responder, not clinically stable for discharge and requires continued stay, Both: ◆ Wheezing, persistent and, ≥ One: ► O₂ sat < 95%(0.95) and < baseline ► PEF or forced expiratory volume at one second (FEV₁) 40-69%(0.40-0.69) or symptoms of airway obstruction ► Respiratory rate > 20 - 30/min, sustained ◆ Intervention, Both: ► Corticosteroid (includes PO) ► Short-acting beta-agonist ≥ 4x/24h	► Partial responder, not clinically stable for discharge and requires continued stay, Both: ◆ Wheezing, persistent and, ≥ One: ► O₂ sat < 95%(0.95) and < baseline ► PEF or forced expiratory volume at one second (FEV₁) 40-69%(0.40-0.69) or symptoms of airway obstruction ► Respiratory rate 22-30/min, sustained ◆ Intervention, Both: ► Corticosteroid (includes PO) ► Short-acting beta-agonist ≥ 4x/24h	

Asthma		
Episode Day 3 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
• ACUTE, One: ► Responder, discharge expected today if clinically stable last 12h, All: ◆ Wheezing, resolved or resolving ◆ O₂ sat within acceptable limits ◆ PEF or forced expiratory volume at one second (FEV₁) within acceptable limits or no symptoms of airway obstruction ◆ Vital signs stable or within acceptable limits ◆ No complication or active comorbidity relevant to this episode of care ► Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ◆ Wheezing, persistent and, Both: ► Finding, ≥ One: ◆ O₂ sat < 90%(0.90) and < baseline ◆ PEF or forced expiratory volume at one second (FEV₁) < 70%(0.70) or symptoms of airway obstruction ◆ Respiratory rate > 20 - 30/min, sustained ► Corticosteroid (includes PO) and, ≥ One: ◆ Short-acting beta-agonist ≥ 4x/24h ◆ Supplemental oxygen ◆ Post Critical care ≤ 24h	• ACUTE, One: ► Responder, discharge expected today if clinically stable last 12h, All: ◆ Wheezing, resolved or resolving ◆ O₂ sat within acceptable limits ◆ PEF or forced expiratory volume at one second (FEV₁) within acceptable limits or no symptoms of airway obstruction ◆ Vital signs stable or within acceptable limits ◆ No complication or active comorbidity relevant to this episode of care ► Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ◆ Wheezing, persistent and, Both: ► Finding, ≥ One: ◆ O₂ sat < 90%(0.90) and < baseline ◆ PEF or forced expiratory volume at one second (FEV₁) < 70%(0.70) or symptoms of airway obstruction ◆ Respiratory rate 22-30/min, sustained ► Corticosteroid (includes PO) and, ≥ One: ◆ Short-acting beta-agonist ≥ 4x/24h ◆ Supplemental oxygen ◆ Post Critical care ≤ 24h	Rationale: • Respiratory rate criterion was updated to align with current Global Initiative for Asthma (GINA) guidelines

COPD



Revision Summary

A new pathway was added to Acute and Intermediate levels of care on Episode Day 1 to account for patients who are hospitalized and develop an acute COPD exacerbation. Frailty criteria were added as a new clinical risk factor to identify patients who may necessitate the Acute level of care.

COPD

Global (ACUTE)

Previously Available Content

- ▶ **Responder**, discharge expected today if clinically stable last 12h, **All**:
 - ♦ Afebrile
 - ♦ Blood gas or O₂ sat within acceptable limits
 - ♦ Dyspnea improving
 - ♦ Complication or comorbidity, **One**:
 - ▶ No complication or active comorbidity relevant to this episode of care
 - ▶ Complication or comorbidity and clinically stable for discharge, **≥ One**:
 - ♦ Blood glucose within acceptable limits
 - ♦ Functional evaluation complete
 - ♦ WBC within acceptable limits

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- ▶ **Responder**, discharge expected today if clinically stable last 12h, **All**:
 - ♦ Afebrile
 - ♦ Blood gas or O₂ sat within acceptable limits
 - ♦ Dyspnea improving
 - ♦ Complication or comorbidity, **One**:
 - ▶ No complication or active comorbidity relevant to this episode of care
 - ▶ Complication or comorbidity and clinically stable for discharge, **≥ One**:
 - ♦ Blood glucose within acceptable limits
 - ♦ Functional evaluation complete
 - ♦ **Heart failure resolving or at baseline**
 - ♦ WBC within acceptable limits

Revision Description

Rationale:

- “Heart failure resolving or at baseline” criteria point was added to better define the patient who is ready for discharge

COPD

Initial Review (OBSERVATION)

Previously Available Content

- OBSERVATION, Both**:
 - ▶ Short-acting beta-agonist administered prior to admission by licensed medical professional **≥ 2** doses
 - ▶ Post treatment finding, **≥ One**:
 - ♦ O₂ sat 90-91%(0.90-0.91) and < baseline
 - ♦ Arterial Po₂ 56-60 mmHg(7.4-8.0 kPa) and < baseline
 - ♦ Arterial or venous Pco₂ 41-44 mmHg(5.5-5.9 kPa) and > baseline
 - ♦ Increased work of breathing, **≥ One**:
 - ▶ Difficulty taking PO due to respiratory effort
 - ▶ Prefers sitting due to respiratory effort
 - ▶ Talks in phrases due to respiratory effort

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- OBSERVATION, Both**:
 - ▶ Short-acting beta-agonist administered prior to admission by licensed medical professional **≥ 2** doses
 - ▶ Post **initial** treatment finding, **≥ One**:
 - ♦ O₂ sat 90-91%(0.90-0.91) and < baseline
 - ♦ Arterial Po₂ 56-60 mmHg(7.4-8.0 kPa) and < baseline
 - ♦ Arterial or venous Pco₂ 41-44 mmHg(5.5-5.9 kPa) and > baseline
 - ♦ Increased work of breathing, **≥ One**:
 - ▶ Difficulty taking PO due to respiratory effort
 - ▶ Prefers sitting due to respiratory effort
 - ▶ Talks in phrases due to respiratory effort

Revision Description

Rationale:

- “Post treatment finding” was changed to “Post initial treatment finding” to capture the patient with findings after treatment provided prior to the decision to admit

COPD		
Initial Review (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
ACUTE, Both: ▶Short-acting beta-agonist administered prior to admission by licensed medical professional ≥ 2 doses ▶Post treatment finding, $\geq$ One: ♦O₂ sat $\leq 89\%$(0.89) and $<$ baseline ♦Arterial Po₂ 40-55 mmHg(5.3-7.3 kPa) and pH 7.45-7.49 ♦Increased work of breathing, $\geq$ One: ▶Unable to take PO due to respiratory effort ▶Hunched over position ♦High risk for adverse event, Both: ▶Finding, $\geq$ One: ♦O₂ sat 90-91%(0.90-0.91) and $<$ baseline ♦Arterial Po₂ 56-60 mmHg(7.4-8.0 kPa) and $<$ baseline ♦Arterial or venous Pco₂ 41-44 mmHg(5.5-5.9 kPa) and $>$ baseline ♦Increased work of breathing, $\geq$ One: ▶Difficulty taking PO due to respiratory effort ▶Prefers sitting due to respiratory effort ▶Talks in phrases due to respiratory effort ▶Clinical risk factor, $\geq$ One: ♦Finding, $\geq$ One: ▶Blood urea nitrogen (BUN) > 25.0 mg/dL(8.9 mmol/L) ▶Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦Post-bronchodilator FEV₁ $< 50\%$(0.50) predicted ♦Home oxygen therapy ≥ 15 h/24h ♦Prior endotracheal intubation for respiratory distress within last 12 mos ♦Cor pulmonale ♦Heart failure, NYHA Class III or Class IV ♦Pneumonia actual or suspected and, $\geq$ One: ▶Temperature $\geq 100.4^{\circ}\text{F}$(38.0°C) ▶WBC $\geq 12,000/\text{cu.mm}$($12 \times 10^9/\text{L}$) ▶WBC $\leq 4,000/\text{cu.mm}$($4 \times 10^9/\text{L}$) ▶Bands $> 10\%$(0.10) ♦Severe and persistent mental illness, cognitive impairment, or substance use disorder and, Both: ▶Patient unreliable and unable to adhere to outpatient instructions or treatment ▶Caregiver unavailable or unable to manage care	ACUTE, Both: ▶Short-acting beta-agonist administered prior to admission by licensed medical professional ≥ 2 doses ▶Post initial treatment finding, $\geq$ One: ♦O₂ sat $\leq 89\%$(0.89) and $<$ baseline ♦Arterial Po₂ 40-55 mmHg(5.3-7.3 kPa) and pH 7.45-7.49 ♦Increased work of breathing, $\geq$ One: ▶Unable to take PO due to respiratory effort ▶Hunched over position ♦High risk for adverse event, Both: ▶Post treatment finding, $\geq$ One: ♦O₂ sat 90-91%(0.90-0.91) and $<$ baseline ♦Arterial Po₂ 56-60 mmHg(7.4-8.0 kPa) and $<$ baseline ♦Arterial or venous Pco₂ 41-44 mmHg(5.5-5.9 kPa) and $>$ baseline ♦Increased work of breathing, $\geq$ One: ▶Difficulty taking PO due to respiratory effort ▶Prefers sitting due to respiratory effort ▶Talks in phrases due to respiratory effort ▶Clinical risk factor, $\geq$ One: ♦Finding, $\geq$ One: ▶Blood urea nitrogen (BUN) > 25.0 mg/dL(8.9 mmol/L) ▶Mental status change (excludes coma, stupor, or obtundation) or GCS 9-14 ♦Post-bronchodilator FEV₁ $< 50\%$(0.50) predicted ♦Home oxygen therapy ≥ 15 h/24h ♦Prior endotracheal intubation for respiratory distress within last 12 mos ♦Cor pulmonale ♦Frailty, Both: ▶Age ≥ 65 yrs ▶Finding, $\geq$ One: ♦Frailty, active diagnosis ♦Frailty characteristics, $\geq$ Two: ▶Chronic condition(s), $\geq$ One: ♦Dementia or cognitive impairment ♦Multimorbidity, ≥ 2 chronic illnesses ▶Functional impairment ▶Malnutrition ▶Polypharmacy, ≥ 5 chronic medications ♦Heart failure, NYHA Class III or Class IV ♦Pneumonia actual or suspected and, $\geq$ One: ▶Temperature $\geq 100.4^{\circ}\text{F}$(38.0°C) ▶WBC $\geq 12,000/\text{cu.mm}$($12 \times 10^9/\text{L}$) ▶WBC $\leq 4,000/\text{cu.mm}$($4 \times 10^9/\text{L}$) ▶Bands $> 10\%$(0.10) ♦Severe and persistent mental illness, cognitive impairment, or substance use	Rationale: • “Post treatment finding” was changed to “Post initial treatment finding” to capture the patient with findings after treatment provided prior to the decision to admit • Frailty criteria were added as a new clinical risk factor to identify frail individuals who may necessitate the Acute level of care during an acute COPD exacerbation due to their increased risk of adverse outcomes, including mortality

COPD		
Initial Review (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
	disorder and, Both: ►Patient unreliable and unable to adhere to outpatient instructions or treatment ►Caregiver unavailable or unable to manage care	

COPD		
Episode Day 1 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, All: ►Short-acting beta-agonist administered prior to admission by licensed medical professional ≥ 2 doses ►Post treatment finding, $\geq$ One: ♦O₂ sat 90-91%(0.90-0.91) and < baseline ♦Arterial Po₂ 56-60 mmHg(7.4-8.0 kPa) and < baseline ♦Arterial or venous Pco₂ 41-44 mmHg(5.5-5.9 kPa) and > baseline ♦Increased work of breathing, $\geq$ One: ►Difficulty taking PO due to respiratory effort ►Prefers sitting due to respiratory effort ►Talks in phrases due to respiratory effort ►Intervention, Both: ♦Bronchodilator $\geq 4x/24h$ (includes inhaled) ♦Corticosteroid (includes PO or inhaled)	•OBSERVATION, All: ►Short-acting beta-agonist administered prior to admission by licensed medical professional ≥ 2 doses ►Post initial treatment finding, $\geq$ One: ♦O₂ sat 90-91%(0.90-0.91) and < baseline ♦Arterial Po₂ 56-60 mmHg(7.4-8.0 kPa) and < baseline ♦Arterial or venous Pco₂ 41-44 mmHg(5.5-5.9 kPa) and > baseline ♦Increased work of breathing, $\geq$ One: ►Difficulty taking PO due to respiratory effort ►Prefers sitting due to respiratory effort ►Talks in phrases due to respiratory effort ►Intervention, Both: ♦Bronchodilator $\geq 4x/24h$ (includes inhaled) ♦Corticosteroid (includes PO or inhaled)	Rationale: • “Post treatment finding” was changed to “Post initial treatment finding” to capture the patient with findings after treatment provided prior to the decision to admit

COPD		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
•ACUTE, All: ►Short-acting beta-agonist administered prior to admission by licensed medical professional ≥ 2 doses ►Post treatment finding, $\geq$ One: ♦O₂ sat $\leq 89\%$(0.89) and < baseline ♦Arterial Po₂ 40-55 mmHg(5.3-7.3 kPa) and pH 7.45-7.49 ♦Increased work of breathing, $\geq$ One:	•ACUTE, One: ►COPD exacerbation, All: ♦Short-acting beta-agonist administered prior to admission by licensed medical professional ≥ 2 doses ♦Post initial treatment finding, $\geq$ One: ►O₂ sat $\leq 89\%$(0.89) and < baseline ►Arterial Po₂ 40-55 mmHg(5.3-7.3 kPa) and	Rationale: • A new pathway was added to allow patients who were hospitalized for another condition and then developed an acute COPD exacerbation as a new

COPD		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
▶ Unable to take PO due to respiratory effort ▶ Hunched over position ♦ High risk for adverse event, Both: ▶ Post treatment finding, ≥ One: ♦ O₂ sat 90-91%(0.90-0.91) and < baseline ♦ Arterial Po₂ 56-60 mmHg(7.4-8.0 kPa) and < baseline ♦ Arterial or venous Pco₂ 41-44 mmHg(5.5-5.9 kPa) and > baseline ♦ Increased work of breathing, ≥ One: ▶ Difficulty taking PO due to respiratory effort ▶ Prefers sitting due to respiratory effort ▶ Talks in phrases due to respiratory effort ▶ Clinical risk factor, ≥ One: ♦ Finding, ≥ One: ▶ Blood urea nitrogen (BUN) > 25.0 mg/dL(8.9 mmol/L) ▶ Mental status change (excludes coma, stupor, or obtundation) or GCS 9-14 ♦ Post-bronchodilator FEV₁ < 50%(0.50) predicted ♦ Home oxygen therapy ≥ 15 h/24h ♦ Prior endotracheal intubation for respiratory distress within last 12 mos ♦ Cor pulmonale ♦ Heart failure, NYHA Class III or Class IV ♦ Pneumonia actual or suspected and, ≥ One: ▶ Temperature ≥ 100.4°F(38.0°C) ▶ WBC ≥ 12,000/cu.mm(12x10⁹/L) ▶ WBC ≤ 4,000/cu.mm(4 x10⁹/L) ▶ Bands > 10%(0.10) ♦ Severe and persistent mental illness, cognitive impairment, or substance use disorder and, Both: ▶ Patient unreliable and unable to adhere to outpatient instructions or treatment ▶ Caregiver unavailable or unable to manage care ▶ Intervention, Both: ♦ Bronchodilator ≥ 4x/24h (includes inhaled) ♦ Corticosteroid (includes PO)	pH 7.45-7.49 ▶ Increased work of breathing, ≥ One: ♦ Unable to take PO due to respiratory effort ♦ Hunched over position ▶ High risk for adverse event, Both: ♦ Post treatment finding, ≥ One: ▶ O₂ sat 90-91%(0.90-0.91) and < baseline ▶ Arterial Po₂ 56-60 mmHg(7.4-8.0 kPa) and < baseline ▶ Arterial or venous Pco₂ 41-44 mmHg(5.5-5.9 kPa) and > baseline ♦ Increased work of breathing, ≥ One: ▶ Difficulty taking PO due to respiratory effort ▶ Prefers sitting due to respiratory effort ▶ Talks in phrases due to respiratory effort ♦ Clinical risk factor, ≥ One: ♦ Finding, ≥ One: ▶ Blood urea nitrogen (BUN) > 25.0 mg/dL(8.9 mmol/L) ▶ Mental status change (excludes coma, stupor, or obtundation) or GCS 9-14 ♦ Post-bronchodilator FEV₁ < 50%(0.50) predicted ♦ Home oxygen therapy ≥ 15 h/24h ♦ Prior endotracheal intubation for respiratory distress within last 12 mos ♦ Cor pulmonale ♦ Frailty, Both: ♦ Age ≥ 65 yrs ♦ Finding, ≥ One: ▶ Frailty, active diagnosis ▶ Frailty characteristics, ≥ Two: ♦ Chronic condition(s), ≥ One: -Dementia or cognitive impairment -Multimorbidity, ≥ 2 chronic illnesses ♦ Functional impairment ♦ Malnutrition ♦ Polypharmacy, ≥ 5 chronic medications ▶ Heart failure, NYHA Class III or Class IV ▶ Pneumonia actual or suspected and, ≥ One: ♦ Temperature ≥ 100.4°F(38.0°C) ♦ WBC ≥ 12,000/cu.mm(12x10⁹/L) ♦ WBC ≤ 4,000/cu.mm(4 x10⁹/L) ♦ Bands > 10%(0.10) ▶ Severe and persistent mental illness, cognitive impairment, or substance use disorder and, Both: ♦ Patient unreliable and unable to adhere to outpatient instructions or treatment ♦ Caregiver unavailable or unable to manage care	primary condition a place to be reviewed on Episode Day 1 at the Acute level of care • “Post treatment finding” was changed to “Post initial treatment finding” to capture the patient with findings after treatment provided prior to the decision to admit • Frailty criteria were added as a new clinical risk factor to identify frail individuals who may necessitate the Acute level of care during an acute COPD exacerbation due to their increased risk of adverse outcomes, including mortality

COPD		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
	♦Intervention, Both: ▶Bronchodilator ≥ 4x/24h (includes inhaled) ▶Corticosteroid (includes PO) ▶COPD exacerbation in a hospitalized patient and, Both: ♦Finding, ≥ One: ▶O₂ sat ≤ 89%(0.89) and < baseline ▶Arterial Po₂ 40-55 mmHg(5.3-7.3 kPa) and pH 7.45-7.49 ▶Increased work of breathing, ≥ One: ♦Unable to take PO due to respiratory effort ♦Hunched over position ♦Intervention, Both: ▶Bronchodilator ≥ 4x/24h (includes inhaled) ▶Corticosteroid (includes PO)	

COPD		
Episode Day 1 (INTERMEDIATE)		
Previously Available Content	2024 March Release	Revision Description
♦INTERMEDIATE, All: ▶Short-acting beta-agonist administered prior to admission by licensed medical professional ≥ 2 doses ▶Post treatment finding, ≥ One: ♦Arterial Po₂ 40-55 mmHg(5.3-7.3 kPa) ♦Arterial or venous Pco₂ 45-60 mmHg(5.9-7.9 kPa) and pH ≤ 7.35 ♦Cyanosis, new onset or worsening ♦Intervention, All: ♦Bronchodilator ≥ 4x/24h (includes inhaled) ♦Corticosteroid (includes PO) ♦Oxygenation, One: ▶HFNC and respiratory monitoring every 3-4h ▶NIPPV	♦INTERMEDIATE, One: ▶COPD exacerbation, All: ♦Short-acting beta-agonist administered prior to admission by licensed medical professional ≥ 2 doses ♦Post initial treatment finding, ≥ One: ▶Arterial Po₂ 40-55 mmHg(5.3-7.3 kPa) ▶Arterial or venous Pco₂ 45-60 mmHg(5.9-7.9 kPa) and pH ≤ 7.35 ▶Cyanosis, new onset or worsening ♦Intervention, All: ▶Bronchodilator ≥ 4x/24h (includes inhaled) ▶Corticosteroid (includes PO) ▶Oxygenation, One: ♦HFNC and respiratory monitoring every 3-4h ♦NIPPV ▶COPD exacerbation in a hospitalized patient and, Both: ♦Finding, ≥ One: ▶Arterial Po₂ 40-55 mmHg(5.3-7.3 kPa) ▶Arterial or venous Pco₂ 45-60 mmHg(5.9-7.9 kPa) and pH ≤ 7.35 ▶Cyanosis, new onset or worsening ♦Intervention, All: ▶Bronchodilator ≥ 4x/24h (includes inhaled) ▶Corticosteroid (includes PO)	Rationale: - A new pathway was added to allow patients who were hospitalized for another condition and then developed an acute COPD exacerbation as a new primary condition a place to be reviewed on Episode Day 1 at the Intermediate level of care “Post treatment finding” was changed to “Post initial treatment finding” to capture the patient with findings after treatment provided prior to the decision to admit

COPD		
Episode Day 1 (INTERMEDIATE)		
Previously Available Content	2024 March Release	Revision Description

- ▶Oxygenation, **One:**
 - ◆HFNC and respiratory monitoring every 3-4h
 - ◆NIPPV

Deep Vein Thrombosis

 Revision Summary	“DVT confirmed by ultrasound” criterion was removed from Initial Review and Episode Day 1 at the Observation and Acute levels of care, and the subset introduction note now indicates that a patient must have a confirmed deep vein thrombosis on imaging to be reviewed in this subset. DVT content was updated to better define the patient who requires the Observation level of care, which is those with an uncomplicated DVT who are unable to administer anticoagulation and outpatient services are unavailable. At the Acute level of care, splanchnic vein thrombosis criteria were added to capture patients requiring hospitalization for a thrombus in their portal, mesenteric, and/or splenic vein(s). In addition, heparin-induced thrombocytopenia (HIT) criteria were added to the Acute level of care since it is a complication that could develop with exposure to heparin.
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Deep Vein Thrombosis		
Initial Review (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, Both: ▶DVT confirmed by ultrasound ▶Risk for bleeding requiring monitoring and anticoagulation (includes PO) and, One: ♦Direct oral anticoagulant (DOAC) ♦Low molecular weight heparin (LMWH) ♦Fondaparinux ♦Argatroban ♦Bivalirudin	•OBSERVATION, One: ▶DVT and anticoagulation (includes PO) and, Both: ♦Patient or caregiver unable to administer ♦Outpatient services unavailable	Rationale: • “Confirmed by ultrasound” was removed and now the introduction note indicates that a patient must have a DVT confirmed by imaging to be reviewed in this subset • “Risk for bleeding requiring monitoring” criteria were removed from the Observation level of care as patients with a high risk for bleeding are appropriate at the Acute level of care while being monitored after the initiation of unfractionated heparin drip • Argatroban and bivalirudin were removed from the Observation level of care as these medications are used in patients with HIT and should be reviewed at the Acute level of care for close monitoring • “Patient or caregiver unable to administer” and “Outpatient services unavailable” were added to describe the patient with low-risk DVT who requires the Observation level of care to allow for time to arrange outpatient services

Deep Vein Thrombosis		
Initial Review (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
•ACUTE, One: ▶DVT confirmed by ultrasound and, One: ◆Unfractionated heparin, continuous and, ≥ One: ▶Creatinine clearance ≤ 30 mL/min ▶Morbidity obesity (BMI ≥ 40 kg/m² or weight > 120 kg) ▶Antiphospholipid antibody syndrome ▶Bleeding, active or increased risk ◆Risk of limb compromise and anticoagulation ◆IVC filter placement performed ≤ 24h	•ACUTE, One: ◆Anticoagulation (includes PO) and, ≥ One: ▶Extensive DVT and neurovascular assessment at least 6x/24h ▶Splanchnic vein thrombosis ◆UFH, continuous and, ≥ One: ▶Antiphospholipid antibody syndrome ▶Bleeding, active or increased risk ▶BMI ≥ 40 kg/m² or weight > 120 kg ▶Creatinine clearance ≤ 30 mL/min ◆HIT, suspected or by history and anticoagulation ◆IVC filter placement performed ≤ 24h	Rationale: • “DVT confirmed by ultrasound” was removed and is now located in the introduction note indicating that a patient must have a DVT confirmed by imaging to be reviewed in this subset • “Risk of limb compromise” was changed to “Extensive DVT” to clarify the intent of the criteria • “includes PO” was added to anticoagulation for the treatment of extensive DVT since the driver for admission to the Acute level of care is not the treatment, but rather that an extensive DVT is considered a complicated DVT which requires frequent monitoring • “Neurovascular assessment at least 6x/24h” was added to extensive DVT since frequent monitoring is the driver for admission to the Acute level of care • “Splanchnic vein thrombosis” was added to be more inclusive of potential locations where a DVT can develop in patients requiring hospitalization • HIT criteria were added as it is a complication that is associated with heparin use that requires the Acute level of care due to the risk of bleeding and need for close monitoring while on alternative anticoagulation

Deep Vein Thrombosis		
Episode Day 1 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, Both: ▶DVT confirmed by ultrasound ▶Risk for bleeding requiring monitoring and anticoagulation (includes PO) and, One: ◆Direct oral anticoagulant (DOAC) ◆Low molecular weight heparin (LMWH)	•OBSERVATION, One: ▶DVT and anticoagulation (includes PO) and, Both: ◆Patient or caregiver unable to administer ◆Outpatient services unavailable	Rationale: • “DVT confirmed by ultrasound” was removed and is now located in the introduction note indicating

Deep Vein Thrombosis		
Episode Day 1 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
♦Fondaparinux ♦Argatroban ♦Bivalirudin		that a patient must have a DVT confirmed by imaging to be reviewed in this subset • “Risk for bleeding requiring monitoring” criteria were removed from the Observation level of care as patients with a high risk for bleeding are appropriate at the Acute level of care while being monitored after the initiation of unfractionated heparin drip • Argatroban and bivalirudin were removed from the Observation level of care as these medications are used in patients with HIT and should be reviewed at the Acute level of care for close monitoring • “Patient or caregiver unable to administer” and “Outpatient services unavailable” were added to describe the patient with low-risk DVT who requires the Observation level of care to allow for time to arrange outpatient services

Deep Vein Thrombosis		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦ACUTE, One: ▶DVT confirmed by ultrasound and, One: ♦Unfractionated heparin, continuous and, ≥ One: ▶Creatinine clearance ≤ 30 mL/min ▶Morbidity (BMI ≥ 40 kg/m² or weight > 120 kg) ▶Bleeding, active or increased risk ▶Antiphospholipid antibody syndrome ♦Risk of limb compromise and anticoagulation ♦Hospital acquired and initiation of anticoagulation (includes PO) ≤ 24h ♦IVC filter placement performed ≤ 24h	♦ACUTE, One: ♦Anticoagulation (includes PO) and, ≥ One: ▶Extensive DVT and neurovascular assessment at least 6x/24h ▶Splanchnic vein thrombosis ♦UFH, continuous and, ≥ One: ▶Antiphospholipid antibody syndrome ▶Bleeding, active or increased risk ▶BMI ≥ 40 kg/m² or weight > 120 kg ▶Creatinine clearance ≤ 30 mL/min ♦HIT, suspected or by history and anticoagulation ♦Hospital acquired and initiation of anticoagulation (includes PO) ≤ 24h ♦IVC filter placement performed ≤ 24h	Rationale: • “DVT confirmed by ultrasound” was removed and is now located in the introduction note indicating that a patient must have a DVT confirmed by imaging to be reviewed in this subset • “Risk of limb compromise” was changed to “Extensive DVT” to clarify the intent of the criteria • “Includes PO” was added to anticoagulation for the treatment of extensive DVT since the driver for admission to the Acute care is not the treatment, but rather that an extensive DVT is considered a

Deep Vein Thrombosis		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
		complicated DVT which requires frequent monitoring • “Neurovascular assessment at least 6x/24h” was added to extensive DVT since frequent monitoring is the driver for admission to the Acute level of care • “Splanchnic vein thrombosis” was added to be more inclusive of potential locations where a DVT can develop in patients requiring hospitalization • HIT criteria were added as it is a complication that is associated with heparin use that requires the Acute level of care due to the risk of bleeding and need for close monitoring while on alternative anticoagulation

Deep Vein Thrombosis		
Episode Day 2 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, Both: ♦Anticoagulation regimen established ♦Coordination of outpatient services completed if needed ▶Partial responder, not clinically stable for discharge and requires continued stay, One: ♦Risk for bleeding requiring monitoring and anticoagulation (includes PO) and, One: ▶Direct oral anticoagulant (DOAC) ▶Low molecular weight heparin (LMWH) ▶Fondaparinux ▶Argatroban ▶Bivalirudin	•OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, Both: ♦Anticoagulation regimen established ♦Coordination of outpatient services completed ▶Partial responder, not clinically stable for discharge and requires continued stay, One: ♦Anticoagulation (includes PO) and, Both: ▶Patient or caregiver unable to administer ▶Outpatient services unavailable	Rationale: • “If needed” was removed from coordination of outpatient services since the inability to administer anticoagulation is the driver for admission to the Observation level of care; therefore, outpatient services must be arranged prior to discharge • Partial responder criteria were changed to align with changes made to Episode Day 1

Deep Vein Thrombosis		
Episode Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
• ACUTE, One: ▶ Early responder, discharge expected today if clinically stable last 12h, Both: ♦ Anticoagulation regimen established ♦ No complication or active comorbidity relevant to this episode of care ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦ Unfractionated heparin, continuous and, ≥ One: ▶ Creatinine clearance ≤ 30 mL/min ▶ Morbid obesity (BMI ≥ 40 kg/m² or weight > 120 kg) ▶ Bleeding, active or increased risk ▶ Antiphospholipid antibody syndrome ♦ Risk of limb compromise and anticoagulation ♦ IVC filter placement performed ≤ 24h ♦ Bleeding, actual or suspected and, ≥ One: ▶ Blood product transfusion or IV fluid ▶ Hct or Hb monitoring at least 2x/24h, ≤ 2d ♦ Post Critical care ≤ 24h	• ACUTE, One: ▶ Early responder, discharge expected today if clinically stable last 12h, Both: ♦ Anticoagulation regimen, One: ▶ Established ▶ Contraindicated ♦ No complication or active comorbidity relevant to this episode of care ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦ Anticoagulation (includes PO) and, ≥ One: ▶ Extensive DVT and neurovascular assessment at least 6x/24h ▶ Splanchnic vein thrombosis ♦ UFH, continuous and, ≥ One: ▶ Antiphospholipid antibody syndrome ▶ Bleeding, active or increased risk ▶ BMI ≥ 40 kg/m² or weight > 120 kg ▶ Creatinine clearance ≤ 30 mL/min ♦ HIT, suspected or by history and anticoagulation ♦ IVC filter placement performed ≤ 24h ♦ Bleeding, actual or suspected and, ≥ One: ▶ Blood product transfusion ▶ Hct or Hb monitoring at least 2x/24h, ≤ 2d ♦ Post Critical care ≤ 24h	Rationale: • Anticoagulation regimen “contraindicated” was added to Early responder criteria to capture the patient that is ready for discharge but will not be on anticoagulation due to a contraindication (i.e., patient receiving IVC filter) • “Risk of limb compromise” was changed to “Extensive DVT” to clarify the intent of the criteria • “Includes PO” was added to anticoagulation for the treatment of extensive DVT since the driver for admission to the Acute care is not the treatment, but rather that an extensive DVT is considered a complicated DVT which requires frequent monitoring • “Neurovascular assessment at least 6x/24h” was added to extensive DVT since frequent monitoring is the driver for admission to the Acute level of care • “Splanchnic vein thrombosis” was added to be more inclusive of potential locations where a DVT can develop in patients requiring hospitalization • HIT criteria were added as it is a complication that is associated with heparin use that requires the Acute level of care due to the risk of bleeding and need for close monitoring while on alternative anticoagulation • IV fluids were removed as an intervention for “Bleeding, actual or suspected” since it would be a temporary intervention and a blood transfusion would better address blood loss

Deep Vein Thrombosis		
Episode Day 3 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
• OBSERVATION, One: ▶ Responder, discharge expected today if clinically stable last 12h, One: ♦ Anticoagulation regimen established ▶ Non-responder, not clinically stable for discharge and exceeds episode days at the Observation level of care for this condition, One: ♦ For DVT use Episode Day 3 at a higher level of care ♦ For a condition other than DVT use appropriate subset (Episode Day 1) based on clinical findings ♦ Refer for secondary review	• OBSERVATION, One: ▶ Responder, discharge expected today if clinically stable last 12h, Both: ♦ Anticoagulation regimen established ♦ Coordination of outpatient services completed ▶ Non-responder, not clinically stable for discharge and exceeds episode days at the Observation level of care for this condition, One: ♦ For DVT use Episode Day 3 at a higher level of care ♦ For a condition other than DVT use appropriate subset (Episode Day 1) based on clinical findings ♦ Refer for secondary review	Rationale: • “Coordination of outpatient services completed” was added to Responder criteria to better describe the patient who is ready for discharge from the Observation level of care

Deep Vein Thrombosis		
Episode Day 3 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
• ACUTE, One: ▶ Responder, discharge expected today if clinically stable last 12h, Both: ♦ Anticoagulation regimen established ♦ Complication or comorbidity, One: ▶ No complication or active comorbidity relevant to this episode of care ▶ Complication or comorbidity and clinically stable for discharge, ≥ One: ♦ Bleeding controlled or resolved ♦ Laboratory values within acceptable limits ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦ Unfractionated heparin, continuous and, ≥ One: ▶ Creatinine clearance ≤ 30 mL/min ▶ Morbid obesity (BMI ≥ 40 kg/m² or weight > 120 kg) ▶ Bleeding, active or increased risk ▶ Antiphospholipid antibody syndrome ▶ Risk of limb compromise ♦ IVC filter placement performed ≤ 24h ♦ Patient or caregiver unable to administer anticoagulation (includes PO) and, Both: ▶ Outpatient services unavailable ▶ INR non-therapeutic ♦ Bleeding, actual or suspected and, ≥ One: ▶ Blood product transfusion or IV fluid ▶ Hct or Hb monitoring at least 2x/24h, ≤ 2d	• ACUTE, One: ▶ Responder, discharge expected today if clinically stable last 12h, Both: ♦ Anticoagulation regimen, One: ▶ Established ▶ Contraindicated ♦ Complication or comorbidity, One: ▶ No complication or active comorbidity relevant to this episode of care ▶ Complication or comorbidity and clinically stable for discharge, ≥ One: ♦ Bleeding controlled or resolved ♦ Laboratory values within acceptable limits ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦ Anticoagulation (includes PO) and, ≥ One: ▶ Extensive DVT and neurovascular assessment at least 6x/24h ▶ Splanchnic vein thrombosis ♦ UFH, continuous and, ≥ One: ▶ Antiphospholipid antibody syndrome ▶ Bleeding, active or increased risk ▶ BMI ≥ 40 kg/m² or weight > 120 kg ▶ Creatinine clearance ≤ 30 mL/min ♦ HIT and anticoagulation ♦ IVC filter placement performed ≤ 24h ♦ Pretreatment or bridging anticoagulation, All: ▶ Patient or caregiver unable to administer	Rationale: • Anticoagulation regimen “contraindicated” was added to Responder criteria to capture the patient that is ready for discharge but will not be on anticoagulation due to a contraindication (i.e., patient receiving IVC filter) • “Risk of limb compromise” was changed to “Extensive DVT” to clarify the intent of the criteria • “Includes PO” was added to anticoagulation for the treatment of extensive DVT since the driver for admission to the Acute care is not the treatment, but rather that an extensive DVT is considered a complicated DVT which requires frequent monitoring • “Neurovascular assessment at least 6x/24h” was added to extensive DVT since frequent monitoring is the driver for admission to the Acute level of care • “Splanchnic vein thrombosis” was added to be more inclusive of potential locations where a DVT can develop in patients requiring hospitalization

Deep Vein Thrombosis		
Episode Day 3 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦Post Critical care ≤ 24h	▶Outpatient services unavailable ▶Intervention, One: ♦INR non-therapeutic and anticoagulation (includes PO) ♦Anticoagulation ≤ 5d since initiation ♦Bleeding, actual or suspected and, ≥ One: ▶Blood product transfusion ▶Hct or Hb monitoring at least 2x/24h, ≤ 2d ♦Post Critical care ≤ 24h	• HIT criteria were added as it is a complication that is associated with heparin use that requires the Acute level of care due to the risk of bleeding and need for close monitoring while on alternative anticoagulation • “Pretreatment or bridging” header was added to clarify the intent of the criteria which is to allow a patient to continue to meet for bridging or pretreatment prior to being switched to an oral anticoagulation if the patient is unable to administer the parenteral anticoagulation and outpatient service are unavailable • “Anticoagulation (includes PO)” was added to “INR non-therapeutic” to clarify the intent of the criteria as it is intended for patients on an oral anticoagulation who have yet to achieve a therapeutic INR • “Anticoagulation ≤ 5d since initiation” was added to capture patients who require parenteral anticoagulation pretreatment prior to being switched to dabigatran or edoxaban • IV fluids were removed as an intervention for “Bleeding, actual or suspected” since it would be a temporary intervention and a blood transfusion would better address blood loss

Deep Vein Thrombosis		
Episode Day 4 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
•ACUTE, One: ▶Responder, discharge expected today if clinically stable last 12h, Both: ♦Anticoagulation regimen established ♦Complication or comorbidity, One: ▶No complication or active comorbidity relevant to this episode of care	•ACUTE, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Anticoagulation regimen, One: ▶Established ▶Contraindicated ♦Coordination of outpatient services completed or	• Rationale: • Anticoagulation regimen “contraindicated” was added to Responder criteria to capture the patient that is ready for discharge but will not be on

Deep Vein Thrombosis

Episode Day 4 (ACUTE)

Previously Available Content	2024 March Release	Revision Description
►Complication or comorbidity and clinically stable for discharge, ≥ One: ◆Bleeding controlled or resolved ◆Laboratory values within acceptable limits ►Non-responder, not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, One: ◆Use Extended Stay subset ◆Refer for secondary review	not indicated ◆Complication or comorbidity, One: ►No complication or active comorbidity relevant to this episode of care ►Complication or comorbidity and clinically stable for discharge, ≥ One: ◆Bleeding controlled or resolved ◆Laboratory values within acceptable limits ►Non-responder, not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, One: ◆Use Extended Stay subset ◆Refer for secondary review	anticoagulation due to a contraindication (i.e., patient receiving IVC filter) • “Coordination of outpatients services completed or not indicated” was added to Responder criteria to better describe the patient who is ready for discharge from the Acute level of care

Dehydration or Gastroenteritis

 Revision Summary	Glasgow Coma Scale 9-14 was removed from the Observation level of care throughout this subset to better describe the patient with mental status changes who would be appropriate at the Observation level of care.
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Dehydration or Gastroenteritis		
Global (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
♦Mental status changes (excludes coma, stupor, or obtundation) or Glasgow Coma Scale 9–14	▶Mental status changes (excludes coma, stupor, or obtundation)	Rationale: Glasgow Coma Scale 9-14 was removed from the Observation level of care throughout this subset to better describe the patient with mental status changes who would be appropriate at the Observation level of care

Diabetes Mellitus

 Revision Summary	Pregnancy and pregestational diabetes mellitus criteria have been removed from the Diabetes Mellitus subset and added to the Antepartum subset.
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Diabetes Mellitus		
Initial Review (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ▶Diabetes mellitus and, One: ♦Blood glucose > 400 mg/dL(22.2 mmol/L) and new onset diabetes mellitus ♦Blood glucose > 500 mg/dL(27.8 mmol/L) and, ≥ One: ▶Creatinine, Both: ♦≥ 1.5x to < 3x baseline or estimated baseline ♦< 4 mg/dL(353.6 μmol/L) ▶Heart rate > 100/min, sustained ▶Inability to tolerate or adhere to prescribed regimen (including diet) ▶Intractable nausea and vomiting ▶Mental status changes (excludes coma, stupor, or obtundation) ▶Na > 150 mEq/L(150 mmol/L) ▶Orthostatic hypotension, Both: ♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Blood pressure, ≥ One: ▶Systolic drop ≥ 20 mmHg ▶Diastolic drop ≥ 10 mmHg ▶Pregnancy and pregestational diabetes mellitus and, Both: ♦Blood glucose, ≥ One: ▶Fasting ≥ 95 mg/dL(5.3 mmol/L) ▶1h postprandial ≥ 140 mg/dL(7.8 mmol/L) or 2h postprandial ≥ 120 mg/dL(6.7 mmol/L) ♦Failed outpatient insulin or oral glucose-lowering medication management ≥ 5d	•OBSERVATION, One: ▶Diabetes mellitus and, One: ♦Blood glucose > 400 mg/dL(22.2 mmol/L) and new onset diabetes mellitus ♦Blood glucose > 500 mg/dL(27.8 mmol/L) and, ≥ One: ▶Creatinine, Both: ♦≥ 1.5x to < 3x baseline or estimated baseline ♦< 4 mg/dL(353.6 μmol/L) ▶Heart rate > 100/min, sustained ▶Inability to tolerate or adhere to prescribed regimen (including diet) ▶Intractable nausea and vomiting ▶Mental status changes (excludes coma, stupor, or obtundation) ▶Na > 150 mEq/L(150 mmol/L) ▶Orthostatic hypotension, Both: ♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Blood pressure, ≥ One: ▶Systolic drop ≥ 20 mmHg ▶Diastolic drop ≥ 10 mmHg	Rationale: • Pregnancy and pregestational diabetes mellitus criteria were removed from the Diabetes Mellitus subset and added to the Antepartum subset with gestational diabetes mellitus to streamline the review process and improve organization

Diabetes Mellitus		
Episode Day 1 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ▶Diabetes mellitus and, Both: ♦Finding, One: ▶Blood glucose > 400 mg/dL(22.2 mmol/L) and new onset diabetes mellitus ▶Blood glucose > 500 mg/dL(27.8 mmol/L) and, ≥ One: ♦Creatinine, Both: ▶≥ 1.5x to < 3x baseline or estimated baseline ▶< 4 mg/dL(353.6 μmol/L) ♦Heart rate > 100/min, sustained ♦Inability to tolerate or adhere to prescribed regimen (including diet) ♦Intractable nausea and vomiting ♦Mental status changes (excludes coma, stupor, or obtundation) ♦Na > 150 mEq/L(150 mmol/L) ♦Orthostatic hypotension, Both: ▶Sustained drop in blood pressure within 3 min of sitting or standing ▶Blood pressure, ≥ One: ♦Systolic drop ≥ 20 mmHg ♦Diastolic drop ≥ 10 mmHg ♦Intervention, Both: ▶Insulin or oral glucose-lowering medication initiation or adjustment ▶IV fluid, One: ♦≥ 75 mL/h and, ≥ One: ▶Weight < 60 kg ▶Age ≥ 65 ▶Renal insufficiency ▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ▶Pregnancy and pregestational diabetes mellitus and, All: ♦Blood glucose, ≥ One: ▶Fasting ≥ 95 mg/dL(5.3 mmol/L) ▶1h postprandial ≥ 140 mg/dL(7.8 mmol/L) or 2h postprandial ≥ 120 mg/dL(6.7 mmol/L) ♦Failed outpatient insulin or oral glucose-lowering medication management ≥ 5d ♦Insulin or oral glucose-lowering medication initiation or adjustment ♦IV fluid, One: ▶≥ 75 mL/h and, ≥ One: ♦Weight < 60 kg ♦Renal insufficiency ♦Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg	•OBSERVATION, One: ▶Diabetes mellitus and, Both: ♦Finding, One: ▶Blood glucose > 400 mg/dL(22.2 mmol/L) and new onset diabetes mellitus ▶Blood glucose > 500 mg/dL(27.8 mmol/L) and, ≥ One: ♦Creatinine, Both: ▶≥ 1.5x to < 3x baseline or estimated baseline ▶< 4 mg/dL(353.6 μmol/L) ♦Heart rate > 100/min, sustained ♦Inability to tolerate or adhere to prescribed regimen (including diet) ♦Intractable nausea and vomiting ♦Mental status changes (excludes coma, stupor, or obtundation) ♦Na > 150 mEq/L(150 mmol/L) ♦Orthostatic hypotension, Both: ▶Sustained drop in blood pressure within 3 min of sitting or standing ▶Blood pressure, ≥ One: ♦Systolic drop ≥ 20 mmHg ♦Diastolic drop ≥ 10 mmHg ♦Intervention, Both: ▶Insulin or oral glucose-lowering medication initiation or adjustment ▶IV fluid	Rationale: • Pregnancy and pregestational diabetes mellitus criteria were removed from the Diabetes Mellitus subset and added to the Antepartum subset with gestational diabetes mellitus to streamline the review process and improve organization

Diabetes Mellitus		
Episode Day 2 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Blood glucose within acceptable limits ♦Patient or caregiver knowledgeable about disease and understands the regimen ♦Tolerating PO ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦Blood glucose, Both: ▶Finding, One: ♦Glucose, ≥ One: ▶> 250 mg/dL(13.9 mmol/L) ▶< 70 mg/dL(3.9 mmol/L) ♦Pregnant and, ≥ One: ▶Fasting ≥ 95 mg/dL(5.3 mmol/L) ▶1h postprandial ≥ 140 mg/dL(7.8 mmol/L) or 2h postprandial ≥ 120 mg/dL(6.7 mmol/L) ▶Insulin or oral glucose-lowering medication initiation, adjustment, or dose held ♦Inadequate oral intake and IV fluid and, One: ▶≥ 75 mL/h and, ≥ One: ♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ▶≥ 100 mL/h and weight ≥ 60 kg ♦Patient or caregiver knowledge deficit and diabetes education	•OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Blood glucose within acceptable limits ♦Patient or caregiver knowledgeable about disease and understands the regimen ♦Tolerating PO ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦Blood glucose, Both: ▶Finding, One: ♦Glucose, ≥ One: ▶> 250 mg/dL(13.9 mmol/L) ▶< 70 mg/dL(3.9 mmol/L) ▶Insulin or oral glucose-lowering medication initiation, adjustment, or dose held ♦Inadequate oral intake and IV fluid ♦Patient or caregiver knowledge deficit and diabetes education	Rationale: • Pregnant criteria were removed from the Diabetes Mellitus subset and added to the Antepartum subset to streamline the review process and improve organization

Extended Stay

 Revision Summary	Bleeding and hemolytic anemia findings were updated in the General section at the Acute level of care to clarify the intent of the criteria. Multiple changes were made to the anticoagulation criteria in the General section at the Acute level of care to include, "Heparin-induced thrombocytopenia" criteria were updated to streamline the review process, "Patient or caregiver unable to administer anticoagulation" criteria were restructured to clarify the intent of the criteria, and criteria were added to capture patients on a UFH infusion who have not achieved a therapeutic INR. High-dose proton pump inhibitor criterion was added to Partial Responder criteria to account for the patient with upper gastrointestinal non-variceal bleeding who requires continued stay after endoscopic therapy at the Acute level of care who exhausted the days in the Gastrointestinal Bleeding subset. Acute level of care Obstetrics criteria were updated with changes to the preeclampsia content to clarify the intent of the criteria and the addition of postpartum endometritis criteria as a pathway for the individual who requires an extended stay for anti-infectives. Post embolization criterion was added at the Critical level of care to reflect the patient who underwent embolization within the last 24 hours and requires an extended stay for monitoring.
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Extended Stay		
ACUTE		
Previously Available Content	2024 March Release	Revision Description
ACUTE, ≥ One: • General, One: ▶ Responder, discharge expected today if clinically stable last 12h, One: ♦ Clinical stability, ≥ One: ▶ Anemia controlled or resolved ▶ Bleeding controlled or resolved ▶ Blood glucose within acceptable limits ▶ Hct or Hb within acceptable limits ▶ Functional impairment and, One: ♦ Discharge to home and, One: ▶ Patient or caregiver able to safely manage impairment ▶ Home care services arranged ♦ Discharge to inpatient facility for rehabilitative services ▶ Pain controlled or manageable ▶ Therapeutic anticoagulation, ≥ One: ♦ Anticoagulation regimen established and laboratory values within acceptable limits ♦ IVC filter ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦ Bleeding and, Both:	ACUTE, ≥ One: • General, One: ▶ Responder, discharge expected today if clinically stable last 12h, One: ♦ Clinical stability, ≥ One: ▶ Anemia controlled or resolved ▶ Bleeding controlled or resolved ▶ Blood glucose within acceptable limits ▶ Hct or Hb within acceptable limits ▶ Functional impairment and, One: ♦ Discharge to home and, One: ▶ Patient or caregiver able to safely manage impairment ▶ Home care services arranged ♦ Discharge to inpatient facility for rehabilitative services ▶ Pain controlled or manageable ▶ Therapeutic anticoagulation, ≥ One: ♦ Anticoagulation regimen established and laboratory values within acceptable limits ♦ IVC filter ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦ Bleeding and, Both:	Rationale: • "Presyncope" was removed as literature does not speak to presyncope as a finding that requires hospitalization in patients with bleeding • "Worse than baseline" was added to "Exertional dyspnea" to clarify that the intent of the criteria point is not appropriate for patients who have chronic dyspnea related to a comorbid condition and describing the patient who has exertional dyspnea over and above their chronic state • "Heparin-induced thrombocytopenia" criteria were changed to "HIT and anticoagulation" to streamline the review process • "LMWH ≤ 3d" was removed and patients can now be reviewed using the new criterion "Therapeutic anticoagulation ≤ 5 days since initiation" • "Unfractionated heparin ≤ 5 days since initiation" was changed to "Therapeutic anticoagulation ≤ 5

Extended Stay		
ACUTE		
Previously Available Content	2024 March Release	Revision Description
▶ Finding, ≥ One: ♦ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦ Exertional dyspnea ♦ Hct or Hb decreased last 24h ♦ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦ Orthostatic hypotension, Both: ▶ Sustained drop in blood pressure within 3 min of sitting or standing ▶ Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ♦ Presyncope or syncope ▶ Requiring Hct or Hb monitoring at least 2x/24h, ≤ 2d, ≥ One: ♦ Blood product transfusion ♦ Return to OR within last 24h ♦ Blood glucose ≤ 24h, All: ▶ Finding, ≥ One: ♦ > 250 mg/dL(13.9 mmol/L) ♦ < 70 mg/dL(3.9 mmol/L) ▶ Blood glucose monitoring ▶ Insulin (includes SC) adjustment or dose held ♦ Functional impairment (new) requiring rehabilitation initiation ≤ 24h and, ≥ One: ▶ PT evaluation and training ▶ OT evaluation and training ▶ Speech-language evaluation and training ▶ Cognitive evaluation and retraining ▶ Swallowing evaluation and retraining ♦ Hemolytic anemia and, Both: ▶ Finding, ≥ One: ♦ Hct < 30%(0.30) or Hb < 10.0 g/dL(100 g/L) ♦ Exertional dyspnea ♦ Presyncope or syncope ▶ Hct or Hb monitoring at least 2x/24h and, ≥ One: ♦ Blood product transfusion ♦ Corticosteroid ♦ Immunotherapy (includes monoclonal antibody treatment) ≤ 24h ♦ Immunoglobulin ♦ Plasmapheresis ♦ Pain uncontrolled and, ≥ One: ▶ Analgesic, continuous ▶ Analgesic ≥ 3x/24h ▶ Requiring change in medication, dose, or frequency ≤ 2d ▶ Transition to PO analgesic ≤ 24h ♦ Therapeutic anticoagulation, One: ▶ LMWH ≤ 3d ▶ Unfractionated heparin ≤ 5d since initiation ▶ Patient or caregiver unable to administer anticoagulation and, Both: ♦ Outpatient services unavailable	▶ Finding, ≥ One: ♦ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦ Exertional dyspnea worse than baseline ♦ Hct or Hb decreased last 24h ♦ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦ Orthostatic hypotension, Both: ▶ Sustained drop in blood pressure within 3 min of sitting or standing ▶ Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ♦ Syncope ▶ Requiring Hct or Hb monitoring at least 2x/24h, ≤ 2d, ≥ One: ♦ Blood product transfusion ♦ Return to OR within last 24h ♦ Blood glucose ≤ 24h, All: ▶ Finding, ≥ One: ♦ > 250 mg/dL(13.9 mmol/L) ♦ < 70 mg/dL(3.9 mmol/L) ▶ Blood glucose monitoring ▶ Insulin (includes SC) adjustment or dose held ♦ Functional impairment (new) requiring rehabilitation initiation ≤ 24h and, ≥ One: ▶ PT evaluation and training ▶ OT evaluation and training ▶ Speech-language evaluation and training ▶ Cognitive evaluation and retraining ▶ Swallowing evaluation and retraining ♦ Hemolytic anemia and, Both: ▶ Hct < 30%(0.30) or Hb < 10.0 g/dL(100 g/L) ▶ Hct or Hb monitoring at least 2x/24h and, ≥ One: ♦ Blood product transfusion ♦ Corticosteroid ♦ Immunotherapy (includes monoclonal antibody treatment) ≤ 24h ♦ Immunoglobulin ♦ Plasmapheresis ♦ HIT and anticoagulation ♦ Pain uncontrolled and, ≥ One: ▶ Analgesic, continuous ▶ Analgesic ≥ 3x/24h ▶ Requiring change in medication, dose, or frequency ≤ 2d ▶ Transition to PO analgesic ≤ 24h ♦ Pretreatment or bridging anticoagulation, ≥ One: ▶ Compliance risk, All: ♦ Patient or caregiver unable to administer ♦ Outpatient services unavailable ♦ INR non-therapeutic and anticoagulation (includes PO) ▶ UFH, continuous and INR non-therapeutic ♦ Therapeutic anticoagulation ≤ 5d since initiation	days since initiation” to be more inclusive of other anticoagulation agents and to allow patients receiving pretreatment for dabigatran or edoxaban a place to be reviewed • “Patient or caregiver unable to administer anticoagulation” criteria were restructured to clarify the intent of the criteria and are now located under the header “Pretreatment or bridging anticoagulation” • “UFH, continuous and INR non-therapeutic” criteria were added to capture patients on a UFH infusion who have yet to achieve a therapeutic INR and therefore require continued hospitalization

Extended Stay		
ACUTE		
Previously Available Content	2024 March Release	Revision Description
♦INR non-therapeutic ▶Heparin-induced thrombocytopenia suspected or by history and, Both: ♦Warfarin (includes PO), administered or contraindicated ♦Intervention, One: ▶Argatroban ▶Bivalirudin ▶Fondaparinux ♦Post Critical care ≤ 24h ▶Non-responder, not clinically stable for discharge and does not meet partial responder criteria (refer for secondary review)	♦Post Critical care ≤ 24h ▶Non-responder, not clinically stable for discharge and does not meet partial responder criteria (refer for secondary review)	

Extended Stay		
ACUTE		
Previously Available Content	2024 March Release	Revision Description
ACUTE, ≥ One: •Gastrointestinal or biliary, One: ▶Responder, discharge expected today if clinically stable last 12h, One: ♦Clinical stability, ≥ One: ▶Ascites controlled or improving and tolerating PO or nutritional route established ▶Bilirubin and, All: ♦Laboratory values normal or within acceptable limits ♦Neurological stability ♦Tolerating PO or nutritional route established ▶Hepatic encephalopathy, Both: ♦Grade I or within acceptable limits ♦Neurological stability last 24h ▶Ileus resolved and tolerating PO or nutritional route established ▶Tolerating PO or nutritional route established ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦Ascites, ≤ 7d since initiation of treatment and, Both: ▶Finding, ≥ One: ♦Abdominal girth or weight, ≥ One: ▶Increasing ▶Subtherapeutic response to treatment ▶Unchanged from admission ♦Dyspnea > baseline	ACUTE, ≥ One: •Gastrointestinal or biliary, One: ▶Responder, discharge expected today if clinically stable last 12h, One: ♦Clinical stability, ≥ One: ▶Ascites controlled or improving and tolerating PO or nutritional route established ▶Bilirubin and, All: ♦Laboratory values normal or within acceptable limits ♦Neurological stability ♦Tolerating PO or nutritional route established ▶Upper gastrointestinal (GI) non-variceal bleeding, Both: ♦Bleeding controlled or resolved ♦High-dose proton pump inhibitor regimen completed or established ▶Hepatic encephalopathy, Both: ♦Grade I or within acceptable limits ♦Neurological stability last 24h ▶Ileus resolved and tolerating PO or nutritional route established ▶Tolerating PO or nutritional route established ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦Ascites, ≤ 7d since initiation of treatment and, Both: ▶Finding, ≥ One:	Rationale: • Responder criteria were updated to reflect the patient appropriate for discharge • High-dose proton pump inhibitor criterion was added to Partial Responder criteria to account for the patient with upper gastrointestinal non-variceal bleeding who requires continued stay after endoscopic therapy at the Acute level of care who exhausted the days in the Gastrointestinal Bleeding subset

Extended Stay		
ACUTE		
Previously Available Content	2024 March Release	Revision Description
- Intervention, ≥ One: - Albumin - Diuretic $\geq 2x/24h$ - Sodium and fluid restriction $\leq 3d$ - Therapeutic paracentesis performed within last 24h - Transjugular intrahepatic portosystemic shunt (TIPS) performed $\leq 24h$ - Bilirubin $> 3.0 \text{ mg/dL}$ ($51.3 \text{ } \mu\text{mol/L}$) and, ≥ One: - Albumin level $< 2.0 \text{ g/dL}$ (20 g/L) and, ≥ One: - Albumin - Feeding tube insertion performed within last 24h and feedings initiated - TPN or PPN - Inadequate oral intake $\leq 4d$ or active vomiting and, ≥ One: - Antiemetic $\geq 3x/24h$ - Serotonin antagonist $> 2x/24h$ - IV fluid, One: $\geq 75 \text{ mL/h}$ and, ≥ One: - Weight $< 60 \text{ kg}$ - Age ≥ 65 - Renal insufficiency - Heart failure, chronic $\geq 100 \text{ mL/h}$ and weight $\geq 60 \text{ kg}$ - Liver biopsy results pending $\leq 24h$ - PT $\geq 1.5x \text{ ULN}$ or INR ≥ 2.0 and fresh frozen plasma (FFP) - Hepatic encephalopathy Grade II $\leq 7d$ since initiation of treatment and, ≥ One: - Branched-chain amino acids (BCAA) (includes PO) - Lactulose (includes PO or PR) - L-ornithine L-aspartate (LOLA) (includes PO) - Neomycin (includes PO) - Rifaximin (includes PO) - Ileus, One: - Resolved and initiating PO or tube feeding $\leq 2d$ - Unresolved and $\leq 5d$ since initiation of treatment and, ≥ One: - IV fluid, One: $\geq 75 \text{ mL/h}$ and, ≥ One: - Weight $< 60 \text{ kg}$ - Age ≥ 65 - Renal insufficiency - Heart failure, chronic $\geq 100 \text{ mL/h}$ and weight $\geq 60 \text{ kg}$ - TPN or PPN - Inadequate oral intake and, ≥ One: - IV fluid $\leq 2d$, One: $\geq 75 \text{ mL/h}$ and, ≥ One: - Weight $< 60 \text{ kg}$ - Age ≥ 65 - Renal insufficiency	- Abdominal girth or weight, ≥ One: - Increasing - Subtherapeutic response to treatment - Unchanged from admission - Dyspnea $> \text{baseline}$ - Intervention, ≥ One: - Albumin - Diuretic $\geq 2x/24h$ - Sodium and fluid restriction $\leq 3d$ - Therapeutic paracentesis performed within last 24h - Transjugular intrahepatic portosystemic shunt (TIPS) performed $\leq 24h$ - Bilirubin $> 3.0 \text{ mg/dL}$ ($51.3 \text{ } \mu\text{mol/L}$) and, ≥ One: - Albumin level $< 2.0 \text{ g/dL}$ (20 g/L) and, ≥ One: - Albumin - Feeding tube insertion performed within last 24h and feedings initiated - TPN or PPN - Inadequate oral intake $\leq 4d$ or active vomiting and, ≥ One: - Antiemetic $\geq 3x/24h$ - Serotonin antagonist $> 2x/24h$ - IV fluid - Liver biopsy results pending $\leq 24h$ - PT $\geq 1.5x \text{ ULN}$ or INR ≥ 2.0 and fresh frozen plasma (FFP) - Upper gastrointestinal (GI) non-variceal bleeding and high-dose proton pump inhibitor $\leq 3d$ since initiation - Hepatic encephalopathy Grade II $\leq 7d$ since initiation of treatment and, ≥ One: - Branched-chain amino acids (BCAA) (includes PO) - Lactulose (includes PO or PR) - L-ornithine L-aspartate (LOLA) (includes PO) - Neomycin (includes PO) - Rifaximin (includes PO) - Ileus, One: - Resolved and initiating PO or tube feeding $\leq 2d$ - Unresolved and $\leq 5d$ since initiation of treatment and, ≥ One: - IV fluid - TPN or PPN - Inadequate oral intake and, ≥ One: - IV fluid - Parenteral nutrition $\leq 2d$ - Initiation or adjustment of PO or tube feeding $\leq 2d$ - Post Critical care $\leq 24h$ - Non-responder, not clinically stable for discharge and does not meet partial responder criteria (refer for secondary review)	

Extended Stay		
ACUTE		
Previously Available Content	2024 March Release	Revision Description
▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ▶Parenteral nutrition ≤ 2d ▶Initiation or adjustment of PO or tube feeding ≤ 2d ♦Post Critical care ≤ 24h ▶Non-responder, not clinically stable for discharge and does not meet partial responder criteria (refer for secondary review)		

Extended Stay		
ACUTE		
Previously Available Content	2024 March Release	Revision Description
ACUTE, ≥ One: •Infection, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Afebrile ♦Anti-infective therapy completed, arranged, or not indicated ♦Laboratory values within acceptable limits ♦Tolerating PO ♦Clinical stability, ≥ One: ▶Infection resolving and, ≥ One: ♦Abscess and anti-infective regimen established and tolerated ♦Drain removed or drainage manageable ♦<i>C. difficile</i> infection and decrease in frequency and severity of diarrhea ▶Infection ruled out ▶Functional impairment and, One: ♦Discharge to home and, One: ▶Patient or caregiver able to safely manage impairment ▶Home care services arranged ♦Discharge to inpatient facility for rehabilitative services	ACUTE, ≥ One: •Infection, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Afebrile ♦Laboratory values within acceptable limits ♦Tolerating PO ♦Clinical stability, ≥ One: ▶Infection resolving, ≥ One: ♦Anti-infective therapy completed, arranged, or not indicated ♦Drain removed or drainage manageable ♦<i>C. difficile</i> infection and decrease in frequency and severity of diarrhea ▶Infection ruled out ▶Functional impairment and, One: ♦Discharge to home and, One: ▶Patient or caregiver able to safely manage impairment ▶Home care services arranged ♦Discharge to inpatient facility for rehabilitative services	Rationale: • Responder criteria were updated to reflect the patient appropriate for discharge

Extended Stay		
ACUTE		
Previously Available Content	2024 March Release	Revision Description
ACUTE, ≥ One: • Obstetrics, One: ▶ Responder, One: discharge expected today if clinically stable last 12h, One: ♦ Preeclampsia with severe features and, All: ▶ Blood pressure within acceptable limits ▶ Reassuring fetal testing ▶ Laboratory values within acceptable limits ▶ Neurological stability ▶ Partial responder, not clinically stable for discharge and requires continued stay, One: ♦ Preeclampsia with severe features and, ≥ One: ▶ Expectant management, Both: ♦ < 34 wks gestation ♦ Intervention, Both: ▶ Blood pressure management, ≥ One: ♦ Antihypertensive (includes PO) ▶ Blood pressure monitoring at least 3x/24h ▶ Fetal heart rate monitoring ▶ Magnesium sulfate ▶ Post Critical care ≤ 24h ▶ Non-responder, not clinically stable for discharge and does not meet partial responder criteria (refer for secondary review)	ACUTE, ≥ One: • Obstetrics, One: ▶ Responder, discharge expected today if clinically stable last 12h, One: ♦ Endometritis resolving and, ≥ One: ▶ Anti-infective regimen established and tolerated ▶ Symptoms resolving ♦ Preeclampsia with severe features and, All: ▶ Blood pressure within acceptable limits ▶ Laboratory values within acceptable limits ▶ Neurological stability ▶ Partial responder, not clinically stable for discharge and requires continued stay, One: ♦ Postpartum endometritis and anti-infective ♦ Preeclampsia with severe features and, ≥ One: ▶ Antihypertensive (includes PO) ▶ Magnesium sulfate ▶ Post Critical care ≤ 24h ▶ Non-responder, not clinically stable for discharge and does not meet partial responder criteria (refer for secondary review)	Rationale: • Responder criteria were updated to reflect the patient appropriate for discharge • Postpartum endometritis criteria were added to Partial Responder criteria as a pathway for the patient who requires an extended stay for antibiotic administration and monitoring, as literature notes that antibiotics should be continued for a minimum of 24 to 48 hours after symptoms have resolved • Preeclampsia criteria were updated to define the patient who requires an extended stay for continued treatment and monitoring at the Acute level of care

Extended Stay		
CRITICAL		
Previously Available Content	2024 March Release	Revision Description
CRITICAL, One: • Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ▶ Acute humoral rejection and apheresis ▶ AKI and, Both: ♦ Finding, ≥ One: ▶ Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: ♦ Loss of P wave ♦ Multifocal PVCs ♦ Neuromuscular deficit ♦ Widening QRS ▶ Sodium, One: ♦ < 120 mEq/L(120 mmol/L) and volume overload ♦ > 160 mEq/L(160 mmol/L)	CRITICAL, One: • Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ▶ Acute humoral rejection and apheresis ▶ AKI and, Both: ♦ Finding, ≥ One: ▶ Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: ♦ Loss of P wave ♦ Multifocal PVCs ♦ Neuromuscular deficit ♦ Widening QRS ▶ Sodium, One: ♦ < 120 mEq/L(120 mmol/L) and volume overload ♦ > 160 mEq/L(160 mmol/L)	Rationale: • “Post embolization ≤ 24h” criterion was added to reflect the patient who underwent embolization within the last 24 hours and requires an extended stay at the Critical level of care for monitoring

Extended Stay		
CRITICAL		
Previously Available Content	2024 March Release	Revision Description
♦Intervention, ≥ One: ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶Dialysis, One: ♦Hemodialysis ≤ 7d since initiation ♦Peritoneal dialysis ≤ 7d since initiation ▶IV fluid resuscitation ≤ 2d ▶Anticonvulsant and, ≥ One: ♦Recurrent seizure ♦Refractory seizure ▶Arrhythmia, ≥ One: ♦Urgent electrical cardioversion performed within last 24h ♦Defibrillation performed within last 24h ♦Temporary pacemaker insertion performed within last 2d ♦Symptomatic recurrent or refractory arrhythmia requiring antiarrhythmic and, ≥ One: ▶Chest pain ▶Heart failure and, ≥ One: ♦Arterial Po₂ < 56 mmHg(7.4 kPa) ♦O₂ sat < 89%(0.89) and < baseline ▶Hemodynamic instability, ≥ One: ♦Heart rate > 120/min, sustained ♦Systolic blood pressure, ≥ One: ▶< 90 mmHg ▶< 110 mmHg with chronic hypertension ▶> 30 mmHg decrease from baseline ▶Labile ♦MAP < 65 mmHg ▶CRRT ▶Cerebral edema and, ≥ One: ♦ICP monitoring (includes OR placement) ♦Corticosteroid ♦Hyperosmolar therapy, One: ▶Mannitol ▶Hypertonic saline ≥ 3%(0.03) solution ▶DIC and blood product transfusion ▶ECMO or ECLS ▶Fulminant <i>C. difficile</i> infection and, Both: ♦Ileus ♦IV fluid resuscitation ≤ 2d ▶Heart failure and, Both: ♦Finding, ≥ One: ▶Arterial Po₂ < 56 mmHg(7.4 kPa) ▶O₂ sat < 89%(0.89) and < baseline ♦Diuretic every 1-2h or continuous ▶Hemodynamic monitoring, invasive ≤ 24h ▶Hepatic encephalopathy Grade IV ≤ 7d since initiation of treatment and, ≥ One: ♦Branched-chain amino acids (BCAA) (includes PO) ♦Lactulose (includes PO or PR)	♦Intervention, ≥ One: ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶Dialysis, One: ♦Hemodialysis ≤ 7d since initiation ♦Peritoneal dialysis ≤ 7d since initiation ▶IV fluid resuscitation ≤ 2d ▶Anticonvulsant and, ≥ One: ♦Recurrent seizure ♦Refractory seizure ▶Arrhythmia, ≥ One: ♦Urgent electrical cardioversion performed within last 24h ♦Defibrillation performed within last 24h ♦Temporary pacemaker insertion performed within last 2d ♦Symptomatic recurrent or refractory arrhythmia requiring antiarrhythmic and, ≥ One: ▶Chest pain ▶Heart failure and, ≥ One: ♦Arterial Po₂ < 56 mmHg(7.4 kPa) ♦O₂ sat < 89%(0.89) and < baseline ▶Hemodynamic instability, ≥ One: ♦Heart rate > 120/min, sustained ♦Systolic blood pressure, ≥ One: ▶< 90 mmHg ▶< 110 mmHg with chronic hypertension ▶> 30 mmHg decrease from baseline ▶Labile ♦MAP < 65 mmHg ▶CRRT ▶Cerebral edema and, ≥ One: ♦ICP monitoring (includes OR placement) ♦Corticosteroid ♦Hyperosmolar therapy, One: ▶Mannitol ▶Hypertonic saline ≥ 3%(0.03) solution ▶DIC and blood product transfusion ▶ECMO or ECLS ▶Fulminant <i>C. difficile</i> infection and, Both: ♦Ileus ♦IV fluid resuscitation ≤ 2d ▶Heart failure and, Both: ♦Finding, ≥ One: ▶Arterial Po₂ < 56 mmHg(7.4 kPa) ▶O₂ sat < 89%(0.89) and < baseline ♦Diuretic every 1-2h or continuous ▶Hemodynamic monitoring, invasive ≤ 24h ▶Hepatic encephalopathy Grade IV ≤ 7d since initiation of treatment and, ≥ One: ♦Branched-chain amino acids (BCAA) (includes PO) ♦Lactulose (includes PO or PR)	

Extended Stay		
CRITICAL		
Previously Available Content	2024 March Release	Revision Description
♦L-ornithine L-aspartate (LOLA) ♦Neomycin (includes PO) ♦Rifaximin (includes PO) ►Hepatorenal syndrome ≤ 8d since initiation of treatment and, ≥ One: ♦CRRT ♦Hemodialysis or peritoneal dialysis 2d since initiation ♦Norepinephrine ►HFNC and respiratory monitoring every 1-2h ►Hypercalcemia, Both: ♦Calcium ≥ 14.0 mg/dL(3.5 mmol/L) ♦Intervention, ≥ One: ►CRRT ►Dialysis, hemodialysis or peritoneal ►Treatment, Both: ♦Calcitonin ♦Saline hydration, One: ►≥ 75 mL/h and, ≥ One: ♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ►≥ 100 mL/h and weight ≥ 60 kg ►Hypocalcemia, Both: ♦Calcium < 5.0 mg/dL(1.25 mmol/L) ♦Calcium repletion ►Hyperkalemia, Both: ♦Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: ►Loss of P wave ►Multifocal PVCs ►Neuromuscular deficit ►Widening QRS ♦Intervention, ≥ One: ►Calcium chloride ►Calcium gluconate ►Dialysis, One: ♦CRRT ♦Dialysis, hemodialysis or peritoneal ►Glucose 50%(0.50) with insulin ►Hypokalemia, All: ♦Potassium < 3.0 mEq/L(3.0 mmol/L) ♦Electrocardiogram (ECG) findings, ≥ One: ►Multifocal PVCs ►R on T phenomenon ►T and U wave fusion ►U wave ♦Potassium repletion ►Hypomagnesemia, Both: ♦Magnesium < 1.0 mg/dL(0.41 mmol/L) ♦Magnesium repletion ►Hypernatremia, Both: ♦Sodium > 160 mEq/L(160 mmol/L) ♦Intervention, ≥ One: ►CRRT ►Dialysis, hemodialysis or peritoneal	♦L-ornithine L-aspartate (LOLA) ♦Neomycin (includes PO) ♦Rifaximin (includes PO) ►Hepatorenal syndrome ≤ 8d since initiation of treatment and, ≥ One: ♦CRRT ♦Hemodialysis or peritoneal dialysis 2d since initiation ♦Norepinephrine ►HFNC and respiratory monitoring every 1-2h ►Hypercalcemia, Both: ♦Calcium ≥ 14.0 mg/dL(3.5 mmol/L) ♦Intervention, ≥ One: ►CRRT ►Dialysis, hemodialysis or peritoneal ►Treatment, Both: ♦Calcitonin ♦Saline hydration ►Hypocalcemia, Both: ♦Calcium < 5.0 mg/dL(1.25 mmol/L) ♦Calcium repletion ►Hyperkalemia, Both: ♦Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: ►Loss of P wave ►Multifocal PVCs ►Neuromuscular deficit ►Widening QRS ♦Intervention, ≥ One: ►Calcium chloride or gluconate ►Dialysis, One: ♦CRRT ♦Dialysis, hemodialysis or peritoneal ►Glucose 50%(0.50) with insulin ►Hypokalemia, All: ♦Potassium < 3.0 mEq/L(3.0 mmol/L) ♦Electrocardiogram (ECG) findings, ≥ One: ►Multifocal PVCs ►R on T phenomenon ►T and U wave fusion ►U wave ♦Potassium repletion ►Hypomagnesemia, Both: ♦Magnesium < 1.0 mg/dL(0.41 mmol/L) ♦Magnesium repletion ►Hypernatremia, Both: ♦Sodium > 160 mEq/L(160 mmol/L) ♦Intervention, ≥ One: ►CRRT ►Dialysis, hemodialysis or peritoneal ►Hyponatremia, Both: ♦Sodium < 120 mEq/L(120 mmol/L) and symptomatic ♦Intervention, ≥ One:	

Extended Stay		
CRITICAL		
Previously Available Content	2024 March Release	Revision Description
▶CRRT ▶Dialysis, hemodialysis or peritoneal ▶IV fluid, One: ♦≥ 75 mL/h and, ≥ One: ▶Weight < 60 kg ▶Age ≥ 65 ▶Renal insufficiency ▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ▶Hyponatremia, Both: ♦Sodium < 120 mEq/L(120 mmol/L) and symptomatic ♦Intervention, ≥ One: ▶Conivaptan or tolvaptan (includes PO) ▶Diuretic ≥ 2x/24h ▶Fluid restriction ▶Sodium chloride 3%(0.03) ▶Hypophosphatemia, Both: ♦Phosphate < 1.0 mg/dL(0.32 mmol/L) ♦Phosphate repletion ▶IABP ▶Mechanical ventilation, discharge planning and, ≥ One: ♦FiO₂ > 50%(0.50) and > baseline ♦PEEP > 10 cm H₂O and > baseline ♦Respiratory interventions every 1-2h ♦Active weaning < 3 attempts ♦Failed weaning ≥ 3 attempts and tracheostomy placed ≤ 24h ▶NIPPV and, ≥ One: ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Pericardial effusion and, ≥ One: ♦Symptomatic ≤ 24h, ≥ One: ▶Dyspnea ▶Jugular venous distention ▶Orthopnea ♦Insertion of pericardial drain performed ≤ 24h ♦Pericardiocentesis performed ≤ 24h ▶Thrombolytic ▶Toxic megacolon and, ≥ One: ♦Blood product transfusion ♦Colonic decompression ♦IV fluid resuscitation ≤ 2d ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) ▶VAD, percutaneous ▶VAD and, ≥ One: ♦Blood pressure, ≥ One: ▶< baseline and inadequate perfusion ▶MAP < 70 mmHg and continuous flow device ▶Systolic < 90 mmHg and pulsatile flow device ♦Decreased level of consciousness	▶Conivaptan or tolvaptan (includes PO) ▶Diuretic ≥ 2x/24h ▶Fluid restriction ▶Sodium chloride 3%(0.03) ▶Hypophosphatemia, Both: ♦Phosphate < 1.0 mg/dL(0.32 mmol/L) ♦Phosphate repletion ▶IABP ▶Mechanical ventilation, discharge planning and, ≥ One: ♦FiO₂ > 50%(0.50) and > baseline ♦PEEP > 10 cm H₂O and > baseline ♦Respiratory interventions every 1-2h ♦Active weaning < 3 attempts ♦Failed weaning ≥ 3 attempts and tracheostomy placed ≤ 24h ▶NIPPV and, ≥ One: ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Pericardial effusion and, ≥ One: ♦Symptomatic ≤ 24h, ≥ One: ▶Dyspnea ▶Jugular venous distention ▶Orthopnea ♦Insertion of pericardial drain performed ≤ 24h ♦Pericardiocentesis performed ≤ 24h ▶Post embolization ≤ 24h ▶Thrombolytic ▶Toxic megacolon and, ≥ One: ♦Blood product transfusion ♦Colonic decompression ♦IV fluid resuscitation ≤ 2d ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) ▶VAD, percutaneous ▶VAD and, ≥ One: ♦Blood pressure, ≥ One: ▶< baseline and inadequate perfusion ▶MAP < 70 mmHg and continuous flow device ▶Systolic < 90 mmHg and pulsatile flow device ♦Decreased level of consciousness ♦Decreased VAD flows and inadequate perfusion ♦O₂ sat < 89%(0.89)	•Non-responder, not clinically stable for discharge and does not meet partial responder criteria for Critical, Intermediate, or Acute (refer for secondary review)

Extended Stay		
CRITICAL		
Previously Available Content	2024 March Release	Revision Description
♦Decreased VAD flows and inadequate perfusion ♦O₂ sat < 89%(0.89) ♦Non-responder, not clinically stable for discharge and does not meet partial responder criteria for Critical, Intermediate, or Acute (refer for secondary review)		

Gallbladder Disorders

 Revision Summary	The Cholecystitis subset was renamed to Gallbladder Disorders as Choledocholithiasis was introduced as a new condition and includes appropriate diagnostics and interventions at the Acute level of care.
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Gallbladder Disorders		
Initial Review (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
N/A	• ACUTE, One: ▶ Choledocholithiasis, actual or suspected and, One: ♦ Pain, right upper abdomen and, ≥ One: ▶ Common bile duct stone or dilation on imaging ▶ Total bilirubin > 1.7 mg/dL and abnormal LFTs	Rationale: • Criteria were added to capture the patient with acute or suspected choledocholithiasis who would be appropriate for the Acute level of care

Gallbladder Disorders		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
N/A	• ACUTE, One: ▶ Choledocholithiasis, actual or suspected and, Both: ♦ Pain, right upper abdomen and, ≥ One: ▶ Common bile duct stone or dilation on imaging ▶ Total bilirubin > 1.7 mg/dL and abnormal LFTs ♦ Intervention, ≥ One: ▶ ERCP performed within 24h ▶ Endoscopic ultrasound performed within 24h ▶ MRCP performed within 24h ▶ NPO and IV fluid ▶ Analgesic ≥ 3x/24h	Rationale: • Criteria were added to capture the patient with acute or suspected choledocholithiasis who would be appropriate for the Acute level of care

Gallbladder Disorders		
Episode Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
N/A	• ACUTE, One: ▶ Responder, discharge expected today if clinically stable last 12h, All: ♦ Tolerating PO ♦ Laboratory values within acceptable limits ♦ Pain controlled or manageable ♦ No complication or active comorbidity relevant to this episode of care ▶ Partial responder, not clinically stable for discharge and requires continued stay, One: ♦ Cholelithiasis, actual or suspected and, ≥ One: ▶ ERCP performed within 24h ▶ Endoscopic ultrasound performed within 24h ▶ Surgery performed within 24h ▶ NPO and IV fluid ▶ Analgesic ≥ 3x/24h	Rationale: • Responder criteria were added to identify patients with choledocholithiasis who are stable and appropriate for discharge from the Acute level of care • Partial responder criteria were added to capture the patient with suspected or confirmed choledocholithiasis who requires continued stay at the Acute level of care

Gallbladder Disorders		
Episode Day 3 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
N/A	• ACUTE, One: ▶ Responder, discharge expected today if clinically stable last 12h, All: ♦ Tolerating PO ♦ Laboratory values within acceptable limits ♦ Pain controlled or manageable ♦ No complication or active comorbidity relevant to this episode of care ▶ Partial responder, not clinically stable for discharge and requires continued stay, One: ♦ Cholelithiasis and, ≥ One: ▶ ERCP performed within 24h ▶ Surgery performed within 24h ▶ Inadequate oral intake and, ≥ One: ♦ IV fluid ♦ Advancing diet as tolerated ▶ Analgesic ≥ 3x/24h	Rationale: • Responder criteria were added to identify patients with choledocholithiasis who are stable and appropriate for discharge from the Acute level of care • Partial responder criteria were added to capture the patient with suspected or confirmed choledocholithiasis who requires continued stay at the Acute level of care

Gallbladder Disorders		
Episode Day 4 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
N/A	• ACUTE, One: ▸ Responder, discharge expected today if clinically stable last 12h, All: ♦ Tolerating PO ♦ Laboratory values within acceptable limits ♦ Pain controlled or manageable ♦ No complication or active comorbidity relevant to this episode of care ▸ Non-responder, not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, One: ♦ Use Extended Stay subset ♦ Refer for secondary review	Rationale: • Responder criteria were added to identify patients with choledocholithiasis who are stable and appropriate for discharge from the Acute level of care • Non-responder criteria were added to capture the patient who is not clinically stable for discharge and exceeds episode days in this subset

Gastrointestinal (GI) Bleeding



Revision Summary

Gastrointestinal (GI) bleeding content was updated to reflect current guidelines and streamline the review process. "≥ 1 liter IV fluid or blood product transfusion prior to the decision to admit" was removed from upper and lower gastrointestinal bleeding criteria to account for the patient who requires admission prior to the completion of initial resuscitation based on the findings (on Initial Review and Episode Day 1 at the Observation and Acute levels of care) and for the hospitalized patient (on Episode Day 1 at the Acute level of care). Appropriate findings and interventions were added to reflect the patient with lower or upper GI bleeding who requires management at the Acute level of care. Embolization criterion was added to reflect the patient who cannot tolerate endoscopy, has recurrent bleeding post repeated endoscopic therapy, or demonstrates extravasation on computer tomography angiography (CTA) and requires the Critical level of care.

Gastrointestinal (GI) Bleeding		
Initial Review (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ▶Gastrointestinal (GI) bleeding and ≥ 1 liter IV fluid or blood product transfusion prior to decision to admit and, Both: ♦Coffee ground emesis, hematemesis, hematochezia, or melena ♦Hct ≥ 21%(0.21) or Hb ≥ 7.0 g/dL(70 g/L)	•OBSERVATION, One: ▶Gastrointestinal (GI) bleeding, Both: ♦Coffee ground emesis, hematemesis, hematochezia, or melena ♦Hct ≥ 21%(0.21) or Hb ≥ 7.0 g/dL(70 g/L)	Rationale: • "≥ 1 liter IV fluid or blood product transfusion prior to decision to admit" criteria were removed to account for the hospitalized patient who presents with acute GI bleeding and may require admission prior to completion of initial resuscitation efforts for example blood transfusion or IV fluids. This also accounts for emergency department quality improvement projects. Another important reason for this change is to allow the hospitalized patient admitted with another condition who then develops an acute GI bleeding as a new primary condition a place to be reviewed on Episode Day 1 at the Acute level of care

Gastrointestinal (GI) Bleeding		
Initial Review (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
•ACUTE, ≥ One: ▶Lower gastrointestinal (GI) bleeding and ≥ 1 liter IV fluid or blood product transfusion prior to	•ACUTE, ≥ One: ▶Lower gastrointestinal (GI) bleeding and, One: ♦Hematochezia or melena and, ≥ One:	Rationale:

Gastrointestinal (GI) Bleeding		
Initial Review (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
decision to admit and, One: ▶ Hematochezia or melena and, ≥ One: ▶ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ▶ Exertional dyspnea ▶ INR ≥ 2.0 ▶ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ▶ Orthostatic hypotension, Both: ◆ Sustained drop in blood pressure within 3 min of sitting or standing ◆ Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ▶ Presyncope or syncope ▶ Upper gastrointestinal (GI) non-variceal bleeding and ≥ 1 liter IV fluid or blood product transfusion prior to decision to admit and, Both: ◆ Coffee ground emesis, hematemesis, hematochezia, or melena ◆ Finding, ≥ One: ▶ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ▶ Exertional dyspnea ▶ INR ≥ 2.0 ▶ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ▶ Orthostatic hypotension, Both: ◆ Sustained drop in blood pressure within 3 min of sitting or standing ◆ Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ▶ Presyncope or syncope	▶ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ▶ Exertional dyspnea worse than baseline ▶ INR ≥ 2.0 ▶ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ▶ Orthostatic hypotension, Both: ◆ Sustained drop in blood pressure within 3 min of sitting or standing ◆ Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ▶ Syncope ▶ Upper gastrointestinal (GI) non-variceal bleeding and, One: ◆ Coffee ground emesis, hematemesis, hematochezia, or melena, and ≥ One: ▶ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ▶ Exertional dyspnea worse than baseline ▶ INR ≥ 2.0 ▶ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ▶ Orthostatic hypotension, Both: ◆ Sustained drop in blood pressure within 3 min of sitting or standing ◆ Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ▶ Syncope	• "≥ 1 liter IV fluid or blood product transfusion prior to decision to admit" criteria were removed to account for the hospitalized patient who presents with acute GI bleeding and may require admission prior to completion of initial resuscitation efforts for example blood transfusion or IV fluids. This also accounts for emergency department quality improvement projects. Another important reason for this change is to allow the hospitalized patient admitted with another condition who then develops an acute GI bleeding as a new primary condition a place to be reviewed on Episode Day 1 at the Acute level of care. This applies to both lower and upper GI bleeding. • "Worse than baseline" was added to "Exertional dyspnea" to clarify that the intent of the criteria point is not appropriate for patients who have chronic dyspnea related to a comorbid condition and describing the patient who has exertional dyspnea over and above their chronic state • "Presyncope" was removed as literature does not speak to presyncope as a driver for admission in patients with gastrointestinal bleeding • Upper gastrointestinal non-variceal bleeding criteria were restructured to improve usability

Gastrointestinal (GI) Bleeding		
Initial Review (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
• CRITICAL, ≥ One: ▶ Variceal bleeding ▶ Hemodynamic instability, All: ◆ Initial treatment, ≥ One: ▶ ≥ 2 units blood product transfusion ▶ ≥ 2 liters IV fluid ▶ ≥ 1 unit blood product transfusion and ≥ 1	• CRITICAL, ≥ One: ▶ Embolization performed within 24h ▶ Variceal bleeding ▶ Hemodynamic instability, All: ◆ Initial treatment, ≥ One: ▶ ≥ 2 units blood product transfusion ▶ ≥ 2 liters IV fluid	Rationale: • "Embolization performed within 24h" criterion was added to reflect the patient who cannot tolerate endoscopy, has recurrent bleeding post repeated endoscopic therapy, or demonstrates extravasation

Gastrointestinal (GI) Bleeding		
Initial Review (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
- liter IV fluid ♦Findings after initial treatment, ≥ One: ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Requiring ongoing resuscitation ▶Mechanical ventilation ▶NIPPV and, ≥ One: ♦Arterial Po₂ ≤ 39 mmHg(5.2 kPa) ♦Arterial or venous Pco₂ ≥ 60 mmHg(8.0 kPa) and, One: ▶COPD and arterial or venous pH ≤ 7.24 ▶No history of COPD ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	▶≥ 1 unit blood product transfusion and ≥ 1 liter IV fluid ♦Findings after initial treatment, ≥ One: ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Requiring ongoing resuscitation ▶Mechanical ventilation ▶NIPPV ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	- on computer tomography angiography (CTA) who requires the Critical level of care • NIPPV subcriteria were removed because any patient who is newly requiring ventilator support is appropriate for the Critical level of care

Gastrointestinal (GI) Bleeding		
Episode Day 1 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, Both: ▶Gastrointestinal (GI) bleeding and ≥ 1 liter IV fluid or blood product transfusion prior to decision to admit and, Both: ♦Coffee ground emesis, hematemesis, hematochezia, or melena ♦Hct ≥ 21%(0.21) or Hb ≥ 7.0 g/dL(70 g/L) ▶Intervention, ≥ One: ♦Hct or Hb monitoring at least 2x/24h ♦Blood product transfusion	•OBSERVATION, Both: ▶Gastrointestinal (GI) bleeding, Both: ♦Coffee ground emesis, hematemesis, hematochezia, or melena ♦Hct ≥ 21%(0.21) or Hb ≥ 7.0 g/dL(70 g/L) ▶Intervention, ≥ One: ♦Blood product transfusion ♦Hct or Hb monitoring at least 2x/24h	Rationale: • “≥ 1 liter IV fluid or blood product transfusion prior to decision to admit” criteria were removed to account for the hospitalized patient who presents with acute GI bleeding and may require admission prior to completion of initial resuscitation efforts for example blood transfusion or IV fluids. This also accounts for emergency department quality improvement projects. Another important reason for this change is to allow the hospitalized patient admitted with another condition who then develops an acute GI bleeding as a new primary condition a place to be reviewed on Episode Day 1 at the Acute level of care

Gastrointestinal (GI) Bleeding		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
• ACUTE, ≥ One: ▶ Lower gastrointestinal (GI) bleeding and ≥ 1 liter IV fluid or blood product transfusion prior to decision to admit and, Both: ♦ Hematochezia or melena and, ≥ One: ▶ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ▶ Exertional dyspnea ▶ INR ≥ 2.0 ▶ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ▶ Orthostatic hypotension, Both: ♦ Sustained drop in blood pressure within 3 min of sitting or standing ♦ Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ▶ Presyncope or syncope ♦ Hct or Hb monitoring at least 2x/24h and, Both: ▶ Colonoscopy performed within 24h ▶ Blood product transfusion ▶ Upper gastrointestinal (GI) non-variceal bleeding and ≥ 1 liter IV fluid or blood product transfusion prior to decision to admit and, Both: ♦ Coffee ground emesis, hematemesis, hematochezia, or melena and, ≥ One: ▶ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ▶ Exertional dyspnea ▶ INR ≥ 2.0 ▶ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ▶ Orthostatic hypotension, Both: ♦ Sustained drop in blood pressure within 3 min of sitting or standing ♦ Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ▶ Presyncope or syncope ♦ Hct or Hb monitoring at least 2x/24h and, Both: ▶ Endoscopy performed within 24h ▶ Blood product transfusion	• ACUTE, ≥ One: ▶ Lower gastrointestinal (GI) bleeding and, Both: ♦ Hematochezia or melena and, ≥ One: ▶ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ▶ Exertional dyspnea worse than baseline ▶ INR ≥ 2.0 ▶ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ▶ Orthostatic hypotension, Both: ♦ Sustained drop in blood pressure within 3 min of sitting or standing ♦ Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ▶ Syncope ♦ Intervention, ≥ One: ▶ Blood product transfusion ▶ Colonoscopy performed within 24h ▶ Hct or Hb monitoring at least 2x/24h ▶ Upper gastrointestinal (GI) non-variceal bleeding and, Both: ♦ Coffee ground emesis, hematemesis, hematochezia, or melena and, ≥ One: ▶ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ▶ Exertional dyspnea worse than baseline ▶ INR ≥ 2.0 ▶ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ▶ Orthostatic hypotension, Both: ♦ Sustained drop in blood pressure within 3 min of sitting or standing ♦ Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ▶ Syncope ♦ Hct or Hb monitoring at least 2x/24h and, ≥ One: ▶ Blood product transfusion ▶ Endoscopy performed within 24h ▶ High-dose proton pump inhibitor ≤ 3d	Rationale: • "≥ 1 liter IV fluid or blood product transfusion prior to decision to admit" criteria were removed to account for the hospitalized patient who presents with acute GI bleeding and may require admission prior to completion of initial resuscitation efforts for example blood transfusion or IV fluids. This also accounts for emergency department quality improvement projects. Another important reason for this change is to allow the hospitalized patient admitted with another condition who then develops an acute GI bleeding as a new primary condition a place to be reviewed on Episode Day 1 at the Acute level of care. This applies to both lower and upper GI bleeding. • "Worse than baseline" was added to "Exertional dyspnea" to clarify that the intent of the criteria point is not appropriate for patients who have chronic dyspnea related to a comorbid condition and describing the patient who has exertional dyspnea over and above their chronic state • "Presyncope" was removed as literature does not speak to presyncope as a driver for admission in patients with gastrointestinal bleeding • Interventions for lower and upper gastrointestinal bleeding were updated to reflect current guidelines and to better capture the patient who is appropriate for the Acute level of care • "High-dose proton pump inhibitor ≤ 3d" criterion was added as a most recent guideline-appropriate intervention after endoscopic therapy at the Acute level of care

Gastrointestinal (GI) Bleeding		
Episode Day 1 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
•CRITICAL, ≥ One: ▶Variceal bleeding and, One: ♦Balloon tamponade ♦Therapeutic endoscopy and, ≥ One: ▶Octreotide ▶Somatostatin ♦Transjugular intrahepatic portosystemic shunt (TIPS) ▶Hemodynamic instability, All: ♦Initial treatment, ≥ One: ▶≥ 2 units blood product transfusion ▶≥ 2 liters IV fluid ▶≥ 1 unit blood product transfusion and ≥ 1 liter IV fluid ♦Findings after initial treatment, ≥ One: ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Requiring ongoing resuscitation ▶Hemodynamic monitoring, invasive ▶Mechanical ventilation ▶NIPPV and, ≥ One: ♦Arterial $P_{O_2} \leq 39$ mmHg(5.2 kPa) ♦Arterial or venous $P_{CO_2} \geq 60$ mmHg(8.0 kPa) and, One: ▶COPD and arterial or venous pH ≤ 7.24 ▶No history of COPD ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	•CRITICAL, ≥ One: ▶Embolization performed within 24h ▶Variceal bleeding and, ≥ One: ♦Balloon tamponade ♦Therapeutic endoscopy and, ≥ One: ▶Octreotide ▶Somatostatin ♦Transjugular intrahepatic portosystemic shunt (TIPS) ▶Hemodynamic instability, All: ♦Initial treatment, ≥ One: ▶≥ 2 units blood product transfusion ▶≥ 2 liters IV fluid ▶≥ 1 unit blood product transfusion and ≥ 1 liter IV fluid ♦Findings after initial treatment, ≥ One: ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Requiring ongoing resuscitation ▶Hemodynamic monitoring, invasive ▶Mechanical ventilation ▶NIPPV ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	Rationale: • "Embolization performed within 24h" criterion was added to reflect the patient who cannot tolerate endoscopy, has recurrent bleeding post repeated endoscopic therapy, or demonstrates extravasation on computer tomography angiography (CTA) who requires the Critical level of care • Variceal bleeding rule was changed to at least one to clarify the intent of the criteria • NIPPV subcriteria were removed because any patient who is newly requiring ventilator support is appropriate for the Critical level of care on Episode Day 1

Gastrointestinal (GI) Bleeding		
Episode Day 2 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Bleeding controlled or resolved ♦Hct or Hb within acceptable limits ♦No complication or active comorbidity relevant to this episode of care ▶Non-responder, not clinically stable for discharge and exceeds initial 24h Observation for this condition, One: ♦For gastrointestinal (GI) bleeding use Episode	•OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Gastrointestinal (GI) bleeding controlled or resolved ♦Hct or Hb within acceptable limits ♦No complication or active comorbidity relevant to this episode of care ▶Non-responder, not clinically stable for discharge and exceeds initial 24h Observation for this condition, One:	Rationale: • Responder criteria were updated to reflect the patient appropriate for discharge

Gastrointestinal (GI) Bleeding		
Episode Day 2 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
Day 2 at a higher level of care ♦For a condition other than gastrointestinal (GI) bleeding use appropriate subset (Episode Day 1) based on clinical findings ♦Refer for secondary review	♦For gastrointestinal (GI) bleeding use Episode Day 2 at a higher level of care ♦For a condition other than gastrointestinal (GI) bleeding use appropriate subset (Episode Day 1) based on clinical findings ♦Refer for secondary review	

Gastrointestinal (GI) Bleeding		
Episode Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦ACUTE, One: ►Responder, discharge expected today if clinically stable last 12h, All: ♦Bleeding controlled or resolved ♦Dyspnea or presyncope/syncope improving or resolved (if present during admission) ♦Hct or Hb within acceptable limits ♦Mental status or GCS within normal limits or at baseline ♦Vital signs within acceptable limits ♦No complication or active comorbidity relevant to this episode of care ►Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦Lower gastrointestinal (GI) bleeding and, Both: ►Finding, ≥ One: ♦Angiodysplasia and risk of rebleed ♦Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦Hct or Hb decreased within last 24h ♦Exertional dyspnea ♦Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦Orthostatic hypotension, Both: ►Sustained drop in blood pressure within 3 min of sitting or standing ►Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ♦Presyncope or syncope ♦Persistent bleeding ►Hct or Hb monitoring at least 2x/24h and, Both: ♦Colonoscopy performed within 24h ♦Blood product transfusion ♦Upper gastrointestinal (GI) non-variceal	♦ACUTE, One: ►Responder, discharge expected today if clinically stable last 12h, All: ♦Gastrointestinal (GI) bleeding controlled or resolved ♦Hct or Hb within acceptable limits ♦Vital signs within acceptable limits ♦No complication or active comorbidity relevant to this episode of care ►Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦Lower gastrointestinal (GI) bleeding and, Both: ►Finding, ≥ One: ♦Angiodysplasia and risk of rebleed ♦Hb decrease ≥ 2.0 g/dL(20g/L) within last 24h ♦Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦Exertional dyspnea worse than baseline ♦Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦Orthostatic hypotension, Both: ►Sustained drop in blood pressure within 3 min of sitting or standing ►Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ♦Persistent bleeding ♦Syncope ►Hct or Hb monitoring at least 2x/24h and, ≥ One: ♦Blood product transfusion ♦Colonoscopy performed within 24h ♦Upper gastrointestinal (GI) non-variceal bleeding and, Both: ►Finding, ≥ One:	Rationale: • Responder criteria were updated to reflect the patient appropriate for discharge • "Hct of Hb decreased within last 24h" criterion was changed to "Hb decrease ≥ 2.0 g/dL within last 24h" to accurately reflect the patient with rebleeding who requires continued stay at the Acute level of care • "Worse than baseline" was added to "Exertional dyspnea" to clarify that the intent of the criteria point is not appropriate for patients who have chronic dyspnea related to a comorbid condition and describing the patient who has exertional dyspnea over and above their chronic state • "Presyncope" was removed as literature does not speak to presyncope as a driver for admission in patients with gastrointestinal bleeding • Interventions for lower and upper gastrointestinal bleeding were updated to reflect current guidelines and to better capture the patient who is appropriate for the Acute level of care • "High-dose proton pump inhibitor ≤ 3d" criterion was added as a most recent guideline-appropriate intervention after endoscopic therapy at the Acute level of care

Gastrointestinal (GI) Bleeding		
Episode Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
bleeding and, Both: ▶ Finding, ≥ One: ♦ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦ Hct or Hb decreased within last 24h ♦ Exertional dyspnea ♦ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦ Orthostatic hypotension, Both: ▶ Sustained drop in blood pressure within 3 min of sitting or standing ▶ Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ♦ Presyncope or syncope ♦ Persistent bleeding ♦ Risk of rebleed, ≥ One: ▶ Adherent clot ▶ Bleeding disorder ▶ Ulcer size ≥ 2 cm ▶ Visible non-bleeding vessel ▶ Hct or Hb monitoring at least 2x/24h and, Both: ♦ Endoscopy performed within 24h ♦ Blood product transfusion ♦ Post Critical care ≤ 24h	♦ Hb decrease ≥ 2.0 g/dL(20g/L) within last 24h ♦ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦ Exertional dyspnea worse than baseline ♦ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦ Orthostatic hypotension, Both: ▶ Sustained drop in blood pressure within 3 min of sitting or standing ▶ Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ♦ Persistent bleeding ♦ Risk of rebleed, ≥ One: ▶ Adherent clot ▶ Bleeding disorder ▶ Ulcer size ≥ 2 cm ▶ Visible non-bleeding vessel ♦ Syncope ▶ Hct or Hb monitoring at least 2x/24h and, ≥ One: ♦ Blood product transfusion ♦ Endoscopy performed within 24h ♦ High-dose proton pump inhibitor ≤ 3d ♦ Post Critical care ≤ 24h	

Gastrointestinal (GI) Bleeding		
Episode Day 2 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
♦ CRITICAL, ≥ One: ▶ Variceal bleeding controlled ≤ 2d and, Both: ♦ Hemodynamic stability ♦ Octreotide or somatostatin ▶ AKI and, Both: ♦ Finding, ≥ One: ▶ Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: ♦ Loss of P wave ♦ Multifocal PVCs ♦ Neuromuscular deficit ♦ Widening QRS ▶ Sodium, One: ♦ < 120 mEq/L(120 mmol/L) and volume overload ♦ > 160 mEq/L(160 mmol/L)	♦ CRITICAL, ≥ One: ▶ Embolization performed within 24h ▶ Post embolization ≤ 24h ▶ Variceal bleeding controlled ≤ 2d and, Both: ♦ Hemodynamic stability ♦ Octreotide or somatostatin ▶ Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion and, Both: ♦ Finding, ≥ One: ▶ Heart rate > 120/min ▶ Systolic blood pressure < 100 mmHg and < baseline ▶ MAP < 65 mmHg ♦ Intervention, ≥ One: ▶ Blood product transfusion	Rationale: "Embolization performed within 24h" criterion was added to reflect the patient who cannot tolerate endoscopy, has recurrent bleeding post repeated endoscopic therapy, or demonstrates extravasation on computer tomography angiography (CTA) who requires the Critical level of care "Post embolization ≤ 24h" criterion was added to reflect the patient who underwent embolization in the last 24h and continues to require the Critical level of care for monitoring

Gastrointestinal (GI) Bleeding		
Episode Day 2 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
♦Intervention, ≥ One: ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶Dialysis, One: ♦Hemodialysis ≤ 7d since initiation ♦Peritoneal dialysis ≤ 7d since initiation ▶IV fluid resuscitation ≤ 2d ▶CRRT ▶Hemodynamic instability persisting after ≥1L of IVF resuscitation or ≥1 unit blood product transfusion and, Both: ♦Finding, ≥ One: ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: ▶Blood product transfusion ▶IV fluid resuscitation ≥1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶Mechanical ventilation ▶NIPPV and, ≥ One: ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	▶IV fluid resuscitation ≥ 1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶Mechanical ventilation ▶NIPPV and, ≥ One: ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	• AKI and CRRT criteria were removed as an individual with AKI or requiring CRRT would be appropriate to be reviewed in the AKI subset

Gastrointestinal (GI) Bleeding		
Episode Day 3 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦ACUTE, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Bleeding controlled or resolved ♦Dyspnea or presyncope/syncope improving or resolved (if present during admission) ♦Hct or Hb within acceptable limits ♦Mental status or GCS within normal limits or at baseline ♦Vital signs within acceptable limits ♦Complication or comorbidity, One: ▶No complication or active comorbidity relevant to this episode of care	♦ACUTE, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Gastrointestinal (GI) bleeding controlled or resolved ♦Hct or Hb within acceptable limits ♦Vital signs within acceptable limits ♦No complication or active comorbidity relevant to this episode of care ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦Lower gastrointestinal (GI) bleeding and, Both: ▶Finding, ≥ One:	Rationale: • Responder criteria were updated to reflect the patient appropriate for discharge • "Hct of Hb decreased within last 24h" criterion was changed to "Hb decrease ≥ 2.0 g/dL within last 24h" to accurately reflect the patient with rebleeding who requires continued stay at the Acute level of care • "Worse than baseline" was added to "Exertional dyspnea" to clarify that the intent of the criteria

Gastrointestinal (GI) Bleeding		
Episode Day 3 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
►Complication or comorbidity and clinically stable for discharge, One: ♦AKI, ≥ One: ►Kidney function improving or within acceptable limits ►Outpatient dialysis regimen established ►Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦Lower gastrointestinal (GI) bleeding and, Both: ►Finding, ≥ One: ♦Angiodysplasia and risk of rebleed ♦Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦Hct or Hb decreased within last 24h ♦Exertional dyspnea ♦Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦Orthostatic hypotension, Both: ►Sustained drop in blood pressure within 3 min of sitting or standing ►Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ♦Presyncope or syncope ►Intervention, Both: ♦Hct or Hb monitoring at least 2x/24h ♦Blood product transfusion ♦Upper gastrointestinal (GI) non-variceal bleeding and, Both: ►Finding, ≥ One: ♦Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦Hct or Hb decreased within last 24h ♦Exertional dyspnea ♦Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦Orthostatic hypotension, Both: ►Sustained drop in blood pressure within 3 min of sitting or standing ►Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ♦Presyncope or syncope ♦Risk of rebleed, ≥ One: ►Adherent clot ►Bleeding disorder ►Ulcer size ≥ 2 cm ►Visible non-bleeding vessel ►Intervention, Both: ♦Hct or Hb monitoring at least 2x/24h ♦Blood product transfusion ♦Upper gastrointestinal (GI) variceal bleeding and, Both: ►Bleeding controlled > 2d and, ≥ One: ♦Medium or large varices ♦Red wale marks found on endoscopy ♦Child-Pugh score 9 or higher ►Hct or Hb monitoring at least 2x/24h and, ≥ One: ♦Blood product transfusion ♦Octreotide or somatostatin	♦Angiodysplasia and risk of rebleed ♦Hb decrease ≥ 2.0 g/dL(20g/L) within last 24h ♦Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦Exertional dyspnea worse than baseline ♦Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦Orthostatic hypotension, Both: ►Sustained drop in blood pressure within 3 min of sitting or standing ►Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ♦Syncope ►Intervention, Both: ♦Blood product transfusion ♦Hct or Hb monitoring at least 2x/24h ♦Upper gastrointestinal (GI) non-variceal bleeding and, Both: ►Finding, ≥ One: ♦Hb decrease ≥ 2.0 g/dL(20g/L) within last 24h ♦Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦Exertional dyspnea worse than baseline ♦Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦Orthostatic hypotension, Both: ►Sustained drop in blood pressure within 3 min of sitting or standing ►Systolic blood pressure drop ≥ 20 mmHg or diastolic blood pressure drop ≥ 10 mmHg ♦Risk of rebleed, ≥ One: ►Adherent clot ►Bleeding disorder ►Ulcer size ≥ 2 cm ►Visible non-bleeding vessel ♦Syncope ►Hct or Hb monitoring at least 2x/24h and, ≥ One: ♦Blood product transfusion ♦High-dose proton pump inhibitor ≤ 3d ♦Upper gastrointestinal (GI) variceal bleeding and, Both: ►Bleeding controlled > 2d and, ≥ One: ♦Medium or large varices ♦Red wale marks found on endoscopy ♦Child-Pugh score 9 or higher ►Hct or Hb monitoring at least 2x/24h and, ≥ One: ♦Blood product transfusion ♦Octreotide or somatostatin ♦Post Critical care ≤ 24h	point is not appropriate for patients who have chronic dyspnea related to a comorbid condition and describing the patient who has exertional dyspnea over and above their chronic state • "Presyncope" was removed as literature does not speak to presyncope as a driver for admission in patients with gastrointestinal bleeding • Interventions for lower and upper gastrointestinal bleeding were updated to reflect current guidelines and to better capture the patient who is appropriate for the Acute level of care • "High-dose proton pump inhibitor ≤ 3d" criterion was added as a most recent guideline-appropriate intervention after endoscopic therapy at the Acute level of care

Gastrointestinal (GI) Bleeding		
Episode Day 3 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦Child-Pugh score 9 or higher ▶Hct or Hb monitoring at least 2x/24h and, ≥ One: ♦Blood product transfusion ♦Octreotide or somatostatin ♦Post Critical care ≤ 24h		

Gastrointestinal (GI) Bleeding		
Episode Day 3 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
♦CRITICAL, ≥ One: ▶Variceal bleeding controlled ≤ 2d and, Both: ♦Hemodynamic stability ♦Octreotide or somatostatin ▶AKI and, Both: ♦Finding, ≥ One: ▶Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: ♦Loss of P wave ♦Multifocal PVCs ♦Neuromuscular deficit ♦Widening QRS ▶Sodium, One: ♦< 120 mEq/L(120 mmol/L) and volume overload ♦> 160 mEq/L(160 mmol/L) ♦Intervention, ≥ One: ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶Dialysis, One: ♦Hemodialysis ≤ 7d since initiation ♦Peritoneal dialysis ≤ 7d since initiation ▶IV fluid resuscitation ≤ 2d ▶CRRT ▶Hemodynamic instability persisting after ≥1L of IVF resuscitation or ≥1 unit blood product transfusion and, Both: ♦Finding, ≥ One: ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: ▶Blood product transfusion ▶IV fluid resuscitation ≥ 1L ≤ 24h	♦CRITICAL, ≥ One: ▶Embolization performed within 24h ▶Post embolization ≤ 24h ▶Variceal bleeding controlled ≤ 2d and, Both: ♦Hemodynamic stability ♦Octreotide or somatostatin ▶Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion and, Both: ♦Finding, ≥ One: ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: ▶Blood product transfusion ▶IV fluid resuscitation ≥ 1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶Mechanical ventilation ▶NIPPV and, ≥ One: ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	Rationale: • "Embolization performed within 24h" criterion was added to reflect the patient who cannot tolerate endoscopy, has recurrent bleeding post repeated endoscopic therapy, or demonstrates extravasation on computer tomography angiography (CTA) who requires the Critical level of care • "Post embolization ≤ 24h" criterion was added to reflect the patient who underwent embolization in the last 24h and continues to require the Critical level of care for monitoring • AKI and CRRT criteria were removed as an individual with AKI or requiring CRRT would be appropriate to be reviewed in the AKI subset

Gastrointestinal (GI) Bleeding		
Episode Day 3 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
▶Blood product transfusion ▶IV fluid resuscitation $\geq 1\text{L} \leq 24\text{h}$ ▶Hemodynamic monitoring, invasive ▶Mechanical ventilation ▶NIPPV and, $\geq$ One: ♦$\text{FiO}_2 > 50\%$(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)		

Gastrointestinal (GI) Bleeding		
Episode Day 4 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
•ACUTE, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Bleeding controlled or resolved ♦Dyspnea or presyncope/syncope improving or resolved (if present during admission) ♦Hct or Hb within acceptable limits ♦Mental status or GCS within normal limits or at baseline ♦Vital signs within acceptable limits ▶Complication or comorbidity, One: ▶No complication or active comorbidity relevant to this episode of care ▶Complication or comorbidity and clinically stable for discharge, One: ♦AKI, $\geq$ One: ▶Kidney function improving or within acceptable limits ▶Outpatient dialysis regimen established ▶Non-responder, not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, One: ♦Use Extended Stay subset ♦Refer for secondary review		
	•ACUTE, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Gastrointestinal (GI) bleeding controlled or resolved ♦Hct or Hb within acceptable limits ♦Vital signs within acceptable limits ♦No complication or active comorbidity relevant to this episode of care ▶Non-responder, not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, One: ♦Use Extended Stay subset ♦Refer for secondary review	Rationale: • Responder criteria were updated to reflect the patient appropriate for discharge

General Medical

 Revision Summary	Heparin-induced thrombocytopenia criteria were updated to streamline the review process in the Cardiovascular or peripheral vascular section at the Acute level of care on all Episode Days and the Intermediate level of care on Episode Days 1 & 2, as well as the Transplant section at the Acute level of care on Episode Day 3-X. Several high risk factors were updated and the inclusion of patients with hypoglycemia in the Eating Disorder criteria to align with the 2023 American Psychiatric Association (APA) Practice Guidelines.
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General Medical		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
•ACUTE, ≥ One: ▶Cardiovascular or peripheral vascular, One: ♦Other cardiovascular diagnosis, actual or suspected, One: ▶Acute limb ischemia, actual or suspected and, Both: ♦Finding, ≥ One: ▶Confirmed by imaging ▶Coldness, mottling, pallor, or numbness ▶Limb swelling ▶Pain at rest ♦Intervention, Both: ▶Anticoagulant, One: ♦Unfractionated heparin, continuous ≤ 5d ♦Heparin-induced thrombocytopenia suspected or by history and, One: ▶Argatroban ▶Bivalirudin ▶Fondaparinux ▶Neurovascular assessment at least 6x/24h	•ACUTE, ≥ One: ▶Cardiovascular or peripheral vascular, One: ♦Other cardiovascular diagnosis, actual or suspected, One: ▶Acute limb ischemia, actual or suspected and, Both: ♦Finding, ≥ One: ▶Confirmed by imaging ▶Coldness, mottling, pallor, or numbness ▶Limb swelling ▶Pain at rest ♦Intervention, Both: ▶Anticoagulant, One: ♦Unfractionated heparin, continuous ≤ 5d ♦HIT, suspected or by history and anticoagulation ▶Neurovascular assessment at least 6x/24h	Rationale: • “Heparin-induced thrombocytopenia” criteria were updated to streamline the review process

General Medical		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
•ACUTE, ≥ One: ▶Eating disorder, actual or suspected, All: ♦Requiring medical stabilization ♦Finding, ≥ One: ▶Blood pressure, ≥ One:	•ACUTE, ≥ One: ▶Eating disorder and, All: ♦Requiring medical stabilization ♦Finding, ≥ One: ▶Blood glucose < 60 mg/dL(3.3 mmol/L)	Rationale: • “Actual or suspected” was removed from the Eating Disorder content on Episode Day 1 because

General Medical		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦Systolic < 90 mmHg ♦Diastolic < 60 mmHg ▶Cardiomyopathy and, One: ♦Cardiomegaly by imaging ♦Tricuspid or mitral valve dysfunction by physical examination ♦EF < 40%(0.40) ▶Co-occurring medical condition and unstable lab values requiring daily monitoring ▶Dehydration and, ≥ One: ♦Blood urea nitrogen (BUN) 25-45 mg/dL(8.9-16.1 mmol/L) ♦Creatinine 1.5-3.0 mg/dL(132.6-265.2 µmol/L) ♦Heart rate sustained > 100/min ♦Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦Orthostatic hypotension, Both: ▶Sustained drop in blood pressure within 3 min of sitting or standing ▶Blood pressure, ≥ One: ♦Systolic drop ≥ 20 mmHg ♦Diastolic drop ≥ 10 mmHg ♦Urine specific gravity > 1.030 ▶Electrolyte imbalance, ≥ One: ♦Magnesium 1.0-1.4 mg/dL(0.41-0.58 mmol/L) ♦Phosphate 1.0-2.0 mg/dL(0.32-0.65 mmol/L) ♦Potassium < 3.0 mEq/L(3.0 mmol/L) without electrocardiogram (ECG) changes ♦Sodium, One: ▶151-160 mEq/L(151-160 mmol/L) ▶120-129 mEq/L(120-129 mmol/L) ▶Failed outpatient management ▶Heart rate, One: ♦< 40/min, sustained ♦> 110/min, sustained ▶Orthostatic hypotension, Both: ♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Blood pressure, ≥ One: ▶Systolic drop ≥ 20 mmHg ▶Diastolic drop ≥ 10 mmHg ▶Orthostatic changes in heart rate ≥ 20/min ▶QTc interval prolonged, One: ♦> 440 ms in males ♦> 460 ms in females ▶Temperature < 97.0°F(36.1°C) ▶Weight, ≥ One: ♦≤ 75%(0.75) ideal body weight ♦BMI < 16 ♦Weight loss ≥ 15%(0.15) in last month ♦Intervention, Both:	▶Blood pressure, Both: ♦Systolic < 90 mmHg ♦Diastolic < 60 mmHg ▶Cardiomyopathy and, One: ♦Cardiomegaly by imaging ♦Tricuspid or mitral valve dysfunction by physical examination ♦EF < 40%(0.40) ▶Co-occurring medical condition and unstable lab values requiring daily monitoring ▶Dehydration and, ≥ One: ♦BUN 25-45 mg/dL(8.9-16.1 mmol/L) ♦Creatinine 1.5-3.0 mg/dL(132.6-265.2 µmol/L) ♦Heart rate sustained > 100/min ♦Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦Urine specific gravity > 1.030 ▶Electrolyte imbalance, ≥ One: ♦Magnesium 1.0-1.4 mg/dL(0.41-0.58 mmol/L) ♦Phosphate 1.0-1.4 mg/dL(0.32-0.45 mmol/L) ♦Potassium < 3.0 mEq/L(3.0 mmol/L) without ECG changes ♦Sodium, One: ▶151-160 mEq/L(151-160 mmol/L) ▶120-129 mEq/L(120-129 mmol/L) ▶Lack of improvement with outpatient treatment ▶Heart rate < 50/min, sustained ▶Orthostatic hypotension, Both: ♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Blood pressure, ≥ One: ▶Systolic drop ≥ 20 mmHg ▶Diastolic drop ≥ 10 mmHg ▶Orthostatic changes in heart rate > 30/min ▶QTc interval > 450 ms ▶Temperature < 96.8°F(36.0°C) ▶Weight, ≥ One: ♦BMI < 15 kg/m² ♦Weight loss > 10%(0.10) in last 6 months or > 20%(0.20) in last year ♦Intervention, Both: ▶Monitoring, All: ♦Calorie count ♦Daily weight ♦Electrolytes monitored at least once daily ♦Observation and documentation of oral or tube feeding pattern ♦Vital signs at least 3x/24h ▶Treatment, ≥ One:	the patient appropriate for these criteria is to have a confirmed diagnosis of an eating disorder • “Blood glucose < 60 mg/dL” was added as a finding for patients with eating disorders as it is listed in the 2023 APA Guidelines as a high risk factor supporting hospitalization for patients with eating disorders • The rule for “Blood pressure” criteria was changed to “Both” because patients admitted with an eating disorder and have both a systolic blood pressure of less than 90 mmHg and a diastolic blood pressure of less than 60 mmHg are appropriate for inpatient treatment based on the 2023 APA Guidelines • The phosphate range for patients with eating disorders at Acute level of care was changed from “1.0-2.0 mg/dL (0.32-0.65 mmol/L)” to “1.0-1.4 mg/dL (0.32-0.45 mmol/L)” to be consistent with other content within the product as a phosphate level of greater than 1.4 mg/dL does not routinely require inpatient treatment • “Failed outpatient management” was changed to “lack of improvement with outpatient treatment” to clarify that although patients with an eating disorder may have received outpatient care, they did not improve significantly enough and now require inpatient treatment • The range of “< 40/min, sustained” and “> 110/min, sustained” was removed and “Heart rate < 50/min” was added as a finding for patients with eating disorders to align with the 2023 APA Guidelines as a risk factor supporting hospitalization • Orthostatic changes in heart rate were changed from “≥ 20/min” to “> 30/min” based on the 2023 APA Guidelines • QTc interval was updated based on the 2023 APA Guidelines • The temperature threshold for patients with an eating disorder was changed from

General Medical		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
▶Monitoring, All: ♦Calorie count ♦Daily weight ♦Electrolyte monitoring, One: ▶Electrolyte imbalance and electrolytes monitored at least once daily ▶Electrolytes within normal range and monitoring electrolytes not required ♦Observation and documentation of oral or tube feeding pattern ▶Treatment, ≥ One: ♦Complete bed rest ♦Fluid, supplements, or medication to correct electrolyte abnormalities ♦IV fluid, One: ▶≥ 75 mL/h and, ≥ One: ♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ▶≥ 100 mL/h and weight ≥ 60 kg ♦Tube feeding or parenteral nutrition ♦Vital signs at least every 4h for medical instability ♦Weight-gain-specific treatment plan and, All: ▶Accompanied or monitored bathroom visits ▶Nutritional assessment daily ▶Re-evaluation of plan effectiveness daily ▶Supervised meals and snacks ▶Vital signs at least 3x/24h	♦Blood glucose monitoring at least 4x/24h ♦Complete bed rest ♦Fluid, supplements, or medication to correct electrolyte abnormalities ♦IV fluid ♦Tube feeding or parenteral nutrition ♦Weight-gain-specific treatment plan and, All: ▶Accompanied or monitored bathroom visits ▶Nutritional assessment daily ▶Re-evaluation of plan effectiveness daily ▶Supervised meals and snacks	"< 97.0°F(36.1°C)" to "< 96.8°F(36.0°C)" based on the 2023 APA Guidelines • The BMI criteria were changed from "BMI < 16" to "BMI < 15 kg/m²" to align with the 2023 APA Guidelines • "≤ 75%(0.75) ideal body weight" was removed as this finding is not included in the 2023 APA Guidelines for adult patients with eating disorders • "Weight loss ≥ 15%(0.15) in last month" was changed to "weight loss > 10%(0.10) in last 6 months or > 20%(0.20) in last year" based on the updated 2023 APA Guidelines • Electrolyte monitoring criteria were restructured to align with clinical practice as these patients are at risk of drastic electrolyte abnormalities and their electrolyte levels are monitored at least once daily even if they were previously within normal range • "Vital signs at least 3x/24h" was moved as a criterion under "monitoring" to streamline the review process • "Blood glucose monitoring at least 4x/24h" was added as an intervention for those patients with an eating disorder who are admitted with hypoglycemia • "Vital signs at least every 4h for medical instability" was removed from criteria as this is a standard of care for patients with eating disorders who are admitted to the Acute level of care

General Medical		
Episode Day 1 (INTERMEDIATE)		
Previously Available Content	2024 March Release	Revision Description
•INTERMEDIATE, ≥ One: ▶Cardiovascular or peripheral vascular, One:	•INTERMEDIATE, ≥ One: ▶Cardiovascular or peripheral vascular, One:	Rationale:

General Medical		
Episode Day 1 (INTERMEDIATE)		
Previously Available Content	2024 March Release	Revision Description
♦Other cardiovascular diagnosis, actual or suspected, ≥ One: ▶Pacemaker or ICD replacement and, ≥ One: ♦Lead fractured or displaced ♦Malfunction ▶Pericardial effusion and high risk for cardiac tamponade ≤ 24h and, Both: ♦Asymptomatic ♦Effusion size ≥ 1 cm ▶Acute limb ischemia, actual or suspected ≤ 2d and, Both: ♦Finding, ≥ One: ▶Confirmed by imaging ▶Coldness, mottling, pallor, or numbness ▶Limb swelling ▶Pain at rest ♦Intervention, Both: ▶Anticoagulant and high risk for bleeding and, One: ♦Unfractionated heparin, continuous ♦Heparin-induced thrombocytopenia suspected or by history and, One: ▶Argatroban ▶Bivalirudin ▶Fondaparinux ▶Neurovascular assessment every 3-4h ▶Post PCI complication and continuous cardiac monitoring (excludes Holter) ≤ 24h ▶VAD requiring readmission and, ≥ One: ♦Anticoagulation, ≥ One: ▶Direct thrombin inhibitor ▶GP IIb or IIIa inhibitor ▶Heparin ▶Thrombolytic ♦Bleeding actual or suspected and, ≥ One: ▶Hct or Hb monitoring at least 3x/24h, ≤ 3d ▶Volume expander or blood product ♦Inadequate nutrition and, ≥ One: ▶Calorie count ≤ 3d ▶Requiring daily tube feeding adjustment ▶TPN initiation ≤ 24h ♦Infection actual or suspected and, Both: ▶Finding, ≥ One: ♦Temperature ≥ 100.4°F(38.0°C) ♦WBC ≥ 12,000/cu.mm(12x10⁹/L) ♦WBC ≤ 4,000/cu.mm(4x10⁹/L) ♦Bands > 10%(0.10) ♦ANC < 500/cu.mm(500x10⁶/L) ▶Anti-infective	♦Other cardiovascular diagnosis, actual or suspected, ≥ One: ▶Pacemaker or ICD replacement and, ≥ One: ♦Lead fractured or displaced ♦Malfunction ▶Pericardial effusion and high risk for cardiac tamponade ≤ 24h and, Both: ♦Asymptomatic ♦Effusion size ≥ 1 cm ▶Acute limb ischemia, actual or suspected ≤ 2d and, Both: ♦Finding, ≥ One: ▶Confirmed by imaging ▶Coldness, mottling, pallor, or numbness ▶Limb swelling ▶Pain at rest ♦Intervention, Both: ▶Anticoagulant and high risk for bleeding and, One: ♦Unfractionated heparin, continuous ♦HIT, suspected or by history and anticoagulation ▶Neurovascular assessment every 3-4h ▶Post PCI complication and continuous cardiac monitoring (excludes Holter) ≤ 24h ▶VAD requiring readmission and, ≥ One: ♦Anticoagulation, ≥ One: ▶Direct thrombin inhibitor ▶GP IIb or IIIa inhibitor ▶Heparin ▶Thrombolytic ♦Bleeding actual or suspected and, ≥ One: ▶Hct or Hb monitoring at least 3x/24h, ≤ 3d ▶Volume expander or blood product ♦Inadequate nutrition and, ≥ One: ▶Calorie count ≤ 3d ▶Requiring daily tube feeding adjustment ▶TPN initiation ≤ 24h ♦Infection actual or suspected and, Both: ▶Finding, ≥ One: ♦Temperature ≥ 100.4°F(38.0°C) ♦WBC ≥ 12,000/cu.mm(12x10⁹/L) ♦WBC ≤ 4,000/cu.7.5mm(4x10⁹/L) ♦Bands > 10%(0.10) ♦ANC < 500/cu.mm(500x10⁶/L) ▶Anti-infective	• “Heparin-induced thrombocytopenia” criteria were updated to streamline the review process

General Medical		
Episode Day 1 (INTERMEDIATE)		
Previously Available Content	2024 March Release	Revision Description
N/A	• INTERMEDIATE, ≥ One: ▶ Eating disorder and, ≥ One: ♦ Hypokalemia, All: ▶ Potassium < 3.0 mEq/L(3.0 mmol/L) ▶ PVCs > 6/min ▶ Potassium repletion ♦ Seizure and, Both: ▶ Activity resolved ▶ Neurological assessment every 3-4h	Rationale: - Hypokalemia criteria were added to the Intermediate level of care as patients with eating disorders are at high risk for arrhythmia and require monitoring while their electrolytes are repleted - Seizure criteria were added to the Intermediate level of care as the 2023 APA Guidelines state seizures are an acute medical complication of severe electrolyte disturbances or hypoglycemia in patients with eating disorders

General Medical		
Episode Day 1 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
N/A	• CRITICAL, ≥ One: ▶ Eating disorder and, Both: ♦ Finding, ≥ One: ▶ Magnesium < 1.0 mg/dL(0.41 mmol/L) ▶ Phosphate < 1.0 mg/dL(0.32 mmol/L) ▶ Potassium < 3.0 mEq/L(3.0 mmol/L) and, ≥ One: ♦ Multifocal PVCs ♦ R on T phenomenon ♦ T and U wave fusion ♦ U wave ▶ Sodium > 160 mEq/L(160 mmol/L) ▶ Sodium < 120 mEq/L(120 mmol/L) and symptomatic ▶ Coma, stupor, obtundation or GCS ≤ 8 ▶ Status epilepticus (excludes epilepsy) ♦ Intervention, ≥ One: ▶ CRRT ▶ Dialysis, hemodialysis or peritoneal ▶ Electrolyte repletion ▶ IV fluid ▶ Neurological assessment every 1-2h ≤ 2d ▶ Sodium chloride 3%(0.03)	Rationale: - Magnesium, phosphate, potassium, and sodium electrolyte levels were added to the Critical level of care for patients who are admitted with eating disorders because these levels put the patients at risk for severe complications “Coma, stupor, obtundation or GCS ≤ 8” and “status epilepticus (excludes epilepsy)” were added as findings for patients admitted with eating disorders as these are indicators of severe electrolyte disturbances or hypoglycemia and require monitoring at the Critical level of care “CRRT,” “dialysis, hemodialysis or peritoneal,” “electrolyte repletion,” “IV fluid,” “neurological assessment every 1-2h,” and “sodium chloride 3%(0.03)” were added to be more inclusive of the interventions which may necessitate the Critical level of care for patients with eating disorders

General Medical		
Episode Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
• ACUTE, ≥ One: ▶ Cardiovascular or peripheral vascular, ≥ One: ♦ Pericardial effusion and, ≥ One: ▶ Pericardial drain removed ≤ 24h ▶ Post Critical care ≤ 24h ♦ Acute limb ischemia and, One: ▶ Treatment, Both: ♦ Anticoagulant, One: ▶ Unfractionated heparin, continuous ≤ 5d ▶ Heparin-induced thrombocytopenia, suspected or by history and, Both: ♦ Warfarin (includes PO) administered or contraindicated ♦ Intervention, One: ▶ Argatroban ▶ Bivalirudin ▶ Fondaparinux ♦ Neurovascular assessment at least 4x/24h	• ACUTE, ≥ One: ▶ Cardiovascular or peripheral vascular, ≥ One: ♦ Pericardial effusion and, ≥ One: ▶ Pericardial drain removed ≤ 24h ▶ Post Critical care ≤ 24h ♦ Acute limb ischemia and, One: ▶ Treatment, Both: ♦ Anticoagulant, One: ▶ Unfractionated heparin, continuous ≤ 5d ▶ HIT, suspected or by history and anticoagulation ♦ Neurovascular assessment at least 4x/24h	Rationale: • “Heparin-induced thrombocytopenia” criteria were updated to streamline the review process

General Medical		
Episode Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
• ACUTE, ≥ One: ▶ Eating disorder, Both: ♦ Monitoring, All: ▶ Calorie count ▶ Daily weight ▶ Electrolyte monitoring, One: ♦ Electrolyte imbalance and electrolytes monitored at least daily ♦ Electrolytes within normal range and monitoring electrolytes not required ▶ Observation and documentation of oral or tube feeding pattern ♦ Treatment, ≥ One: ▶ Complete bed rest ▶ Fluid, supplements, or medication to correct electrolyte abnormalities ▶ IV fluid, One: ♦ ≥ 75 mL/h and, ≥ One: ▶ Weight < 60 kg ▶ Age ≥ 65 ▶ Renal insufficiency	• ACUTE, ≥ One: ▶ Eating disorder, ≥ One: ♦ Eating disorder and, Both: ♦ Finding, ≥ One: ▶ Blood glucose < 60 mg/dL(3.3 mmol/L) ▶ Blood pressure, Both: ♦ Systolic < 90 mmHg ♦ Diastolic < 60 mmHg ▶ Cardiomyopathy and, One: ♦ Cardiomegaly by imaging ♦ Tricuspid or mitral valve dysfunction by physical examination ♦ EF < 40%(0.40) ▶ Co-occurring medical condition and unstable lab values requiring daily monitoring ▶ Dehydration and, ≥ One: ♦ BUN 25-45 mg/dL(8.9-16.1 mmol/L) ♦ Creatinine 1.5-3.0 mg/dL(132.6-265.2 μmol/L) ♦ Heart rate sustained > 100/min	Rationale: • Eating Disorder findings were added to Episode Day 2 to capture the patients who continue to show manifestations and complications related to their eating disorder who require monitoring and treatment • Electrolyte monitoring criteria were restructured to better align with clinical practice as these patients are at high risk of electrolyte abnormalities and require at least daily bloodwork • “Vital signs at least 3x/24h” was moved as a criterion under “monitoring” to streamline the review process • “Vital signs at least every 4h for medical instability” was removed as a treatment as this is a standard

General Medical		
Episode Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ▶Tube feeding or parenteral nutrition ▶Vital signs at least every 4h for medical instability ▶Weight-gain-specific treatment plan and, All: ♦Accompanied or monitored bathroom visits ♦Nutritional assessment daily ♦Re-evaluation of plan effectiveness daily ♦Supervised meals and snacks ♦Vital signs at least 3x/24h	♦Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦Urine specific gravity > 1.030 ▶Electrolyte imbalance, ≥ One: ♦Magnesium 1.0-1.4 mg/dL(0.41-0.58 mmol/L) ♦Phosphate 1.0-1.4 mg/dL(0.32-0.45 mmol/L) ♦Potassium < 3.0 mEq/L(3.0 mmol/L) without ECG changes ♦Sodium, One: ▶151-160 mEq/L(151-160 mmol/L) ▶120-129 mEq/L(120-129 mmol/L) ▶Heart rate < 50/min, sustained ▶Orthostatic hypotension, Both: ♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Blood pressure, ≥ One: ▶Systolic drop ≥ 20 mmHg ▶Diastolic drop ≥ 10 mmHg ▶Orthostatic changes in heart rate > 30/min ▶QTc interval > 450 ms ▶Temperature < 96.8°F(36.0°C) ▶Weight, ≥ One: ♦Continued weight loss ♦Failure to stabilize weight ♦Intervention, Both: ♦Monitoring, All: ▶Calorie count ▶Daily weight ▶Electrolytes monitored at least once daily ▶Observation and documentation of oral or tube feeding pattern ♦Vital signs at least 3x/24h ♦Treatment, ≥ One: ▶Blood glucose monitoring at least 4x/24h ▶Complete bed rest ▶Fluid, supplements, or medication to correct electrolyte abnormalities ▶IV fluid ▶Tube feeding or parenteral nutrition ▶Weight-gain-specific treatment plan and, All: ♦Accompanied or monitored bathroom visits ♦Nutritional assessment daily ♦Re-evaluation of plan effectiveness daily ♦Supervised meals and snacks ♦Post Critical care ≤ 24h	of care for patients admitted to the Acute level of care with an eating disorder • “Blood glucose monitoring at least 4x/24h” was added as an intervention for those patients with an eating disorder who are admitted with hypoglycemia • “Post Critical care ≤ 24h” was added to include the patients who were previously admitted to the Critical level of care and now require treatment and monitoring at the Acute level of care

General Medical		
Episode Day 2 (INTERMEDIATE)		
Previously Available Content	2024 March Release	Revision Description
• INTERMEDIATE, ≥ One: ▶ Cardiovascular or peripheral vascular, ≥ One: ♦Epoprostenol, treprostinil, or iloprost (includes continuous infusion or inhaled) initiation ≤ 2d ♦Pacemaker or ICD placement performed within last 24h ♦Pericardial drain and post Critical care ≤ 24h ♦Pericardial effusion and high risk for cardiac tamponade ≤ 24h and, Both: ▶Asymptomatic ▶Effusion size ≥ 1 cm ♦Acute limb ischemia ≤ 2d and, Both: ▶Anticoagulant and high risk for bleeding and, One: ♦Unfractionated heparin, continuous ♦Heparin-induced thrombocytopenia, suspected or by history and, Both: ▶Warfarin (includes PO) administered or contraindicated ▶Intervention, One: ♦Argatroban ♦Bivalirudin ♦Fondaparinux ▶Neurovascular assessment every 3-4h ♦VAD requiring readmission and, ≥ One: ▶Anticoagulation, ≥ One: ♦Direct thrombin inhibitor ♦GP IIb or IIIa inhibitor ♦Heparin ♦Thrombolytic ▶Bleeding actual or suspected and, ≥ One: ♦Hct or Hb monitoring at least 3x/24h, ≤ 3d ♦Volume expander or blood product ▶Inadequate nutrition and, ≥ One: ♦Calorie count ≤ 3d ♦Requiring daily tube feeding adjustment ♦TPN initiation ≤ 24h ▶Infection actual or suspected and, Both: ♦Finding, ≥ One: ▶Temperature ≥ 100.4°F(38.0°C) ▶WBC ≥ 12,000/cu.mm(12x10⁹/L) ▶WBC ≤ 4,000/cu.mm(4x10⁹/L) ▶Bands > 10%(0.10) ▶ANC < 500/cu.mm(500x10⁶/L) ♦Anti-infective ▶Post acute facility unavailable and, ≥ One: ♦Functional impairment requiring therapy ♦Inadequate social or family support ♦Patient or caregiver unable to demonstrate functional ability to maintain and operate VAD ▶Post Critical care ≤ 3d	• INTERMEDIATE, ≥ One: ▶ Cardiovascular or peripheral vascular, ≥ One: ♦Epoprostenol, treprostinil, or iloprost (includes continuous infusion or inhaled) initiation ≤ 2d ♦Pacemaker or ICD placement performed within last 24h ♦Pericardial drain and post Critical care ≤ 24h ♦Pericardial effusion and high risk for cardiac tamponade ≤ 24h and, Both: ▶Asymptomatic ▶Effusion size ≥ 1 cm ♦Acute limb ischemia ≤ 2d and, Both: ▶Anticoagulant and high risk for bleeding and, One: ♦Unfractionated heparin, continuous ♦HIT, suspected or by history and anticoagulation ▶Neurovascular assessment every 3-4h ♦VAD requiring readmission and, ≥ One: ▶Anticoagulation, ≥ One: ♦Direct thrombin inhibitor ♦GP IIb or IIIa inhibitor ♦Heparin ♦Thrombolytic ▶Bleeding actual or suspected and, ≥ One: ♦Hct or Hb monitoring at least 3x/24h, ≤ 3d ♦Volume expander or blood product ▶Inadequate nutrition and, ≥ One: ♦Calorie count ≤ 3d ♦Requiring daily tube feeding adjustment ♦TPN initiation ≤ 24h ▶Infection actual or suspected and, Both: ♦Finding, ≥ One: ▶Temperature ≥ 100.4°F(38.0°C) ▶WBC ≥ 12,000/cu.mm(12x10⁹/L) ▶WBC ≤ 4,000/cu.mm(4x10⁹/L) ▶Bands > 10%(0.10) ▶ANC < 500/cu.mm(500x10⁶/L) ♦Anti-infective ▶Post acute facility unavailable and, ≥ One: ♦Functional impairment requiring therapy ♦Inadequate social or family support ♦Patient or caregiver unable to demonstrate functional ability to maintain and operate VAD ▶Post Critical care ≤ 3d	Rationale: • “Heparin-induced thrombocytopenia” criteria were updated to streamline the review process

General Medical		
Episode Day 2 (INTERMEDIATE)		
Previously Available Content	2024 March Release	Revision Description
N/A	• INTERMEDIATE, ≥ One: ▶ Eating disorder and, ≥ One: ♦ Hypokalemia, All: ▶ Potassium < 3.0 mEq/L(3.0 mmol/L) ▶ PVCs > 6/min ▶ Potassium repletion ♦ Seizure and, Both: ▶ Activity resolved ▶ Neurological assessment every 3-4h	Rationale: - Hypokalemia criteria were added to the Intermediate level of care as patients with eating disorders are at high risk for arrhythmia and require monitoring while their electrolytes are repleted - Seizure criteria were added to the Intermediate level of care as the 2023 APA Guidelines state seizures are an acute medical complication of severe electrolyte disturbances or hypoglycemia in patients with eating disorders

General Medical		
Episode Day 2 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
N/A	• CRITICAL, ≥ One: ▶ Eating disorder and, Both: ♦ Finding, ≥ One: ▶ Magnesium < 1.0 mg/dL(0.41 mmol/L) ▶ Phosphate < 1.0 mg/dL(0.32 mmol/L) ▶ Potassium < 3.0 mEq/L(3.0 mmol/L) and, ≥ One: ♦ Multifocal PVCs ♦ R on T phenomenon ♦ T and U wave fusion ♦ U wave ▶ Sodium > 160 mEq/L(160 mmol/L) ▶ Sodium < 120 mEq/L(120 mmol/L) and symptomatic ▶ Coma, stupor, obtundation or GCS ≤ 8 ♦ Intervention, ≥ One: ▶ CRRT ▶ Dialysis, hemodialysis or peritoneal ▶ Electrolyte repletion ▶ IV fluid ▶ Neurological assessment every 1-2h, ≤ 2d ▶ Sodium chloride 3%(0.03)	Rationale: - Magnesium, phosphate, potassium, and sodium electrolyte levels were added to the Critical level of care for patients who are admitted with eating disorders because these levels put the patient at risk for severe complications “Coma, stupor, obtundation or GCS ≤ 8” and “status epilepticus (excludes epilepsy)” were added as findings for patients admitted with eating disorders as these are indicators of severe electrolyte disturbances or hypoglycemia and require monitoring at the Critical level of care “CRRT,” “dialysis, hemodialysis or peritoneal,” “electrolyte repletion,” “IV fluid,” “neurological assessment every 1-2h,” and “sodium chloride 3%(0.03)” were added to be more inclusive of the

General Medical		
Episode Day 2 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
		interventions which may necessitate the Critical level of care for patients with eating disorders

General Medical		
Episode Day 3-X (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
• ACUTE, ≥ One: ▶ Cardiovascular or peripheral vascular, One: ♦ Responder, discharge expected today if clinically stable last 12h, One: ▶ Clinical stability, ≥ One: ♦ Pericardial effusion resolving and pain controlled or manageable ♦ Acute limb ischemia resolving, All: ▶ Limb swelling decreased ♦ Antithrombotic therapy, One: ♦ Antiplatelet regimen established ♦ Anticoagulation regimen established ♦ Warfarin with no evidence of bleeding and, One: ▶ INR ≥ 2.0 ▶ INR ≥ 2.5 and mechanical valve (excludes bioprosthetic valves) ▶ Pain controlled or manageable ▶ Peripheral pulses adequate ♦ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ▶ Pericardial drain removed ≤ 24h ▶ Acute limb ischemia ≤ 7d since onset and, ≥ One: ♦ Anticoagulation, One: ▶ Unfractionated heparin, continuous ≤ 5d ♦ Oral anticoagulation and, Both: ♦ Patient or caregiver unable to administer ♦ Outpatient services unavailable and, One: ▶ Warfarin (includes PO) and INR non-therapeutic and, Both: ♦ Finding, One: -INR < 2.0 -INR < 2.5 and mechanical valve (excludes bioprosthetic valves) and, One: -Mitral	• ACUTE, ≥ One: ▶ Cardiovascular or peripheral vascular, One: ♦ Responder, discharge expected today if clinically stable last 12h, One: ▶ Clinical stability, ≥ One: ♦ Pericardial effusion resolving and pain controlled or manageable ♦ Acute limb ischemia resolving, All: ▶ Limb swelling decreased ♦ Antithrombotic therapy, One: ♦ Antiplatelet regimen established ♦ Anticoagulation regimen established ♦ Warfarin with no evidence of bleeding and, One: ▶ INR ≥ 2.0 ▶ INR ≥ 2.5 and mechanical valve (excludes bioprosthetic valves) ▶ Pain controlled or manageable ▶ Peripheral pulses adequate ♦ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ▶ Pericardial drain removed ≤ 24h ▶ Acute limb ischemia ≤ 7d since onset and, ≥ One: ♦ Anticoagulation, One: ▶ Unfractionated heparin, continuous ≤ 5d ♦ Oral anticoagulation and, Both: ♦ Patient or caregiver unable to administer ♦ Outpatient services unavailable and, One: ▶ Warfarin (includes PO) and INR non-therapeutic and, Both: ♦ Finding, One: -INR < 2.0 -INR < 2.5 and mechanical valve (excludes bioprosthetic valves) and, One: -Mitral	Rationale: • “Heparin-induced thrombocytopenia” criteria were updated to streamline the review process

General Medical		
Episode Day 3-X (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
-Mitral and aortic ♦Unfractionated heparin, continuous ▶Heparin-induced thrombocytopenia suspected or by history and, Both: ♦Warfarin (includes PO) administered or contraindicated ♦Intervention, One: ▶Argatroban ▶Bivalirudin ▶Fondaparinux ♦Bleeding actual or suspected and, Both: ▶Finding, ≥ One: ♦Heart rate 100-120/min, sustained ♦Hct or Hb decreased within last 24h ♦Orthostatic hypotension, Both: ▶Sustained drop in blood pressure within 3 min of sitting or standing ▶Blood pressure, ≥ One: ♦Systolic drop ≥ 20 mmHg ♦Diastolic drop ≥ 10 mmHg ▶Intervention, ≥ One: ♦Blood product transfusion and anticoagulation reversed ♦Requiring Hct or Hb monitoring at least 2x/24h, $\leq 2d$ ♦Neurovascular assessment at least 4x/24h, $\leq 24h$ ▶Post Critical care $\leq 24h$ ♦Non-responder, not clinically stable for discharge and does not meet partial responder criteria, One: ▶Use Extended Stay subset ▶Refer for secondary review	-Mitral and aortic ♦Unfractionated heparin, continuous ▶HIT and anticoagulation ♦Bleeding actual or suspected and, Both: ▶Finding, ≥ One: ♦Heart rate 100-120/min, sustained ♦Hct or Hb decreased within last 24h ♦Orthostatic hypotension, Both: ▶Sustained drop in blood pressure within 3 min of sitting or standing ▶Blood pressure, ≥ One: ♦Systolic drop ≥ 20 mmHg ♦Diastolic drop ≥ 10 mmHg ▶Intervention, ≥ One: ♦Blood product transfusion and anticoagulation reversed ♦Requiring Hct or Hb monitoring at least 2x/24h, $\leq 2d$ ♦Neurovascular assessment at least 4x/24h, $\leq 24h$ ▶Post Critical care $\leq 24h$ ♦Non-responder, not clinically stable for discharge and does not meet partial responder criteria, One: ▶Use Extended Stay subset ▶Refer for secondary review	

General Medical		
Episode Day 3-X (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦ACUTE, $\geq$ One: ▶Eating disorder, One: ♦Responder, discharge expected today if clinically stable last 12h, All: ▶Blood pressure, $\geq$ One: ♦Systolic ≥ 90 mmHg or within acceptable limits ♦Diastolic ≥ 60 mmHg or within acceptable	♦ACUTE, $\geq$ One: ▶Eating disorder, One: ♦Responder, discharge expected today if clinically stable last 12h, All: ▶Vital signs within acceptable limits ▶Lab values within acceptable limits ▶Treatment regimen or nutritional route established	Rationale: • Responder criteria were updated to reflect the patients appropriate for discharge • “Blood glucose < 60 mg/dL” was added as a finding for patients with eating disorders as it is listed in the 2023 APA Guidelines as a high risk

General Medical

Episode Day 3-X (ACUTE)

Previously Available Content

- limits
- ▶Heart rate 40-110/min or within acceptable limits
- ▶Orthostatic changes resolved or within acceptable limits
- ▶Lab values within acceptable limits
- ▶Treatment regimen or nutritional route established
- ▶Weight increasing or stabilized
- ♦**Partial responder**, not clinically stable for discharge and requires continued stay, **≥ One**:
 - ▶Eating disorder and, **Both**:
 - ♦Finding, **≥ One**:
 - ▶Blood pressure, **≥ One**:
 - ♦Systolic < 90 mmHg
 - ♦Diastolic < 60 mmHg
 - ▶Cardiomyopathy and, **≥ One**:
 - ♦Cardiomegaly by imaging
 - ♦Tricuspid or mitral valve dysfunction by physical examination
 - ♦EF < 40%(0.40)
 - ▶Co-occurring medical condition and unstable lab values requiring daily monitoring
 - ▶Dehydration and, **≥ One**:
 - ♦Blood urea nitrogen (BUN) 25-45 mg/dL(8.9-16.1 mmol/L)
 - ♦Creatinine 1.5-3.0 mg/dL(132.6-265.2 µmol/L)
 - ♦Heart rate, sustained > 100/min
 - ♦Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14
 - ♦**Orthostatic hypotension, Both**:
 - ▶Sustained drop in blood pressure within 3 min of sitting or standing
 - ♦Blood pressure, **≥ One**:
 - ♦Systolic drop **≥ 20 mmHg**
 - ♦Diastolic drop **≥ 10 mmHg**
 - ♦Urine specific gravity > 1.030
 - ▶Edema or rapid weight gain
 - ▶Electrolyte imbalance, **≥ One**:
 - ♦Magnesium 1.0-1.4 mg/dL(0.41-0.58 mmol/L)
 - ♦Phosphate 1.0-2.0 mg/dL(0.32-0.65 mmol/L)
 - ♦Potassium < 3.0 mEq/L(3.0 mmol/L) without electrocardiogram (ECG) changes
 - ♦Sodium, **One**:
 - ▶151-160 mEq/L(151-160 mmol/L)
 - ▶120-129 mEq/L(120-129 mmol/L)
 - ▶Heart rate, **One**:
 - ♦< 40/min, sustained
 - ♦> 110/min, sustained
 - ▶Orthostatic hypotension, **Both**:

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- ▶Weight increasing or stabilized
- ♦**Partial responder**, not clinically stable for discharge and requires continued stay, **≥ One**:
 - ▶Eating disorder and, **Both**:
 - ♦Finding, **≥ One**:
 - ▶Blood glucose < 60 mg/dL(3.3 mmol/L)
 - ▶Blood pressure, **Both**:
 - ♦Systolic < 90 mmHg
 - ♦Diastolic < 60 mmHg
 - ▶Cardiomyopathy and, **≥ One**:
 - ♦Cardiomegaly by imaging
 - ♦Tricuspid or mitral valve dysfunction by physical examination
 - ♦EF < 40%(0.40)
 - ▶Co-occurring medical condition and unstable lab values requiring daily monitoring
 - ▶Dehydration and, **≥ One**:
 - ♦BUN 25-45 mg/dL(8.9-16.1 mmol/L)
 - ♦Creatinine 1.5-3.0 mg/dL(132.6-265.2 µmol/L)
 - ♦Heart rate sustained > 100/min
 - ♦Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14
 - ♦Urine specific gravity > 1.030
 - ▶Edema or rapid weight gain
 - ▶Electrolyte imbalance, **≥ One**:
 - ♦Magnesium 1.0-1.4 mg/dL(0.41-0.58 mmol/L)
 - ♦Phosphate 1.0-1.4 mg/dL(0.32-0.45 mmol/L)
 - ♦Potassium < 3.0 mEq/L(3.0 mmol/L) without ECG changes
 - ♦Sodium, **One**:
 - ▶151-160 mEq/L(151-160 mmol/L)
 - ▶120-129 mEq/L(120-129 mmol/L)
 - ▶Heart rate < 50/min, sustained
 - ▶Orthostatic hypotension, **Both**:
 - ♦Sustained drop in blood pressure within 3 min of sitting or standing
 - ♦Blood pressure, **≥ One**:
 - ▶Systolic drop **≥ 20 mmHg**
 - ▶Diastolic drop **≥ 10 mmHg**
 - ▶Orthostatic changes in heart rate > 30/min
 - ▶QTc interval > 450 ms
 - ▶Temperature < 96.8°F(36.0°C)
 - ▶Weight, **≥ One**:
 - ♦Continued weight loss
 - ♦Failure to stabilize weight
 - ♦Intervention, **Both**:
 - ▶Monitoring, **All**:
 - ♦Calorie count
 - ♦Daily weight
 - ♦Electrolytes monitored at least once daily
 - ♦Observation and documentation of oral or

Revision Description

- factor supporting hospitalization for patients with eating disorders
- The rule for “Blood pressure” criteria was changed to “Both” because patients admitted with an eating disorder and have both a systolic blood pressure of less than 90 mmHg and a diastolic blood pressure of less than 60 mmHg are appropriate for inpatient treatment based on the 2023 APA Guidelines
- The phosphate range for patients with eating disorders at Acute level of care was changed from “1.0-2.0 mg/dL (0.32-0.65 mmol/L)” to “1.0-1.4 mg/dL (0.32-0.45 mmol/L)” to be consistent with other content within the product as a phosphate level of greater than 1.4 mg/dL does not routinely require inpatient treatment
- The range of “< 40/min, sustained” and “> 110/min, sustained” was removed and “Heart rate < 50/min” was added as a finding for patients with eating disorders to align with the 2023 APA Guidelines as a risk factor supporting hospitalization
- Orthostatic changes in heart rate were changed from “≥ 20/min” to “> 30/min” based on the 2023 APA Guidelines
- QTc interval was updated based on the 2023 APA Guidelines
- The temperature threshold for patients with an eating disorder was changed from “< 97.0°F(36.1°C)” to “< 96.8°F(36.0°C)” based on the 2023 APA Guidelines
- The BMI criteria were changed from “BMI < 16” to “BMI < 15 kg/m²” to align with the 2023 APA Guidelines
- “≤ 75%(0.75) ideal body weight” was removed as this finding is not included in the 2023 APA Guidelines for adult patients with eating disorders
- “Weight loss ≥ 15%(0.15) in last month” was changed to “weight loss > 10%(0.10) in last 6 months or > 20%(0.20) in last year” based on the updated 2023 APA Guidelines

General Medical		
Episode Day 3-X (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Blood pressure, ≥ One: ▶Systolic drop ≥ 20 mmHg ▶Diastolic drop ≥ 10 mmHg ▶Orthostatic changes in heart rate ≥ 20/min ▶QTc interval prolonged, One: ♦> 440 ms in males ♦> 460 ms in females ▶Temperature $< 97.0^{\circ}\text{F}(36.1^{\circ}\text{C})$ ▶Weight, ≥ One: ♦Continued weight loss ♦Failure to stabilize weight ♦Intervention, Both: ▶Monitoring, All: ♦Calorie count ♦Daily weight ♦Electrolyte monitoring, One: ▶Electrolyte imbalance and electrolytes monitored at least daily ▶Electrolytes within normal range and monitoring electrolytes not required ♦Observation and documentation of oral or tube feeding pattern ▶Treatment, ≥ One: ♦Complete bed rest ♦Fluid, supplements, or medication to correct electrolyte abnormalities ♦IV fluid, One: ▶≥ 75 mL/h and, ≥ One: ♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ▶≥ 100 mL/h and weight ≥ 60 kg ♦Tube feeding or parenteral nutrition ♦Vital signs at least every 4h for medical instability ♦Weight-gain-specific treatment plan and, All: ▶Accompanied or monitored bathroom visits ▶Nutritional assessment daily ▶Re-evaluation of plan effectiveness daily ▶Supervised meals and snacks ▶Vital signs at least 3x/24h ▶Post Critical care ≤ 24h ♦Non-responder, not clinically stable for discharge and does not meet partial responder criteria, One: ▶Use Extended Stay subset ▶Refer for secondary review	- tube feeding pattern ♦Vital signs at least 3x/24h ▶Treatment, ≥ One: ♦Blood glucose monitoring at least 4x/24h ♦Complete bed rest ♦Fluid, supplements, or medication to correct electrolyte abnormalities ♦IV fluid ♦Tube feeding or parenteral nutrition ♦Weight-gain-specific treatment plan and, All: ▶Accompanied or monitored bathroom visits ▶Nutritional assessment daily ▶Re-evaluation of plan effectiveness daily ▶Supervised meals and snacks ▶Post Critical care ≤ 24h ♦Non-responder, not clinically stable for discharge and does not meet partial responder criteria, One: ▶Use Extended Stay subset ▶Refer for secondary review	• Electrolyte monitoring criteria were restructured to align with clinical practice as these patients are at high risk of electrolyte abnormalities, and their electrolyte levels are monitored at least once daily even if they were previously within normal range • “Vital signs at least 3x/24h” was moved as a criterion under “monitoring” to streamline the review process • “Blood glucose monitoring at least 4x/24h” was added as an intervention for those patients with an eating disorder who are admitted with hypoglycemia • “Vital signs at least every 4h for medical instability” was removed from criteria as this is a standard of care for patients with eating disorders who are admitted to the Acute level of care

General Medical		
Episode Day 3-X (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
► Transplant, One: ♦ Responder, discharge expected today if clinically stable last 12h, One: ► Clinical stability, ≥ One: ♦ Graft function stable and, Both: ► Rejection controlled ► Immunosuppressant drug tolerated ♦ Kidney and fluid collection resolving ♦ Liver and thrombosis resolving and, One: ► Anticoagulation regimen established or not indicated ► Warfarin with no evidence of bleeding and, One: ♦ INR ≥ 2.0 ♦ INR ≥ 2.5 and mechanical valve (excludes bioprosthetic valves) ♦ Pancreas and, ≥ One: ► Duodenal segment leak resolved ► Thrombosis resolving, One: ♦ Anticoagulation regimen established or not indicated ♦ Warfarin with no evidence of bleeding and, One: ► INR ≥ 2.0 ► INR ≥ 2.5 and mechanical valve (excludes bioprosthetic valves) ♦ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ► Graft dysfunction and immunosuppressant and, ≥ One: ♦ Conversion from IV to PO $\leq 2d$ ♦ Medication adjustment (includes PO) $\leq 3d$ ♦ New medication initiation ► Kidney and, Both: ♦ Hematoma, seroma, or lymphocele ♦ Persistent percutaneous drainage ► Liver and, Both: ♦ Finding, ≥ One: ► Hepatic vein thrombosis ► Portal vein thrombosis ♦ Intervention, ≥ One: ► Anticoagulation, One: ♦ LMWH $\leq 3d$ ♦ Unfractionated heparin $\leq 5d$ ♦ Oral anticoagulation and, Both: ► Patient or caregiver unable to administer ► Outpatient services unavailable and, One: ♦ Warfarin (includes PO) and INR non-therapeutic and, Both: - Finding, One:	► Transplant, One: ♦ Responder, discharge expected today if clinically stable last 12h, One: ► Clinical stability, ≥ One: ♦ Graft function stable and, Both: ► Rejection controlled ► Immunosuppressant drug tolerated ♦ Kidney and fluid collection resolving ♦ Liver and thrombosis resolving and, One: ► Anticoagulation regimen established or not indicated ► Warfarin with no evidence of bleeding and, One: ♦ INR ≥ 2.0 ♦ INR ≥ 2.5 and mechanical valve (excludes bioprosthetic valves) ♦ Pancreas and, ≥ One: ► Duodenal segment leak resolved ► Thrombosis resolving, One: ♦ Anticoagulation regimen established or not indicated ♦ Warfarin with no evidence of bleeding and, One: ► INR ≥ 2.0 ► INR ≥ 2.5 and mechanical valve (excludes bioprosthetic valves) ♦ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ► Graft dysfunction and immunosuppressant and, ≥ One: ♦ Conversion from IV to PO $\leq 2d$ ♦ Medication adjustment (includes PO) $\leq 3d$ ♦ New medication initiation ► Kidney and, Both: ♦ Hematoma, seroma, or lymphocele ♦ Persistent percutaneous drainage ► Liver and, Both: ♦ Finding, ≥ One: ► Hepatic vein thrombosis ► Portal vein thrombosis ♦ Intervention, ≥ One: ► Anticoagulation, One: ♦ LMWH $\leq 3d$ ♦ Unfractionated heparin $\leq 5d$ ♦ Oral anticoagulation and, Both: ► Patient or caregiver unable to administer ► Outpatient services unavailable and, One: ♦ Warfarin (includes PO) and INR non-therapeutic and, Both: - Finding, One:	Rationale: • “Heparin-induced thrombocytopenia” criteria were updated to streamline the review process

General Medical		
Episode Day 3-X (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
-INR < 2.0 -INR < 2.5 and mechanical valve (excludes bioprosthetic valves) and, One: -Mitral -Mitral and aortic -Intervention, One: -Fondaparinux -LMWH -Unfractionated heparin ♦Non-vitamin K oral anticoagulant (includes PO) ≤ 5d and, One: -LMWH -Fondaparinux -Unfractionated heparin ♦Heparin-induced thrombocytopenia suspected or by history and, Both: ▶Warfarin (includes PO) administered or contraindicated ▶Intervention, One: ♦Argatroban ♦Bivalirudin ♦Fondaparinux ▶Bleeding actual or suspected and, Both: ♦Finding, ≥ One: ▶Heart rate 100-120/min, sustained ▶Hct or Hb decreased within last 24h ▶Orthostatic hypotension, Both: ♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Blood pressure, ≥ One: -Systolic drop ≥ 20 mmHg -Diastolic drop ≥ 10 mmHg ♦Intervention, ≥ One: ▶Blood product transfusion and anticoagulation reversed ▶Requiring Hct or Hb monitoring at least 2x/24h ≤ 2d ▶Thrombectomy performed within last 24h ▶Pancreas and duodenal segment leak with percutaneous drainage ▶Pancreas and thrombosis and, ≥ One: ♦Anticoagulation, One: ▶LMWH ≤ 3d ▶Unfractionated heparin ≤ 5d ♦Oral anticoagulation and, Both: ♦Patient or caregiver unable to administer ♦Outpatient services unavailable and, One: ▶Warfarin (includes PO) and INR non-therapeutic and, Both: ♦Finding, One: -INR < 2.0 -INR < 2.5 and mechanical valve (excludes bioprosthetic valves) - and, One: -Mitral -Mitral and aortic ♦Intervention, One: -Fondaparinux	-INR < 2.0 -INR < 2.5 and mechanical valve (excludes bioprosthetic valves) and, One: -Mitral -Mitral and aortic -Intervention, One: -Fondaparinux -LMWH -Unfractionated heparin ♦Non-vitamin K oral anticoagulant (includes PO) ≤ 5d and, One: -LMWH -Fondaparinux -Unfractionated heparin ♦HIT and anticoagulation ▶Bleeding actual or suspected and, Both: ♦Finding, ≥ One: ▶Heart rate 100-120/min, sustained ▶Hct or Hb decreased within last 24h ▶Orthostatic hypotension, Both: ♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Blood pressure, ≥ One: -Systolic drop ≥ 20 mmHg -Diastolic drop ≥ 10 mmHg ♦Intervention, ≥ One: ▶Blood product transfusion and anticoagulation reversed ▶Requiring Hct or Hb monitoring at least 2x/24h ≤ 2d ▶Thrombectomy performed within last 24h ▶Pancreas and duodenal segment leak with percutaneous drainage ▶Pancreas and thrombosis and, ≥ One: ♦Anticoagulation, One: ▶LMWH ≤ 3d ▶Unfractionated heparin ≤ 5d ♦Oral anticoagulation and, Both: ♦Patient or caregiver unable to administer ♦Outpatient services unavailable and, One: ▶Warfarin (includes PO) and INR non-therapeutic and, Both: ♦Finding, One: -INR < 2.0 -INR < 2.5 and mechanical valve (excludes bioprosthetic valves) - and, One: -Mitral -Mitral and aortic ♦Intervention, One: -Fondaparinux	

General Medical		
Episode Day 3-X (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
(excludes bioprosthetic valves) and, One: -Mitral -Mitral and aortic ♦Intervention, One: -Fondaparinux -LMWH -Unfractionated heparin ▶Non-vitamin K oral anticoagulant (includes PO) ≤ 5d and, One: ♦LMWH ♦Fondaparinux ♦Unfractionated heparin ▶Heparin-induced thrombocytopenia suspected or by history and, Both: ♦Warfarin (includes PO) administered or contraindicated ♦Intervention, One: ▶Argatroban ▶Bivalirudin ▶Fondaparinux ♦Bleeding, actual or suspected and, Both: ▶Finding, ≥ One: ♦Hct < 25%(0.25) or Hb < 8.3 g/dL(83 g/L) ♦Platelet count < 60,000/cu.mm(60x10⁹/L) or > 1,000,000/cu.mm(1000x10⁹/L) ♦Heart rate 100-120/min, sustained ♦Hct or Hb decreased last 24h ♦Orthostatic hypotension, Both: ▶Sustained drop in blood pressure within 3 min of sitting or standing ▶Blood pressure, ≥ One: ♦Systolic drop ≥ 20 mmHg ♦Diastolic drop ≥ 10 mmHg ♦PT ≥ 1.5x upper limit normal (ULN) or INR ≥ 2.0 ♦PTT ≥ 1.5x upper limit normal (ULN) ▶Intervention, ≥ One: ♦Blood product transfusion and anticoagulation reversed ♦Requiring Hct or Hb monitoring at least 2x/24h, ≤ 2d ▶Post Critical care ≤ 24h ♦Non-responder, not clinically stable for discharge and does not meet partial responder criteria, One: ▶Use Extended Stay subset ▶Refer for secondary review	-LMWH -Unfractionated heparin ▶Non-vitamin K oral anticoagulant (includes PO) ≤ 5d and, One: ♦LMWH ♦Fondaparinux ♦Unfractionated heparin ▶HIT and anticoagulation ♦Bleeding, actual or suspected and, Both: ▶Finding, ≥ One: ♦Hct < 25%(0.25) or Hb < 8.3 g/dL(83 g/L) ♦Platelet count < 60,000/cu.mm(60x10⁹/L) or > 1,000,000/cu.mm(1000x10⁹/L) ♦Heart rate 100-120/min, sustained ♦Hct or Hb decreased last 24h ♦Orthostatic hypotension, Both: ▶Sustained drop in blood pressure within 3 min of sitting or standing ▶Blood pressure, ≥ One: ♦Systolic drop ≥ 20 mmHg ♦Diastolic drop ≥ 10 mmHg ♦PT ≥ 1.5x upper limit normal (ULN) or INR ≥ 2.0 ♦PTT ≥ 1.5x upper limit normal (ULN) ▶Intervention, ≥ One: ♦Blood product transfusion and anticoagulation reversed ♦Requiring Hct or Hb monitoring at least 2x/24h, ≤ 2d ▶Post Critical care ≤ 24h ♦Non-responder, not clinically stable for discharge and does not meet partial responder criteria, One: ▶Use Extended Stay subset ▶Refer for secondary review	

General Medical		
Episode Day 3-X (INTERMEDIATE)		
Previously Available Content	2024 March Release	Revision Description
N/A	• INTERMEDIATE, One: ▶ Partial responder, not clinically stable for discharge and requires continued stay, $\geq$ One: ♦ Eating disorder and, One: ♦ Hypokalemia, All: ▶ Potassium < 3.0 mEq/L(3.0 mmol/L) ▶ PVCs > 6/min ▶ Potassium repletion ♦ Seizure and, Both: ▶ Activity resolved ▶ Neurological assessment every 3-4h	Rationale: • Hypokalemia criteria were added to the Intermediate level of care as patients with eating disorders are at high risk for arrhythmia and require monitoring while their electrolytes are repleted • Seizure criteria were added to the Intermediate level of care as the 2023 APA Guidelines state seizures are an acute medical complication of severe electrolyte disturbances or hypoglycemia in patients with eating disorders

General Medical		
Episode Day 3-X (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
N/A	• CRITICAL, One: ▶ Partial Responder, not clinically stable for discharge and requires continued stay, $\geq$ One: ♦ Eating disorder and, Both: ▶ Finding, $\geq$ One: ♦ Magnesium < 1.0 mg/dL(0.41 mmol/L) ♦ Phosphate < 1.0 mg/dL(0.32 mmol/L) ♦ Potassium < 3.0 mEq/L(3.0 mmol/L) and, $\geq$ One: ▶ Multifocal PVCs ▶ R on T phenomenon ▶ T and U wave fusion ▶ U wave ♦ Sodium > 160 mEq/L(160 mmol/L) ♦ Sodium < 120 mEq/L(120 mmol/L) and symptomatic ♦ Coma, stupor, obtundation or GCS $\leq$ 8 ▶ Intervention, $\geq$ One: ♦ CRRT ♦ Dialysis, hemodialysis or peritoneal ♦ Electrolyte repletion ♦ IV fluid ♦ Neurological assessment every 1-2h $\leq$ 2d ♦ Sodium chloride 3%(0.03)	Rationale: • Magnesium, phosphate, potassium and sodium electrolyte levels were added to the Critical level of care for patients who are admitted with eating disorders because these levels put the patient at risk for severe complications • “Coma, stupor, obtundation or GCS $\leq$ 8” and “status epilepticus (excludes epilepsy)” were added as findings for patients admitted with eating disorders as these are indicators of severe electrolyte disturbances or hypoglycemia and require monitoring at the Critical level of care • “CRRT,” “dialysis, hemodialysis or peritoneal,” “electrolyte repletion,” “IV fluid,” “neurological assessment every 1-2h,” and “sodium chloride 3%(0.03)” were added to be more inclusive of the interventions which may necessitate the Critical level of care for patients with eating disorders

General Surgical



Revision Summary

The Observation level of care was added to the Pre-op Day to capture patients with a traumatic injury who are awaiting an ambulatory procedure and have severe pain requiring admission for pain management before the procedure. Traumatic injury criteria were added to the Acute level of care on Pre-op Day to capture the patients who are awaiting a designated inpatient procedure and require pain and/or IV fluid management. Post open fracture repair criteria were added to Operative and Post-op Day 1 at the Observation level of care to capture patients who have an open fracture repair procedure performed (not designated as an inpatient procedure) and require post-operative prophylactic antibiotics. Post external fixation criteria were added to the Acute level of care on Operative Day and Post-op Days 1, 2, and 3-21 to capture patients who have an open fracture stabilization procedure performed (not designated as an inpatient procedure) and require post-operative care prior to internal fixation. Frailty criteria were added as a new comorbid condition at the Acute level of care on Operative Day to identify patients who have an ambulatory procedure and may necessitate the Acute level of care. Heparin-induced thrombocytopenia criteria were updated at the Acute level of care on Post-op Days 2 and 3-21 to streamline the review process.

General Surgical		
Pre-op Day (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
N/A	•OBSERVATION, One: ▶Traumatic injury and, Both: ♦Ambulatory surgery scheduled within 24h ♦Pain, severe, Both: ▶Unresponsive to ≥ 2 doses of analgesic prior to admit decision ▶Requiring additional analgesic ≥ 2 doses after admission	Rationale: The Observation level of care and “Traumatic injury” criteria were added to capture patients with a traumatic injury who are awaiting an ambulatory procedure and have severe pain requiring pain management

General Surgical		
Pre-op Day (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
•ACUTE, One: ▶Solid organ transplant to be performed within 24h of admission ▶High risk for thromboembolism, Both: ♦Finding, ≥ One: ▶Atrial fibrillation and, ≥ One: ♦CHADS ₂ (congestive heart failure, hypertension, age years, diabetes mellitus, stroke) score > 2	•ACUTE, One: ▶Solid organ transplant to be performed within 24h of admission ▶High risk for thromboembolism, Both: ♦Finding, ≥ One: ▶Atrial fibrillation and, ≥ One: ♦CHADS ₂ (congestive heart failure, hypertension, age years, diabetes mellitus, stroke) score > 2	Rationale: “Traumatic injury” criteria were added to the Acute level of care to capture patients with a traumatic injury who are awaiting a designated inpatient procedure and require pain and/or IV fluid management

General Surgical		
Pre-op Day (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦Rheumatic valvular disease ♦Stroke or TIA within last 3 mos ▶Mechanical heart valve and, ≥ One: ♦Mitral valve ♦Older aortic valve (e.g., caged-ball, tilting disc) ♦Stroke or TIA within last 6 mos ▶Venous thromboembolism (VTE) and, ≥ One: ♦Within last 3 mos ♦Severe thrombophilia ♦Anticoagulant, One: ▶LMWH ▶Fondaparinux ▶Unfractionated heparin ▶Post cardiac catheterization and urgent cardiac surgery scheduled within 24h	♦Rheumatic valvular disease ♦Stroke or TIA within last 3 mos ▶Mechanical heart valve and, ≥ One: ♦Mitral valve ♦Older aortic valve (e.g., caged-ball, tilting disc) ♦Stroke or TIA within last 6 mos ▶Venous thromboembolism (VTE) and, ≥ One: ♦Within last 3 mos ♦Severe thrombophilia ♦Anticoagulant, One: ▶LMWH ▶Fondaparinux ▶Unfractionated heparin ▶Post cardiac catheterization and urgent cardiac surgery scheduled within 24h ▶Traumatic injury and, Both: ♦Designated inpatient surgery scheduled within 24h ♦Intervention, ≥ One: ▶IV fluid ▶Analgesic ≥ 4x/24h or continuous	

General Surgical		
Operative Day (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, ≥ One: ▶Post ambulatory surgery or procedure and complication, ≥ One: ♦Arrhythmia, All: ▶Antiarrhythmic (includes PO) ▶Continuous cardiac monitoring (excludes Holter) ▶Worsening of a previously controlled arrhythmia ♦Bleeding, ≥ One: ▶Active bleeding and, ≥ One: ♦Blood product transfusion ♦Factor replacement ♦Hct or Hb monitoring at least 2x/24h ▶Excessive intraoperative blood loss and, Both: ♦Hct or Hb monitoring at least 2x/24h ♦Intervention, ≥ One: ▶Blood product transfusion ▶IV fluid, One: ♦≥ 75 mL/h and, ≥ One: ▶Weight < 60 kg	•OBSERVATION, ≥ One: ▶Post ambulatory surgery or procedure and complication, ≥ One: ♦Arrhythmia, All: ▶Antiarrhythmic (includes PO) ▶Continuous cardiac monitoring (excludes Holter) ▶Worsening of a previously controlled arrhythmia ♦Bleeding, ≥ One: ▶Active bleeding and, ≥ One: ♦Blood product transfusion ♦Factor replacement ♦Hct or Hb monitoring at least 2x/24h ▶Excessive intraoperative blood loss and, Both: ♦Hct or Hb monitoring at least 2x/24h ♦Intervention, ≥ One: ▶Blood product transfusion ▶IV fluid	Rationale: • “Post open fracture repair and prophylactic anti-infective” criteria were added to the Observation level of care to capture patients who have an open fracture repair procedure performed (not designated as an inpatient procedure) and require post-operative prophylactic antibiotics

General Surgical		
Operative Day (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
▶Age ≥ 65 ▶Renal insufficiency ▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ▶Hematuria and, ≥ One: ♦Bladder irrigation, continuous or cyclic ♦Blood product transfusion ♦Hct or Hb monitoring at least 2x/24h ♦IV fluid, One: ▶≥ 75 mL/h and, ≥ One: ♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ▶≥ 100 mL/h and weight ≥ 60 kg ♦Delayed recovery from anesthesia and, ≥ One: ▶Assessment and monitoring at least 6x/24h and, ≥ One: ♦Breathing ♦Level of consciousness ♦Mobility ♦Sensation ▶O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ♦Inability to remove sheath post cardiac catheterization and, ≥ One: ▶ACT > 180 sec ▶PTT > 60 sec ♦Pain uncontrolled and, Both: ▶Pain assessment ▶Requiring change in medication, dose, or frequency ♦Spinal headache and, ≥ One: ▶Epidural blood patch performed within last 24h ▶IV caffeine and fluid ♦Unable to void and requiring urinary catheter ♦Vomiting, protracted and, Both: ▶Antiemetic, ≥ One: ♦Antiemetic ≥ 3x/24h ♦Serotonin antagonist ≥ 2x/24h ▶IV fluid, One: ♦≥ 75 mL/h and, ≥ One: ▶Weight < 60 kg ▶Age ≥ 65 ▶Renal insufficiency ▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ▶Post-operative functional impairment requiring rehabilitation initiation ≤ 24h	▶Hematuria and, ≥ One: ♦Bladder irrigation, continuous or cyclic ♦Blood product transfusion ♦Hct or Hb monitoring at least 2x/24h ♦IV fluid ♦Delayed recovery from anesthesia and, ≥ One: ▶Assessment and monitoring at least 6x/24h and, ≥ One: ♦Breathing ♦Level of consciousness ♦Mobility ♦Sensation ▶O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ♦Inability to remove sheath post cardiac catheterization and, ≥ One: ▶ACT > 180 sec ▶PTT > 60 sec ♦Post open fracture repair and prophylactic anti-infective ♦Pain uncontrolled and, Both: ▶Pain assessment ▶Requiring change in medication, dose, or frequency ♦Spinal headache and, ≥ One: ▶Epidural blood patch performed within last 24h ▶IV caffeine and fluid ♦Unable to void and requiring urinary catheter ♦Vomiting, protracted and, Both: ▶Antiemetic, ≥ One: ♦Antiemetic ≥ 3x/24h ♦Serotonin antagonist ≥ 2x/24h ▶IV fluid ▶Post-operative functional impairment requiring rehabilitation initiation ≤ 24h	

General Surgical		
Operative Day (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
• ACUTE, ≥ One: ► Surgery or invasive procedure and designated inpatient setting (includes kidney transplant) ► Ambulatory procedure (excludes low risk procedure) and high risk for complications requiring inpatient management, Both: ♦ Physician documentation supports the need for post procedure management for at least 2d ♦ Comorbid or high risk medical history, ≥ One: ► American Society of Anesthesiologists (ASA) class III or IV ► Severe valvular heart disease ► Ascites within 30d of surgery ► COPD (excludes asthma, diffuse interstitial fibrosis, or sarcoidosis), ≥ One: ♦ Functional disability from COPD ♦ Chronic bronchodilator therapy (includes oral or inhaled agents) ♦ FEV1 of < 75%(0.75) of predicted ► Diabetes Mellitus and HbA1c ≥ 7%(0.07) ► ESRD and chronic dialysis ► Congestive heart failure exacerbation within 30d of surgery ► Immunocompromised ► Malnutrition ► Morbid obesity (BMI > 40kg/m²) ► Coagulopathy ► High risk for thromboembolism, Both: ♦ Finding, ≥ One: ► Atrial fibrillation and, ≥ One: ♦ CHADS₂ (congestive heart failure, hypertension, age years, diabetes mellitus, stroke) score > 2 ♦ Rheumatic valvular disease ♦ Stroke or TIA within last 3 mos ► Mechanical heart valve and, ≥ One: ♦ Mitral valve ♦ Older aortic valve (e.g., caged-ball, tilting disc) ♦ Stroke or TIA within last 6 mos ► Venous thromboembolism (VTE) and, ≥ One: ♦ Within last 3 mos ♦ Severe thrombophilia ♦ Anticoagulant, One: ► LMWH ► Fondaparinux ► Unfractionated heparin	• ACUTE, ≥ One: ► Surgery and designated inpatient setting or prior authorization ► Ambulatory procedure (excludes low risk procedure) and high risk for complications requiring inpatient management, Both: ♦ Physician documentation supports the need for post procedure management for at least 2d ♦ Comorbid or high risk medical history, ≥ One: ► American Society of Anesthesiologists (ASA) class III or IV ► Severe valvular heart disease ► Ascites within 30d of surgery ► COPD (excludes asthma, diffuse interstitial fibrosis, or sarcoidosis), ≥ One: ♦ Functional disability from COPD ♦ Chronic bronchodilator therapy (includes oral or inhaled agents) ♦ FEV1 of < 75%(0.75) of predicted ► Diabetes Mellitus and HbA1c ≥ 7%(0.07) ► ESRD and chronic dialysis ► Congestive heart failure exacerbation within 30d of surgery ► Immunocompromised ► Malnutrition ► BMI ≥ 40kg/m² ► Coagulopathy ► Frailty, Both: ♦ Age ≥ 65 yrs ♦ Finding, ≥ One: ► Frailty, active diagnosis ► Frailty characteristics, ≥ Two: ♦ Chronic condition(s), ≥ One: ► Dementia or cognitive impairment ► Multimorbidity, ≥ 2 chronic illnesses ♦ Functional impairment ♦ Malnutrition ♦ Polypharmacy, ≥ 5 chronic medications ► High risk for thromboembolism, Both: ♦ Finding, ≥ One: ► Atrial fibrillation and, ≥ One: ♦ CHADS₂ (congestive heart failure, hypertension, age years, diabetes mellitus, stroke) score > 2 ♦ Rheumatic valvular disease ♦ Stroke or TIA within last 3 mos ► Mechanical heart valve and, ≥ One: ♦ Mitral valve ♦ Older aortic valve (e.g., caged-ball, tilting disc) ♦ Stroke or TIA within last 6 mos ► Venous thromboembolism (VTE) and, ≥ One: ♦ Within last 3 mos	Rationale: • “Surgery or invasive procedure and designated inpatient setting (includes kidney transplant) was changed to “Surgery and designated inpatient setting or prior authorization” to clarify the intent of the criteria • BMI criterion was updated to the new classification of obesity and the BMI value was change from “> 40 kg/m²” to “≥ 40 kg/m²” • Frailty criteria were added as a new comorbid condition to identify patients who have an ambulatory procedure and may necessitate the Acute level of care as even low-risk procedures (e.g., laparoscopic appendectomy or cholecystectomy) in frail individuals have been associated with a higher risk of postoperative complications, readmission, and all-cause mortality • “Post external fixation of open fracture awaiting internal fixation” criterion was added to capture patients who had open fracture procedure performed (not designated as an inpatient procedure) and require post-operative care prior to internal fixation

General Surgical		
Operative Day (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
	♦Severe thrombophilia ♦Anticoagulant, One: ▶LMWH ▶Fondaparinux ▶Unfractionated heparin ▶Post external fixation of open fracture awaiting internal fixation ≤ 5d	

General Surgical		
Post-op Day 1 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Functional status, One: ▶Able to perform ADLs or return to baseline ▶Functional impairment and discharge to home, One: ♦Patient or caregiver able to safely manage impairment ♦Home care services arranged ▶Functional impairment and discharge to inpatient facility for rehabilitative services ♦Pain controlled or manageable ♦Tolerating PO or nutritional route established ♦Complication or comorbidity, One: ▶No complication or active comorbidity relevant to this episode of care ▶Complication or comorbidity and clinically stable for discharge, ≥ One: ♦Arrhythmia controlled ♦Bleeding controlled or resolved ♦Passing urine without urinary retention ♦Postoperative vomiting resolved ♦Recovered from anesthesia, All: ▶Mobility and coordination at baseline ▶O₂ sat ≥ 92%(0.92) or within acceptable limits ▶Sensation intact ▶Stable level of consciousness ♦Sheath removed ▶Partial responder, not clinically stable for discharge and requires continued stay, One: ♦Post ambulatory surgery or procedure and complication requiring management, ≥ One:	•OBSERVATION, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Functional status, One: ▶Able to perform ADLs or return to baseline ▶Functional impairment and discharge to home, One: ♦Patient or caregiver able to safely manage impairment ♦Home care services arranged ▶Functional impairment and discharge to inpatient facility for rehabilitative services ♦Pain controlled or manageable ♦Tolerating PO or nutritional route established ♦Complication or comorbidity, One: ▶No complication or active comorbidity relevant to this episode of care ▶Complication or comorbidity and clinically stable for discharge, ≥ One: ♦Arrhythmia controlled ♦Bleeding controlled or resolved ♦Open fracture repair and, Both: ▶Prophylactic anti-infective completed ▶Wound without signs of infection ♦Passing urine without urinary retention ♦Postoperative vomiting resolved ♦Recovered from anesthesia, All: ▶Mobility and coordination at baseline ▶O₂ sat ≥ 92%(0.92) or within acceptable limits ▶Sensation intact ▶Stable level of consciousness ♦Sheath removed ▶Partial responder, not clinically stable for	Rationale: • “Open fracture repair” responder criteria were added to reflect the patient who is appropriate for discharge • “Post open fracture repair and prophylactic anti-infective” criterion was added to account for patients who had an open fracture repair procedure performed (not designated as an inpatient procedure) and require post-operative prophylactic antibiotics

General Surgical		
Post-op Day 1 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
▶Arrhythmia, All: ♦Antiarrhythmic (includes PO) ♦Continuous cardiac monitoring (excludes Holter) ♦Worsening of a previously controlled arrhythmia ▶Bleeding, ≥ One: ♦Active bleeding and, ≥ One: ▶Blood product transfusion ▶Factor replacement ▶Hct or Hb monitoring at least 2x/24h ♦Excessive intraoperative blood loss and, Both: ▶Hct or Hb monitoring at least 2x/24h ▶Intervention, ≥ One: ♦Blood product transfusion ♦IV fluid, One: ▶≥ 75 mL/h and, ≥ One: ♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ▶≥ 100 mL/h and weight ≥ 60 kg ♦Hematuria and, ≥ One: ▶Bladder irrigation, continuous or cyclic ▶Blood product transfusion ▶Hct or Hb monitoring at least 2x/24h ▶IV fluid, One: ♦≥ 75 mL/h and, ≥ One: ▶Weight < 60 kg ▶Age ≥ 65 ▶Renal insufficiency ▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ▶Delayed recovery from anesthesia and, ≥ One: ♦Assessment and monitoring at least 6x/24h and, ≥ One: ▶Breathing ▶Level of consciousness ▶Mobility ▶Sensation ♦O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ▶Inability to remove sheath post cardiac catheterization and, ≥ One: ♦ACT > 180 sec ♦PTT > 60 sec ▶Pain uncontrolled and, Both: ♦Pain assessment ♦Requiring change in medication, dose, or frequency ▶Spinal headache and, ≥ One: ♦Epidural blood patch performed within last 24h	discharge and requires continued stay, One: ♦Post ambulatory surgery or procedure and complication requiring management, ≥ One: ▶Arrhythmia, All: ♦Antiarrhythmic (includes PO) ♦Continuous cardiac monitoring (excludes Holter) ♦Worsening of a previously controlled arrhythmia ▶Bleeding, ≥ One: ♦Active bleeding and, ≥ One: ▶Blood product transfusion ▶Factor replacement ▶Hct or Hb monitoring at least 2x/24h ♦Excessive intraoperative blood loss and, Both: ▶Hct or Hb monitoring at least 2x/24h ▶Intervention, ≥ One: ♦Blood product transfusion ♦IV fluid ♦Hematuria and, ≥ One: ▶Bladder irrigation, continuous or cyclic ▶Blood product transfusion ▶Hct or Hb monitoring at least 2x/24h ▶IV fluid ▶Delayed recovery from anesthesia and, ≥ One: ♦Assessment and monitoring at least 6x/24h and, ≥ One: ▶Breathing ▶Level of consciousness ▶Mobility ▶Sensation ♦O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ▶Inability to remove sheath post cardiac catheterization and, ≥ One: ♦ACT > 180 sec ♦PTT > 60 sec ▶Post open fracture repair and prophylactic anti-infective ▶Pain uncontrolled and, Both: ♦Pain assessment ♦Requiring change in medication, dose, or frequency ▶Spinal headache and, ≥ One: ♦Epidural blood patch performed within last 24h ♦IV caffeine and fluid ▶Unable to void and requiring urinary catheter ▶Vomiting, protracted and, Both: ♦Antiemetic, ≥ One: ▶Antiemetic ≥ 3x/24h ▶Serotonin antagonist ≥ 2x/24h ♦IV fluid	

General Surgical		
Post-op Day 1 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
♦IV caffeine and fluid ▶Unable to void and requiring urinary catheter ▶Vomiting, protracted and, Both: ♦Antiemetic, ≥ One: ▶Antiemetic $\geq 3 \times / 24 \text{h}$ ▶Serotonin antagonist $\geq 2 \times / 24 \text{h}$ ♦IV fluid, One: ▶$\geq 75 \text{ mL/h}$ and, ≥ One: ♦Weight $< 60 \text{ kg}$ ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ▶$\geq 100 \text{ mL/h}$ and weight $\geq 60 \text{ kg}$		

General Surgical		
Post-op Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
•ACUTE, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Afebrile ♦Able to perform ADLs or return to baseline ♦Pain controlled or manageable ♦Tolerating PO or nutritional route established ♦Complication or comorbidity, One: ▶No complication or active comorbidity relevant to this episode of care ▶Complication or comorbidity and clinically stable for discharge, ≥ One: ♦Bleeding controlled or resolved ♦Chest tube removed or Heimlich valve ♦Electrolytes within acceptable limits ♦Heart failure resolving ♦Passing urine without urinary retention ♦O₂ sat $\geq 92\%(0.92)$ or within acceptable limits ♦VAD, established ▶Partial responder, not clinically stable for discharge and requires continued stay, One: ♦Post Ambulatory procedure and high risk for complications determined on Operative Day requiring post operative management at least 2d ♦Routine post-op stay, One: ▶Moderate stay within the last 5d ▶Long stay within the last 6d	•ACUTE, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Afebrile ♦Able to perform ADLs or return to baseline ♦Pain controlled or manageable ♦Tolerating PO or nutritional route established ♦Complication or comorbidity, One: ▶No complication or active comorbidity relevant to this episode of care ▶Complication or comorbidity and clinically stable for discharge, ≥ One: ♦Bleeding controlled or resolved ♦Chest tube removed or Heimlich valve ♦Electrolytes within acceptable limits ♦Heart failure resolving ♦Passing urine without urinary retention ♦O₂ sat $\geq 92\%(0.92)$ or within acceptable limits ♦VAD, established ▶Partial responder, not clinically stable for discharge and requires continued stay, One: ♦Post Ambulatory procedure and high risk for complications determined on Operative Day requiring post operative management at least 2d ♦Routine post-op stay, One: ▶Moderate stay within the last 5d ▶Long stay within the last 6d	Rationale: • “Post external fixation of open fracture awaiting internal fixation” criterion was added to capture patients who had open fracture stabilization procedure performed (not designated as an inpatient procedure) and require post-operative care prior to internal fixation

General Surgical		
Post-op Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
▶Solid organ transplant, ≥ One: ♦Heart within the last 11d ♦Heart-lung within the last 15d ♦Kidney within the last 6d ♦Liver within the last 11d ♦Lung within the last 13d ♦Pancreas within the last 8d ♦Small bowel within the last 18d ♦Post surgery or invasive procedure and complication, ≥ One: ▶Bleeding actual or suspected and, Both: ♦Finding, ≥ One: ▶Heart rate 100-120/min, sustained ▶Hct or Hb decreased within last 24h ▶Orthostatic hypotension, Both: ♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Blood pressure, ≥ One: ▶Systolic drop ≥ 20 mmHg ▶Diastolic drop ≥ 10 mmHg ♦Intervention, ≥ One: ▶Blood product transfusion ▶Return to OR within last 24h ▶Requiring Hct or Hb monitoring at least 2x/24h, $\leq 2d$ ▶Chest tube and, ≥ One: ♦Suction, continuous ♦Drainage ≥ 250 mL/24h ♦Repositioning within last 24h ♦Water seal within last 24h ▶Heart failure and, Both: ♦Dyspnea $>$ baseline and, ≥ One: ▶O₂ sat $\leq 91\%(0.91)$ and $<$ baseline ▶Worsening edema ▶Heart failure confirmed by imaging ♦Intervention, ≥ One: ▶Diuretic, One: ♦$\geq 2x/24h$ (includes PO) ♦At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: ▶Blood urea nitrogen (BUN) increased from baseline ▶Creatinine increased from baseline ▶Dialysis ▶Hypercalcemia, Both: ♦Calcium 11.1-13.9 mg/dL(2.78-3.48 mmol/L) ♦Intervention, ≥ One: ▶Bisphosphonate ▶Corticosteroid (includes PO) ▶Saline hydration, One: ♦≥ 75 mL/h and, ≥ One: ▶Weight < 60 kg ▶Age ≥ 65	▶Post external fixation of open fracture awaiting internal fixation $\leq 5d$ ▶Solid organ transplant, ≥ One: ♦Heart within the last 11d ♦Heart-lung within the last 15d ♦Kidney within the last 6d ♦Liver within the last 11d ♦Lung within the last 13d ♦Pancreas within the last 8d ♦Small bowel within the last 18d ♦Post surgery or invasive procedure and complication, ≥ One: ▶Bleeding actual or suspected and, Both: ♦Finding, ≥ One: ▶Heart rate 100-120/min, sustained ▶Hct or Hb decreased within last 24h ▶Orthostatic hypotension, Both: ♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Blood pressure, ≥ One: ▶Systolic drop ≥ 20 mmHg ▶Diastolic drop ≥ 10 mmHg ♦Intervention, ≥ One: ▶Blood product transfusion ▶Return to OR within last 24h ▶Requiring Hct or Hb monitoring at least 2x/24h, $\leq 2d$ ▶Chest tube and, ≥ One: ♦Suction, continuous ♦Drainage ≥ 250 mL/24h ♦Repositioning within last 24h ♦Water seal within last 24h ▶Heart failure and, Both: ♦Dyspnea $>$ baseline and, ≥ One: ▶O₂ sat $\leq 91\%(0.91)$ and $<$ baseline ▶Worsening edema ▶Heart failure confirmed by imaging ♦Intervention, ≥ One: ▶Diuretic, One: ♦$\geq 2x/24h$ (includes PO) ♦At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: ▶Blood urea nitrogen (BUN) increased from baseline ▶Creatinine increased from baseline ▶Dialysis ▶Hypercalcemia, Both: ♦Calcium 11.1-13.9 mg/dL(2.78-3.48 mmol/L) ♦Intervention, ≥ One: ▶Bisphosphonate ▶Corticosteroid (includes PO) ▶Saline hydration ▶Hypocalcemia, Both:	

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Post-op Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
▶Renal insufficiency ▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ▶Hypocalcemia, Both: ♦Calcium 5.0-7.4 mg/dL(1.25-1.85 mmol/L) ♦Calcium repletion ▶Hyperkalemia, All: ♦Potassium > 5.5 mEq/L(5.5 mmol/L) ♦No electrocardiogram (ECG) changes ♦Intervention, ≥ One: ▶Calcium chloride or gluconate ▶Diuretic (includes PO) ▶Glucose 50%(0.50) with insulin ▶IV fluid ▶Potassium binding agent (includes PO or PR) ▶Hypokalemia, All: ♦Potassium < 3.0 mEq/L(3.0 mmol/L) ♦No electrocardiogram (ECG) changes ♦Potassium repletion (includes PO) ♦Intervention, ≥ One: ▶Calcium chloride ▶Calcium gluconate ▶Diuretic (includes PO) ▶IV fluid, One: ♦≥ 75 mL/h and, ≥ One: ▶Weight < 60 kg ▶Age ≥ 65 ▶Renal insufficiency ▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ▶Sodium polystyrene (Kayexalate) (Includes PO or PR) ▶Hypokalemia, All: ♦Potassium < 3.0 mEq/L(3.0 mmol/L) ♦No electrocardiogram (ECG) changes ♦Potassium repletion (includes PO) ▶Hypermagnesemia, Both: ♦Magnesium > 2.5 mg/dL(1.03 mmol/L) ♦Intervention, ≥ One: ▶Diuretic ≥ 2x/24h (includes PO) ▶IV fluid, One: ♦≥ 75 mL/h and, ≥ One: ▶Weight < 60 kg ▶Age ≥ 65 ▶Renal insufficiency ▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ▶Hypomagnesemia, Both: ♦Magnesium 1.0-1.4 mg/dL(0.41-0.58 mmol/L) ♦Magnesium repletion ▶Hypernatremia, Both: ♦Sodium 151-160 mEq/L(151-160 mmol/L) ♦IV fluid ▶Hyponatremia or SIADH, Both: ♦Sodium 120-129 mEq/L(120-129 mmol/L) ♦Intervention, ≥ One: ▶Diuretic and oral sodium supplement ▶Fluid restriction ▶Medication administration, ≥ One: ♦Demeclocycline (includes PO) ♦Diuretic ≥ 2x/24h ▶Sodium chloride 0.9%(0.009) ▶Hypophosphatemia, Both: ♦Phosphate 1.0-2.0 mg/dL(0.32-0.65 mmol/L) ♦Phosphate repletion (includes PO) ▶Ileus and, One: ♦Resolved and initiating PO or tube feeding ≤ 2d ♦Unresolved and IV fluid ≤ 5d ▶Inability to void requiring urinary catheter ▶Inadequate oral intake and, ≥ One: ♦IV fluid ♦Advancing diet as tolerated ▶Pain uncontrolled and, Both: ♦Pain assessment ♦Requiring change in medication, dose, or frequency ▶Respiratory impairment (including pneumonia) and, Both:	♦Calcium 5.0-7.4 mg/dL(1.25-1.85 mmol/L) ♦Calcium repletion ▶Hyperkalemia, All: ♦Potassium > 5.5 mEq/L(5.5 mmol/L) ♦No electrocardiogram (ECG) changes ♦Intervention, ≥ One: ▶Calcium chloride or gluconate ▶Diuretic (includes PO) ▶Glucose 50%(0.50) with insulin ▶IV fluid ▶Potassium binding agent (includes PO or PR) ▶Hypokalemia, All: ♦Potassium < 3.0 mEq/L(3.0 mmol/L) ♦No electrocardiogram (ECG) changes ♦Potassium repletion (includes PO) ▶Hypermagnesemia, Both: ♦Magnesium > 2.5 mg/dL(1.03 mmol/L) ♦Intervention, ≥ One: ▶Diuretic ≥ 2x/24h (includes PO) ▶IV fluid ▶Hypomagnesemia, Both: ♦Magnesium 1.0-1.4 mg/dL(0.41-0.58 mmol/L) ♦Magnesium repletion ▶Hypernatremia, Both: ♦Sodium 151-160 mEq/L(151-160 mmol/L) ♦IV fluid ▶Hyponatremia or SIADH, Both: ♦Sodium 120-129 mEq/L(120-129 mmol/L) ♦Intervention, ≥ One: ▶Diuretic and oral sodium supplement ▶Fluid restriction ▶Medication administration, ≥ One: ♦Demeclocycline (includes PO) ♦Diuretic ≥ 2x/24h ▶Sodium chloride 0.9%(0.009) ▶Hypophosphatemia, Both: ♦Phosphate 1.0-2.0 mg/dL(0.32-0.65 mmol/L) ♦Phosphate repletion (includes PO) ▶Ileus and, One: ♦Resolved and initiating PO or tube feeding ≤ 2d ♦Unresolved and IV fluid ≤ 5d ▶Inability to void requiring urinary catheter ▶Inadequate oral intake and, ≥ One: ♦IV fluid ♦Advancing diet as tolerated ▶Pain uncontrolled and, Both: ♦Pain assessment ♦Requiring change in medication, dose, or frequency ▶Respiratory impairment (including pneumonia) and, Both:	

General Surgical		
Post-op Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
▶Fluid restriction ▶Medication administration, ≥ One: ♦Demeclocycline (includes PO) ♦Diuretic $\geq 2x/24h$ ♦Sodium chloride 0.9%(0.009) ▶Hypophosphatemia, Both: ♦Phosphate 1.0-2.0 mg/dL(0.32-0.65 mmol/L) ♦Phosphate repletion (includes PO) ▶Ileus and, One: ♦Resolved and initiating PO or tube feeding $\leq 2d$ ♦Unresolved and IV fluid $\leq 5d$ and, One: ▶≥ 75 mL/h and, ≥ One: ♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ▶≥ 100 mL/h and weight ≥ 60 kg ▶Inability to void requiring urinary catheter ▶Inadequate oral intake and, ≥ One: ♦IV fluid, One: ▶≥ 75 mL/h and, ≥ One: ♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ▶≥ 100 mL/h and weight ≥ 60 kg ♦Advancing diet as tolerated ▶Pain uncontrolled and, Both: ♦Pain assessment ♦Requiring change in medication, dose, or frequency ▶Respiratory impairment (including pneumonia) and, Both: ♦Finding, ≥ One: ▶Temperature $\geq 100.4^{\circ}F(38.0^{\circ}C)$ ▶Inability to clear secretions $\leq 24h$ ▶O₂ sat $\leq 91\%(0.91)$ and $<$ baseline ♦Intervention, ≥ One: ▶Anti-infective (includes PO) ▶Requiring supplemental oxygen ▶Respiratory interventions or incentive spirometry at least 6x/24h ▶Spinal headache and, ≥ One: ♦Epidural blood patch performed within last 24h ♦IV caffeine and fluid ▶Post Critical care $\leq 24h$	♦Finding, ≥ One: ▶Temperature $\geq 100.4^{\circ}F(38.0^{\circ}C)$ ▶Inability to clear secretions $\leq 24h$ ▶O₂ sat $\leq 91\%(0.91)$ and $<$ baseline ♦Intervention, ≥ One: ▶Anti-infective (includes PO) ▶Requiring supplemental oxygen ▶Respiratory interventions or incentive spirometry at least 6x/24h ▶Spinal headache and, ≥ One: ♦Epidural blood patch performed within last 24h ♦IV caffeine and fluid ▶Post Critical care $\leq 24h$	

General Surgical		
Post-op Day 2 (OBSERVATION)		
Previously Available Content	2024 March Release	Revision Description
•OBSERVATION, One: ►Responder, discharge expected today if clinically stable last 12h, All: ♦Able to perform ADLs or return to baseline ♦Pain controlled or manageable ♦Tolerating PO or nutritional route established ♦Complication or comorbidity, One: ►No complication or active comorbidity relevant to this episode of care ►Complication or comorbidity and clinically stable for discharge, ≥ One: ♦Arrhythmia controlled ♦Bleeding controlled or resolved ♦Passing urine without urinary retention ♦Postoperative vomiting resolved ♦Recovered from anesthesia, All: ►Mobility and coordination at baseline ►O₂ sat ≥ 92%(0.92) or within acceptable limits ►Sensation intact ►Stable level of consciousness ♦Sheath removed ►Non-responder, not clinically stable for discharge and exceeds post-op days at the Observation level of care for this condition, One: ♦Use appropriate subset (Episode Day 1) based on clinical findings ♦Refer for secondary review	•OBSERVATION, One: ►Responder, discharge expected today if clinically stable last 12h, All: ♦Able to perform ADLs or return to baseline ♦Pain controlled or manageable ♦Tolerating PO or nutritional route established ♦Complication or comorbidity, One: ►No complication or active comorbidity relevant to this episode of care ►Complication or comorbidity and clinically stable for discharge, ≥ One: ♦Arrhythmia controlled ♦Bleeding controlled or resolved ♦Open fracture repair and, Both: ►Prophylactic anti-infective completed ►Wound without signs of infection ♦Passing urine without urinary retention ♦Postoperative vomiting resolved ♦Recovered from anesthesia, All: ►Mobility and coordination at baseline ►O₂ sat ≥ 92%(0.92) or within acceptable limits ►Sensation intact ►Stable level of consciousness ♦Sheath removed ►Non-responder, not clinically stable for discharge and exceeds post-op days at the Observation level of care for this condition, One: ♦Use appropriate subset (Episode Day 1) based on clinical findings ♦Refer for secondary review	Rationale: • “Open fracture repair” responder criteria were added to reflect the patient who is appropriate for discharge

General Surgical		
Post-op Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
•ACUTE, One: ►Responder, discharge expected today if clinically stable last 12h, All: ♦Afebrile ♦Able to perform ADLs or return to baseline ♦Pain controlled or manageable ♦Tolerating PO or nutritional route established ♦Complication or comorbidity, One: ►No complication or active comorbidity relevant to this episode of care	•ACUTE, One: ►Responder, discharge expected today if clinically stable last 12h, All: ♦Afebrile ♦Able to perform ADLs or return to baseline ♦Pain controlled or manageable ♦Tolerating PO or nutritional route established ♦Complication or comorbidity, One: ►No complication or active comorbidity relevant to this episode of care	• Rationale: • “Post external fixation of open fracture awaiting internal fixation” criterion was added to capture patients who had open fracture stabilization procedure performed (not designated as an inpatient procedure) and require post-operative care prior to internal fixation

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Post-op Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
►Complication or comorbidity and clinically stable for discharge, ≥ One: ♦AKI, ≥ One: ►Kidney function improving or within acceptable limits ►Outpatient dialysis regimen established ♦Anticoagulation regimen established ♦Bleeding controlled or resolved ♦<i>C. difficile</i> infection, symptoms resolving, and tolerating PO ♦Chest tube removed or Heimlich valve ♦Electrolytes within acceptable limits ♦Functional impairment, One: ►Discharge to home, One: ♦Patient or caregiver able to safely manage impairment ♦Home care services arranged ►Discharge to inpatient facility for rehabilitative services ♦Heart failure resolving ♦Neurological stability ♦Passing urine without urinary retention ♦O₂ sat ≥ 92%(0.92) or within acceptable limits ♦VAD, established ♦Vital signs within acceptable limits ♦Wound regimen established and adequate healing ►Partial responder, not clinically stable for discharge and requires continued stay, One: ♦Routine post-op stay, One: ►Moderate stay within the last 5d ►Long stay within the last 6d ►Solid organ transplant, ≥ One: ♦Heart within the last 11d ♦Heart-lung within the last 15d ♦Kidney within the last 6d ♦Liver within the last 11d ♦Lung within the last 13d ♦Pancreas within the last 8d ♦Small bowel within the last 18d ♦Complication or comorbid condition, ≥ One: ►Abnormal vital signs, Both: ♦Finding, ≥ One: ►Heart rate > 100/min and > baseline ►Systolic blood pressure > 140 mmHg and > baseline ♦Medication adjustment (includes PO) ►Bleeding actual or suspected and, Both: ♦Finding, ≥ One: ►Heart rate 100-120/min, sustained ►Hct or Hb decreased within last 24h ►Orthostatic hypotension, Both: ♦Sustained drop in blood pressure within 3	►Complication or comorbidity and clinically stable for discharge, ≥ One: ♦AKI, ≥ One: ►Kidney function improving or within acceptable limits ►Outpatient dialysis regimen established ♦Anticoagulation regimen established ♦Bleeding controlled or resolved ♦<i>C. difficile</i> infection, symptoms resolving, and tolerating PO ♦Chest tube removed or Heimlich valve ♦Electrolytes within acceptable limits ♦Functional impairment, One: ►Discharge to home, One: ♦Patient or caregiver able to safely manage impairment ♦Home care services arranged ►Discharge to inpatient facility for rehabilitative services ♦Heart failure resolving ♦Neurological stability ♦Passing urine without urinary retention ♦O₂ sat ≥ 92%(0.92) or within acceptable limits ♦VAD, established ♦Vital signs within acceptable limits ♦Wound regimen established and adequate healing ►Partial responder, not clinically stable for discharge and requires continued stay, One: ♦Routine post-op stay, One: ►Moderate stay within the last 5d ►Long stay within the last 6d ►Post external fixation of open fracture awaiting internal fixation ≤ 5d ►Solid organ transplant, ≥ One: ♦Heart within the last 11d ♦Heart-lung within the last 15d ♦Kidney within the last 6d ♦Liver within the last 11d ♦Lung within the last 13d ♦Pancreas within the last 8d ♦Small bowel within the last 18d ♦Complication or comorbid condition, ≥ One: ►Abnormal vital signs, Both: ♦Finding, ≥ One: ►Heart rate > 100/min and > baseline ►Systolic blood pressure > 140 mmHg and > baseline ♦Medication adjustment (includes PO) ►Bleeding actual or suspected and, Both: ♦Finding, ≥ One: ►Heart rate 100-120/min, sustained ►Hct or Hb decreased within last 24h	• “continuous” was added to UFH to clarify the intent of the criteria • “Heparin-induced thrombocytopenia” criteria were updated to streamline the review process • “Oral anticoagulation” criteria were updated to better define the patient who is unable to administer parenteral anticoagulation therapy at home and requires hospitalization and can now be found under the header “Pretreatment or bridging anticoagulation”

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Post-op Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
- min of sitting or standing ♦Blood pressure, ≥ One: ▸Systolic drop ≥ 20 mmHg ▸Diastolic drop ≥ 10 mmHg ♦Intervention, ≥ One: ▸Blood product transfusion ▸Return to OR within last 24h ▸Requiring Hct or Hb monitoring at least 2x/24h, $\leq 2d$ ▸C. <i>difficile</i> infection, actual or suspected and, Both: ♦Anti-infective (includes PO) $\leq 5d$ ♦Inadequate oral intake ▸Chest tube and, ≥ One: ♦Suction, continuous ♦Drainage ≥ 250 mL/24h ♦Repositioning within last 24h ♦Water seal within last 24h ▸Fever and anti-infective and, One: ♦Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ ▸Functional impairment (new) requiring rehabilitation initiation $\leq 24h$ and, ≥ One: ♦PT evaluation and training ♦OT evaluation and training ♦Speech-language evaluation and training ♦Cognitive evaluation and retraining ♦Swallowing evaluation and retraining ▸Graft dysfunction and immunosuppressant and, ≥ One: ♦IV ♦Conversion from IV to PO $\leq 2d$ ♦Medication adjustment (includes PO) ▸Heart failure and, Both: ♦Dyspnea $>$ baseline and, ≥ One: ▸O₂ sat $\leq 91\%(0.91)$ and $<$ baseline ▸Worsening edema ▸Heart failure confirmed by imaging ♦Intervention, ≥ One: ▸Diuretic, One: ♦$\geq 2x/24h$ (includes PO) ♦At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: ▸Blood urea nitrogen (BUN) increased from baseline ▸Creatinine increased from baseline ▸Dialysis ▸Hypercalcemia, Both: ♦Calcium 11.1-13.9 mg/dL(2.78-3.48 mmol/L) ♦Intervention, ≥ One: ▸Bisphosphonate ▸Corticosteroid (includes PO) ▸Saline hydration, One: ♦≥ 75 mL/h and, ≥ One:	▸Orthostatic hypotension, Both: ♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Blood pressure, ≥ One: ▸Systolic drop ≥ 20 mmHg ▸Diastolic drop ≥ 10 mmHg ♦Intervention, ≥ One: ▸Blood product transfusion ▸Return to OR within last 24h ▸Requiring Hct or Hb monitoring at least 2x/24h, $\leq 2d$ ▸C. <i>difficile</i> infection, actual or suspected and, Both: ♦Anti-infective (includes PO) $\leq 5d$ ♦Inadequate oral intake ▸Chest tube and, ≥ One: ♦Suction, continuous ♦Drainage ≥ 250 mL/24h ♦Repositioning within last 24h ♦Water seal within last 24h ▸Fever and anti-infective and, One: ♦Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ ▸Functional impairment (new) requiring rehabilitation initiation $\leq 24h$ and, ≥ One: ♦PT evaluation and training ♦OT evaluation and training ♦Speech-language evaluation and training ♦Cognitive evaluation and retraining ♦Swallowing evaluation and retraining ▸Graft dysfunction and immunosuppressant and, ≥ One: ♦IV ♦Conversion from IV to PO $\leq 2d$ ♦Medication adjustment (includes PO) ▸Heart failure and, Both: ♦Dyspnea $>$ baseline and, ≥ One: ▸O₂ sat $\leq 91\%(0.91)$ and $<$ baseline ▸Worsening edema ▸Heart failure confirmed by imaging ♦Intervention, ≥ One: ▸Diuretic, One: ♦$\geq 2x/24h$ (includes PO) ♦At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: ▸Blood urea nitrogen (BUN) increased from baseline ▸Creatinine increased from baseline ▸Dialysis ▸Hypercalcemia, Both: ♦Calcium 11.1-13.9 mg/dL(2.78-3.48 mmol/L) ♦Intervention, ≥ One: ▸Bisphosphonate ▸Corticosteroid (includes PO)	

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Post-op Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
▶Weight < 60 kg ▶Age ≥ 65 ▶Renal insufficiency ▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ▶Hypocalcemia, Both: ♦Calcium 5.0-7.4 mg/dL(1.25-1.85 mmol/L) ♦Calcium repletion ▶Hyperkalemia, All: ♦Potassium > 5.5 mEq/L(5.5 mmol/L) ♦No electrocardiogram (ECG) changes ♦Intervention, ≥ One: ▶Calcium chloride ▶Calcium gluconate ▶Diuretic (includes PO) ♦IV fluid, One: ♦≥ 75 mL/h and, ≥ One: ▶Weight < 60 kg ▶Age ≥ 65 ▶Renal insufficiency ▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ▶Sodium polystyrene (Kayexalate) (Includes PO or PR) ▶Hypokalemia, All: ♦Potassium < 3.0 mEq/L(3.0 mmol/L) ♦No electrocardiogram (ECG) changes ♦Potassium repletion (includes PO) ▶Hypermagnesemia, Both: ♦Magnesium > 2.5 mg/dL(1.03 mmol/L) ♦Intervention, ≥ One: ▶Diuretic ≥ 2x/24h (includes PO) ♦IV fluid, One: ♦≥ 75 mL/h and, ≥ One: ▶Weight < 60 kg ▶Age ≥ 65 ▶Renal insufficiency ▶Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ▶Hypomagnesemia, Both: ♦Magnesium 1.0-1.4 mg/dL(0.41-0.58 mmol/L) ♦Magnesium repletion ▶Hypernatremia, Both: ♦Sodium 151-160 mEq/L(151-160 mmol/L) ♦IV fluid, One: ♦≥ 75 mL/h and, ≥ One: ♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ▶Hyponatremia or SIADH, Both: ♦Sodium 120-129 mEq/L(120-129 mmol/L)	▶Saline hydration ▶Hypocalcemia, Both: ♦Calcium 5.0-7.4 mg/dL(1.25-1.85 mmol/L) ♦Calcium repletion ▶Hyperkalemia, All: ♦Potassium > 5.5 mEq/L(5.5 mmol/L) ♦No electrocardiogram (ECG) changes ♦Intervention, ≥ One: ▶Calcium chloride or gluconate ▶Diuretic (includes PO) ♦Glucose 50%(0.50) with insulin ♦IV fluid ♦Potassium binding agent (includes PO or PR) ▶Hypokalemia, All: ♦Potassium < 3.0 mEq/L(3.0 mmol/L) ♦No electrocardiogram (ECG) changes ♦Potassium repletion (includes PO) ▶Hypermagnesemia, Both: ♦Magnesium > 2.5 mg/dL(1.03 mmol/L) ♦Intervention, ≥ One: ▶Diuretic ≥ 2x/24h (includes PO) ♦IV fluid ▶Hypomagnesemia, Both: ♦Magnesium 1.0-1.4 mg/dL(0.41-0.58 mmol/L) ♦Magnesium repletion ▶Hypernatremia, Both: ♦Sodium 151-160 mEq/L(151-160 mmol/L) ♦IV fluid ▶Hyponatremia or SIADH, Both: ♦Sodium 120-129 mEq/L(120-129 mmol/L) ♦Intervention, ≥ One: ▶Diuretic and oral sodium supplement ▶Fluid restriction ▶Medication administration, ≥ One: ♦Demeclocycline (includes PO) ♦Diuretic ≥ 2x/24h ♦Sodium chloride 0.9%(0.009) ▶Hypophosphatemia, Both: ♦Phosphate 1.0-2.0 mg/dL(0.32-0.65 mmol/L) ♦Phosphate repletion (includes PO) ▶Ileus and, One: ♦Resolved and initiating PO or tube feeding ≤ 2d ♦Unresolved and IV fluid ≤ 5d ▶Immunosuppressant level abnormal or signs of toxicity and medication adjustment (includes PO) ▶Inadequate oral intake and, ≥ One: ♦IV fluid ♦Advancing diet as tolerated ▶Neurological impairment or condition and, Both: ♦Finding, ≥ One:	

General Surgical		
Post-op Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦Intervention, ≥ One: ►Diuretic and oral sodium supplement ►Fluid restriction ►Medication administration, ≥ One: ♦Demeclocycline (includes PO) ♦Diuretic ≥ 2x/24h ►Sodium chloride 0.9%(0.009) ►Hypophosphatemia, Both: ♦Phosphate 1.0-2.0 mg/dL(0.32-0.65 mmol/L) ♦Phosphate repletion (includes PO) ►Ileus and, One: ♦Resolved and initiating PO or tube feeding ≤ 2d ♦Unresolved and IV fluid ≤ 5d and, One: ►≥ 75 mL/h and, ≥ One: ♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ►≥ 100 mL/h and weight ≥ 60 kg ►Immunosuppressant level abnormal or signs of toxicity and medication adjustment (includes PO) ►Inadequate oral intake and, ≥ One: ♦IV fluid, One: ►≥ 75 mL/h and, ≥ One: ♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ►≥ 100 mL/h and weight ≥ 60 kg ♦Advancing diet as tolerated ►Neurological impairment or condition and, Both: ♦Finding, ≥ One: ►Acute onset ►Deterioration ♦Neurological assessment at least 3x/24h, ≤ 2d ►O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ►Pain uncontrolled and, Both: ♦Pain assessment ♦Requiring change in medication, dose, or frequency ►Respiratory impairment (including pneumonia) and, Both: ♦Finding, ≥ One: ►Temperature ≥ 100.4°F(38.0°C) ►Inability to clear secretions ≤ 24h ►O₂ sat ≤ 91%(0.91) and < baseline ♦Intervention, ≥ One: ►Anti-infective (includes PO) ►Requiring supplemental oxygen ►Respiratory interventions or incentive spirometry at least 6x/24h ►Spinal headache and, ≥ One: ♦Epidural blood patch performed within last 24h ♦IV caffeine and fluid ►Therapeutic anticoagulation, including DVT and, One: ♦LMWH ≤ 3d ♦UFH, continuous ≤ 5d ♦HIT, suspected or by history and anticoagulation ♦Pretreatment or bridging anticoagulation, All: ►Patient or caregiver unable to administer ►Outpatient services unavailable ♦Intervention, One: ♦INR non-therapeutic and anticoagulation (includes PO) ♦Anticoagulation ≤ 5d since initiation ►Wound, ≥ One: ♦Complex wound care ≤ 5d and, ≥ One: ►Wound care ≥ 3x/24h and, ≥ One: ♦Requiring parenteral analgesic ♦≥ 15 min in duration ►Wound management teaching ≤ 24h ♦Return to OR within last 24h ►Post Critical care ≤ 24h	►Acute onset ►Deterioration ♦Neurological assessment at least 3x/24h, ≤ 2d ►O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ►Pain uncontrolled and, Both: ♦Pain assessment ♦Requiring change in medication, dose, or frequency ►Respiratory impairment (including pneumonia) and, Both: ♦Finding, ≥ One: ►Temperature ≥ 100.4°F(38.0°C) ►Inability to clear secretions ≤ 24h ►O₂ sat ≤ 91%(0.91) and < baseline ♦Intervention, ≥ One: ►Anti-infective (includes PO) ►Requiring supplemental oxygen ►Respiratory interventions or incentive spirometry at least 6x/24h ►Spinal headache and, ≥ One: ♦Epidural blood patch performed within last 24h ♦IV caffeine and fluid ►Therapeutic anticoagulation, including DVT and, One: ♦LMWH ≤ 3d ♦UFH, continuous ≤ 5d ♦HIT, suspected or by history and anticoagulation ♦Pretreatment or bridging anticoagulation, All: ►Patient or caregiver unable to administer ►Outpatient services unavailable ♦Intervention, One: ♦INR non-therapeutic and anticoagulation (includes PO) ♦Anticoagulation ≤ 5d since initiation ►Wound, ≥ One: ♦Complex wound care ≤ 5d and, ≥ One: ►Wound care ≥ 3x/24h and, ≥ One: ♦Requiring parenteral analgesic ♦≥ 15 min in duration ►Wound management teaching ≤ 24h ♦Return to OR within last 24h ►Post Critical care ≤ 24h	►Acute onset ►Deterioration ♦Neurological assessment at least 3x/24h, ≤ 2d ►O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ►Pain uncontrolled and, Both: ♦Pain assessment ♦Requiring change in medication, dose, or frequency ►Respiratory impairment (including pneumonia) and, Both: ♦Finding, ≥ One: ►Temperature ≥ 100.4°F(38.0°C) ►Inability to clear secretions ≤ 24h ►O₂ sat ≤ 91%(0.91) and < baseline ♦Intervention, ≥ One: ►Anti-infective (includes PO) ►Requiring supplemental oxygen ►Respiratory interventions or incentive spirometry at least 6x/24h ►Spinal headache and, ≥ One: ♦Epidural blood patch performed within last 24h ♦IV caffeine and fluid ►Therapeutic anticoagulation, including DVT and, One: ♦LMWH ≤ 3d ♦UFH, continuous ≤ 5d ♦HIT, suspected or by history and anticoagulation ♦Pretreatment or bridging anticoagulation, All: ►Patient or caregiver unable to administer ►Outpatient services unavailable ♦Intervention, One: ♦INR non-therapeutic and anticoagulation (includes PO) ♦Anticoagulation ≤ 5d since initiation ►Wound, ≥ One: ♦Complex wound care ≤ 5d and, ≥ One: ►Wound care ≥ 3x/24h and, ≥ One: ♦Requiring parenteral analgesic ♦≥ 15 min in duration ►Wound management teaching ≤ 24h ♦Return to OR within last 24h ►Post Critical care ≤ 24h

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Post-op Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
- spirometry at least 6x/24h ▶Spinal headache and, ≥ One: ♦Epidural blood patch performed within last 24h ♦IV caffeine and fluid ▶Therapeutic anticoagulation, including DVT and, One: ♦LMWH ≤ 3d ♦Unfractionated heparin ≤ 5d ♦Oral anticoagulation and, All: ▶Patient or caregiver unable to administer ▶Outpatient services unavailable and, One: ♦Warfarin (includes PO) and INR non-therapeutic ♦Dabigatran or edoxaban (includes PO) ≤ 5d ▶Intervention, One: ♦Fondaparinux ♦LMWH ♦Unfractionated heparin ♦Heparin-induced thrombocytopenia suspected or by history and, Both: ▶Warfarin (includes PO), administered or contraindicated ▶Intervention, One: ♦Argatroban ♦Bivalirudin ♦Fondaparinux ▶Wound, ≥ One: ♦Complex wound care ≤ 5d and, ≥ One: ▶Wound care ≥ 3x/24h and, ≥ One: ♦Requiring parenteral analgesic ♦≥ 15 min in duration ▶Wound management teaching ≤ 24h ♦Return to OR within last 24h ▶Post Critical care ≤ 24h		

General Surgical		
Post-op Day 3-21 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
•ACUTE, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Afebrile ♦Able to perform ADLs or return to baseline	•ACUTE, One: ▶Responder, discharge expected today if clinically stable last 12h, All: ♦Afebrile ♦Able to perform ADLs or return to baseline	Rationale: • “Post external fixation of open fracture awaiting internal fixation” criterion was added to capture patients who had open fracture stabilization

General Surgical		
Post-op Day 3-21 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦Pain controlled or manageable ♦Tolerating PO or nutritional route established ♦Complication or comorbidity, One: ▶No complication or active comorbidity relevant to this episode of care ▶Complication or comorbidity and clinically stable for discharge, ≥ One: ♦AKI, ≥ One: ▶Kidney function improving or within acceptable limits ▶Outpatient dialysis regimen established ♦Anticoagulation regimen established ♦Bleeding controlled or resolved ♦<i>C. difficile</i> infection, symptoms resolving, and tolerating PO ♦Chest tube removed or Heimlich valve ♦Electrolytes within acceptable limits ♦Functional impairment, One: ▶Discharge to home, One: ♦Patient or caregiver able to safely manage impairment ♦Home care services arranged ▶Discharge to inpatient facility for rehabilitative services ♦Graft function established and stable ♦Heart failure resolving ♦Immunosuppressant regimen tolerated and levels within acceptable limits ♦Neurological stability ♦Passing urine without urinary retention ♦O₂ sat ≥ 92%(0.92) or within acceptable limits ♦Vital signs within acceptable limits ♦Wound regimen established and adequate healing ▶Partial responder, not clinically stable for discharge and requires continued stay, One: ♦Routine post-op stay, One: ▶Moderate stay within the last 5d ▶Long stay within the last 6d ▶Solid organ transplant, ≥ One: ♦Heart within the last 11d ♦Heart-lung within the last 15d ♦Kidney within the last 6d ♦Liver within the last 11d ♦Lung within the last 13d ♦Pancreas within the last 8d ♦Small bowel within the last 18d ♦Complication or comorbid condition, ≥ One: ▶Abnormal vital signs, Both: ♦Finding, ≥ One: ▶Heart rate > 100/min and > baseline ▶Systolic blood pressure > 140 mmHg and > baseline	♦Pain controlled or manageable ♦Tolerating PO or nutritional route established ♦Complication or comorbidity, One: ▶No complication or active comorbidity relevant to this episode of care ▶Complication or comorbidity and clinically stable for discharge, ≥ One: ♦AKI, ≥ One: ▶Kidney function improving or within acceptable limits ▶Outpatient dialysis regimen established ♦Anticoagulation regimen established ♦Bleeding controlled or resolved ♦<i>C. difficile</i> infection, symptoms resolving, and tolerating PO ♦Chest tube removed or Heimlich valve ♦Electrolytes within acceptable limits ♦Functional impairment, One: ▶Discharge to home, One: ♦Patient or caregiver able to safely manage impairment ♦Home care services arranged ▶Discharge to inpatient facility for rehabilitative services ♦Graft function established and stable ♦Heart failure resolving ♦Immunosuppressant regimen tolerated and levels within acceptable limits ♦Neurological stability ♦Passing urine without urinary retention ♦O₂ sat ≥ 92%(0.92) or within acceptable limits ♦Vital signs within acceptable limits ♦Wound regimen established and adequate healing ▶Partial responder, not clinically stable for discharge and requires continued stay, One: ♦Routine post-op stay, One: ▶Moderate stay within the last 5d ▶Long stay within the last 6d ▶Post external fixation of open fracture awaiting internal fixation ≤ 5d ▶Solid organ transplant, ≥ One: ♦Heart within the last 11d ♦Heart-lung within the last 15d ♦Kidney within the last 6d ♦Liver within the last 11d ♦Lung within the last 13d ♦Pancreas within the last 8d ♦Small bowel within the last 18d ♦Complication or comorbid condition, ≥ One: ▶Abnormal vital signs, Both: ♦Finding, ≥ One: ▶Heart rate > 100/min and > baseline	procedure performed (not designated as an inpatient procedure) and require post-operative care prior to internal fixation • “continuous” was added to UFH to clarify the intent of the criteria • “Heparin-induced thrombocytopenia” criteria were updated to streamline the review process • “Oral anticoagulation” criteria were updated to better define the patient who is unable to administer parenteral anticoagulation therapy at home and requires hospitalization and can now be found under the header “Pretreatment or bridging anticoagulation”

General Surgical		
Post-op Day 3-21 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦Medication adjustment (includes PO) ▶Bleeding actual or suspected and, Both: ♦Finding, ≥ One: ▶Heart rate 100-120/min, sustained ▶Hct or Hb decreased within last 24h ▶Orthostatic hypotension, Both: ♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Blood pressure, ≥ One: ▶Systolic drop ≥ 20 mmHg ▶Diastolic drop ≥ 10 mmHg ♦Intervention, ≥ One: ▶Blood product transfusion ▶Return to OR within last 24h ▶Requiring Hct or Hb monitoring at least 2x/24h, $\leq 2d$ ▶C. <i>difficile</i> infection, actual or suspected and, Both: ♦Anti-infective (includes PO) $\leq 5d$ ♦Inadequate oral intake ▶Chest tube and, ≥ One: ♦Suction, continuous ♦Drainage ≥ 250 mL/24h ♦Repositioning within last 24h ♦Water seal within last 24h ▶Fever and anti-infective and, One: ♦Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ ▶Fever, new onset and, Both: ♦Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ ♦Intervention, ≥ One: ▶Culture pending $\leq 2d$ ▶Drug-induced fever suspected and precipitating drug discontinued $\leq 3d$ ▶Imaging study $\leq 24h$ ▶Graft dysfunction and immunosuppressant and, ≥ One: ♦IV ♦Conversion from IV to PO $\leq 2d$ ♦Medication adjustment (includes PO) ▶Heart failure and, Both: ♦Dyspnea $>$ baseline and, ≥ One: ▶O₂ sat $\leq 91\%(0.91)$ and $<$ baseline ▶Worsening edema ▶Heart failure confirmed by imaging ♦Intervention, ≥ One: ▶Diuretic, One: ♦$\geq 2x/24h$ (includes PO) ♦At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: ▶Blood urea nitrogen (BUN) increased from baseline ▶Creatinine increased from baseline ▶Dialysis	▶Systolic blood pressure > 140 mmHg and $>$ baseline ♦Medication adjustment (includes PO) ▶Bleeding actual or suspected and, Both: ♦Finding, ≥ One: ▶Heart rate 100-120/min, sustained ▶Hct or Hb decreased within last 24h ▶Orthostatic hypotension, Both: ♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Blood pressure, ≥ One: ▶Systolic drop ≥ 20 mmHg ▶Diastolic drop ≥ 10 mmHg ♦Intervention, ≥ One: ▶Blood product transfusion ▶Return to OR within last 24h ▶Requiring Hct or Hb monitoring at least 2x/24h, $\leq 2d$ ▶C. <i>difficile</i> infection, actual or suspected and, Both: ♦Anti-infective (includes PO) $\leq 5d$ ♦Inadequate oral intake ▶Chest tube and, ≥ One: ♦Suction, continuous ♦Drainage ≥ 250 mL/24h ♦Repositioning within last 24h ♦Water seal within last 24h ▶Fever and anti-infective and, One: ♦Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ ▶Fever, new onset and, Both: ♦Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ ♦Intervention, ≥ One: ▶Culture pending $\leq 2d$ ▶Drug-induced fever suspected and precipitating drug discontinued $\leq 3d$ ▶Imaging study $\leq 24h$ ▶Graft dysfunction and immunosuppressant and, ≥ One: ♦IV ♦Conversion from IV to PO $\leq 2d$ ♦Medication adjustment (includes PO) ▶Heart failure and, Both: ♦Dyspnea $>$ baseline and, ≥ One: ▶O₂ sat $\leq 91\%(0.91)$ and $<$ baseline ▶Worsening edema ▶Heart failure confirmed by imaging ♦Intervention, ≥ One: ▶Diuretic, One: ♦$\geq 2x/24h$ (includes PO) ♦At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: ▶Blood urea nitrogen (BUN) increased from baseline	

General Surgical		
Post-op Day 3-21 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
►Hypercalcemia, Both: ♦Calcium 11.1-13.9 mg/dL(2.78-3.48 mmol/L) ♦Intervention, ≥ One: ►Bisphosphonate ►Corticosteroid (includes PO) ►Saline hydration, One: ♦≥ 75 mL/h and, ≥ One: ►Weight < 60 kg ►Age ≥ 65 ►Renal insufficiency ►Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ►Hypocalcemia, Both: ♦Calcium 5.0-7.4 mg/dL(1.25-1.85 mmol/L) ♦Calcium repletion ►Hyperkalemia, All: ♦Potassium > 5.5 mEq/L(5.5 mmol/L) ♦No electrocardiogram (ECG) changes ♦Intervention, ≥ One: ►Calcium chloride or gluconate ►Diuretic (includes PO) ►IV fluid, One: ♦≥ 75 mL/h and, ≥ One: ►Weight < 60 kg ►Age ≥ 65 ►Renal insufficiency ►Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ►Sodium polystyrene (Kayexalate) (Includes PO or PR) ►Hypokalemia, All: ♦Potassium < 3.0 mEq/L(3.0 mmol/L) ♦No electrocardiogram (ECG) changes ♦Potassium repletion (includes PO) ►Hypermagnesemia, Both: ♦Magnesium > 2.5 mg/dL(1.03 mmol/L) ♦Intervention, ≥ One: ►Diuretic ≥ 2x/24h (includes PO) ►IV fluid, One: ♦≥ 75 mL/h and, ≥ One: ►Weight < 60 kg ►Age ≥ 65 ►Renal insufficiency ►Heart failure, chronic ♦≥ 100 mL/h and weight ≥ 60 kg ►Hypomagnesemia, Both: ♦Magnesium 1.0-1.4 mg/dL(0.41-0.58 mmol/L) ♦Magnesium repletion ►Hypernatremia, Both: ♦Sodium 151-160 mEq/L(151-160 mmol/L) ♦IV fluid, One: ♦≥ 75 mL/h and, ≥ One:	►Creatinine increased from baseline ►Dialysis ►Hypercalcemia, Both: ♦Calcium 11.1-13.9 mg/dL(2.78-3.48 mmol/L) ♦Intervention, ≥ One: ►Bisphosphonate ►Corticosteroid (includes PO) ►Saline hydration ►Hypocalcemia, Both: ♦Calcium 5.0-7.4 mg/dL(1.25-1.85 mmol/L) ♦Calcium repletion ►Hyperkalemia, All: ♦Potassium > 5.5 mEq/L(5.5 mmol/L) ♦No electrocardiogram (ECG) changes ♦Intervention, ≥ One: ►Calcium chloride or gluconate ►Diuretic (includes PO) ►Glucose 50%(0.50) with insulin ►IV fluid ►Potassium binding agent (includes PO or PR) ►Hypokalemia, All: ♦Potassium < 3.0 mEq/L(3.0 mmol/L) ♦No electrocardiogram (ECG) changes ♦Potassium repletion (includes PO) ►Hypermagnesemia, Both: ♦Magnesium > 2.5 mg/dL(1.03 mmol/L) ♦Intervention, ≥ One: ►Diuretic ≥ 2x/24h (includes PO) ►IV fluid ►Hypomagnesemia, Both: ♦Magnesium 1.0-1.4 mg/dL(0.41-0.58 mmol/L) ♦Magnesium repletion ►Hypernatremia, Both: ♦Sodium 151-160 mEq/L(151-160 mmol/L) ♦IV fluid ►Hyponatremia or SIADH, Both: ♦Sodium 120-129 mEq/L(120-129 mmol/L) ♦Intervention, ≥ One: ►Diuretic and oral sodium supplement ►Fluid restriction ►Medication administration, ≥ One: ♦Demeclocycline (includes PO) ♦Diuretic ≥ 2x/24h ►Sodium chloride 0.9%(0.009) ►Hypophosphatemia, Both: ♦Phosphate 1.0-2.0 mg/dL(0.32-0.65 mmol/L) ♦Phosphate repletion (includes PO) ►Ileus and, One: ♦Resolved and initiating PO or tube feeding ≤ 2d ♦Unresolved and IV fluid ≤ 5d ►Functional impairment (new) requiring	

General Surgical		
Post-op Day 3-21 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ▶≥ 100 mL/h and weight ≥ 60 kg ▶Hyponatremia or SIADH, Both: ♦Sodium 120-129 mEq/L(120-129 mmol/L) ♦Intervention, ≥ One: ▶Diuretic and oral sodium supplement ▶Fluid restriction ▶Medication administration, ≥ One: ♦Demeclocycline (includes PO) ♦Diuretic ≥ 2x/24h ▶Sodium chloride 0.9%(0.009) ▶Hypophosphatemia, Both: ♦Phosphate 1.0-2.0 mg/dL(0.32-0.65 mmol/L) ♦Phosphate repletion (includes PO) ▶Ileus and, One: ♦Resolved and initiating PO or tube feeding ≤ 2d ♦Unresolved and IV fluid ≤ 5d and, One: ▶≥ 75 mL/h and, ≥ One: ♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ▶≥ 100 mL/h and weight ≥ 60 kg ▶Functional impairment (new) requiring rehabilitation initiation ≤ 24h and, ≥ One: ♦PT evaluation and training ♦OT evaluation and training ♦Speech-language evaluation and training ♦Cognitive evaluation and retraining ♦Swallowing evaluation and retraining ▶Immunosuppressant level abnormal or signs of toxicity and medication adjustment (includes PO) ▶Inadequate oral intake and, ≥ One: ♦IV fluid, One: ▶≥ 75 mL/h and, ≥ One: ♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ▶≥ 100 mL/h and weight ≥ 60 kg ♦Advancing diet as tolerated ▶Neurological impairment or condition and, Both: ♦Finding, ≥ One: ▶Acute onset ▶Deterioration ♦Neurological assessment at least 3x/24h, ≤ 2d ▶O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen	- rehabilitation initiation ≤ 24h and, ≥ One: ♦PT evaluation and training ♦OT evaluation and training ♦Speech-language evaluation and training ♦Cognitive evaluation and retraining ♦Swallowing evaluation and retraining ▶Immunosuppressant level abnormal or signs of toxicity and medication adjustment (includes PO) ▶Inadequate oral intake and, ≥ One: ♦IV fluid ♦Advancing diet as tolerated ▶Neurological impairment or condition and, Both: ♦Finding, ≥ One: ▶Acute onset ▶Deterioration ♦Neurological assessment at least 3x/24h, ≤ 2d ▶O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ▶Pain uncontrolled and, Both: ♦Pain assessment ♦Requiring change in medication, dose, or frequency ▶Respiratory impairment (including pneumonia) and, Both: ♦Finding, ≥ One: ▶Temperature ≥ 100.4°F(38.0°C) ▶Inability to clear secretions ≤ 24h ▶O₂ sat ≤ 91%(0.91) and < baseline ♦Intervention, ≥ One: ▶Anti-infective (includes PO) ▶Requiring supplemental oxygen ▶Respiratory interventions or incentive spirometry at least 6x/24h ▶Spinal headache and, ≥ One: ♦Epidural blood patch performed within last 24h ♦IV caffeine and fluid ▶Therapeutic anticoagulation, including DVT and, One: ♦LMWH ≤ 3d ♦UFH, continuous ≤ 5d ♦HIT and anticoagulation ♦Pretreatment or bridging anticoagulation, All: ▶Patient or caregiver unable to administer ▶Outpatient services unavailable ♦Intervention, One: ♦INR non-therapeutic and anticoagulation (includes PO) ♦Anticoagulation ≤ 5d since initiation ▶Wound, ≥ One: ♦Complex wound care ≤ 5d and, ≥ One: ▶Wound care ≥ 3x/24h and, ≥ One:	

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Post-op Day 3-21 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
▶Pain uncontrolled and, Both: ♦Pain assessment ♦Requiring change in medication, dose, or frequency ▶Respiratory impairment (including pneumonia) and, Both: ♦Finding, ≥ One: ▶Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ ▶Inability to clear secretions $\leq 24\text{h}$ ▶O₂ sat $\leq 91\%(0.91)$ and $<$ baseline ♦Intervention, ≥ One: ▶Anti-infective (includes PO) ▶Requiring supplemental oxygen ▶Respiratory interventions or incentive spirometry at least 6x/24h ▶Spinal headache and, ≥ One: ♦Epidural blood patch performed within last 24h ♦IV caffeine and fluid ▶Therapeutic anticoagulation, including DVT and, One: ♦LMWH $\leq 3\text{d}$ ♦Unfractionated heparin $\leq 5\text{d}$ ♦Oral anticoagulation and, All: ▶Patient or caregiver unable to administer ▶Outpatient services unavailable and, One: ♦Warfarin (includes PO) and INR non-therapeutic ♦Dabigatran or edoxaban (includes PO) $\leq 5\text{d}$ ▶Intervention, One: ♦Fondaparinux ♦LMWH ♦Unfractionated heparin ♦Heparin-induced thrombocytopenia suspected or by history and, Both: ▶Warfarin (includes PO), administered or contraindicated ▶Intervention, One: ♦Argatroban ♦Bivalirudin ♦Fondaparinux ▶Wound, ≥ One: ♦Complex wound care $\leq 5\text{d}$ and, ≥ One: ▶Wound care $\geq 3\text{x}/24\text{h}$ and, ≥ One: ♦Requiring parenteral analgesic ♦$\geq 15\text{ min}$ in duration ▶Wound management teaching $\leq 24\text{h}$ ♦Return to OR within last 24h ▶Post Critical care $\leq 24\text{h}$	♦Requiring parenteral analgesic ♦$\geq 15\text{ min}$ in duration ▶Wound management teaching $\leq 24\text{h}$ ♦Return to OR within last 24h ▶Post Critical care $\leq 24\text{h}$	

General Trauma



Revision Summary

The General Trauma subset underwent significant changes, including modifications to the workflow, criteria structure, and content. The following workflow changes were made:

- Patients awaiting an ambulatory surgical procedure who require hospitalization prior to the procedure for severe pain may now be reviewed at the Observation level of care in the General Surgical subset
- Patients awaiting a designated inpatient surgical procedure who require pain and/or fluid management may now be reviewed at the Acute level of care in the General Surgical subset
- Criteria have been incorporated in the General Trauma subset to address injuries that require staged operative procedures in addition to medical management (e.g., burns) or require an ambulatory procedure as part of their treatment plan (e.g., eye injuries)
- During each review within the Trauma subset, the patient may now be evaluated under a different category (e.g., general, respiratory, neurological), injury, or intervention depending on the continued need for hospitalization. Despite a change in the reason for continued stay, the review should be conducted for the next Episode Day, rather than starting from Episode Day 1, since the injury originated from the initial trauma
- If the patient sustains an injury while hospitalized (e.g., a fall from the bed), it would be appropriate to apply Episode Day 1 criteria, since this is a brand new condition

The criteria structure was updated to simplify the review process. Categories such as general, cardiovascular, and respiratory have been removed from the Intermediate and Critical levels of care. This change was made because interventions typically drive the need for a higher level of care and can be applied to multiple injuries/conditions.

The significant content changes are as follows:

- Abuse/neglect criteria were added to the Observation level of care to capture patients who sustain a minor injury from suspected or actual abuse/neglect that are not included in the General Trauma subset; however, they require admission to determine a safe discharge plan
- Chest trauma criteria for patients with normal initial ECG and troponin were moved from the Acute level of care to the Observation level on Episode Day 1 as these tests are sensitive to rule out blunt cardiac injury. Although some patients may require continuous cardiac monitoring, it is appropriate at the Observation level of care.
- Chest trauma criteria at the Intermediate level of care were updated to include patients with an abnormal ECG as the Eastern Association for Surgery of Trauma (EAST) recommends a monitored setting for patients with an abnormal ECG and/or elevated troponin
- Chest trauma criteria were removed from the Critical level of care as these patients can be reviewed at the Intermediate level of care
- Abdominal organ injury criteria were broadened to include other organs such as the kidneys, pancreas, intestines, ureter, and urethra. Interventions to appropriately address these organ injuries were added to the Acute level of care.
- Spleen and liver grade I injuries were removed from the Observation level of care and all low-grade injuries (e.g., AAST grade < III liver, pancreas, or spleen injury, grade < IV kidney injury) can now be reviewed at the Acute level of care on Episode Days 1 & 2 due to the risk for bleeding
- High grade organ injury criteria were added to the Critical level of care on Episode Day 2 to allow patients with these injuries an additional day at the Critical level of care for continuous monitoring due to the risk of non-operative management failure

	• “Post angioembolization ≤ 24h” criterion was added to capture patients who underwent angioembolization of an abdominal organ injury within the last 24 hours and require continued stay at the Critical level of care for up to 24 hours of monitoring • Chest wall fracture criteria were added to the Observation level of care on Episode Day 1 and the Acute level of care on all Episode Days to capture patients who are at high risk for complications at the Observation level of care and those with signs of respiratory compromise or significant injury (e.g., > 65 yrs and > 2 rib fractures) at the Acute level of care • Musculoskeletal injury criteria were added to the Observation level of care to capture patients who may require a surgical consultation to be cleared for discharge or to determine if further treatment is warranted • Partial responder criteria at the Observation level of care were added on Episode Day 2 for conditions or injuries (e.g., abuse or neglect, foreign body requiring post extraction monitoring or repeat imaging if not removed, uncontrolled pain, new functional impairment due to a musculoskeletal injury, hypoxia requiring supplemental oxygen, burn injury in a patient who is unable to adhere to an outpatient treatment regimen) requiring continued monitoring and/or treatment • Head trauma criteria were revised at the Observation and Acute level of care to better define patients who are appropriate at each level of care. The Observation level of care is meant for patients with a negative head CT and a mild traumatic brain injury (GCS 13–14) among other injuries (e.g., anticoagulant or antiplatelet use, difficult neurological assessment), while the Acute level of care is meant to capture patients with a negative head CT but a moderate traumatic brain injury (GCS 9-12) among other injuries (e.g., basilar skull fracture, CSF leak). • Head trauma criteria on Episode Day 2 at the Acute level of care now includes the finding “GCS 9-14 and < baseline” to allow patients who have not returned to their neurological baseline to step up to the Acute level of care and to account for patients previously at the Acute level of care that are improving • Nonfatal drowning criteria were updated to better define patients who require admission and were moved from the Observation to the Acute level of care due to the risk for respiratory decompensation • Pneumothorax criteria were added to the Observation level of care on Episode Day 1 for patients requiring a small-bore chest tube without an increase in oxygen requirement after the procedure, and those requiring only oximetry monitoring and repeat CXR as these are appropriate interventions for this level of care • Thoracic injury criteria were added to the Observation level of care on Episode Day 1 as patients requiring oximetry monitoring and repeat CXR are appropriate for this level of care • Burn criteria were added to the Observation level of care to capture patients who sustain a second-degree burn who would typically be treated as an outpatient but are unable to adhere to the outpatient treatment regimen • The “Burn therapy” rule at the Acute level of care was changed from “≥ Two” to “≥ One” on all Episode Days as admission is appropriate to the Acute level of care if at least one of the listed interventions is required • Bleeding criteria at the Acute and Intermediate levels of care on Episode Day 1 were removed as the driver for admission is the injury sustained • Bleeding criteria at the Intermediate level of care were removed on continued stay as patients who are hemodynamically stable in the setting of bleeding should be reviewed at the Acute level of care and those who are hemodynamically unstable should be reviewed at the Critical level of care on continued stay • Bleeding criteria for patients requiring anticoagulation reversal criteria were added to the Acute level of care on Episode Day 1 as the driver for admission for this patient population is the risk of significant bleeding rather than the injury sustained • Frailty criteria were added to the Acute level of care to identify patients who sustain a mild trauma that is not captured in the existing criteria but requires admission due to their risk of a significant decline in function and associated negative outcomes related to trauma • Eye injury criteria at the Acute level of care were updated to better define patients who require admission for surgical intervention and in continued stay, those requiring pain control with analgesics and/or anti-infectives
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	• Fracture, subluxation, or dislocation criteria at the Acute level of care were updated to be more inclusive of patients (e.g., risk for airway compromise, CSF leak, requiring a blood transfusion) who may require this level of care • The IV fluid criterion for fracture, subluxation, or dislocation injuries (excluding facial fractures) at the Acute level of care was removed as it is not a driver for admission • Antiarrhythmic criteria at the Intermediate level of care and arrhythmia criteria at the Critical level of care were removed on Episode Day 1 as patients should be reviewed at the appropriate level of care based on the injury that caused the arrhythmia or the appropriate arrhythmia subset if it is the driver for admission • Electrical injury criteria were revised and moved from the Intermediate to the Critical level of care based on literature indicating that patients with these injuries should be monitored in an intensive care unit for at least 24 hours • Smoke inhalation criteria at the Intermediate level of care on Episode Day 1 were removed as these patients should be reviewed based on the respiratory intervention required • Complex wound care criteria were removed from the Intermediate and Critical levels of care on Episode Day 1 as the driver for admission is the injury sustained from the trauma • Aortic injury criteria were added to the Critical level of care to capture patients who require non-operative management of this injury • Aortic injury criteria were added to the Acute level of care on Episode Day 3-X to capture patients that are appropriate to step down from the Critical level of care, which includes those requiring blood pressure management and/or repeat imaging prior to discharge • “Emergent tracheostomy or cricothyrotomy” criteria replaced “Airway placement” criteria at the Critical level of care to clarify the intent of the criteria • “Depressed skull fracture” and “Spinal cord injury” were added to the Critical level of care on Episode Day 1 to be more inclusive of the neurological injuries that may necessitate this level of care • Pelvic fracture interventions at the Critical level of care were updated to be more inclusive of interventions that may be performed in hemodynamically unstable patients • Snakebite criteria were added to the Critical level of care to capture patients with envenomation requiring antivenom treatment or close neurovascular and/or neurological monitoring on Episode Day 1 • Snakebite criteria were added to the Critical level of care on Episode Day 2 to capture patients who remain symptomatic and require further treatment with antivenom • “Post antivenom” criteria were added to the Critical level of care on Episode Days 2 & 3 to capture patients who require monitoring for 24 hours post-completion of antivenom treatment to ensure symptoms and coagulopathy have resolved • “Post endovascular revascularization” criterion was added to the Critical level of care on continued stay to capture patients who underwent endovascular revascularization for limb ischemia within the last 24 hours and require continued monitoring • “Infection, actual or suspected” criteria were added to Episode 3-X as it is a common complication of traumatic injuries and may result in the need for continued stay
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Heart Failure



Revision Summary

Frailty criteria were added as a new clinical risk factor to identify patients with acute on chronic heart failure who may necessitate the Acute level of care.

Heart Failure		
Initial Review (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
• ACUTE, One: ▶ Heart failure, new onset and, Both: ♦ Symptom, ≥ One: ▶ Dyspnea ▶ Orthopnea ▶ Paroxysmal nocturnal dyspnea ♦ Finding, ≥ One: ▶ Rales (crackles) on physical examination ▶ Gallop (S3, S4) on physical examination ▶ Pleural effusion or pulmonary edema or cardiomegaly confirmed by CXR ▶ Edema of lower extremities ▶ Hepatomegaly ▶ Jugular venous distention ▶ BNP or NT-proBNP > ULN ▶ Heart failure, acute on chronic and, Both: ♦ Continued symptoms after at least 1 dose of diuretic and ≥ 2h treatment and, ≥ One: ▶ Dyspnea, not returned to baseline ▶ O₂ sat < 89%(0.89) and < baseline ♦ Finding, ≥ One: ▶ Inadequate diuresis, ≥ One: ♦ Urine sodium < 50 mEq/L(50 mmol/L) at 2h after diuretic ♦ Urine output < 150 ml/h at 2h after diuretic ▶ Clinical risk factor, ≥ One: ♦ Troponin > ULN ♦ Creatinine ≥ 1.5x baseline or estimated baseline ♦ Chronic kidney disease (excludes chronic dialysis) and creatinine ≥ 2.75 mg/dL ♦ BUN ≥ 43 mg/dL ♦ Sodium < 130 mEq/L(130 mmol/L) ♦ Heart rate 100-120/min, sustained ♦ SBP < 120 mmHg and < baseline ♦ Severe and persistent mental illness, cognitive impairment, or substance use disorder and, Both: ▶ Patient unreliable and unable to adhere to outpatient instructions or treatment	• ACUTE, One: ▶ Heart failure, new onset and, Both: ♦ Symptom, ≥ One: ▶ Dyspnea ▶ Orthopnea ▶ Paroxysmal nocturnal dyspnea ♦ Finding, ≥ One: ▶ Rales (crackles) on physical examination ▶ Gallop (S3, S4) on physical examination ▶ Pleural effusion or pulmonary edema or cardiomegaly confirmed by CXR ▶ Edema of lower extremities ▶ Hepatomegaly ▶ Jugular venous distention ▶ BNP or NT-proBNP > ULN ▶ Heart failure, acute on chronic and, Both: ♦ Continued symptoms after at least 1 dose of diuretic and ≥ 2h treatment and, ≥ One: ▶ Dyspnea, not returned to baseline ▶ O₂ sat < 89%(0.89) and < baseline ♦ Finding, ≥ One: ▶ Inadequate diuresis, ≥ One: ♦ Urine sodium < 50 mEq/L(50 mmol/L) at 2h after diuretic ♦ Urine output < 150 ml/h at 2h after diuretic ▶ Clinical risk factor, ≥ One: ♦ Troponin > ULN ♦ Creatinine ≥ 1.5x baseline or estimated baseline ♦ Chronic kidney disease (excludes chronic dialysis) and creatinine ≥ 2.75 mg/dL ♦ BUN ≥ 43 mg/dL ♦ Sodium < 130 mEq/L(130 mmol/L) ♦ Heart rate 100-120/min, sustained ♦ SBP < 120 mmHg and < baseline ♦ Frailty, Both: ▶ Age ≥ 65 yrs ▶ Finding, ≥ One: ♦ Frailty, active diagnosis ♦ Frailty characteristics, ≥ Two:	Rationale: • Frailty criteria were added as a new clinical risk factor to identify patients with acute on chronic heart failure who may necessitate the Acute level of care as the presence of frailty in patients with heart failure has been associated with increased length of stay, risk for readmission, and mortality

Heart Failure		
Initial Review (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
►Caregiver unavailable or unable to manage care	►Chronic condition(s), ≥ One: ◆Dementia or cognitive impairment ◆Multimorbidity, ≥ 2 chronic illnesses ►Functional impairment ►Malnutrition ►Polypharmacy, ≥ 5 chronic medications ◆Severe and persistent mental illness, cognitive impairment, or substance use disorder and, Both: ►Patient unreliable and unable to adhere to outpatient instructions or treatment ►Caregiver unavailable or unable to manage care	

Heart Failure		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
●ACUTE, One: ►Heart failure, new onset and, All: ◆Symptom, ≥ One: ►Dyspnea ►Orthopnea ►Paroxysmal nocturnal dyspnea ◆Finding, ≥ One: ►Rales (crackles) on physical examination ►Gallop (S3, S4) on physical examination ►Pleural effusion or pulmonary edema or cardiomegaly confirmed by CXR ►Edema of lower extremities ►Hepatomegaly ►Jugular venous distention ►BNP or NT-proBNP > ULN ◆Intervention, ≥ One: ►Diuretic ≥ 1 dose ►Dialysis (includes ultrafiltration) ►Heart failure, acute on chronic and, All: ◆Continued symptoms after at least 1 dose of diuretic and ≥ 2h treatment and, ≥ One: ►Dyspnea, not returned to baseline ►O₂ sat < 89%(0.89) and < baseline ◆Finding, ≥ One: ►Inadequate diuresis, ≥ One: ◆Urine sodium < 50 mEq/L(50 mmol/L) at 2h after diuretic ◆Urine output < 150ml/h at 2h after diuretic	●ACUTE, One: ►Heart failure, new onset and, All: ◆Symptom, ≥ One: ►Dyspnea ►Orthopnea ►Paroxysmal nocturnal dyspnea ◆Finding, ≥ One: ►Rales (crackles) on physical examination ►Gallop (S3, S4) on physical examination ►Pleural effusion or pulmonary edema or cardiomegaly confirmed by CXR ►Edema of lower extremities ►Hepatomegaly ►Jugular venous distention ►BNP or NT-proBNP > ULN ◆Intervention, ≥ One: ►Diuretic ≥ 1 dose ►Dialysis (includes ultrafiltration) ►Heart failure, acute on chronic and, All: ◆Continued symptoms after at least 1 dose of diuretic and ≥ 2h treatment and, ≥ One: ►Dyspnea, not returned to baseline ►O₂ sat < 89%(0.89) and < baseline ◆Finding, ≥ One: ►Inadequate diuresis, ≥ One: ◆Urine sodium < 50 mEq/L(50 mmol/L) at 2h after diuretic ◆Urine output < 150ml/h at 2h after diuretic	Rationale: • Frailty criteria were added as a new clinical risk factor to identify patients with acute on chronic heart failure who may necessitate the Acute level of care as the presence of frailty in patients with heart failure has been associated with increased length of stay, risk for readmission, and mortality

Heart Failure		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
▶Clinical risk factor, ≥ One: ♦Troponin > ULN ♦Creatinine ≥ 1.5x baseline or estimated baseline ♦Chronic kidney disease (excludes chronic dialysis) and creatinine ≥ 2.75 mg/dL ♦BUN ≥ 43 mg/dL ♦Sodium < 130 mEq/L(130 mmol/L) ♦Heart rate 100-120/min, sustained ♦SBP < 120 mmHg and < baseline ♦Severe and persistent mental illness, cognitive impairment, or substance use disorder and, Both: ▶Patient unreliable and unable to adhere to outpatient instructions or treatment ▶Caregiver unavailable or unable to manage care ♦Intervention, ≥ One: ▶Diuretic, ≥ One: ♦Diuretic ≥ 2 doses ♦Escalation or change of diuretic ▶Dialysis (includes ultrafiltration)	▶Clinical risk factor, ≥ One: ♦Troponin > ULN ♦Creatinine ≥ 1.5x baseline or estimated baseline ♦Chronic kidney disease (excludes chronic dialysis) and creatinine ≥ 2.75 mg/dL ♦BUN ≥ 43 mg/dL ♦Sodium < 130 mEq/L(130 mmol/L) ♦Heart rate 100-120/min, sustained ♦SBP < 120 mmHg and < baseline ♦Frailty, Both: ▶Age ≥ 65 yrs ▶Finding, ≥ One: ♦Frailty, active diagnosis ♦Frailty characteristics, ≥ Two: ▶Chronic condition(s), ≥ One: ♦Dementia or cognitive impairment ♦Multimorbidity, ≥ 2 chronic illnesses ▶Functional impairment ▶Malnutrition ▶Polypharmacy, ≥ 5 chronic medications ♦Severe and persistent mental illness, cognitive impairment, or substance use disorder and, Both: ▶Patient unreliable and unable to adhere to outpatient instructions or treatment ▶Caregiver unavailable or unable to manage care ♦Intervention, ≥ One: ▶Diuretic, ≥ One: ♦Diuretic ≥ 2 doses ♦Escalation or change of diuretic ▶Dialysis (includes ultrafiltration)	

Hematology/Oncology: Treatments



Revision Summary

Elranatamab, Talquetamab-tgvs or Teclistamab-cqyv were added to the chemotherapy single or multidrug regimen criteria as the dosing regimen for these medications requires several days of inpatient monitoring for serious adverse effects. The verbiage “by imaging” was removed from “Brain malignancy, brain metastasis or spinal cord compression” criteria line as these diagnoses would be confirmed by imaging prior to inpatient admission for radiation therapy.

Hematology/Oncology: Treatments

Global (ACUTE)

Previously Available Content

- ▶Single or multidrug regimen, **≥ One:**
 - ♦EPOCH, DA-EPOCH or R-EPOCH
 - ♦High dose ifosfamide
 - ♦Hyper-CVAD

2024 March Release

- ▶Single or multidrug regimen, **≥ One:**
 - ♦**Elranatamab, Talquetamab-tgvs or Teclistamab-cqyv**
 - ♦EPOCH, DA-EPOCH or R-EPOCH
 - ♦High dose ifosfamide
 - ♦Hyper-CVAD

Revision Description

Rationale:

- Elranatamab, Talquetamab-tgvs or Teclistamab-cqyv were added to the chemotherapy single or multidrug regimen criteria as the dosing regimen for these medications requires several days of inpatient monitoring for serious adverse effects

Hematology/Oncology: Treatments

Episode Day 1 (ACUTE)

Previously Available Content

- ▶Radiation and, **≥ One:**
 - ♦Brain malignancy, brain metastasis or spinal cord compression **by imaging**
 - ♦Iodine ablation > 30 mCi
 - ♦Radioactive implant
 - ♦SVC syndrome confirmed by imaging

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- ▶Radiation and, **≥ One:**
 - ♦Brain malignancy, brain metastasis or spinal cord compression
 - ♦Iodine ablation > 30 mCi
 - ♦Radioactive implant
 - ♦SVC syndrome confirmed by imaging

Revision Description

Rationale:

- “by imaging” was removed as brain malignancy, brain metastasis or spinal cord compression diagnoses would be confirmed by imaging prior to inpatient admission for radiation therapy

Hematology/Oncology: Treatments		
Episode Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
►Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ◆Radiation ≤ 7d and, ≥ One: ►Brain malignancy, brain metastasis or spinal cord compression by imaging ►Iodine ablation > 30 mCi ►Radioactive implant ►SVC syndrome confirmed by imaging	►Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ◆Radiation ≤ 7d and, ≥ One: ►Brain malignancy, brain metastasis or spinal cord compression ►Iodine ablation > 30 mCi ►Radioactive implant ►SVC syndrome confirmed by imaging	Rationale: • “by imaging” was removed as brain malignancy, brain metastasis or spinal cord compression diagnoses would be confirmed by imaging prior to inpatient admission for radiation therapy

Hematology/Oncology: Treatments		
Episode Day 3-X (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
►Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ◆Radiation ≤ 7d and, ≥ One: ►Brain malignancy, brain metastasis or spinal cord compression by imaging ►Iodine ablation > 30 mCi ►Radioactive implant ►SVC syndrome confirmed by imaging	►Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ◆Radiation ≤ 7d and, ≥ One: ►Brain malignancy, brain metastasis or spinal cord compression ►Iodine ablation > 30 mCi ►Radioactive implant ►SVC syndrome confirmed by imaging	Rationale: • “by imaging” was removed as brain malignancy, brain metastasis or spinal cord compression diagnoses would be confirmed by imaging prior to inpatient admission for radiation therapy

Hypertensive Disorders of Pregnancy



Revision Summary

Hypertensive Disorders of Pregnancy content was updated to align with current guidelines. Criteria changes were made to streamline the review process and clarify the intent. Continuous antihypertensive criteria were added to the Critical level of care to be more inclusive of treatment options for pregnant individuals with severe hypertension refractory to treatment. Gestational hypertension or preeclampsia without severe features criteria were added as a pathway for those with coexisting conditions and risk factors that increase the complexity of care and may preclude expectant management in the outpatient setting. The Episode Day structure changed from 4 days to 3-X to account for multiple conditions requiring an extended length of stay for expectant management.

Hypertensive Disorders of Pregnancy

Episode Day 1 (OBSERVATION)

Previously Available Content

•OBSERVATION, One:

- ▶Rule out preeclampsia, **All:**
 - ♦Gestation ≥ 20 wks
 - ♦Systolic blood pressure 140-159 mmHg or diastolic blood pressure 90-109 mmHg
- ♦Laboratory evaluation, **All:**
 - ▶Urine protein analysis, ≥ **One:**
 - ♦24h urine collection
 - ♦Random urine sample
 - ♦Reagent-strip test
 - ▶Complete blood count (CBC)
 - ▶Creatinine
 - ▶Liver enzymes
 - ▶Platelet count
- ♦Blood pressure monitoring at least 3x/24h
- ♦Fetal evaluation, **Both:**
 - ▶Ultrasound assessment
 - ▶Fetal heart rate monitoring

2024 March Release

•OBSERVATION, One:

- ▶Rule out **gestational hypertension** or preeclampsia, **All:**
 - ♦Gestation ≥ 20 wks
 - ♦Systolic blood pressure 140-159 mmHg or diastolic blood pressure 90-109 mmHg
- ♦Laboratory evaluation, **All:**
 - ▶Urine protein analysis
 - ▶CBC
 - ▶Creatinine
 - ▶LFTs
 - ▶Platelet count
- ♦Blood pressure monitoring at least 3x/24h
- ♦Fetal evaluation, **Both:**
 - ▶Ultrasound assessment
 - ▶Fetal heart rate monitoring
- ▶**Hypertension, chronic, Both:**
 - ♦Systolic blood pressure ≥ 160 mmHg or diastolic blood pressure ≥ 110 mmHg lasting at least 15 min
- ♦Intervention, **All:**
 - ▶Antihypertensive (includes PO)
 - ▶Blood pressure monitoring at least 3x/24h
 - ▶Fetal heart rate monitoring

Revision Description

Rationale:

- “Gestational hypertension” has been added to preeclampsia criteria to align with current guidelines, which group both conditions together and states that they may not represent two separate and distinct conditions
- “Laboratory evaluation” criteria have been updated and “Urine protein analysis” tests are now included in a hover note to clarify the intent and streamline the review process
- “Hypertension, chronic” criteria were moved from the Acute level of care to the Observation level of care to align with current guidelines and allow for initial surveillance, medication titration, and evaluation for superimposed preeclampsia for up to 48 hours in cases of diagnostic uncertainty

Hypertensive Disorders of Pregnancy		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
• ACUTE, ≥ One: ▶ HELLP syndrome ≤ 2d, All: ♦ Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 70 U/L (1.17 µkat/L) ♦ Platelet count 50,000-100,000/cu.mm (50-100x10⁹/L) ♦ Finding, ≥ One: ▶ Fragmented or contracted red cells on blood smear ▶ Total serum lactate dehydrogenase (LDH) ≥ 600 IU/L (10.02 µkat/L) ▶ Total bilirubin > 1.20 mg/dL (20.52 µmol/L) ♦ Blood pressure management, ≥ One: ▶ Antihypertensive (includes PO) ▶ Blood pressure monitoring at least 3x/24h ♦ Fetal heart rate monitoring ♦ Laboratory monitoring, All: ▶ Complete blood count (CBC) ▶ Liver function tests (LFT) ▶ Platelet count ▶ Hypertension, chronic or gestational, Both: ♦ Systolic blood pressure ≥ 160 mmHg or diastolic blood pressure ≥ 110 mmHg lasting at least 15 min ♦ Intervention, All: ▶ Antihypertensive (includes PO) ▶ Blood pressure monitoring at least 3x/24h ▶ Fetal heart rate monitoring ▶ Preeclampsia with severe features and, All: ♦ Finding, One: ▶ Systolic blood pressure ≥ 160 mmHg or diastolic blood pressure ≥ 110 mmHg on ≥ 2 measurements ▶ Systolic blood pressure 140-159 mmHg or diastolic blood pressure 90-109 mmHg on ≥ 2 measurements and, ≥ One: ♦ AST or ALT > 2x ULN ♦ Creatinine, ≥ One: ▶ > 1.1 mg/dL (97.2 µmol/L) ▶ > 2x baseline ♦ Pain, severe persistent and unresponsive to medication and, ≥ One: ▶ Epigastric ▶ Headache, new onset ▶ Right upper quadrant ♦ Platelet count < 100,000/cu.mm (100x10⁹/L) ♦ Visual disturbances, persistent ♦ Monitoring, All: ▶ Blood pressure monitoring at least 3x/24h ▶ Fetal heart rate monitoring ▶ Laboratory monitoring, All: ♦ CBC ♦ Creatinine ♦ LFTs ♦ Platelet count ♦ Intervention, ≥ One: ▶ Antihypertensive (includes PO) ▶ Magnesium sulfate	• ACUTE, ≥ One: ▶ HELLP syndrome ≤ 2d, All: ♦ AST or ALT > 2x ULN ♦ Platelet count 50,000-100,000/cu.mm (50-100x10⁹/L) ♦ LDH ≥ 600 IU/L (10.02 µkat/L) ♦ Blood pressure management, ≥ One: ▶ Antihypertensive (includes PO) ▶ Blood pressure monitoring at least 3x/24h ♦ Fetal heart rate monitoring ♦ Laboratory monitoring, All: ▶ CBC ▶ LFTs ▶ Platelet count ▶ Gestational hypertension or preeclampsia with severe features and, All: ♦ Finding, One: ▶ Systolic blood pressure ≥ 160 mmHg or diastolic blood pressure ≥ 110 mmHg on ≥ 2 measurements ▶ Systolic blood pressure 140-159 mmHg or diastolic blood pressure 90-109 mmHg on ≥ 2 measurements and, ≥ One: ♦ AST or ALT > 2x ULN ♦ Creatinine, ≥ One: ▶ > 1.1 mg/dL (97.2 µmol/L) ▶ > 2x baseline ♦ Pain, severe persistent and unresponsive to medication and, ≥ One: ▶ Epigastric ▶ Headache, new onset ▶ Right upper quadrant ♦ Platelet count < 100,000/cu.mm (100x10⁹/L) ♦ Visual disturbances, persistent ♦ Monitoring, All: ▶ Blood pressure monitoring at least 3x/24h ▶ Fetal heart rate monitoring ▶ Laboratory monitoring, All: ♦ CBC ♦ Creatinine ♦ LFTs ♦ Platelet count ♦ Intervention, ≥ One: ▶ Antihypertensive (includes PO) ▶ Magnesium sulfate	Rationale: • HELLP criteria were streamlined, and a specified number for AST or ALT was changed to greater than 2x ULN to account for institutional assay variations and to align with the diagnostic criteria noted in current guidelines • Based on current guidelines, HELLP criteria were revised by aligning with primary interventions and more appropriate diagnostic benchmarks, removal of total bilirubin and fragmented or contracted red blood cells criteria, as these are not universally utilized • “Hypertension, chronic” criteria were moved from the Acute level of care to the Observation level of care to align with current guidelines and allow for initial surveillance, medication titration, and evaluation for superimposed preeclampsia for up to 48 hours in cases of diagnostic uncertainty • “Gestational hypertension” has been added to preeclampsia criteria to align with current guidelines, which group both conditions together and states that they may not represent two separate and distinct conditions

Hypertensive Disorders of Pregnancy		
Episode Day 1 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦Complete blood count (CBC) ♦Creatinine ♦Liver function tests (LFT) ♦Platelet count ♦Intervention, ≥ One: ▶Antihypertensive (includes PO) ▶Magnesium sulfate		

Hypertensive Disorders of Pregnancy		
Episode Day 1 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
•CRITICAL, ≥ One: ▶Acute kidney injury (AKI) and, Both: ♦Finding, ≥ One: ▶Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: ♦Loss of P wave ♦Multifocal PVCs ♦Neuromuscular deficit ♦Widening QRS ▶Sodium, One: ♦< 120 mEq/L(120 mmol/L) and volume overload ♦> 160 mEq/L(160 mmol/L) ♦Intervention, ≥ One: ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶Hemodialysis ≤ 7d since initiation ▶IV fluid resuscitation ≤ 2d ▶Arrhythmia, ≥ One: ♦Urgent electrical cardioversion performed within last 24h ♦Defibrillation performed within last 24h ♦Temporary pacemaker insertion performed within last 2d ♦Symptomatic recurrent or refractory arrhythmia requiring antiarrhythmic and, ≥ One: ▶Chest pain ▶Heart failure and, ≥ One: ♦Arterial Po₂ < 56 mmHg(7.4 kPa) ♦O₂ sat < 89%(0.89) and < baseline ▶Hemodynamic instability, ≥ One: ♦Heart rate > 120/min, sustained	•CRITICAL, ≥ One: ▶Antihypertensive, continuous ▶DIC and blood product transfusion ▶Eclampsia and neurological assessment every 1-2h, ≤ 2d ▶ECMO or ECLS ▶Heart failure and, Both: ♦Finding, ≥ One: ▶Arterial Po₂ < 56 mmHg(7.4 kPa) ▶O₂ sat < 89%(0.89) and < baseline ♦Diuretic every 1-2h or continuous ▶Hemodynamic instability, All: ♦ED treatment, ≥ One: ≥ 2 units blood product transfusion ≥ 2 liters IV fluid ≥ 1 unit blood product transfusion and ≥ 1 liter IV fluid ♦Findings after ED treatment, ≥ One: ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Requiring ongoing resuscitation ▶Hemodynamic monitoring, invasive ▶HELLP syndrome ≤ 2d, Both: ♦Finding, ≥ One: ▶Lab values, All: ♦Platelet count < 50,000/cu.mm(50x10⁹/L) ♦AST or ALT > 2x ULN ♦LDH ≥ 600 IU/L(10.02 µkat/L) ▶Increased risk of bleeding, ≥ One: ♦PT ≥ 1.5x ULN or INR ≥ 2.0 ♦PTT ≥ 1.5x ULN ♦Intervention, ≥ One:	Rationale: • Acute Kidney Injury (AKI) and CRRT criteria were removed as an individual with AKI or requiring CRRT can be reviewed in the AKI subset • Criterion for continuous antihypertensive infusion was added to the Critical level of care to align with guidelines and provide a pathway for pregnant individuals with hypertensive disorders of pregnancy and severe hypertension refractory to treatment • “Arrhythmia” content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • HELLP criteria were streamlined, and a specified number for AST or ALT was changed to greater than 2x ULN to account for institutional assay variations and to align with the diagnostic criteria noted in current guidelines • Based on current guidelines, HELLP criteria were revised by aligning with primary interventions and more appropriate diagnostic benchmarks, removal of total bilirubin and fragmented or contracted red blood cells criteria, as these are not universally utilized

Hypertensive Disorders of Pregnancy

Episode Day 1 (CRITICAL)

Previously Available Content	2024 March Release	Revision Description
♦Systolic blood pressure, ≥ One: ▶< 90 mmHg ▶< 110 mmHg with chronic hypertension ▶> 30 mmHg decrease from baseline ▶Labile ♦MAP < 65 mmHg ▶CRRT ▶DIC and blood product transfusion ▶Eclampsia and neurological assessment every 1-2h, ≤ 2d ▶ECMO or ECLS ▶Heart failure and, Both: ♦Finding, ≥ One: ▶Arterial Po₂ < 56 mmHg(7.4 kPa) ▶O₂ sat < 89%(0.89) and < baseline ♦Diuretic every 1-2h or continuous ▶Hemodynamic instability, All: ♦ED treatment, ≥ One: ▶≥ 2 units blood product transfusion ▶≥ 2 liters IV fluid ▶≥ 1 unit blood product transfusion and ≥ 1 liter IV fluid ♦Findings after ED treatment, ≥ One: ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Requiring ongoing resuscitation ▶Hemodynamic monitoring, invasive ▶HELLP syndrome ≤ 2d, Both: ♦Finding, ≥ One: ▶Lab values, All: ♦Platelet count < 50,000/cu.mm(50x10⁹/L) ♦Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 70 U/L(1.17 µkat/L) ♦Finding, ≥ One: ▶Fragmented or contracted red cells on blood smear ▶Total serum lactate dehydrogenase (LDH) ≥ 600 IU/L(10.02 µkat/L) ▶Total bilirubin > 1.20 mg/dL(20.52 µmol/L) ▶Increased risk of bleeding, ≥ One: ♦PT ≥ 1.5x upper limit of normal (ULN) or INR ≥ 2.0 ♦PTT ≥ 1.5x upper limit of normal (ULN) ♦Intervention, ≥ One: ▶Blood product transfusion ▶Laboratory monitoring, All: ♦Complete blood count (CBC) ♦Liver function tests (LFT) ♦Platelet count ▶HFNC and respiratory monitoring every 1-2h	▶Blood product transfusion ▶Laboratory monitoring, All: ♦CBC ♦LFTs ♦Platelet count ▶HFNC ▶Mechanical ventilation ▶NIPPV ▶Oxygen ≥ 40%(0.40), ≤ 2d ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	• “HFNC” criterion was added to reflect the patient who is appropriate at the Critical level of care • NIPPV subcriteria were removed because any patient who is newly requiring noninvasive ventilator support is appropriate for the Critical level of care on Episode Day 1 • Oxygen ≥ 40%” criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Critical level of care • “Seizure, recurrent or refractory requiring anticonvulsant” criteria were removed as this subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology

Hypertensive Disorders of Pregnancy		
Episode Day 1 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
▶Mechanical ventilation ▶NIPPV and, ≥ One: ♦Arterial Po₂ ≤ 39 mmHg(5.2 kPa) ♦Arterial or venous Pco₂ ≥ 60 mmHg(8.0 kPa) and, One: ▶COPD and arterial or venous pH ≤ 7.24 ▶No hx of COPD ▶Oxygen ≥ 40%(0.40), ≤ 2d ▶Seizure, recurrent or refractory requiring anticonvulsant ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure)		

Hypertensive Disorders of Pregnancy		
Episode Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
•ACUTE, ≥ One: ▶Delivery, One: ♦Planned within 48h and, Both: ▶≥ 24 and < 37 wks gestation ▶Corticosteroid administered within last 48h ▶Fever and, Both: ♦Temperature ≥ 100.4°F(38.0°C) ♦Intervention, One: ▶Known source and anti-infective ▶New onset and, ≥ One: ♦Culture pending ≤ 2d ♦Drug-induced fever suspected and precipitating drug discontinued ≤ 3d ♦Imaging study ≤ 24h ▶HELLP syndrome ≤ 2d, All: ♦Blood pressure management, ≥ One: ▶Antihypertensive (includes PO) ▶Blood pressure monitoring at least 3x/24h ♦Fetal heart rate monitoring ♦Laboratory monitoring, All: ▶Complete blood count (CBC) ▶Liver function tests (LFT) ▶Platelet count ▶Hypertension and, All: ♦Antihypertensive (includes PO) ♦Blood pressure monitoring at least 3x/24h ♦Fetal heart rate monitoring ▶Preeclampsia with severe features and, ≥ One:	•ACUTE, ≥ One: ▶Delivery, One: ♦Planned within 48h and, Both: ▶≥ 22 and < 37 wks gestation ▶Corticosteroid administered within last 48h ▶HELLP syndrome ≤ 2d, All: ♦Blood pressure management, ≥ One: ▶Antihypertensive (includes PO) ▶Blood pressure monitoring at least 3x/24h ♦Fetal heart rate monitoring ♦Laboratory monitoring, All: ▶CBC ▶LFTs ▶Platelet count ▶Gestational hypertension or preeclampsia with severe features and, ≥ One: ♦Expectant management, Both: ▶< 37 wks gestation ▶Intervention, Both: ♦Blood pressure management, ≥ One: ▶Antihypertensive (includes PO) ▶Blood pressure monitoring at least 3x/24h ♦Fetal heart rate monitoring ♦Magnesium sulfate ▶Gestational hypertension or preeclampsia without severe features and, Both: ♦< 37 wks gestation ♦Expectant management, Both:	Rationale: • Delivery “Planned within 48h and” criteria were revised to allow for antenatal corticosteroid administration at a gestational age of 22 weeks or more to align with current guidelines • Fever criteria were removed as patients with a fever can be reviewed in the Antepartum subset • Chronic hypertension criteria were moved from the Acute level of care to the Observation level of care to align with current guidelines and allow for initial surveillance, medication titration, and evaluation for superimposed preeclampsia for up to 48 hours in cases of diagnostic uncertainty • “Gestational hypertension” has been added to preeclampsia criteria to align with current guidelines, which group both conditions together and states that they may not represent two separate and distinct conditions • Severe preeclampsia criteria have been updated to allow expectant management for up to 37 weeks gestation, which aligns with external peer reviewer

•**ACUTE, ≥ One:**

- ▶Delivery, **One:**
 - ♦Planned within 48h and, **Both:**
 - ▶≥ 22 and < 37 wks gestation
 - ▶Corticosteroid administered within last 48h
- ▶HELLP syndrome ≤ 2d, **All:**
 - ♦Blood pressure management, **≥ One:**
 - ▶Antihypertensive (includes PO)
 - ▶Blood pressure monitoring at least 3x/24h
 - ♦Fetal heart rate monitoring
 - ♦Laboratory monitoring, **All:**
 - ▶CBC
 - ▶LFTs
 - ▶Platelet count
- ▶**Gestational hypertension or** preeclampsia with severe features and, **≥ One:**
 - ♦Expectant management, **Both:**
 - ▶< 37 wks gestation
 - ▶Intervention, **Both:**
 - ♦Blood pressure management, **≥ One:**
 - ▶Antihypertensive (includes PO)
 - ▶Blood pressure monitoring at least 3x/24h
 - ♦Fetal heart rate monitoring
- ♦Magnesium sulfate
- ▶**Gestational hypertension or preeclampsia without severe features and, Both:**
 - ♦< 37 wks gestation
 - ♦Expectant management, **Both:**

Rationale:

- Delivery “Planned within 48h and” criteria were revised to allow for antenatal corticosteroid administration at a gestational age of 22 weeks or more to align with current guidelines
- Fever criteria were removed as patients with a fever can be reviewed in the Antepartum subset
- Chronic hypertension criteria were moved from the Acute level of care to the Observation level of care to align with current guidelines and allow for initial surveillance, medication titration, and evaluation for superimposed preeclampsia for up to 48 hours in cases of diagnostic uncertainty
- “Gestational hypertension” has been added to preeclampsia criteria to align with current guidelines, which group both conditions together and states that they may not represent two separate and distinct conditions
- Severe preeclampsia criteria have been updated to allow expectant management for up to 37 weeks gestation, which aligns with external peer reviewer

Hypertensive Disorders of Pregnancy		
Episode Day 2 (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
♦Expectant management, Both: ▶< 34 wks gestation ♦Intervention, Both: ♦Blood pressure management, ≥ One: ▶Antihypertensive (includes PO) ▶Blood pressure monitoring at least 3x/24h ♦Fetal heart rate monitoring ♦Magnesium sulfate	▶Finding, ≥ One: ♦Fetal growth restriction ♦Oligohydramnios ♦Limited access to prompt medical care and arrangements for outpatient services pending ≤ 24h ♦Outpatient services unavailable ♦Patient or caregiver unreliable and unable to adhere to outpatient instructions or treatment ♦Severe and persistent mental illness, cognitive impairment, or substance use disorder ♦Intervention, Both: ♦Blood pressure management, ≥ One: ▶Antihypertensive (includes PO) ▶Blood pressure monitoring at least 3x/24h ♦Fetal heart rate monitoring ▶Post Critical care ≤ 24h	feedback due to the lack of consensus amongst obstetric organizations regarding the optimal delivery timeframe for severe preeclampsia when maternal and fetal condition is stable • Gestational hypertension or preeclampsia without severe features criteria have been added to align with guidelines and provide a pathway for expectant management at the Acute level of care for those without severe features that have coexisting comorbidities, risk factors, social determinants of health (SDOH), or barriers to discharge that preclude expectant management in the outpatient setting • “Post critical care ≤ 24h” criterion was added to provide a pathway for the patient that is stepping down from the Critical to the Acute level of care

Hypertensive Disorders of Pregnancy		
Episode Day 2 (INTERMEDIATE)		
Previously Available Content	2024 March Release	Revision Description
♦INTERMEDIATE, ≥ One: ▶Arrhythmia, ≥ One: ♦IV antiarrhythmic and hemodynamically stable ♦Antiarrhythmic conversion from IV to PO, One: ▶No proarrhythmic risk ≤ 24h ▶Proarrhythmic risk and continuous cardiac monitoring (excludes Holter) ≤ 3d ▶Eclampsia and neurological assessment every 3-4h, ≤ 2d ▶NIPPV	♦INTERMEDIATE, ≥ One: ▶Eclampsia and neurological assessment every 3-4h, ≤ 2d ▶HFNC and respiratory monitoring every 3-4h ▶NIPPV	Rationale: • “Arrhythmia” content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • “HFNC and respiratory monitoring every 3-4h” criterion was added to reflect the patient who is appropriate at the Intermediate level of care

Hypertensive Disorders of Pregnancy		
Episode Day 2 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
• CRITICAL, ≥ One: ▶ Acute kidney injury (AKI) and, Both: ♦ Finding, ≥ One: ▶ Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: ♦ Loss of P wave ♦ Multifocal PVCs ♦ Neuromuscular deficit ♦ Widening QRS ▶ Sodium, One: ♦ < 120 mEq/L(120 mmol/L) and volume overload ♦ > 160 mEq/L(160 mmol/L) ♦ Intervention, ≥ One: ▶ Calcium chloride ▶ Calcium gluconate ▶ Glucose 50%(0.50) with insulin ▶ Diuretic, continuous ▶ Hemodialysis ≤ 7d since initiation ▶ IV fluid resuscitation ≤ 2d ▶ Arrhythmia, ≥ One: ♦ Urgent electrical cardioversion performed within last 24h ♦ Defibrillation performed within last 24h ♦ Temporary pacemaker insertion performed within last 2d ♦ Symptomatic recurrent or refractory arrhythmia requiring antiarrhythmic and, ≥ One: ▶ Chest pain ▶ Heart failure and, ≥ One: ♦ Arterial Po₂ < 56 mmHg(7.4 kPa) ♦ O₂ sat < 89%(0.89) and < baseline ▶ Hemodynamic instability, ≥ One: ♦ Heart rate > 120/min, sustained ♦ Systolic blood pressure, ≥ One: ▶ < 90 mmHg ▶ < 110 mmHg with chronic hypertension ▶ > 30 mmHg decrease from baseline ▶ Labile ♦ MAP < 65 mmHg ▶ CRRT ▶ DIC and blood product transfusion ▶ Eclampsia and neurological assessment every 1-2h, ≤ 2d ▶ ECMO or ECLS ▶ Heart failure and, Both: ♦ Finding, ≥ One: ▶ Arterial Po₂ < 56 mmHg(7.4 kPa) ▶ O₂ sat < 89%(0.89) and < baseline ♦ Diuretic every 1-2h or continuous ▶ Hemodynamic instability persisting after ≥1L of IVF resuscitation or ≥1 unit blood product transfusion and, Both:	• CRITICAL, ≥ One: ▶ Antihypertensive, continuous ▶ DIC and blood product transfusion ▶ Eclampsia and neurological assessment every 1-2h, ≤ 2d ▶ ECMO or ECLS ▶ Heart failure and, Both: ♦ Finding, ≥ One: ▶ Arterial Po₂ < 56 mmHg(7.4 kPa) ▶ O₂ sat < 89%(0.89) and < baseline ♦ Diuretic every 1-2h or continuous ▶ Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion and, Both: ♦ Finding, ≥ One: ▶ Heart rate > 120/min ▶ Systolic blood pressure < 100 mmHg and < baseline ▶ MAP < 65 mmHg ♦ Intervention, ≥ One: ▶ Blood product transfusion ▶ IV fluid resuscitation ≥ 1L ≤ 24h ▶ Hemodynamic monitoring, invasive ▶ HELLP syndrome ≤ 2d, Both: ♦ Finding, ≥ One: ▶ Lab values, All: ♦ Platelet count < 50,000/cu.mm(50x10⁹/L) ♦ AST or ALT > 2x ULN ♦ LDH ≥ 600 IU/L(10.02 µkat/L) ▶ Increased risk of bleeding, ≥ One: ♦ PT ≥ 1.5x ULN or INR ≥ 2.0 ♦ PTT ≥ 1.5x ULN ♦ Intervention, ≥ One: ▶ Blood product transfusion ▶ Laboratory monitoring, All: ♦ CBC ♦ LFTs ♦ Platelet count ▶ HFNC and respiratory monitoring every 1-2h ▶ Mechanical ventilation ▶ NIPPV and, ≥ One: ♦ FiO₂ > 50%(0.50) ♦ Respiratory intervention every 1-2h ▶ Oxygen ≥ 40%(0.40) ▶ Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	Rationale: • Acute Kidney Injury (AKI) and CRRT criteria were removed as an individual with AKI or requiring CRRT can be reviewed in the AKI subset • Criteria for continuous antihypertensive infusion were added to the Critical level of care to align with guidelines and provide a pathway for pregnant individuals with hypertensive disorders of pregnancy and severe hypertension refractory to treatment who require continuous antihypertensive infusion • "Arrhythmia" content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • HELLP criteria were streamlined, and a specified number for AST or ALT was changed to greater than 2x ULN to account for institutional assay variations and to align with the diagnostic criteria noted in current guidelines • Based on current guidelines, HELLP criteria were revised by aligning with primary interventions and more appropriate diagnostic benchmarks, removal of total bilirubin and fragmented or contracted red blood cells criteria, as these are not universally utilized • Oxygen ≥ 40%" criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Critical level of care • "Seizure, recurrent or refractory requiring anticonvulsant criteria" was removed because this subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy, or General Medical subset depending on the etiology

Hypertensive Disorders of Pregnancy		
Episode Day 2 (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
♦Finding, ≥ One: ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: ▶Blood product transfusion ▶IV fluid resuscitation ≥1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶HELLP syndrome ≤ 2d, Both: ♦Finding, ≥ One: ▶Lab values, All: ♦Platelet count < 50,000/cu.mm(50x10⁹/L) ♦Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 70 U/L(1.17 µkat/L) ♦Finding, ≥ One: ▶Fragmented or contracted red cells on blood smear ▶Total serum lactate dehydrogenase (LDH) ≥ 600 IU/L(10.02 µkat/L) ▶Total bilirubin > 1.20 mg/dL(20.52 µmol/L) ▶Increased risk of bleeding, ≥ One: ♦PT ≥ 1.5x upper limit of normal (ULN) or INR ≥ 2.0 ♦PTT ≥ 1.5x upper limit of normal (ULN) ♦Intervention, ≥ One: ▶Blood product transfusion ▶Laboratory monitoring, All: ♦Complete blood count (CBC) ♦Liver function tests (LFT) ♦Platelet count ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Oxygen ≥ 40%(0.40), ≤ 2d ▶Seizure, recurrent or refractory requiring anticonvulsant ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure)		

Hypertensive Disorders of Pregnancy

Episode Day 3-X (ACUTE)

Previously Available Content	2024 March Release	Revision Description
• ACUTE, One: ► Responder, discharge expected today if clinically stable last 12h, All: ♦ Afebrile ♦ Blood pressure within acceptable limits ♦ Reassuring fetal testing ♦ Lab values within acceptable limits ♦ Neurological stability ♦ Pain controlled or manageable ► Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦ Delivery, One: ► Planned within 48h and, Both: ♦ ≥ 24 and < 37 wks gestation ♦ Corticosteroid administered within last 48h ♦ Fever and, Both: ► Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ ♦ Intervention, One: ► Known source and anti-infective ► New onset and, $\geq 2\text{d}$ ► Culture pending $\leq 2\text{d}$ ► Drug-induced fever suspected and precipitating drug discontinued $\leq 3\text{d}$ ► Imaging study $\leq 24\text{h}$ ♦ HELLP syndrome $\leq 2\text{d}$, All: ► Platelet count 50,000-100,000/cu.mm(50-100x10⁹/L) ► Blood pressure management, ≥ One: ♦ Antihypertensive (includes PO) ♦ Blood pressure monitoring at least 3x/24h ► Fetal heart rate monitoring ► Laboratory monitoring, All: ♦ Complete blood count (CBC) ♦ Liver function tests (LFT) ♦ Platelet count ♦ Hypertension uncontrolled $\leq 2\text{d}$, All: ► Systolic blood pressure ≥ 150 mmHg or diastolic blood pressure ≥ 100 mmHg last 24h ► Antihypertensive (includes PO) ► Blood pressure monitoring at least 3x/24h ► Fetal heart rate monitoring ♦ Preeclampsia with severe features and, $\geq \text{One}$: ► Expectant management, Both: ♦ < 34 wks gestation ♦ Intervention, Both: ► Blood pressure management, ≥ One: ♦ Antihypertensive (includes PO) ♦ Blood pressure monitoring at least 3x/24h ► Fetal heart rate monitoring ► Magnesium sulfate ♦ Post Critical care $\leq 24\text{h}$	• ACUTE, One: ► Responder, discharge expected today if clinically stable last 12h, All: ♦ Blood pressure within acceptable limits ♦ Reassuring fetal testing ♦ Lab values within acceptable limits ♦ Neurological stability ♦ Pain controlled or manageable ► Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦ Delivery, One: ► Planned within 48h and, Both: ♦ ≥ 22 and < 37 wks gestation ♦ Corticosteroid administered within last 48h ♦ HELLP syndrome $\leq 2\text{d}$, All: ► Platelet count 50,000-100,000/cu.mm(50-100x10⁹/L) ► Blood pressure management, ≥ One: ♦ Antihypertensive (includes PO) ♦ Blood pressure monitoring at least 3x/24h ► Fetal heart rate monitoring ► Laboratory monitoring, All: ♦ CBC ♦ LFTs ♦ Platelet count ♦ Gestational hypertension or preeclampsia with severe features and, $\geq \text{One}$: ► Expectant management, Both: ♦ < 37 wks gestation ♦ Intervention, Both: ► Blood pressure management, ≥ One: ♦ Antihypertensive (includes PO) ♦ Blood pressure monitoring at least 3x/24h ► Fetal heart rate monitoring ► Magnesium sulfate ♦ Gestational hypertension or preeclampsia without severe features and, Both: ► < 37 wks gestation ► Expectant management, Both: ♦ Finding, $\geq \text{One}$: ► Fetal growth restriction ► Oligohydramnios ► Limited access to prompt medical care and arrangements for outpatient services pending $\leq 24\text{h}$ ► Outpatient services unavailable ► Patient or caregiver unreliable and unable to adhere to outpatient instructions or treatment ► Severe and persistent mental illness, cognitive impairment, or substance use disorder	Rationale: • The Episode Day structure changed from 4 days to 3-X to account for multiple conditions requiring an extended length of stay for expectant management • “Afebrile” criterion was removed from Responder criteria to align with the removal of fever criteria within this subset • Delivery “Planned within 48h and” criteria were revised to allow for antenatal corticosteroid administration at a gestational age of 22 weeks or more to align with current guidelines • Fever criteria were removed as patients with a fever can be reviewed in the Antepartum subset • Chronic hypertension criteria were moved from the Acute level of care to the Observation level of care to align with current guidelines and allow for initial surveillance, medication titration, and evaluation for superimposed preeclampsia up to 48 hours in cases of diagnostic uncertainty • “Gestational hypertension” has been added to preeclampsia criteria to align with current guidelines, which group both conditions together and states that they may not represent two separate and distinct conditions • Severe preeclampsia criteria have been updated to allow expectant management for up to 37 weeks gestation, which aligns with external peer reviewer feedback due to the lack of consensus amongst obstetric organizations regarding the optimal delivery timeframe for severe preeclampsia when maternal and fetal condition is stable • Gestational hypertension or preeclampsia without severe features criteria have been added to align with guidelines and provide a pathway for expectant management at the Acute level of care for those without severe features that have coexisting comorbidities, risk factors, SDOH, or barriers to discharge that preclude expectant management in the outpatient setting

Hypertensive Disorders of Pregnancy		
Episode Day 3-X (ACUTE)		
Previously Available Content	2024 March Release	Revision Description
	♦Intervention, Both: ▶Blood pressure management, ≥ One: ♦Antihypertensive (includes PO) ♦Blood pressure monitoring at least 3x/24h ▶Fetal heart rate monitoring ♦Post Critical care ≤ 24h ▶Non-responder, not clinically stable for discharge and does not meet partial responder criteria, One: ♦Use Extended Stay subset ♦Refer for secondary review	Non-responder criteria were added for the patient who is not clinically stable for discharge and exceeds the designated endpoints at the Acute level of care

Hypertensive Disorders of Pregnancy		
Episode Day 3-X (INTERMEDIATE)		
Previously Available Content	2024 March Release	Revision Description
♦INTERMEDIATE, ≥ One: ▶Arrhythmia, ≥ One: ♦IV antiarrhythmic and hemodynamically stable ♦Antiarrhythmic conversion from IV to PO, One: ▶No proarrhythmic risk ≤ 24h ▶Proarrhythmic risk and continuous cardiac monitoring (excludes Holter) ≤ 3d ▶Eclampsia and neurological assessment every 3-4h, ≤ 2d ▶NIPPV	♦INTERMEDIATE, ≥ One: ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦Eclampsia and neurological assessment every 3-4h, ≤ 2d ♦HFNC and respiratory monitoring every 3-4h ♦NIPPV ▶Non-responder, not clinically stable for discharge, One: ♦Does not meet partial responder criteria or at the Acute level of care, One: ▶Use Extended Stay subset ▶Refer for secondary review	Rationale: - The Episode Day structure changed from 4 days to 3-X to account for multiple conditions requiring an extended length of stay for expectant management “Arrhythmia” content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (Blocks, Atrial, or Ventricular or Abnormal ECG Finding) “HFNC and respiratory monitoring every 3-4h” criterion was added to reflect the patient who is appropriate at the Intermediate level of care - Non-responder criteria were added for the patient who is not clinically stable for discharge and exceeds the designated endpoints at the Intermediate level of care

Hypertensive Disorders of Pregnancy		
Episode Day 3-X (CRITICAL)		
Previously Available Content	2024 March Release	Revision Description
•CRITICAL, ≥ One: ▶Acute kidney injury (AKI) and, Both: ♦Finding, ≥ One: ▶Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: ♦Loss of P wave ♦Multifocal PVCs ♦Neuromuscular deficit ♦Widening QRS ▶Sodium, One: ♦< 120 mEq/L(120 mmol/L) and volume overload ♦> 160 mEq/L(160 mmol/L) ♦Intervention, ≥ One: ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶Hemodialysis ≤ 7d since initiation ▶IV fluid resuscitation ≤ 2d ▶Arrhythmia, ≥ One: ♦Urgent electrical cardioversion performed within last 24h ♦Defibrillation performed within last 24h ♦Temporary pacemaker insertion performed within last 2d ♦Symptomatic recurrent or refractory arrhythmia requiring antiarrhythmic and, ≥ One: ▶Chest pain ▶Heart failure and, ≥ One: ♦Arterial Po₂ < 56 mmHg(7.4 kPa) ♦O₂ sat < 89%(0.89) and < baseline ▶Hemodynamic instability, ≥ One: ♦Heart rate > 120/min, sustained ♦Systolic blood pressure, ≥ One: ▶< 90 mmHg ▶< 110 mmHg with chronic hypertension ▶> 30 mmHg decrease from baseline ▶Labile ♦MAP < 65 mmHg ▶CRRT ▶DIC and blood product transfusion ▶Eclampsia and neurological assessment every 1-2h, ≤ 2d ▶ECMO or ECLS ▶Heart failure and, Both: ♦Finding, ≥ One: ▶Arterial Po₂ < 56 mmHg(7.4 kPa) ▶O₂ sat < 89%(0.89) and < baseline ♦Diuretic every 1-2h or continuous ▶Hemodynamic instability persisting after ≥1L of IVF resuscitation or ≥1 unit blood product transfusion and, Both:	•CRITICAL, ≥ One: ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: ♦Antihypertensive, continuous ♦DIC and blood product transfusion ♦Eclampsia and neurological assessment every 1-2h, ≤ 2d ♦ECMO or ECLS ♦Heart failure and, Both: ▶Finding, ≥ One: ♦Arterial Po₂ < 56 mmHg(7.4 kPa) ♦O₂ sat < 89%(0.89) and < baseline ▶Diuretic every 1-2h or continuous ♦Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion and, Both: ▶Finding, ≥ One: ♦Heart rate > 120/min ♦Systolic blood pressure < 100 mmHg and < baseline ♦MAP < 65 mmHg ▶Intervention, ≥ One: ♦Blood product transfusion ♦IV fluid resuscitation ≥ 1L ≤ 24h ♦Hemodynamic monitoring, invasive ♦HELLP syndrome ≤ 2d, Both: ▶Finding, ≥ One: ♦Lab values, All: ▶Platelet count < 50,000/cu.mm(50x10⁹/L) ▶AST or ALT > 2x ULN ▶LDH ≥ 600 IU/L(10.02 µkat/L) ♦Increased risk of bleeding, ≥ One: ▶PT ≥ 1.5x ULN or INR ≥ 2.0 ▶PTT ≥ 1.5x ULN ▶Intervention, ≥ One: ♦Blood product transfusion ♦Laboratory monitoring, All: ▶CBC ▶LFTs ▶Platelet count ♦HFNC and respiratory monitoring every 1-2h ♦Mechanical ventilation ♦NIPPV and, ≥ One: ▶FiO₂ > 50%(0.50) ▶Respiratory intervention every 1-2h ♦Oxygen ≥ 40%(0.40) ♦Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) ▶Non-responder, not clinically stable for discharge, One: ♦Does not meet partial responder criteria or at the Acute or Intermediate level of care, One:	Rationale: • The Episode Day structure changed from 4 days to 3-X to account for multiple conditions requiring an extended length of stay for expectant management • Acute Kidney Injury (AKI) and CRRT criteria were removed as an individual with AKI or requiring CRRT can be reviewed in the AKI subset • Criterion for continuous antihypertensive infusion was added to the Critical level of care to align with guidelines and provide a pathway for pregnant individuals with hypertensive disorders of pregnancy and severe hypertension refractory to treatment • "Arrhythmia" content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • HELLP criteria were streamlined, and a specified number for AST or ALT was changed to greater than 2x ULN to account for institutional assay variations and to align with the diagnostic criteria noted in current guidelines • Based on current guidelines, HELLP criteria were revised by aligning with primary interventions and more appropriate diagnostic benchmarks, removal of total bilirubin and fragmented or contracted red blood cells criteria, as these are not universally utilized • Oxygen ≥ 40%" criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Critical level of care • "Seizure, recurrent or refractory requiring anticonvulsant criteria" was removed because this subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy, or General Medical subset depending on the etiology

Hypertensive Disorders of Pregnancy

Episode Day 3-X (CRITICAL)

Previously Available Content	2024 March Release	Revision Description
♦Finding, ≥ One: ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: ▶Blood product transfusion ▶IV fluid resuscitation ≥1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶HELLP syndrome ≤ 2d, Both: ♦Finding, ≥ One: ▶Lab values, All: ♦Platelet count < 50,000/cu.mm(50x10⁹/L) ♦Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 70 U/L(1.17 µkat/L) ♦Finding, ≥ One: ▶Fragmented or contracted red cells on blood smear ▶Total serum lactate dehydrogenase (LDH) ≥ 600 IU/L(10.02 µkat/L) ▶Total bilirubin > 1.20 mg/dL(20.52 µmol/L) ▶Increased risk of bleeding, ≥ One: ♦PT ≥ 1.5x upper limit of normal (ULN) or INR ≥ 2.0 ♦PTT ≥ 1.5x upper limit of normal (ULN) ♦Intervention, ≥ One: ▶Blood product transfusion ▶Laboratory monitoring, All: ♦Complete blood count (CBC) ♦Liver function tests (LFT) ♦Platelet count ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Oxygen ≥ 40%(0.40), ≤ 2d ▶Seizure, recurrent or refractory requiring anticonvulsant ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure)	▶Use Extended Stay subset ▶Refer for secondary review	• Non-responder criteria were added for the patient who is not clinically stable for discharge and exceeds the designated endpoints at the Critical level of care

Infection: COVID-19



Revision Summary

Frailty criteria were added as a new clinical risk factor to identify patients who may necessitate the Acute level of care. Functional impairment criteria were added to the Acute level of care on Episode Day 3-X to capture patients who develop a new functional impairment requiring continued stay for rehabilitation initiation.

Infection: COVID-19

Initial Review (ACUTE)

Previously Available Content

- **ACUTE, One:**
 - ▶ COVID-19 test positive or high clinical suspicion and, **Both:**
 - ♦ Finding, **≥ One:**
 - ▶ O₂ sat < 94%(0.94) and < baseline
 - ▶ PaO₂/FiO₂ < 300 mmHg(40.0 kPa)
 - ▶ High risk for severe disease, **Both:**
 - ♦ Risk factor, **≥ One:**
 - ▶ Age ≥ 65
 - ▶ Comorbid condition or disease, **≥ One:**
 - ♦ Stable angina, CCS Class III or IV
 - ♦ Cerebrovascular disease
 - ♦ Chronic kidney disease and GFR < 60 mL/min(1.73 m²)
 - ♦ Chronic liver disease
 - ♦ Chronic lung disease
 - ♦ Diabetes mellitus, type 1 or type 2
 - ♦ Down syndrome
 - ♦ Heart failure, NYHA Class III or IV (current)
 - ♦ Immunocompromised
 - ♦ Malignancy
 - ♦ Obesity
 - ♦ Pregnancy
 - ♦ Sickle cell disease
 - ♦ Severe and persistent mental illness, cognitive impairment, or substance use disorder, **Both:**
 - ▶ Patient unreliable and unable to adhere to outpatient instructions or treatment
 - ▶ Caregiver unavailable or unable to manage care
 - ▶ Inflammatory or end-organ injury markers, **≥ Two:**
 - ♦ Creatinine ≥ 1.5x baseline or estimated baseline
 - ♦ CRP > 10 mg/L(1 mg/dl)
 - ♦ D-dimer > 0.5 mg/L(500 ng/ml)
 - ♦ LDH > ULN
 - ♦ Ferritin > ULN

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- **ACUTE, One:**
 - ▶ COVID-19 test positive or high clinical suspicion and, **Both:**
 - ♦ Finding, **≥ One:**
 - ▶ O₂ sat < 94%(0.94) and < baseline
 - ▶ PaO₂/FiO₂ < 300 mmHg(40.0 kPa)
 - ▶ High risk for severe disease, **Both:**
 - ♦ Risk factor, **≥ One:**
 - ▶ Comorbid condition or disease, **≥ One:**
 - ♦ Cerebrovascular disease
 - ♦ Chronic kidney disease and GFR < 60 mL/min(1.73 m²)
 - ♦ Chronic liver disease
 - ♦ Chronic lung disease
 - ♦ Diabetes mellitus, type 1 or type 2
 - ♦ Down syndrome
 - ♦ Frailty, **Both:**
 - ▶ Age ≥ 65 yrs
 - ▶ Finding, **≥ One:**
 - ♦ Frailty, active diagnosis
 - ♦ Frailty characteristics, **≥ Two:**
 - ▶ Chronic condition(s), **≥ One:**
 - ♦ Dementia or cognitive impairment
 - ♦ Multimorbidity, **≥ 2** chronic illnesses
 - ▶ Functional impairment
 - ▶ Malnutrition
 - ▶ Polypharmacy, **≥ 5** chronic medications
 - ▶ Heart failure, NYHA Class III or IV (current)
 - ♦ Immunocompromised
 - ♦ Malignancy
 - ♦ Obesity
 - ♦ Pregnancy
 - ♦ Stable angina, CCS Class III or IV
 - ♦ Sickle cell disease
 - ♦ Severe and persistent mental illness, cognitive impairment, or substance use

Rationale:

- Frailty criteria were added as a new clinical risk factor to identify frail individuals with COVID-19 who may necessitate the Acute level of care as frailty has been shown to be an independent predictor of mortality among hospitalized patients with COVID-19
- Mental status change criteria were added as it is a common symptom seen in individuals with frailty who have COVID-19
- The rule was updated to clarify the intent of the criteria and better define the patient with "Severe and persistent mental illness, cognitive impairment, or substance use disorder." In order to satisfy this rule, this criterion and both of the subcriteria below have to be selected.

| Infection: COVID-19 | | |
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| Initial Review (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦Troponin > ULN ♦Signs or symptoms, ≥ One: <ul style="list-style-type: none"> ▶Pneumonia confirmed by imaging ▶Systemic infection or inflammation, ≥ One: <ul style="list-style-type: none"> ♦Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ or $< 97.0^{\circ}\text{F}(36.1^{\circ}\text{C})$ ♦WBC, ≥ One: <ul style="list-style-type: none"> ▶$> 12,000/\text{cu.mm}(12 \times 10^9/\text{L})$ or $< 4,000/\text{cu.mm}(4 \times 10^9/\text{L})$ ▶Lymphocytes $< 1,500/\text{cu.mm}(1.5 \times 10^9/\text{L})$ ▶Bands $> 10\%(0.10)$ ▶ANC $< 500/\text{cu.mm}(500 \times 10^6/\text{L})$ ♦Heart rate $> 90/\text{min}$, sustained ♦Respiratory rate $> 20/\text{min}$, sustained ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶COVID-19 targeted therapy (includes PO; including approved therapy, emergency use authorization, or clinical trials) ▶Supplemental oxygen | <ul style="list-style-type: none"> disorder and, Both: <ul style="list-style-type: none"> ▶Patient unreliable and unable to adhere to outpatient instructions or treatment ▶Caregiver unavailable or unable to manage care ▶Inflammatory or end-organ injury markers, ≥ Two: <ul style="list-style-type: none"> ♦Creatinine $\geq 1.5 \times$ baseline or estimated baseline ♦CRP $> 10 \text{ mg/L}(1 \text{ mg/dl})$ ♦D-dimer $> 0.5 \text{ mg/L}(500 \text{ ng/ml})$ ♦LDH $> \text{ULN}$ ♦Ferritin $> \text{ULN}$ ♦Troponin $> \text{ULN}$ ♦Signs or symptoms, ≥ One: <ul style="list-style-type: none"> ▶Frailty and mental status changes (excludes coma, stupor, or obtundation) ▶Pneumonia confirmed by imaging ▶Systemic infection or inflammation, ≥ One: <ul style="list-style-type: none"> ♦Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ or $< 97.0^{\circ}\text{F}(36.1^{\circ}\text{C})$ ♦WBC, ≥ One: <ul style="list-style-type: none"> ▶$> 12,000/\text{cu.mm}(12 \times 10^9/\text{L})$ or $< 4,000/\text{cu.mm}(4 \times 10^9/\text{L})$ ▶Lymphocytes $< 1,500/\text{cu.mm}(1.5 \times 10^9/\text{L})$ ▶Bands $> 10\%(0.10)$ ▶ANC $< 500/\text{cu.mm}(500 \times 10^6/\text{L})$ ♦Heart rate $> 90/\text{min}$, sustained ♦Respiratory rate $> 20/\text{min}$, sustained ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶COVID-19 targeted therapy (includes PO; including approved therapy, emergency use authorization, or clinical trials) ▶Supplemental oxygen | |

| Infection: COVID-19 | | |
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| Initial Review (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦INTERMEDIATE, One: <ul style="list-style-type: none"> ▶COVID-19 test positive or high clinical suspicion and, ≥ One: <ul style="list-style-type: none"> ♦NIPPV and respiratory monitoring every 3-4h ♦HFNC and respiratory monitoring every 3-4h ♦Oxygen $\geq 40\%(0.40)$, $\leq 2d$ | <ul style="list-style-type: none"> ♦INTERMEDIATE, One: <ul style="list-style-type: none"> ▶COVID-19 test positive or high clinical suspicion and, ≥ One: <ul style="list-style-type: none"> ♦NIPPV and respiratory monitoring every 3-4h ♦HFNC and respiratory monitoring every 3-4h ♦Oxygen $\geq 40\%(0.40)$ | <p>Rationale:</p> <ul style="list-style-type: none"> • “Oxygen $\geq 40\%$” criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the |

| Infection: COVID-19 | | |
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| Initial Review (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| | | driver that is appropriate for the Intermediate level of care |

| Infection: COVID-19 | | |
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| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• ACUTE, One:</p> <ul style="list-style-type: none"> ▶ COVID-19 test positive or high clinical suspicion and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ O₂ sat < 94%(0.94) and < baseline ▶ PaO₂/FiO₂ < 300 mmHg(40.0 kPa) ▶ High risk for severe disease, Both: <ul style="list-style-type: none"> ♦ Risk factor, ≥ One: <ul style="list-style-type: none"> ▶ Age ≥ 65 ▶ Comorbid condition or disease, ≥ One: <ul style="list-style-type: none"> ♦ Stable angina, CCS Class III or IV ♦ Cerebrovascular disease ♦ Chronic kidney disease and GFR < 60 mL/min(1.73 m²) ♦ Chronic liver disease ♦ Chronic lung disease ♦ Diabetes mellitus, type 1 or type 2 ♦ Down syndrome ♦ Heart failure, NYHA Class III or IV (current) ♦ Immunocompromised ♦ Malignancy ♦ Obesity ♦ Pregnancy ♦ Sickle cell disease ♦ Severe and persistent mental illness, cognitive impairment, or substance use disorder, Both: <ul style="list-style-type: none"> ▶ Patient unreliable and unable to adhere to outpatient instructions or treatment ▶ Caregiver unavailable or unable to manage care ▶ Inflammatory or end-organ injury markers, ≥ Two: <ul style="list-style-type: none"> ♦ Creatinine ≥ 1.5x baseline or estimated baseline ♦ CRP > 10 mg/L(1 mg/dl) ♦ D-dimer > 0.5 mg/L(500 ng/ml) | <p>• ACUTE, One:</p> <ul style="list-style-type: none"> ▶ COVID-19 test positive or high clinical suspicion and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ O₂ sat < 94%(0.94) and < baseline ▶ PaO₂/FiO₂ < 300 mmHg(40.0 kPa) ▶ High risk for severe disease, Both: <ul style="list-style-type: none"> ♦ Risk factor, ≥ One: <ul style="list-style-type: none"> ▶ Comorbid condition or disease, ≥ One: <ul style="list-style-type: none"> ♦ Cerebrovascular disease ♦ Chronic kidney disease and GFR < 60 mL/min(1.73 m²) ♦ Chronic liver disease ♦ Chronic lung disease ♦ Diabetes mellitus, type 1 or type 2 ♦ Down syndrome ♦ Frailty, Both: <ul style="list-style-type: none"> ▶ Age ≥ 65 yrs ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ Frailty, active diagnosis ♦ Frailty characteristics, ≥ Two: <ul style="list-style-type: none"> ▶ Chronic condition(s), ≥ One: <ul style="list-style-type: none"> ♦ Dementia or cognitive impairment ♦ Multimorbidity, ≥ 2 chronic illnesses ▶ Functional impairment ▶ Malnutrition ▶ Polypharmacy, ≥ 5 chronic medications ♦ Heart failure, NYHA Class III or IV (current) ♦ Immunocompromised ♦ Malignancy ♦ Obesity ♦ Pregnancy ♦ Stable angina, CCS Class III or IV ♦ Sickle cell disease | <p>Rationale:</p> <ul style="list-style-type: none"> • Frailty criteria were added as a new clinical risk factor to identify frail individuals with COVID-19 who may necessitate the Acute level of care as frailty has been shown to be an independent predictor of mortality among hospitalized patients with COVID-19 • Mental status change criteria were added as it is a common symptom seen in individuals with frailty who have COVID-19 • The rule was updated to clarify the intent of the criteria and better define the patient with “Severe and persistent mental illness, cognitive impairment, or substance use disorder.” In order to satisfy this rule, this criterion and both of the subcriteria below have to be selected. |

| Infection: COVID-19 | | |
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| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦LDH > ULN ♦Ferritin > ULN ♦Troponin > ULN ♦Signs or symptoms, ≥ One: <ul style="list-style-type: none"> ▶Pneumonia confirmed by imaging ▶Systemic infection or inflammation, ≥ One: <ul style="list-style-type: none"> ♦Temperature ≥ 100.4°F(38.0°C) or < 97.0°F(36.1°C) ♦WBC, ≥ One: <ul style="list-style-type: none"> ▶> 12,000/cu.mm(12x10⁹/L) or < 4,000/cu.mm(4x10⁹/L) ▶Lymphocytes < 1,500/cu.mm(1.5x10⁹/L) ▶Bands > 10%(0.10) ▶ANC < 500/cu.mm(500x10⁶/L) ♦Heart rate > 90/min, sustained ♦Respiratory rate > 20/min, sustained ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶COVID-19 targeted therapy (includes PO; including approved therapy, emergency use authorization, or clinical trials) ▶Supplemental oxygen | <ul style="list-style-type: none"> ♦Severe and persistent mental illness, cognitive impairment, or substance use disorder and, Both: <ul style="list-style-type: none"> ▶Patient unreliable and unable to adhere to outpatient instructions or treatment ▶Caregiver unavailable or unable to manage care ♦Inflammatory or end-organ injury markers, ≥ Two: <ul style="list-style-type: none"> ♦Creatinine ≥ 1.5x baseline or estimated baseline ♦CRP > 10 mg/L(1 mg/dl) ♦D-dimer > 0.5 mg/L(500 ng/ml) ♦LDH > ULN ♦Ferritin > ULN ♦Troponin > ULN ♦Signs or symptoms, ≥ One: <ul style="list-style-type: none"> ▶Frailty and mental status changes (excludes coma, stupor, or obtundation) ▶Pneumonia confirmed by imaging ▶Systemic infection or inflammation, ≥ One: <ul style="list-style-type: none"> ♦Temperature ≥ 100.4°F(38.0°C) or < 97.0°F(36.1°C) ♦WBC, ≥ One: <ul style="list-style-type: none"> ▶> 12,000/cu.mm(12x10⁹/L) or < 4,000/cu.mm(4x10⁹/L) ▶Lymphocytes < 1,500/cu.mm(1.5x10⁹/L) ▶Bands > 10%(0.10) ▶ANC < 500/cu.mm(500x10⁶/L) ♦Heart rate > 90/min, sustained ♦Respiratory rate > 20/min, sustained ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶COVID-19 targeted therapy (includes PO; including approved therapy, emergency use authorization, or clinical trials) ▶Supplemental oxygen | |

| Infection: COVID-19 | | |
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| Episode Day 1 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> •INTERMEDIATE, One: <ul style="list-style-type: none"> ▶COVID-19 test positive or high clinical suspicion and, ≥ One: <ul style="list-style-type: none"> ♦NIPPV and respiratory monitoring every 3-4h | <ul style="list-style-type: none"> •INTERMEDIATE, One: <ul style="list-style-type: none"> ▶COVID-19 test positive or high clinical suspicion and, ≥ One: <ul style="list-style-type: none"> ♦NIPPV and respiratory monitoring every 3-4h | <p>Rationale:</p> <ul style="list-style-type: none"> • “Oxygen ≥ 40%” criterion was updated with removal of the endpoint to clarify the intent of the |

| Infection: COVID-19 | | |
|---|---|---|
| Episode Day 1 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦HFNC and respiratory monitoring every 3-4h ♦Oxygen $\geq 40\%(0.40)$, $\leq 2d$ | <ul style="list-style-type: none"> ♦HFNC and respiratory monitoring every 3-4h ♦Oxygen $\geq 40\%(0.40)$ | <p>criteria point, as the oxygen requirement is the driver that is appropriate for the Intermediate level of care</p> |

| Infection: COVID-19 | | |
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| Episode Day 2 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦INTERMEDIATE, $\geq$ One: <ul style="list-style-type: none"> ►NIPPV and respiratory monitoring every 3-4h ►HFNC and respiratory monitoring every 3-4h ►Oxygen $\geq 40\%(0.40)$, $\leq 2d$ | <ul style="list-style-type: none"> ♦INTERMEDIATE, $\geq$ One: <ul style="list-style-type: none"> ►NIPPV and respiratory monitoring every 3-4h ►HFNC and respiratory monitoring every 3-4h ►Oxygen $\geq 40\%(0.40)$ | <p>Rationale:</p> <ul style="list-style-type: none"> • “Oxygen $\geq 40\%$” criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Intermediate level of care |

| Infection: COVID-19 | | |
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| Episode Day 3-X (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦ACUTE, One: <ul style="list-style-type: none"> ►Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦Afebrile ♦Anti-infective regimen established and tolerated or not indicated ♦Laboratory values normal or within acceptable limits ♦O₂ sat within acceptable limits ♦Vital signs within acceptable limits ♦Tolerating PO or nutritional route established ♦Complication or comorbidity, One: <ul style="list-style-type: none"> ►No complication or active comorbidity relevant to this episode of care ►Complication or comorbidity and clinically stable | <ul style="list-style-type: none"> ♦ACUTE, One: <ul style="list-style-type: none"> ►Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦Afebrile ♦Anti-infective regimen established and tolerated or not indicated ♦Laboratory values normal or within acceptable limits ♦O₂ sat within acceptable limits ♦Vital signs within acceptable limits ♦Tolerating PO or nutritional route established ♦Complication or comorbidity, One: <ul style="list-style-type: none"> ►No complication or active comorbidity relevant to this episode of care ►Complication or comorbidity and clinically stable | <p>Rationale:</p> <ul style="list-style-type: none"> • “Functional evaluation complete” criterion was added to Responder criteria to identify patients with a new functional impairment who are appropriate for discharge • Functional impairment criteria were added to capture patients who develop a new functional impairment requiring continued stay for rehabilitation initiation |

| Infection: COVID-19 | | |
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| Episode Day 3-X (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>for discharge, One:</p> <ul style="list-style-type: none"> ♦AKI and, ≥ One: <ul style="list-style-type: none"> ►Kidney function improving or within acceptable limits ►Outpatient dialysis regimen established <p>►Partial responder, not clinically stable for discharge and requires continued stay, ≥ One:</p> <ul style="list-style-type: none"> ♦COVID-19 targeted therapy (including approved therapy, emergency use authorization, or clinical trials) ♦COVID-19 positive or high clinical suspicion and, Both: <ul style="list-style-type: none"> ►Finding, ≥ One: <ul style="list-style-type: none"> ♦Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ or $< 97.0^{\circ}\text{F}(36.1^{\circ}\text{C})$ ♦O₂ sat $< 94\%(0.94)$ and $<$ baseline ♦PaO₂/FiO₂ < 300 mmHg(40.0 kPa) ♦Heart rate $> 90/\text{min}$, sustained ♦Inflammatory or end-organ injury markers, ≥ One: <ul style="list-style-type: none"> ►D-dimer > 0.5 mg/L(500 ng/ml) ►CRP > 10 mg/L(1 mg/dl) ►LDH $>$ ULN ►Ferritin $>$ ULN ►Troponin $>$ ULN ♦WBC, ≥ One: <ul style="list-style-type: none"> ►$> 12,000/\text{cu.mm}(12 \times 10^9/\text{L})$ or $< 4,000/\text{cu.mm}(4 \times 10^9/\text{L})$ ►Lymphocytes $< 1,500/\text{cu.mm}(1.5 \times 10^9/\text{L})$ ►Bands $> 10\%(0.10)$ ►ANC $< 500/\text{cu.mm}(500 \times 10^6/\text{L})$ ►Intervention, ≥ One: <ul style="list-style-type: none"> ♦COVID-19 targeted therapy (includes PO; including approved therapy, emergency use authorization, or clinical trials) ♦Supplemental oxygen <p>♦AKI, Both:</p> <ul style="list-style-type: none"> ►Finding, ≥ One: <ul style="list-style-type: none"> ♦Oliguria ♦Creatinine, ≥ One: <ul style="list-style-type: none"> ►$\geq 1.5 \times$ baseline or estimated baseline ►≥ 0.3 mg/dl increase within last 48h ♦Dialysis initiated ►Intervention, ≥ One: <ul style="list-style-type: none"> ♦Volume expander $\leq 2\text{d}$ ♦Diuretic $\geq 2 \times /24\text{h}$, $\leq 3\text{d}$ ♦Dialysis (initial) $\leq 7\text{d}$ since initiation ♦Dialysis discontinued requiring lab monitoring $\leq 2\text{d}$ since discontinuation <p>♦Inadequate oral intake and IV fluid $\leq 4\text{d}$ and, One:</p> <ul style="list-style-type: none"> ►≥ 75 mL/h and, ≥ One: <ul style="list-style-type: none"> ♦Weight < 60 kg ♦Age ≥ 65 | <p>for discharge, ≥ One:</p> <ul style="list-style-type: none"> ♦AKI and, ≥ One: <ul style="list-style-type: none"> ►Kidney function improving or within acceptable limits ►Outpatient dialysis regimen established ♦Functional evaluation complete <p>►Partial responder, not clinically stable for discharge and requires continued stay, ≥ One:</p> <ul style="list-style-type: none"> ♦COVID-19 targeted therapy (including approved therapy, emergency use authorization, or clinical trials) ♦COVID-19 positive or high clinical suspicion and, Both: <ul style="list-style-type: none"> ►Finding, ≥ One: <ul style="list-style-type: none"> ♦Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ or $< 97.0^{\circ}\text{F}(36.1^{\circ}\text{C})$ ♦O₂ sat $< 94\%(0.94)$ and $<$ baseline ♦PaO₂/FiO₂ < 300 mmHg(40.0 kPa) ♦Heart rate $> 90/\text{min}$, sustained ♦Inflammatory or end-organ injury markers, ≥ One: <ul style="list-style-type: none"> ►D-dimer > 0.5 mg/L(500 ng/ml) ►CRP > 10 mg/L(1 mg/dl) ►LDH $>$ ULN ►Ferritin $>$ ULN ►Troponin $>$ ULN ♦WBC, ≥ One: <ul style="list-style-type: none"> ►$> 12,000/\text{cu.mm}(12 \times 10^9/\text{L})$ or $< 4,000/\text{cu.mm}(4 \times 10^9/\text{L})$ ►Lymphocytes $< 1,500/\text{cu.mm}(1.5 \times 10^9/\text{L})$ ►Bands $> 10\%(0.10)$ ►ANC $< 500/\text{cu.mm}(500 \times 10^6/\text{L})$ ►Intervention, ≥ One: <ul style="list-style-type: none"> ♦COVID-19 targeted therapy (includes PO; including approved therapy, emergency use authorization, or clinical trials) ♦Supplemental oxygen <p>♦AKI, Both:</p> <ul style="list-style-type: none"> ►Finding, ≥ One: <ul style="list-style-type: none"> ♦Oliguria ♦Creatinine, ≥ One: <ul style="list-style-type: none"> ►$\geq 1.5 \times$ baseline or estimated baseline ►≥ 0.3 mg/dl increase within last 48h ♦Dialysis initiated ►Intervention, ≥ One: <ul style="list-style-type: none"> ♦Volume expander $\leq 2\text{d}$ ♦Diuretic $\geq 2 \times /24\text{h}$, $\leq 3\text{d}$ ♦Dialysis (initial) $\leq 7\text{d}$ since initiation ♦Dialysis discontinued requiring lab monitoring $\leq 2\text{d}$ since discontinuation <p>♦Functional impairment (new) and rehabilitation initiation $\leq 24\text{h}$</p> <p>♦Inadequate oral intake and IV fluid $\leq 4\text{d}$</p> <p>♦Post Critical care $\leq 24\text{h}$</p> | <p>Revision Description</p> |

| Infection: COVID-19 | | |
|---|---|----------------------|
| Episode Day 3-X (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦Renal insufficiency ♦Heart failure, chronic ▶≥ 100 mL/h and weight ≥ 60 kg ♦Post Critical care ≤ 24h ▶Non-responder, not clinically stable for discharge and does not meet partial responder criteria, One: <ul style="list-style-type: none"> ♦Use Extended Stay subset ♦Refer for secondary review | <ul style="list-style-type: none"> ▶Non-responder, not clinically stable for discharge and does not meet partial responder criteria, One: <ul style="list-style-type: none"> ♦Use Extended Stay subset ♦Refer for secondary review | |

| Infection: COVID-19 | | |
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| Episode Day 3-X (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> •INTERMEDIATE, One: <ul style="list-style-type: none"> ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦NIPPV and respiratory monitoring every 3-4h ♦HFNC and respiratory monitoring every 3-4h ♦Oxygen ≥ 40%(0.40), ≤ 2d ▶Non-responder, not clinically stable for discharge, One: <ul style="list-style-type: none"> ♦Does not meet partial responder criteria at the Acute level of care, One: <ul style="list-style-type: none"> ▶Use Extended Stay subset ▶Refer for secondary review | <ul style="list-style-type: none"> •INTERMEDIATE, One: <ul style="list-style-type: none"> ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦NIPPV and respiratory monitoring every 3-4h ♦HFNC and respiratory monitoring every 3-4h ♦Oxygen ≥ 40%(0.40) ▶Non-responder, not clinically stable for discharge, One: <ul style="list-style-type: none"> ♦Does not meet partial responder criteria at the Acute level of care, One: <ul style="list-style-type: none"> ▶Use Extended Stay subset ▶Refer for secondary review | <p>Rationale:</p> <ul style="list-style-type: none"> • “Oxygen ≥ 40%” criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Intermediate level of care |

Infection: Endocarditis



Revision Summary

The Infection: Endocarditis subset has undergone significant updating and restructuring. The length of stay was changed from 22 days to 12 days, with Episode Day 12 being the Non-Responder Day based upon our geometric mean length of stay (GMLOS) benchmark data. Additional changes include the removal of the Modified Duke Criteria, addition of cerebral embolic event and HFNC to the Intermediate and Critical levels of care, respectively. Another change is that we now do not differentiate between subacute and acute endocarditis as both groups of patients still require the same diagnostics and interventions such as echocardiogram and IV anti-infectives.

| Infection: Endocarditis | | |
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| Initial Review (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| •ACUTE, One: <ul style="list-style-type: none"> ▶Endocarditis, subacute, One: <ul style="list-style-type: none"> ♦Modified Duke Criteria, One: <ul style="list-style-type: none"> ▶Confirmed endocarditis, One: <ul style="list-style-type: none"> ♦2 major criteria ♦1 major criterion and 3 minor criteria ♦5 minor criteria ▶Suspected endocarditis, One: <ul style="list-style-type: none"> ♦1 major criterion and 1 minor criterion ♦3 minor criteria | •ACUTE, All: <ul style="list-style-type: none"> ▶Blood culture ▶Anti-infective ▶Echocardiogram performed within 24h | Rationale: <ul style="list-style-type: none"> • The Modified Duke Criteria was removed from content and now lives in a note as it is not the driver of admission to the Acute level of care • “Blood culture,” “Anti-infective,” and “Echocardiogram performed within 24h” were added as these are required for confirmation and treatment at the Acute level of care |

| Infection: Endocarditis | | |
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| Initial Review (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| •CRITICAL, One: <ul style="list-style-type: none"> ▶Endocarditis actual or suspected and, ≥ One: <ul style="list-style-type: none"> ♦Heart failure ♦Acute kidney injury (AKI) and, One: <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> ▶Loss of P wave ▶Multifocal PVCs ▶Neuromuscular deficit ▶Widening QRS ♦Sodium, One: <ul style="list-style-type: none"> ▶< 120 mEq/L(120 mmol/L) and volume | •CRITICAL, ≥ One: <ul style="list-style-type: none"> ▶Cerebral embolic event and neurological assessment every 1-2h, ≤ 2d ▶Heart failure, acute onset ▶AKI and, One: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> ♦Loss of P wave ♦Multifocal PVCs ♦Neuromuscular deficit ♦Widening QRS ▶Sodium, One: | Rationale: <ul style="list-style-type: none"> • “Endocarditis actual or suspected and” line of criteria was removed to streamline the review process • “Cerebral embolic event and neurological assessment every 1-2h, ≤ 2d” was added to capture the patient appropriate for the Critical level of care as this is one of the more common and serious complications of endocarditis |

| Infection: Endocarditis | | |
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| Initial Review (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> overload <ul style="list-style-type: none"> ▶ > 160 mEq/L(160 mmol/L) ♦CRRT ♦Arrhythmia, ≥ One: <ul style="list-style-type: none"> ▶ Urgent electrical cardioversion performed within last 24h ▶ Defibrillation performed within last 24h ▶ Temporary pacemaker insertion performed within last 2d ▶ Antiarrhythmic and, Both: <ul style="list-style-type: none"> ♦Arrhythmia, ≥ One: <ul style="list-style-type: none"> ▶ Recurrent arrhythmia ▶ Refractory arrhythmia ♦Symptomatic, ≥ One: <ul style="list-style-type: none"> ▶ Chest pain ▶ Heart failure and, ≥ One: <ul style="list-style-type: none"> ♦Arterial Po₂ < 56 mmHg(7.4 kPa) ♦O₂ sat < 89%(0.89) and < baseline ▶ Hemodynamic instability, ≥ One: <ul style="list-style-type: none"> ♦Heart rate > 120/min sustained ♦Systolic blood pressure, ≥ One: <ul style="list-style-type: none"> ▶ < 90 mmHg ▶ < 110 mmHg with chronic hypertension ▶ > 30 mmHg decrease from baseline ▶ Labile ♦MAP < 65 mmHg ♦DIC and blood product transfusion ♦ECMO or ECLS ♦Hemodynamic instability unresponsive to ≥ 2 liter fluid bolus, requiring ongoing fluid resuscitation ≤ 24h and, Both: <ul style="list-style-type: none"> ▶ Hypotension, ≥ One: <ul style="list-style-type: none"> ♦MAP < 65 mmHg ♦Systolic blood pressure < 90 mmHg and < baseline ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦Lactic acid > 2 mmol/L(18.0 mg/dL) ♦Urine output < 0.5 mL/kg/h ♦Hemodynamic monitoring, invasive ♦IABP or percutaneous VAD ♦Mechanical ventilation ♦NIPPV and, ≥ One: <ul style="list-style-type: none"> ▶ Arterial Po₂ ≤ 39 mmHg(5.2 kPa) ▶ Arterial or venous Pco₂ ≥ 60 mmHg(8.0 kPa) and, One: <ul style="list-style-type: none"> ♦COPD and arterial or venous pH ≤ 7.24 ♦No hx of COPD ♦Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <ul style="list-style-type: none"> ♦< 120 mEq/L(120 mmol/L) and volume overload ♦> 160 mEq/L(160 mmol/L) ▶ Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦Urgent electrical cardioversion performed within last 24h ♦Defibrillation performed within last 24h ♦Temporary pacemaker insertion performed within last 2d ▶ Antiarrhythmic and, Both: <ul style="list-style-type: none"> ♦Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦Recurrent arrhythmia ♦Refractory arrhythmia ▶ Symptomatic, ≥ One: <ul style="list-style-type: none"> ♦Chest pain ♦Heart failure and, ≥ One: <ul style="list-style-type: none"> ▶ Arterial Po₂ < 56 mmHg(7.4 kPa) ▶ O₂ sat < 89%(0.89) and < baseline ♦Hemodynamic instability, ≥ One: <ul style="list-style-type: none"> ▶ Heart rate > 120/min sustained ▶ Systolic blood pressure, ≥ One: <ul style="list-style-type: none"> ♦< 90 mmHg ♦< 110 mmHg with chronic hypertension ♦> 30 mmHg decrease from baseline ♦Labile ▶ MAP < 65 mmHg ♦CRRT ▶ DIC and blood product transfusion ♦ECMO or ECLS ♦Hemodynamic instability unresponsive to ≥ 2 liter fluid bolus, requiring ongoing fluid resuscitation ≤ 24h and, Both: <ul style="list-style-type: none"> ♦Hypotension, ≥ One: <ul style="list-style-type: none"> ▶ MAP < 65 mmHg ▶ Systolic blood pressure < 90 mmHg and < baseline ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶ Lactic acid > 2 mmol/L(18.0 mg/dL) ▶ Urine output < 0.5 mL/kg/h ♦Hemodynamic monitoring, invasive ♦HFNC ♦IABP or percutaneous VAD ♦Mechanical ventilation ♦NIPPV ♦Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <ul style="list-style-type: none"> • “Acute onset” was added to the Heart failure line of criteria to clarify the intent • “HFNC” was added to reflect the patient who is appropriate for admission to the Critical level of care • NIPPV subcriteria were removed because any patient who is newly requiring noninvasive ventilator support is appropriate for the Critical level of care on Initial Review |

| Infection: Endocarditis | | |
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| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• ACUTE, One:</p> <ul style="list-style-type: none"> ▶ Endocarditis, subacute, Both: <ul style="list-style-type: none"> ♦ Modified Duke Criteria, One: <ul style="list-style-type: none"> ▶ Confirmed endocarditis, One: <ul style="list-style-type: none"> ♦ 2 major criteria ♦ 1 major criterion and 3 minor criteria ♦ 5 minor criteria ▶ Suspected endocarditis, One: <ul style="list-style-type: none"> ♦ 1 major criterion and 1 minor criterion ♦ 3 minor criteria ♦ Intervention, All: <ul style="list-style-type: none"> ▶ Anti-infective ▶ Baseline erythrocyte sedimentation rate (ESR) or C-reactive protein (CRP) ▶ Blood culture ▶ Continuous cardiac monitoring (excludes Holter) ▶ Echocardiogram scheduled or performed within 24h | <p>• ACUTE, All:</p> <ul style="list-style-type: none"> ▶ Blood culture ▶ Anti-infective ▶ Echocardiogram performed within 24h | <p>Rationale:</p> <ul style="list-style-type: none"> • The Modified Duke Criteria was removed from content and now lives in a note as it is not the driver of admission to the Acute level of care • Interventions were updated to streamline the review process and clarify the intent of the criteria |

| Infection: Endocarditis | | |
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| Episode Day 1 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• CRITICAL, One:</p> <ul style="list-style-type: none"> ▶ Endocarditis actual or suspected and, ≥ One: <ul style="list-style-type: none"> ♦ Heart failure and, All: <ul style="list-style-type: none"> ▶ Blood cultures ▶ Monitoring every 1-2h ▶ Surgical evaluation ♦ Acute kidney injury (AKI) and, Both: <ul style="list-style-type: none"> ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> ▶ Loss of P wave ▶ Multifocal PVCs ▶ Neuromuscular deficit ▶ Widening QRS ♦ Sodium, One: <ul style="list-style-type: none"> ▶ < 120 mEq/L(120 mmol/L) and volume overload ▶ > 160 mEq/L(160 mmol/L) ▶ Intervention, ≥ One: <ul style="list-style-type: none"> ♦ Calcium chloride ♦ Calcium gluconate ♦ Glucose 50%(0.50) with insulin ♦ Diuretic, continuous | <p>• CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶ Cerebral embolic event and neurological assessment every 1-2h, ≤ 2d ▶ Heart failure, acute onset ▶ AKI and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> ♦ Loss of P wave ♦ Multifocal PVCs ♦ Neuromuscular deficit ♦ Widening QRS ▶ Sodium, One: <ul style="list-style-type: none"> ♦ < 120 mEq/L(120 mmol/L) and volume overload ♦ > 160 mEq/L(160 mmol/L) ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Calcium chloride ▶ Calcium gluconate ▶ Glucose 50%(0.50) with insulin ▶ Diuretic, continuous ▶ Dialysis, One: <ul style="list-style-type: none"> ♦ Hemodialysis ≤ 7d since initiation | <p>Rationale:</p> <ul style="list-style-type: none"> • “Endocarditis actual or suspected and” line of criteria was removed to streamline the review process • “Cerebral embolic event and neurological assessment every 1-2h, ≤ 2d” was added to capture the patient appropriate for the Critical level of care as this is one of the more common and serious complications of endocarditis • “Acute onset” was added to the Heart failure line of criteria to clarify the intent • “HFNC” was added to reflect the patient who is appropriate for admission to the Critical level of care • NIPPV subcriteria were removed because any patient who is newly requiring noninvasive |

| Infection: Endocarditis | | |
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| Episode Day 1 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦Dialysis, One: <ul style="list-style-type: none"> ▸Hemodialysis ≤ 7d since initiation ▸Peritoneal dialysis ≤ 7d since initiation ♦IV fluid resuscitation ≤ 2d ♦CRRT ♦Arrhythmia, ≥ One: <ul style="list-style-type: none"> ▸Urgent electrical cardioversion performed within last 24h ▸Defibrillation performed within last 24h ▸Temporary pacemaker insertion performed within last 2d ♦Antiarrhythmic and, Both: <ul style="list-style-type: none"> ♦Arrhythmia, ≥ One: <ul style="list-style-type: none"> ▸Recurrent arrhythmia ▸Refractory arrhythmia ♦Symptomatic, ≥ One: <ul style="list-style-type: none"> ▸Chest pain ▸Heart failure and, ≥ One: <ul style="list-style-type: none"> ♦Arterial Po₂ < 56 mmHg(7.4 kPa) ♦O₂ sat < 89%(0.89) and < baseline ▸Hemodynamic instability, ≥ One: <ul style="list-style-type: none"> ♦Heart rate > 120/min sustained ♦Systolic blood pressure, ≥ One: <ul style="list-style-type: none"> ▸< 90 mmHg ▸< 110 mmHg with chronic hypertension ▸> 30 mmHg decrease from baseline ▸Labile ♦MAP < 65 mmHg ♦DIC and blood product transfusion ♦ECMO or ECLS ♦Hemodynamic instability unresponsive to ≥ 2 liter fluid bolus, requiring ongoing fluid resuscitation ≤ 24h and, Both: <ul style="list-style-type: none"> ▸Hypotension, ≥ One: <ul style="list-style-type: none"> ♦MAP < 65 mmHg ♦Systolic blood pressure < 90 mmHg and < baseline ▸Finding, ≥ One: <ul style="list-style-type: none"> ♦Lactic acid > 2 mmol/L(18.0 mg/dL) ♦Urine output < 0.5 mL/kg/h ♦Hemodynamic monitoring, invasive ♦IABP or percutaneous VAD ♦Mechanical ventilation ♦NIPPV and, ≥ One: <ul style="list-style-type: none"> ▸Arterial Po₂ ≤ 39 mmHg(5.2 kPa) ▸Arterial or venous Pco₂ ≥ 60 mmHg(8.0 kPa) and, One: <ul style="list-style-type: none"> ♦COPD and arterial or venous pH ≤ 7.24 ♦No hx of COPD ♦Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <ul style="list-style-type: none"> ♦Peritoneal dialysis ≤ 7d since initiation ▸IV fluid resuscitation ≤ 2d ♦Arrhythmia, ≥ One: <ul style="list-style-type: none"> ▸Urgent electrical cardioversion performed within last 24h ▸Defibrillation performed within last 24h ▸Temporary pacemaker insertion performed within last 2d ♦Antiarrhythmic and, Both: <ul style="list-style-type: none"> ♦Arrhythmia, ≥ One: <ul style="list-style-type: none"> ▸Recurrent arrhythmia ▸Refractory arrhythmia ♦Symptomatic, ≥ One: <ul style="list-style-type: none"> ▸Chest pain ▸Heart failure and, ≥ One: <ul style="list-style-type: none"> ▸Arterial Po₂ < 56 mmHg(7.4 kPa) ▸O₂ sat < 89%(0.89) and < baseline ▸Hemodynamic instability, ≥ One: <ul style="list-style-type: none"> ▸Heart rate > 120/min sustained ▸Systolic blood pressure, ≥ One: <ul style="list-style-type: none"> ♦< 90 mmHg ♦< 110 mmHg with chronic hypertension ♦> 30 mmHg decrease from baseline ▸Labile ▸MAP < 65 mmHg ♦CRRT ♦DIC and blood product transfusion ♦ECMO or ECLS ♦Hemodynamic instability unresponsive to ≥ 2 liter fluid bolus, requiring ongoing fluid resuscitation ≤ 24h and, Both: <ul style="list-style-type: none"> ♦Hypotension, ≥ One: <ul style="list-style-type: none"> ▸MAP < 65 mmHg ▸Systolic blood pressure < 90 mmHg and < baseline ♦Finding, ≥ One: <ul style="list-style-type: none"> ▸Lactic acid > 2 mmol/L(18.0 mg/dL) ▸Urine output < 0.5 mL/kg/h ♦Hemodynamic monitoring, invasive ♦HFNC ♦IABP or percutaneous VAD ♦Mechanical ventilation ♦NIPPV ♦Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>ventilator support is appropriate for the Critical level of care on Episode Day 1</p> |

| Infection: Endocarditis | | |
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| Episode Day 2-10 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>Episode Day 2, One:</p> <ul style="list-style-type: none"> • ACUTE, One: <ul style="list-style-type: none"> ▶ Endocarditis, subacute and, Both: <ul style="list-style-type: none"> ♦ Anti-infective ♦ Continuous cardiac monitoring (excludes Holter) | <p>Episode Day 2-10, One:</p> <ul style="list-style-type: none"> • ACUTE, ≥ One: <ul style="list-style-type: none"> ▶ Anti-infective ▶ Post Critical care ≤ 24h | <p>Rationale:</p> <ul style="list-style-type: none"> • The subset duration was changed from 22 days to 12 days reflecting the geometric mean length of stay benchmark data. The Episode Day structure was updated to align with this change. • The “Endocarditis, subacute and” line of criteria was removed to streamline the review process • “Continuous cardiac monitoring (excludes Holter)” was removed from the criteria as this is a standard of care • “Post Critical care ≤ 24h” criterion has been added as a pathway to step-down from the Critical level of care on Episode Day 2 |

| Infection: Endocarditis | | |
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| Episode Day 2-10 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• INTERMEDIATE, ≥ One:</p> <ul style="list-style-type: none"> ▶ Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦ IV antiarrhythmic and hemodynamically stable ♦ Antiarrhythmic conversion from IV to PO, One: <ul style="list-style-type: none"> ▶ No proarrhythmic risk ≤ 24h ▶ Proarrhythmic risk and continuous cardiac monitoring (excludes Holter) ≤ 3d ▶ NIPPV ▶ Oxygen ≥ 40%(0.40) ≤ 2d | <p>• INTERMEDIATE, ≥ One:</p> <ul style="list-style-type: none"> ▶ Cerebral embolic event and neurological assessment every 3-4h, ≤ 2d ▶ Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦ IV antiarrhythmic and hemodynamically stable ♦ Antiarrhythmic conversion from IV to PO, One: <ul style="list-style-type: none"> ▶ No proarrhythmic risk ≤ 24h ▶ Proarrhythmic risk and continuous cardiac monitoring (excludes Holter) ≤ 3d ▶ HFNC and respiratory monitoring every 3-4h ▶ NIPPV ▶ Oxygen ≥ 40%(0.40) | <p>Rationale:</p> <ul style="list-style-type: none"> • “Cerebral embolic event and neurological assessment every 3-4h, ≤ 2d” was added to capture the patient who is stepping down from the Critical level of care or stepping up from the Acute level of care who now requires neurological assessment every 3-4 hours • “HFNC and respiratory monitoring every 3-4h” was added to reflect the patient who is appropriate for the Intermediate level of care • “Oxygen ≥ 40%” criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the |

| Infection: Endocarditis | | |
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| Episode Day 2-10 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| | | driver that is appropriate for the Intermediate level of care |

| Infection: Endocarditis | | |
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| Episode Day 2-10 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶Acute kidney injury (AKI) and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> ♦Loss of P wave ♦Multifocal PVCs ♦Neuromuscular deficit ♦Widening QRS ▶Sodium, One: <ul style="list-style-type: none"> ♦< 120 mEq/L(120 mmol/L) and volume overload ♦> 160 mEq/L(160 mmol/L) ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶Dialysis, One: <ul style="list-style-type: none"> ♦Hemodialysis ≤ 7d since initiation ♦Peritoneal dialysis ≤ 7d since initiation ▶IV fluid resuscitation ≤ 2d ▶CRRT ▶Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦Urgent electrical cardioversion performed within last 24h ♦Defibrillation performed within last 24h ♦Temporary pacemaker insertion performed within last 2d ♦Antiarrhythmic and, Both: <ul style="list-style-type: none"> ▶Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦Recurrent arrhythmia ♦Refractory arrhythmia ▶Symptomatic, ≥ One: <ul style="list-style-type: none"> ♦Chest pain ♦Heart failure and, ≥ One: <ul style="list-style-type: none"> ▶Arterial Po₂ < 56 mmHg(7.4 kPa) | <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶Cerebral embolic event and neurological assessment every 1-2h, ≤ 2d ▶Heart failure, acute onset ▶AKI and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> ♦Loss of P wave ♦Multifocal PVCs ♦Neuromuscular deficit ♦Widening QRS ▶Sodium, One: <ul style="list-style-type: none"> ♦< 120 mEq/L(120 mmol/L) and volume overload ♦> 160 mEq/L(160 mmol/L) ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶Dialysis, One: <ul style="list-style-type: none"> ♦Hemodialysis ≤ 7d since initiation ♦Peritoneal dialysis ≤ 7d since initiation ▶IV fluid resuscitation ≤ 2d ▶Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦Urgent electrical cardioversion performed within last 24h ♦Defibrillation performed within last 24h ♦Temporary pacemaker insertion performed within last 2d ♦Antiarrhythmic and, Both: <ul style="list-style-type: none"> ▶Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦Recurrent arrhythmia ♦Refractory arrhythmia ▶Symptomatic, ≥ One: <ul style="list-style-type: none"> ♦Chest pain ♦Heart failure and, ≥ One: | <p>Rationale:</p> <ul style="list-style-type: none"> • “Cerebral embolic event and neurological assessment every 1-2h, ≤ 2d” was added to capture the patient appropriate for the Critical level of care as this is one of the more common and serious complications of endocarditis • “Heart failure, acute onset” line of criteria was added to continued stay as these patients may require surgical consultation and close monitoring for worsening of symptoms, development of cardiogenic shock, or worsening valvular pathology • “HFNC and respiratory monitoring every 1-2h” was added to reflect the patient who is appropriate for the Critical level of care |

| Infection: Endocarditis | | |
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| Episode Day 2-10 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ▶O₂ sat < 89%(0.89) and < baseline ♦Hemodynamic instability, ≥ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min, sustained ▶Systolic blood pressure, ≥ One: <ul style="list-style-type: none"> ♦< 90 mmHg ♦< 110 mmHg with chronic hypertension ♦> 30 mmHg decrease from baseline ♦Labile ▶MAP < 65 mmHg ▶DIC and blood product transfusion ▶ECMO or ECLS ▶Hemodynamic instability unresponsive to ≥ 2 liter fluid bolus, requiring ongoing fluid resuscitation ≤ 24h and, Both: <ul style="list-style-type: none"> ♦Hypotension, ≥ One: <ul style="list-style-type: none"> ▶MAP < 65 mmHg ▶Systolic blood pressure < 90 mmHg and < baseline ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Lactic acid > 2 mmol/L(18.0 mg/dL) ▶Urine output < 0.5 mL/kg/h ▶Hemodynamic monitoring, invasive ▶IABP or percutaneous VAD ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <ul style="list-style-type: none"> ▶Arterial Po₂ < 56 mmHg(7.4 kPa) ▶O₂ sat < 89%(0.89) and < baseline ♦Hemodynamic instability, ≥ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min, sustained ▶Systolic blood pressure, ≥ One: <ul style="list-style-type: none"> ♦< 90 mmHg ♦< 110 mmHg with chronic hypertension ♦> 30 mmHg decrease from baseline ♦Labile ▶MAP < 65 mmHg ▶CRRT ▶DIC and blood product transfusion ▶ECMO or ECLS ▶Hemodynamic instability unresponsive to ≥ 2 liter fluid bolus, requiring ongoing fluid resuscitation ≤ 24h and, Both: <ul style="list-style-type: none"> ♦Hypotension, ≥ One: <ul style="list-style-type: none"> ▶MAP < 65 mmHg ▶Systolic blood pressure < 90 mmHg and < baseline ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Lactic acid > 2 mmol/L(18.0 mg/dL) ▶Urine output < 0.5 mL/kg/h ▶Hemodynamic monitoring, invasive ▶HFNC and respiratory monitoring every 1-2h ▶IABP or percutaneous VAD ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | |

| Infection: Endocarditis | | |
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| Episode Day 11 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>Episode Day 3-13, One:</p> <ul style="list-style-type: none"> ♦ACUTE, One: <ul style="list-style-type: none"> ▶Endocarditis, subacute and, Both: <ul style="list-style-type: none"> ♦Anti-infective ♦Continuous cardiac monitoring (excludes Holter) | <p>Episode Day 11, One:</p> <ul style="list-style-type: none"> ♦ACUTE, One: <ul style="list-style-type: none"> ▶Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦Anti-infective, One: <ul style="list-style-type: none"> ▶IV therapy arranged ▶PO therapy established | <p>Rationale:</p> <ul style="list-style-type: none"> • Responder criteria were added to capture the patient appropriate for discharge • Partial responder criteria were added to capture the patient who is appropriate for continued stay at the Acute level of care |

| Infection: Endocarditis | | |
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| Episode Day 11 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| | <ul style="list-style-type: none"> ♦Blood cultures negative ♦Complication or comorbidity, One: <ul style="list-style-type: none"> ▶No complication or active comorbidity relevant to this episode of care ▶Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> ♦AKI, ≥ One: <ul style="list-style-type: none"> ▶Kidney function improving or within acceptable limits ▶Outpatient dialysis regimen established ♦Adverse drug reaction resolving ♦<i>C. difficile</i> infection, symptoms resolving, and tolerating PO ♦Functional impairment and, One: <ul style="list-style-type: none"> ▶Discharge to home and, One: <ul style="list-style-type: none"> ♦Patient or caregiver able to safely manage impairment ♦Home care services arranged ▶Discharge to inpatient facility for rehabilitative services ♦Heart failure resolving and, Both: <ul style="list-style-type: none"> ▶Dyspnea resolving or at baseline ▶O₂ sat ≥ 92%(0.92) or within acceptable limits ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦Endocarditis, One: <ul style="list-style-type: none"> ▶Anti-infective ▶Transition to oral anti-infective ≤ 24h and discharge planning ♦AKI, Both: <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦Oliguria ♦Creatinine, ≥ One: <ul style="list-style-type: none"> ▶≥ 1.5x baseline or estimated baseline ▶≥ 0.3 mg/dl increase within last 48h ♦Dialysis initiated ▶Intervention, ≥ One: <ul style="list-style-type: none"> ♦Volume expander ≤ 2d ♦Diuretic ≥ 2x/24h, ≤ 3d ♦Dialysis (initial) ≤ 7d since initiation ♦Dialysis discontinued requiring lab monitoring ≤ 2d since discontinuation ♦Immunosuppressant therapy ≤ 3d ♦Adverse drug reaction requiring change in medication ≤ 24h ♦<i>C. difficile</i> infection, actual or suspected and, Both: <ul style="list-style-type: none"> ▶Anti-infective (includes PO) ≤ 5d ▶Inadequate oral intake ♦Functional impairment (new) requiring rehabilitation initiation ≤ 24h and, ≥ One: <ul style="list-style-type: none"> ▶PT evaluation and training ▶OT evaluation and training | <ul style="list-style-type: none"> • The Episode Days were restructured based upon the Geometric mean length of stay benchmark data |

| Infection: Endocarditis | | |
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| Episode Day 11 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| | <ul style="list-style-type: none"> ▶Speech-language evaluation and training ▶Cognitive evaluation and retraining ▶Swallowing evaluation and retraining ♦Heart failure and, Both: <ul style="list-style-type: none"> ▶Dyspnea > baseline and, ≥ One: <ul style="list-style-type: none"> ♦O₂ sat ≤ 91%(0.91) and < baseline ♦Worsening edema ♦Heart failure confirmed by imaging ♦Intervention, One: <ul style="list-style-type: none"> ♦Diuretic, One: <ul style="list-style-type: none"> ▶≥ 2x/24h (includes PO) ▶At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: <ul style="list-style-type: none"> ♦Blood urea nitrogen (BUN) increased from baseline ♦Creatinine increased from baseline ♦Dialysis ♦Post Critical care ≤ 24h | |

| Infection: Endocarditis | | |
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| Episode Day 11 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> •INTERMEDIATE, ≥ One: <ul style="list-style-type: none"> ▶Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦IV antiarrhythmic and hemodynamically stable ♦Antiarrhythmic conversion from IV to PO, One: <ul style="list-style-type: none"> ▶No proarrhythmic risk ≤ 24h ▶Proarrhythmic risk and continuous cardiac monitoring (excludes Holter) ≤ 3d ▶NIPPV ▶Oxygen ≥ 40%(0.40) ≤ 2d | <ul style="list-style-type: none"> •INTERMEDIATE, ≥ One: <ul style="list-style-type: none"> ▶Cerebral embolic event and neurological assessment every 3-4h, ≤ 2d ▶Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦IV antiarrhythmic and hemodynamically stable ♦Antiarrhythmic conversion from IV to PO, One: <ul style="list-style-type: none"> ▶No proarrhythmic risk ≤ 24h ▶Proarrhythmic risk and continuous cardiac monitoring (excludes Holter) ≤ 3d ▶HFNC and respiratory monitoring every 3-4h ▶NIPPV ▶Oxygen ≥ 40%(0.40) | <p>Rationale:</p> <ul style="list-style-type: none"> • “Cerebral embolic event and neurological assessment every 3-4h, ≤ 2d” was added to capture the patient who is stepping down from the Critical level of care or stepping up from the Acute level of care who now requires neurological assessment every 3-4 hours • “HFNC and respiratory monitoring every 3-4h” was added to reflect the patient who is appropriate for the Intermediate level of care • “Oxygen ≥ 40%” criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Intermediate level of care |

| Infection: Endocarditis | | |
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| Episode Day 11 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶ Acute kidney injury (AKI) and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> ♦ Loss of P wave ♦ Multifocal PVCs ♦ Neuromuscular deficit ♦ Widening QRS ▶ Sodium, One: <ul style="list-style-type: none"> ♦ < 120 mEq/L(120 mmol/L) and volume overload ♦ > 160 mEq/L(160 mmol/L) ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Calcium chloride ▶ Calcium gluconate ▶ Glucose 50%(0.50) with insulin ▶ Diuretic, continuous ▶ Dialysis, One: <ul style="list-style-type: none"> ♦ Hemodialysis ≤ 7d since initiation ♦ Peritoneal dialysis ≤ 7d since initiation ▶ IV fluid resuscitation ≤ 2d ▶ CRRT ▶ Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦ Urgent electrical cardioversion performed within last 24h ♦ Defibrillation performed within last 24h ♦ Temporary pacemaker insertion performed within last 2d ♦ Antiarrhythmic and, Both: <ul style="list-style-type: none"> ▶ Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦ Recurrent arrhythmia ♦ Refractory arrhythmia ▶ Symptomatic, ≥ One: <ul style="list-style-type: none"> ♦ Chest pain ♦ Heart failure and, ≥ One: <ul style="list-style-type: none"> ▶ Arterial Po₂ < 56 mmHg(7.4 kPa) ▶ O₂ sat < 89%(0.89) and < baseline ♦ Hemodynamic instability, ≥ One: <ul style="list-style-type: none"> ▶ Heart rate > 120/min, sustained ▶ Systolic blood pressure, ≥ One: <ul style="list-style-type: none"> ♦ < 90 mmHg ♦ < 110 mmHg with chronic hypertension ♦ > 30 mmHg decrease from baseline ♦ Labile ▶ MAP < 65 mmHg ▶ DIC and blood product transfusion ▶ ECMO or ECLS ▶ Hemodynamic instability unresponsive to ≥ 2 liter fluid bolus, requiring ongoing fluid resuscitation ≤ 24h and, Both: | <p>• CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶ Cerebral embolic event and neurological assessment every 1-2h, ≤ 2d ▶ Heart failure, acute onset ▶ AKI and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> ♦ Loss of P wave ♦ Multifocal PVCs ♦ Neuromuscular deficit ♦ Widening QRS ▶ Sodium, One: <ul style="list-style-type: none"> ♦ < 120 mEq/L(120 mmol/L) and volume overload ♦ > 160 mEq/L(160 mmol/L) ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Calcium chloride ▶ Calcium gluconate ▶ Glucose 50%(0.50) with insulin ▶ Diuretic, continuous ▶ Dialysis, One: <ul style="list-style-type: none"> ♦ Hemodialysis ≤ 7d since initiation ♦ Peritoneal dialysis ≤ 7d since initiation ▶ IV fluid resuscitation ≤ 2d ▶ Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦ Urgent electrical cardioversion performed within last 24h ♦ Defibrillation performed within last 24h ♦ Temporary pacemaker insertion performed within last 2d ♦ Antiarrhythmic and, Both: <ul style="list-style-type: none"> ▶ Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦ Recurrent arrhythmia ♦ Refractory arrhythmia ▶ Symptomatic, ≥ One: <ul style="list-style-type: none"> ♦ Chest pain ♦ Heart failure and, ≥ One: <ul style="list-style-type: none"> ▶ Arterial Po₂ < 56 mmHg(7.4 kPa) ▶ O₂ sat < 89%(0.89) and < baseline ♦ Hemodynamic instability, ≥ One: <ul style="list-style-type: none"> ▶ Heart rate > 120/min, sustained ▶ Systolic blood pressure, ≥ One: <ul style="list-style-type: none"> ♦ < 90 mmHg ♦ < 110 mmHg with chronic hypertension ♦ > 30 mmHg decrease from baseline ♦ Labile ▶ MAP < 65 mmHg ▶ CRRT ▶ DIC and blood product transfusion ▶ ECMO or ECLS | <p>Rationale:</p> <ul style="list-style-type: none"> • “Cerebral embolic event and neurological assessment every 1-2h, ≤ 2d” was added to capture the patient appropriate for the Critical level of care as this is one of the more common and serious complications of endocarditis • “Heart failure, acute onset” line of criteria was added to continued stay as these patients may require surgical consultation and close monitoring for worsening of symptoms, development of cardiogenic shock, or worsening valvular pathology • “HFNC and respiratory monitoring every 1-2h” was added to reflect the patient who is appropriate for the Critical level of care |

| Infection: Endocarditis | | |
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| Episode Day 11 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦Hypotension, ≥ One: <ul style="list-style-type: none"> ▶MAP < 65 mmHg ▶Systolic blood pressure < 90 mmHg and < baseline ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Lactic acid > 2 mmol/L(18.0 mg/dL) ▶Urine output < 0.5 mL/kg/h ▶Hemodynamic monitoring, invasive ▶IABP or percutaneous VAD ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <ul style="list-style-type: none"> ▶Hemodynamic instability unresponsive to ≥ 2 liter fluid bolus, requiring ongoing fluid resuscitation ≤ 24h and, Both: <ul style="list-style-type: none"> ♦Hypotension, ≥ One: <ul style="list-style-type: none"> ▶MAP < 65 mmHg ▶Systolic blood pressure < 90 mmHg and < baseline ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Lactic acid > 2 mmol/L(18.0 mg/dL) ▶Urine output < 0.5 mL/kg/h ▶Hemodynamic monitoring, invasive ▶HFNC and respiratory monitoring every 1-2h ▶IABP or percutaneous VAD ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | |

Infection: GI/GYN



Revision Summary

Organ dysfunction or hypoperfusion criteria rule was changed to clarify the intent of the criteria.

Infection: GI/GYN

Initial Review (ACUTE)

Previously Available Content

- **ACUTE, ≥ One:**
 - ▶ Abscess, intra-abdominal or pelvic, **Both:**
 - ♦ Confirmed by imaging
 - ♦ Anti-infective
 - ▶ Appendicitis and nonoperative management, **Both:**
 - ♦ Appendiceal abscess or phlegmon by imaging
 - ♦ Anti-infective
 - ▶ Acute (ascending) cholangitis, actual or suspected and, **All:**
 - ♦ Systemic inflammation, **≥ One:**
 - ▶ Temperature ≥ 100.4°F (38.0°C)
 - ▶ Shaking chills
 - ▶ WBC ≥ 10,000/cu.mm (10x10⁹/L)
 - ▶ WBC ≤ 4,000/cu.mm (4x10⁹/L)
 - ▶ CRP ≥ 1mg/dL (10mg/L)
 - ▶ Procalcitonin ≥ ULN
 - ♦ Finding, **≥ One:**
 - ▶ Cholestasis, **≥ One:**
 - ♦ Jaundice or bilirubin ≥ 2mg/dL (34.2μmol/L)
 - ♦ Alkaline phosphatase (ALP) > 1.5x ULN
 - ♦ Alanine transaminase (ALT) > 1.5x ULN
 - ♦ Aspartate aminotransferase (AST) > 1.5x ULN
 - ▶ Imaging finding, **≥ One:**
 - ♦ Biliary dilation
 - ♦ Biliary stone, stricture, or stent
 - ♦ Obstructing mass
 - ♦ Anti-infective
 - ▶ *C. difficile* infection and, **Both:**
 - ♦ Finding, **≥ One:**
 - ▶ Temperature > 101.3°F (38.5°C)
 - ▶ WBC ≥ 15,000/cu.mm (15x10⁹/L)
 - ▶ Creatinine ≥ 1.5x baseline or estimated baseline
 - ♦ Intervention, **Both:**
 - ▶ Anti-infective (includes PO)
 - ▶ IV fluid
 - ▶ Diverticulitis and, **Both:**
 - ♦ Finding, **≥ One:**
 - ▶ Complicated CT finding
 - ♦ Comorbid, **≥ One:**

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- **ACUTE, ≥ One:**
 - ▶ Abscess, intra-abdominal or pelvic, **Both:**
 - ♦ Confirmed by imaging
 - ♦ Anti-infective
 - ▶ Appendicitis and nonoperative management, **Both:**
 - ♦ Appendiceal abscess or phlegmon by imaging
 - ♦ Anti-infective
 - ▶ Acute (ascending) cholangitis, actual or suspected and, **All:**
 - ♦ Systemic inflammation, **≥ One:**
 - ▶ Temperature ≥ 100.4°F (38.0°C)
 - ▶ Shaking chills
 - ▶ WBC ≥ 10,000/cu.mm (10x10⁹/L)
 - ▶ WBC ≤ 4,000/cu.mm (4x10⁹/L)
 - ▶ CRP ≥ 1mg/dL (10mg/L)
 - ▶ Procalcitonin ≥ ULN
 - ♦ Finding, **≥ One:**
 - ▶ Cholestasis, **≥ One:**
 - ♦ Jaundice or bilirubin ≥ 2mg/dL (34.2μmol/L)
 - ♦ Alkaline phosphatase (ALP) > 1.5x ULN
 - ♦ Alanine transaminase (ALT) > 1.5x ULN
 - ♦ Aspartate aminotransferase (AST) > 1.5x ULN
 - ▶ Imaging finding, **≥ One:**
 - ♦ Biliary dilation
 - ♦ Biliary stone, stricture, or stent
 - ♦ Obstructing mass
 - ♦ Anti-infective
 - ▶ *C. difficile* infection and, **Both:**
 - ♦ Finding, **≥ One:**
 - ▶ Temperature > 101.3°F (38.5°C)
 - ▶ WBC ≥ 15,000/cu.mm (15x10⁹/L)
 - ▶ Creatinine ≥ 1.5x baseline or estimated baseline
 - ♦ Intervention, **Both:**
 - ▶ Anti-infective (includes PO)
 - ▶ IV fluid
 - ▶ Diverticulitis and, **Both:**
 - ♦ Finding, **≥ One:**
 - ▶ Complicated CT finding
 - ♦ Comorbid, **≥ One:**

Rationale:

- Organ dysfunction or hypoperfusion criteria rule was changed to ≥ One to clarify the intent of the criteria and to allow lactic acid to be selected as well as the risk findings

| Infection: GI/GYN | | |
|--|--|----------------------|
| Initial Review (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦End stage liver disease ♦Immunocompromised ♦Renal disease and chronic dialysis ▶Organ dysfunction or hypoperfusion, One: <ul style="list-style-type: none"> ♦Moderate risk finding, ≥ Two: <ul style="list-style-type: none"> ▶Creatinine $\geq 1.5\times$ and $< 2\times$ baseline or estimated baseline ▶Mental status changes (excludes coma, stupor, or obtundation) and GCS 13-14 ▶Platelet count 100,000/cu.mm($100\times 10^9/L$) to 150,000/cu.mm($150\times 10^9/L$) and $<$ baseline ▶Total bilirubin 1.2 mg/dL($20.52\text{ }\mu\text{mol/L}$) to 1.9 mg/dL($32.49\text{ }\mu\text{mol/L}$) and $>$ baseline ▶O₂ sat $\leq 91\%$(0.91) and $<$ baseline ♦High risk finding, ≥ One: <ul style="list-style-type: none"> ▶Creatinine $\geq 2\times$ baseline or estimated baseline ▶Mental status changes (excludes coma, stupor, or obtundation) and GCS 10-12 ▶Platelet count $< 100,000/\text{cu.mm}$ ($100\times 10^9/L$) and $<$ baseline ▶Total bilirubin $\geq 2\text{ mg/dL}$($34.2\text{ }\mu\text{mol/L}$) and $>$ baseline ♦Lactic acid $> \text{ULN}$ and $< 4\text{ mmol/L}$(36.0 mg/dL) ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Anti-infective ▶Abscess drainage $\leq 24\text{h}$ ▶Pelvic inflammatory disease and, ≥ One: <ul style="list-style-type: none"> ♦Pregnancy ♦Severe illness, ≥ One: <ul style="list-style-type: none"> ▶Temperature $> 101^\circ\text{F}$(38.3°C) ▶Vomiting, protracted ♦Tubo-ovarian abscess confirmed by imaging ♦Persistent symptoms despite outpatient anti-infective (includes PO) treatment $\geq 3\text{d}$ and, ≥ One: <ul style="list-style-type: none"> ▶Abdominal pain, persistent ▶Temperature $\geq 100.4^\circ\text{F}$(38.0°C) ▶SBP and, All: <ul style="list-style-type: none"> ♦Polymorphonuclear leukocyte count $> 250/\text{cu.mm}$($0.25\times 10^9/L$) ♦Albumin ♦Anti-infective | <ul style="list-style-type: none"> ♦End stage liver disease ♦Immunocompromised ♦Renal disease and chronic dialysis ▶Organ dysfunction or hypoperfusion, ≥ One: <ul style="list-style-type: none"> ♦Moderate risk finding, ≥ Two: <ul style="list-style-type: none"> ▶Creatinine $\geq 1.5\times$ and $< 2\times$ baseline or estimated baseline ▶Mental status changes (excludes coma, stupor, or obtundation) and GCS 13-14 ▶Platelet count 100,000/cu.mm($100\times 10^9/L$) to 150,000/cu.mm($150\times 10^9/L$) and $<$ baseline ▶Total bilirubin 1.2 mg/dL($20.52\text{ }\mu\text{mol/L}$) to 1.9 mg/dL($32.49\text{ }\mu\text{mol/L}$) and $>$ baseline ▶O₂ sat $\leq 91\%$(0.91) and $<$ baseline ♦High risk finding, ≥ One: <ul style="list-style-type: none"> ▶Creatinine $\geq 2\times$ baseline or estimated baseline ▶Mental status changes (excludes coma, stupor, or obtundation) and GCS 10-12 ▶Platelet count $< 100,000/\text{cu.mm}$ ($100\times 10^9/L$) and $<$ baseline ▶Total bilirubin $\geq 2\text{ mg/dL}$($34.2\text{ }\mu\text{mol/L}$) and $>$ baseline ♦Lactic acid $> \text{ULN}$ and $< 4\text{ mmol/L}$(36.0 mg/dL) ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Anti-infective ▶Abscess drainage $\leq 24\text{h}$ ▶Pelvic inflammatory disease and, ≥ One: <ul style="list-style-type: none"> ♦Pregnancy ♦Severe illness, ≥ One: <ul style="list-style-type: none"> ▶Temperature $> 101^\circ\text{F}$(38.3°C) ▶Vomiting, protracted ♦Tubo-ovarian abscess confirmed by imaging ♦Persistent symptoms despite outpatient anti-infective (includes PO) treatment $\geq 3\text{d}$ and, ≥ One: <ul style="list-style-type: none"> ▶Abdominal pain, persistent ▶Temperature $\geq 100.4^\circ\text{F}$(38.0°C) ▶SBP and, All: <ul style="list-style-type: none"> ♦Polymorphonuclear leukocyte count $> 250/\text{cu.mm}$($0.25\times 10^9/L$) ♦Albumin ♦Anti-infective | |

| Infection: GI/GYN | | |
|--|--|---|
| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• ACUTE, ≥ One:</p> <ul style="list-style-type: none"> ▶ Abscess, intra-abdominal or pelvic, Both: <ul style="list-style-type: none"> ◆ Confirmed by imaging ◆ Anti-infective ▶ Appendicitis and nonoperative management, Both: <ul style="list-style-type: none"> ◆ Appendiceal abscess or phlegmon by imaging ◆ Anti-infective ▶ Acute (ascending) cholangitis, actual or suspected and, All: <ul style="list-style-type: none"> ◆ Systemic inflammation, ≥ One: <ul style="list-style-type: none"> ▶ Temperature ≥ 100.4°F (38.0°C) ▶ Shaking chills ▶ WBC ≥ 10,000/cu.mm (10x10⁹/L) ▶ WBC ≤ 4,000/cu.mm (4x10⁹/L) ▶ CRP ≥ 1mg/dL (10mg/L) ▶ Procalcitonin ≥ ULN ◆ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Cholestasis, ≥ One: <ul style="list-style-type: none"> ◆ Jaundice or bilirubin ≥ 2mg/dL (34.2μmol/L) ◆ Alkaline phosphatase (ALP) > 1.5x ULN ◆ Alanine transaminase (ALT) > 1.5x ULN ◆ Aspartate aminotransferase (AST) > 1.5x ULN ▶ Imaging finding, ≥ One: <ul style="list-style-type: none"> ◆ Biliary dilation ◆ Biliary stone, stricture, or stent ◆ Obstructing mass ◆ Anti-infective ▶ <i>C. difficile</i> infection and, Both: <ul style="list-style-type: none"> ◆ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Temperature > 101.3°F (38.5°C) ▶ WBC ≥ 15,000/cu.mm (15x10⁹/L) ▶ Creatinine ≥ 1.5x baseline or estimated baseline ▶ Hospital acquired during inpatient hospitalization ◆ Intervention, Both: <ul style="list-style-type: none"> ▶ Anti-infective (includes PO) ▶ IV fluid ▶ Diverticulitis and, Both: <ul style="list-style-type: none"> ◆ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Complicated CT finding ▶ Comorbid, ≥ One: <ul style="list-style-type: none"> ◆ End stage liver disease ◆ Immunocompromised ◆ Renal disease and chronic dialysis ▶ Organ dysfunction or hypoperfusion, One: <ul style="list-style-type: none"> ◆ Moderate risk finding, ≥ Two: <ul style="list-style-type: none"> ▶ Creatinine ≥ 1.5x and < 2x baseline or estimated baseline ▶ Mental status changes (excludes coma, stupor, or obtundation) and GCS 13-14 ▶ Platelet count 100,000/cu.mm (100x10⁹/L) to 150,000/cu.mm (150x10⁹/L) and | <p>• ACUTE, ≥ One:</p> <ul style="list-style-type: none"> ▶ Abscess, intra-abdominal or pelvic, Both: <ul style="list-style-type: none"> ◆ Confirmed by imaging ◆ Anti-infective ▶ Appendicitis and nonoperative management, Both: <ul style="list-style-type: none"> ◆ Appendiceal abscess or phlegmon by imaging ◆ Anti-infective ▶ Acute (ascending) cholangitis, actual or suspected and, All: <ul style="list-style-type: none"> ◆ Systemic inflammation, ≥ One: <ul style="list-style-type: none"> ▶ Temperature ≥ 100.4°F (38.0°C) ▶ Shaking chills ▶ WBC ≥ 10,000/cu.mm (10x10⁹/L) ▶ WBC ≤ 4,000/cu.mm (4x10⁹/L) ▶ CRP ≥ 1mg/dL (10mg/L) ▶ Procalcitonin ≥ ULN ◆ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Cholestasis, ≥ One: <ul style="list-style-type: none"> ◆ Jaundice or bilirubin ≥ 2mg/dL (34.2μmol/L) ◆ Alkaline phosphatase (ALP) > 1.5x ULN ◆ Alanine transaminase (ALT) > 1.5x ULN ◆ Aspartate aminotransferase (AST) > 1.5x ULN ▶ Imaging finding, ≥ One: <ul style="list-style-type: none"> ◆ Biliary dilation ◆ Biliary stone, stricture, or stent ◆ Obstructing mass ◆ Anti-infective ▶ <i>C. difficile</i> infection and, Both: <ul style="list-style-type: none"> ◆ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Temperature > 101.3°F (38.5°C) ▶ WBC ≥ 15,000/cu.mm (15x10⁹/L) ▶ Creatinine ≥ 1.5x baseline or estimated baseline ▶ Hospital acquired during inpatient hospitalization ◆ Intervention, Both: <ul style="list-style-type: none"> ▶ Anti-infective (includes PO) ▶ IV fluid ▶ Diverticulitis and, Both: <ul style="list-style-type: none"> ◆ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Complicated CT finding ▶ Comorbid, ≥ One: <ul style="list-style-type: none"> ◆ End stage liver disease ◆ Immunocompromised ◆ Renal disease and chronic dialysis ▶ Organ dysfunction or hypoperfusion, ≥ One: <ul style="list-style-type: none"> ◆ Moderate risk finding, ≥ Two: <ul style="list-style-type: none"> ▶ Creatinine ≥ 1.5x and < 2x baseline or estimated baseline ▶ Mental status changes (excludes coma, stupor, or obtundation) and GCS 13-14 ▶ Platelet count 100,000/cu.mm (100x10⁹/L) to 150,000/cu.mm (150x10⁹/L) and | <p>Rationale:</p> <ul style="list-style-type: none"> • Organ dysfunction or hypoperfusion criteria rule was changed to ≥ One to clarify the intent of the criteria and to allow lactic acid to be selected as well as the risk findings |

| Infection: GI/GYN | | |
|---|---|----------------------|
| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> < baseline ▶Total bilirubin 1.2 mg/dL(20.52 µmol/L) to 1.9 mg/dL(32.49 µmol/L) and > baseline ▶O₂ sat ≤ 91%(0.91) and < baseline ♦High risk finding, ≥ One: <ul style="list-style-type: none"> ▶Creatinine ≥ 2x baseline or estimated baseline ▶Mental status changes (excludes coma, stupor, or obtundation) and GCS 10-12 ▶Platelet count < 100,000/cu.mm (100x10⁹/L) and < baseline ▶Total bilirubin ≥ 2 mg/dL(34.2 µmol/L) and > baseline ♦Lactic acid > ULN and < 4 mmol/L (36.0 mg/dL) ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Anti-infective ▶Abscess drainage ≤ 24h ▶Pelvic inflammatory disease and, Both: <ul style="list-style-type: none"> ♦Finding, One: <ul style="list-style-type: none"> ▶Pregnancy ▶Severe illness, ≥ One: <ul style="list-style-type: none"> ♦Temperature > 101°F(38.3°C) ♦Vomiting, protracted ▶Tubo-ovarian abscess confirmed by imaging ▶Persistent symptoms despite outpatient anti-infective (includes PO) treatment ≥ 3d and, ≥ One: <ul style="list-style-type: none"> ♦Abdominal pain, persistent ♦Temperature ≥ 100.4°F(38.0°C) ♦Anti-infective ▶SBP and, All: <ul style="list-style-type: none"> ♦Polymorphonuclear leukocyte count > 250/cu.mm(0.25x10⁹/L) ♦Albumin ♦Anti-infective | <ul style="list-style-type: none"> < baseline ▶Total bilirubin 1.2 mg/dL(20.52 µmol/L) to 1.9 mg/dL(32.49 µmol/L) and > baseline ▶O₂ sat ≤ 91%(0.91) and < baseline ♦High risk finding, ≥ One: <ul style="list-style-type: none"> ▶Creatinine ≥ 2x baseline or estimated baseline ▶Mental status changes (excludes coma, stupor, or obtundation) and GCS 10-12 ▶Platelet count < 100,000/cu.mm (100x10⁹/L) and < baseline ▶Total bilirubin ≥ 2 mg/dL(34.2 µmol/L) and > baseline ♦Lactic acid > ULN and < 4 mmol/L (36.0 mg/dL) ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Anti-infective ▶Abscess drainage ≤ 24h ▶Pelvic inflammatory disease and, Both: <ul style="list-style-type: none"> ♦Finding, One: <ul style="list-style-type: none"> ▶Pregnancy ▶Severe illness, ≥ One: <ul style="list-style-type: none"> ♦Temperature > 101°F(38.3°C) ♦Vomiting, protracted ▶Tubo-ovarian abscess confirmed by imaging ▶Persistent symptoms despite outpatient anti-infective (includes PO) treatment ≥ 3d and, ≥ One: <ul style="list-style-type: none"> ♦Abdominal pain, persistent ♦Temperature ≥ 100.4°F(38.0°C) ♦Anti-infective ▶SBP and, All: <ul style="list-style-type: none"> ♦Polymorphonuclear leukocyte count > 250/cu.mm(0.25x10⁹/L) ♦Albumin ♦Anti-infective | |

Infection: Pneumonia



Revision Summary

Frailty criteria were added as a new clinical risk factor to identify patients who may necessitate the Acute level of care. Functional impairment criteria were added to the Acute level of care on Episode Day 2-4 to capture patients who develop a new functional impairment requiring continued stay for rehabilitation initiation.

Infection: Pneumonia

Initial Review (ACUTE)

| Previously Available Content | 2024 March Release | Revision Description |
|---|---|---|
| <p>• ACUTE, Both:</p> <ul style="list-style-type: none"> ▶ Pneumonia confirmed by imaging ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ O₂ sat < 89%(0.89) and < baseline ♦ Arterial Po₂ < 56 mmHg(7.4 kPa) and < baseline ♦ Arterial or venous Pco₂ 45-54 mmHg(6.0-7.2 kPa) and pH 7.31-7.35 ♦ Complicated pneumonia ♦ High risk for adverse event, Both: <ul style="list-style-type: none"> ▶ O₂ sat 89-91%(0.89-0.91) and < baseline ▶ Clinical risk factor, ≥ One: <ul style="list-style-type: none"> ♦ COPD, Grade 3 or 4 (current) ♦ Heart failure, NYHA Class III or IV (current) ♦ Severe and persistent mental illness or substance use disorder, Both: <ul style="list-style-type: none"> ▶ Patient unreliable and unable to adhere to outpatient instructions or treatment ▶ Caregiver unavailable or unable to manage care ♦ Immunocompromised ♦ Pneumonia Severity Index (PSI) Risk Class IV or V (PSI score ≥ 91) ♦ CURB-65 criteria, ≥ Three: <ul style="list-style-type: none"> ▶ New onset confusion ▶ Blood urea nitrogen (BUN) ≥ 20 mg/dL(7 mmol/L) ▶ Respiratory rate ≥ 30/min ▶ Systolic blood pressure < 90 mmHg or diastolic blood pressure ≤ 60 mmHg ▶ Age ≥ 65 years | <p>• ACUTE, Both:</p> <ul style="list-style-type: none"> ▶ Pneumonia confirmed by imaging ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ O₂ sat < 89%(0.89) and < baseline ♦ Arterial Po₂ < 56 mmHg(7.4 kPa) and < baseline ♦ Arterial or venous Pco₂ 45-54 mmHg(6.0-7.2 kPa) and pH 7.31-7.35 ♦ Complicated pneumonia ♦ Frailty, All: <ul style="list-style-type: none"> ▶ Age ≥ 65 yrs ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ Frailty, active diagnosis ♦ Frailty characteristics, ≥ Two: <ul style="list-style-type: none"> ▶ Chronic condition(s), ≥ One: <ul style="list-style-type: none"> ♦ Dementia or cognitive impairment ♦ Multimorbidity, ≥ 2 chronic illnesses ▶ Functional impairment ▶ Malnutrition ▶ Polypharmacy, ≥ 5 chronic medications ▶ Symptom, ≥ One: <ul style="list-style-type: none"> ♦ O₂ sat 89-91%(0.89-0.91) and < baseline ♦ Pneumonia Severity Index (PSI) Risk Class III (PSI score 71-90) ♦ CURB-65 criteria, ≥ One: <ul style="list-style-type: none"> ▶ New onset confusion ▶ Blood urea nitrogen (BUN) ≥ 20 mg/dL(7 mmol/L) ▶ Respiratory rate ≥ 30/min ▶ Systolic blood pressure < 90 mmHg or diastolic blood pressure ≤ 60 mmHg ♦ High risk for adverse event, Both: <ul style="list-style-type: none"> ▶ O₂ sat 89-91%(0.89-0.91) and < baseline ▶ Clinical risk factor, ≥ One: <ul style="list-style-type: none"> ♦ COPD, Grade 3 or 4 (current) ♦ Heart failure, NYHA Class III or IV (current) ♦ Severe and persistent mental illness or substance use disorder and, Both: <ul style="list-style-type: none"> ▶ Patient unreliable and unable to adhere to outpatient instructions or treatment ▶ Caregiver unavailable or unable to manage | <p>Rationale:</p> <ul style="list-style-type: none"> • Frailty criteria were added to identify frail individuals with pneumonia who may necessitate the Acute level of care as this patient population has been associated with more severe disease, higher mortality rates, and an increased length of stay • The rule was updated to clarify the intent of the criteria and better define the patient with "Severe and persistent mental illness, cognitive impairment, or substance use disorder." In order to satisfy this rule, this criterion and both of the subcriteria below have to be selected. |

| Infection: Pneumonia | | |
|------------------------------|--|----------------------|
| Initial Review (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| | care
♦Immunocompromised
♦Pneumonia Severity Index (PSI) Risk Class IV or V (PSI score ≥ 91)
♦CURB-65 criteria, ≥ Three :
▶New onset confusion
▶Blood urea nitrogen (BUN) ≥ 20 mg/dL(7 mmol/L)
▶Respiratory rate ≥ 30/min
▶Systolic blood pressure < 90 mmHg or diastolic blood pressure ≤ 60 mmHg
▶Age ≥ 65 years | |

| Infection: Pneumonia | | |
|---|---|--|
| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| •ACUTE, One:
▶Pneumonia confirmed by imaging and, Both :
♦Finding, ≥ One :
▶O ₂ sat < 89%(0.89) and < baseline
▶Arterial Po ₂ < 56 mmHg(7.4 kPa) and < baseline
▶Arterial or venous Pco ₂ 45-54 mmHg(6.0-7.2 kPa) and pH 7.31-7.35
▶Complicated pneumonia
▶High risk for adverse event, Both :
♦O ₂ sat 89-91%(0.89-0.91) and < baseline
♦Clinical risk factor, ≥ One :
▶COPD, Grade 3 or 4 (current)
▶Heart failure, NYHA Class III or IV (current)
▶Severe and persistent mental illness or substance use disorder, Both :
♦Patient unreliable and unable to adhere to outpatient instructions or treatment
♦Caregiver unavailable or unable to manage care
▶Immunocompromised
▶Pneumonia Severity Index (PSI) Risk Class IV or V (PSI score ≥ 91)
▶CURB-65 criteria, ≥ Three :
♦New onset confusion
♦Blood urea nitrogen (BUN) ≥ 20 mg/dL(7 mmol/L)
♦Respiratory rate ≥ 30/min
♦Systolic blood pressure < 90 mmHg or | •ACUTE, One:
▶Pneumonia confirmed by imaging and, Both :
♦Finding, ≥ One :
▶O ₂ sat < 89%(0.89) and < baseline
▶Arterial Po ₂ < 56 mmHg(7.4 kPa) and < baseline
▶Arterial or venous Pco ₂ 45-54 mmHg(6.0-7.2 kPa) and pH 7.31-7.35
▶Complicated pneumonia
▶Frailty, All :
♦Age ≥ 65 yrs
♦Finding, ≥ One :
▶Frailty, active diagnosis
▶Frailty characteristics, ≥ Two :
♦Chronic condition(s), ≥ One :
▶Dementia or cognitive impairment
▶Multimorbidity, ≥ 2 chronic illnesses
♦Functional impairment
♦Malnutrition
♦Polypharmacy, ≥ 5 chronic medications
♦Symptom, ≥ One :
▶O ₂ sat 89-91%(0.89-0.91) and < baseline
▶Pneumonia Severity Index (PSI) Risk Class III (PSI score 71-90)
▶CURB-65 criteria, ≥ One :
♦New onset confusion
♦Blood urea nitrogen (BUN) ≥ 20 mg/dL(7 mmol/L)
♦Respiratory rate ≥ 30/min | Rationale: <ul style="list-style-type: none"> Frailty criteria were added to identify frail individuals with pneumonia who may necessitate the Acute level of care as this patient population has been associated with more severe disease, higher mortality rates, and an increased length of stay The rule was updated to clarify the intent of the criteria and better define the patient with "Severe and persistent mental illness, cognitive impairment, or substance use disorder." In order to satisfy this rule, this criterion and both of the subcriteria below have to be selected. |

| Infection: Pneumonia | | |
|---|---|----------------------|
| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> diastolic blood pressure $\leq$ 60 mmHg ♦Age $\geq$ 65 years ►Comorbid pneumonia in a hospitalized patient confirmed by imaging ♦Intervention, $\geq$ One: <ul style="list-style-type: none"> ►Anti-infective ►PO anti-infective and, $\geq$ One: <ul style="list-style-type: none"> ♦IV fluid, One: <ul style="list-style-type: none"> ►$\geq$ 75 mL/h and, $\geq$ One: <ul style="list-style-type: none"> ♦Weight $<$ 60 kg ♦Age $\geq$ 65 ♦Renal insufficiency ♦Heart failure, chronic ►$\geq$ 100 mL/h and weight $\geq$ 60 kg ♦Requiring supplemental oxygen | <ul style="list-style-type: none"> ♦Systolic blood pressure $<$ 90 mmHg or diastolic blood pressure $\leq$ 60 mmHg ►High risk for adverse event, Both: <ul style="list-style-type: none"> ♦O₂ sat 89-91%(0.89-0.91) and $<$ baseline ♦Clinical risk factor, $\geq$ One: <ul style="list-style-type: none"> ►COPD, Grade 3 or 4 (current) ►Heart failure, NYHA Class III or IV (current) ►Severe and persistent mental illness or substance use disorder and, Both: <ul style="list-style-type: none"> ♦Patient unreliable and unable to adhere to outpatient instructions or treatment ♦Caregiver unavailable or unable to manage care ►Immunocompromised ►Pneumonia Severity Index (PSI) Risk Class IV or V (PSI score $\geq$ 91) ►CURB-65 criteria, $\geq$ Three: <ul style="list-style-type: none"> ♦New onset confusion ♦Blood urea nitrogen (BUN) $\geq$ 20 mg/dL(7 mmol/L) ♦Respiratory rate $\geq$ 30/min ♦Systolic blood pressure $<$ 90 mmHg or diastolic blood pressure $\leq$ 60 mmHg ♦Age $\geq$ 65 years ►Comorbid pneumonia in a hospitalized patient confirmed by imaging ♦Intervention, $\geq$ One: <ul style="list-style-type: none"> ►Anti-infective ►PO anti-infective and, $\geq$ One: <ul style="list-style-type: none"> ♦IV fluid ♦Requiring supplemental oxygen | |

| Infection: Pneumonia | | |
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| Episode Day 2 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> •ACUTE, One: <ul style="list-style-type: none"> ►Early responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦Anti-infective, One: <ul style="list-style-type: none"> ►Arranged ►Completed ►Not indicated ♦O₂ sat within acceptable limits ♦Laboratory values normal or within acceptable limits | <ul style="list-style-type: none"> •ACUTE, One: <ul style="list-style-type: none"> ►Early responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦Anti-infective, One: <ul style="list-style-type: none"> ►Arranged ►Completed ►Not indicated ♦O₂ sat within acceptable limits ♦Laboratory values normal or within acceptable limits | <ul style="list-style-type: none"> Rationale: <ul style="list-style-type: none"> • Functional impairment criteria were added to capture patients who develop a new functional impairment requiring continued stay for rehabilitation initiation |

| Infection: Pneumonia | | |
|---|---|---|
| Episode Day 2 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦Tolerating PO or nutritional route established ♦No complication or active comorbidity relevant to this episode of care ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦Pneumonia and, ≥ One: <ul style="list-style-type: none"> ▶Anti-infective ▶PO anti-infective and, ≥ One: <ul style="list-style-type: none"> ♦O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ♦Respiratory intervention ≥ 4x/24h ▶Chest tube placement within last 24h ♦Aspiration pneumonitis, actual or suspected and, ≥ One: <ul style="list-style-type: none"> ▶Assessment of swallowing function ≤ 24h ▶Feeding tube insertion within last 24h ♦COPD and, All: <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦O₂ sat ≤ 91%(0.91) and < baseline ♦Arterial Po₂ ≤ 60 mmHg(8.0 kPa) ♦Arterial or venous Pco₂ > baseline ▶Bronchodilator ≥ 4x/24h ▶Corticosteroid (includes PO or inhaled) ▶Oxygenation, One: <ul style="list-style-type: none"> ♦O₂ sat ≥ 92%(0.92) or baseline ♦O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ♦Heart failure and, Both: <ul style="list-style-type: none"> ▶Dyspnea > baseline and, ≥ One: <ul style="list-style-type: none"> ♦O₂ sat ≤ 91%(0.91) and < baseline ♦Worsening edema ♦Heart failure confirmed by imaging ▶Intervention, ≥ One: <ul style="list-style-type: none"> ♦Diuretic, One: <ul style="list-style-type: none"> ▶≥ 2x/24h (includes PO) ▶At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: <ul style="list-style-type: none"> ♦Blood urea nitrogen (BUN) increased from baseline ♦Creatinine increased from baseline ♦Dialysis ♦Inadequate oral intake and IV fluid and, One: <ul style="list-style-type: none"> ▶≥ 75 mL/h and, ≥ One: <ul style="list-style-type: none"> ♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ▶≥ 100 mL/h and weight ≥ 60 kg ♦Post Critical care ≤ 24h | <ul style="list-style-type: none"> ♦Tolerating PO or nutritional route established ♦No complication or active comorbidity relevant to this episode of care ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦Pneumonia and, ≥ One: <ul style="list-style-type: none"> ▶Anti-infective ▶PO anti-infective and, ≥ One: <ul style="list-style-type: none"> ♦O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ♦Respiratory intervention ≥ 4x/24h ▶Chest tube placement within last 24h ♦Aspiration pneumonitis, actual or suspected and, ≥ One: <ul style="list-style-type: none"> ▶Assessment of swallowing function ≤ 24h ▶Feeding tube insertion within last 24h ♦COPD and, All: <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦O₂ sat ≤ 91%(0.91) and < baseline ♦Arterial Po₂ ≤ 60 mmHg(8.0 kPa) ♦Arterial or venous Pco₂ > baseline ▶Bronchodilator ≥ 4x/24h ▶Corticosteroid (includes PO or inhaled) ▶Oxygenation, One: <ul style="list-style-type: none"> ♦O₂ sat ≥ 92%(0.92) or baseline ♦O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ♦Functional impairment (new) and rehabilitation initiation ≤ 24h ♦Heart failure and, Both: <ul style="list-style-type: none"> ▶Dyspnea > baseline and, ≥ One: <ul style="list-style-type: none"> ♦O₂ sat ≤ 91%(0.91) and < baseline ♦Worsening edema ♦Heart failure confirmed by imaging ▶Intervention, ≥ One: <ul style="list-style-type: none"> ♦Diuretic, One: <ul style="list-style-type: none"> ▶≥ 2x/24h (includes PO) ▶At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: <ul style="list-style-type: none"> ♦Blood urea nitrogen (BUN) increased from baseline ♦Creatinine increased from baseline ♦Dialysis ♦Inadequate oral intake and IV fluid ♦Post Critical care ≤ 24h | <ul style="list-style-type: none"> ♦Functional impairment (new) and rehabilitation initiation ≤ 24h |

| Infection: Pneumonia | | |
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| Episode Day 3 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• ACUTE, One:</p> <ul style="list-style-type: none"> ▶ Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦ Anti-infective, One: <ul style="list-style-type: none"> ▶ Arranged ▶ Completed ▶ Not indicated ♦ Laboratory values normal or within acceptable limits ♦ O₂ sat within acceptable limits ♦ Tolerating PO or nutritional route established ♦ Complication or comorbidity, One: <ul style="list-style-type: none"> ▶ No complication or active comorbidity relevant to this episode of care ▶ Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> ♦ Aspiration pneumonitis and, One: <ul style="list-style-type: none"> ▶ Able to swallow without aspirating ▶ Feeding tube insertion ♦ Chest tube and, One: <ul style="list-style-type: none"> ▶ Heimlich valve ▶ Removed and no reaccumulation of air or fluid confirmed by chest x-ray (CXR) ♦ COPD and, ≥ One: <ul style="list-style-type: none"> ▶ O₂ sat within acceptable limits ▶ Arterial Po₂ > 60 mmHg(8.0 kPa) or within acceptable limits ▶ Arterial or venous Pco₂ within acceptable limits ♦ Heart failure resolving and, Both: <ul style="list-style-type: none"> ▶ Dyspnea resolving or at baseline ▶ O₂ sat within acceptable limits ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦ Pneumonia and, ≥ One: <ul style="list-style-type: none"> ▶ Anti-infective ▶ PO anti-infective and, ≥ One: <ul style="list-style-type: none"> ♦ O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ♦ Respiratory intervention ≥ 4x/24h ♦ Aspiration pneumonitis, actual or suspected and, ≥ One: <ul style="list-style-type: none"> ▶ Assessment of swallowing function ≤ 24h ▶ Feeding tube insertion within last 24h ♦ Chest tube and, ≥ One: <ul style="list-style-type: none"> ▶ Suction, continuous ▶ Drainage ≥ 250 mL/24h ▶ Repositioning within last 24h ▶ Water seal within last 24h ♦ COPD and, All: <ul style="list-style-type: none"> ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ O₂ sat ≤ 91%(0.91) and < baseline ♦ Arterial Po₂ ≤ 60 mmHg(8.0 kPa) | <p>• ACUTE, One:</p> <ul style="list-style-type: none"> ▶ Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦ Anti-infective, One: <ul style="list-style-type: none"> ▶ Arranged ▶ Completed ▶ Not indicated ♦ Laboratory values normal or within acceptable limits ♦ O₂ sat within acceptable limits ♦ Tolerating PO or nutritional route established ♦ Complication or comorbidity, One: <ul style="list-style-type: none"> ▶ No complication or active comorbidity relevant to this episode of care ▶ Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> ♦ Aspiration pneumonitis and, One: <ul style="list-style-type: none"> ▶ Able to swallow without aspirating ▶ Feeding tube insertion ♦ Chest tube and, One: <ul style="list-style-type: none"> ▶ Heimlich valve ▶ Removed and no reaccumulation of air or fluid confirmed by chest x-ray (CXR) ♦ COPD and, ≥ One: <ul style="list-style-type: none"> ▶ O₂ sat within acceptable limits ▶ Arterial Po₂ > 60 mmHg(8.0 kPa) or within acceptable limits ▶ Arterial or venous Pco₂ within acceptable limits ♦ Heart failure resolving and, Both: <ul style="list-style-type: none"> ▶ Dyspnea resolving or at baseline ▶ O₂ sat within acceptable limits ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦ Pneumonia and, ≥ One: <ul style="list-style-type: none"> ▶ Anti-infective ▶ PO anti-infective and, ≥ One: <ul style="list-style-type: none"> ♦ O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ♦ Respiratory intervention ≥ 4x/24h ♦ Aspiration pneumonitis, actual or suspected and, ≥ One: <ul style="list-style-type: none"> ▶ Assessment of swallowing function ≤ 24h ▶ Feeding tube insertion within last 24h ♦ Chest tube and, ≥ One: <ul style="list-style-type: none"> ▶ Suction, continuous ▶ Drainage ≥ 250 mL/24h ▶ Repositioning within last 24h ▶ Water seal within last 24h ♦ COPD and, All: <ul style="list-style-type: none"> ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ O₂ sat ≤ 91%(0.91) and < baseline | <p>Rationale:</p> <ul style="list-style-type: none"> • “Functional evaluation complete” criterion was added to Responder criteria to identify patients with a new functional impairment who are appropriate for discharge • Functional impairment criteria were added to capture patients who develop a new functional impairment requiring continued stay for rehabilitation initiation |

| Infection: Pneumonia | | |
|---|--|----------------------|
| Episode Day 3 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦Arterial or venous Pco₂ > baseline ▶Bronchodilator ≥ 4x/24h ▶Corticosteroid (includes PO or inhaled) ▶Oxygenation, One: <ul style="list-style-type: none"> ♦O₂ sat ≥ 92%(0.92) or baseline ♦O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ♦Heart failure and, Both: <ul style="list-style-type: none"> ▶Dyspnea > baseline and, ≥ One: <ul style="list-style-type: none"> ♦O₂ sat ≤ 91%(0.91) and < baseline ♦Worsening edema ♦Heart failure confirmed by imaging ▶Intervention, ≥ One: <ul style="list-style-type: none"> ♦Diuretic, One: <ul style="list-style-type: none"> ▶≥ 2x/24h (includes PO) ▶At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: <ul style="list-style-type: none"> ♦Blood urea nitrogen (BUN) increased from baseline ♦Creatinine increased from baseline ♦Dialysis ♦Inadequate oral intake and IV fluid and, One: <ul style="list-style-type: none"> ▶≥ 75 mL/h and, ≥ One: <ul style="list-style-type: none"> ♦Weight < 60 kg ♦Age ≥ 65 ♦Renal insufficiency ♦Heart failure, chronic ▶≥ 100 mL/h and weight ≥ 60 kg ♦Post Critical care ≤ 24 | <ul style="list-style-type: none"> ♦Arterial Po₂ ≤ 60 mmHg(8.0 kPa) ♦Arterial or venous Pco₂ > baseline ▶Bronchodilator ≥ 4x/24h ▶Corticosteroid (includes PO or inhaled) ▶Oxygenation, One: <ul style="list-style-type: none"> ♦O₂ sat ≥ 92%(0.92) or baseline ♦O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ♦Functional impairment (new) and rehabilitation initiation ≤ 24h ♦Heart failure and, Both: <ul style="list-style-type: none"> ▶Dyspnea > baseline and, ≥ One: <ul style="list-style-type: none"> ♦O₂ sat ≤ 91%(0.91) and < baseline ♦Worsening edema ♦Heart failure confirmed by imaging ▶Intervention, ≥ One: <ul style="list-style-type: none"> ♦Diuretic, One: <ul style="list-style-type: none"> ▶≥ 2x/24h (includes PO) ▶At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: <ul style="list-style-type: none"> ♦Blood urea nitrogen (BUN) increased from baseline ♦Creatinine increased from baseline ♦Dialysis ♦Inadequate oral intake and IV fluid ♦Post Critical care ≤ 24h | |

| Infection: Pneumonia | | |
|---|---|---|
| Episode Day 4 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦ACUTE, One: <ul style="list-style-type: none"> ▶Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦Anti-infective, One: <ul style="list-style-type: none"> ▶Arranged ▶Completed ▶Not indicated ♦Laboratory values normal or within acceptable limits ♦O₂ sat within acceptable limits ♦Tolerating PO or nutritional route established | <ul style="list-style-type: none"> ♦ACUTE, One: <ul style="list-style-type: none"> ▶Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦Anti-infective, One: <ul style="list-style-type: none"> ▶Arranged ▶Completed ▶Not indicated ♦Laboratory values normal or within acceptable limits ♦O₂ sat within acceptable limits ♦Tolerating PO or nutritional route established | <p>Rationale:</p> <ul style="list-style-type: none"> • “Functional evaluation complete” criterion was added to Responder criteria to identify patients with a new functional impairment who are appropriate for discharge • Functional impairment criteria were added to capture patients who develop a new functional |

| Infection: Pneumonia | | |
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| Episode Day 4 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ◆Complication or comorbidity, One: <ul style="list-style-type: none"> ▶No complication or active comorbidity relevant to this episode of care ▶Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> ◆Aspiration pneumonitis and, One: <ul style="list-style-type: none"> ▶Able to swallow without aspirating ▶Feeding tube insertion ◆Chest tube and, One: <ul style="list-style-type: none"> ▶Heimlich valve ▶Removed and no reaccumulation of air or fluid confirmed by chest x-ray (CXR) ◆COPD and, ≥ One: <ul style="list-style-type: none"> ▶O₂ sat within acceptable limits ▶Arterial Po₂ > 60 mmHg(8.0 kPa) or within acceptable limits ▶Arterial or venous Pco₂ within acceptable limits ◆Heart failure resolving and, Both: <ul style="list-style-type: none"> ▶Dyspnea resolving or at baseline ▶O₂ sat within acceptable limits ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ◆Pneumonia and, ≥ One: <ul style="list-style-type: none"> ▶Anti-infective ▶PO anti-infective and, ≥ One: <ul style="list-style-type: none"> ◆O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ◆Respiratory intervention ≥ 4x/24h ◆Aspiration pneumonitis, actual or suspected and, ≥ One: <ul style="list-style-type: none"> ▶Assessment of swallowing function ≤ 24h ▶Feeding tube insertion within last 24h ◆Chest tube and, ≥ One: <ul style="list-style-type: none"> ▶Suction, continuous ▶Drainage ≥ 250 mL/24h ▶Repositioning within last 24h ▶Water seal within last 24h ◆COPD and, All: <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ◆O₂ sat ≤ 91%(0.91) and < baseline ◆Arterial Po₂ ≤ 60 mmHg(8.0 kPa) ◆Arterial or venous Pco₂ > baseline ▶Bronchodilator ≥ 4x/24h ▶Corticosteroid (includes PO or inhaled) ▶Oxygenation, One: <ul style="list-style-type: none"> ◆O₂ sat ≥ 92%(0.92) or baseline ◆O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ◆Heart failure and, Both: <ul style="list-style-type: none"> ▶Dyspnea > baseline and, ≥ One: <ul style="list-style-type: none"> ◆O₂ sat ≤ 91%(0.91) and < baseline ◆Worsening edema ▶Heart failure confirmed by imaging | <ul style="list-style-type: none"> ◆Complication or comorbidity, One: <ul style="list-style-type: none"> ▶No complication or active comorbidity relevant to this episode of care ▶Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> ◆Aspiration pneumonitis and, One: <ul style="list-style-type: none"> ▶Able to swallow without aspirating ▶Feeding tube insertion ◆Chest tube and, One: <ul style="list-style-type: none"> ▶Heimlich valve ▶Removed and no reaccumulation of air or fluid confirmed by chest x-ray (CXR) ◆COPD and, ≥ One: <ul style="list-style-type: none"> ▶O₂ sat within acceptable limits ▶Arterial Po₂ > 60 mmHg(8.0 kPa) or within acceptable limits ▶Arterial or venous Pco₂ within acceptable limits ◆Functional evaluation complete ◆Heart failure resolving and, Both: <ul style="list-style-type: none"> ▶Dyspnea resolving or at baseline ▶O₂ sat within acceptable limits ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ◆Pneumonia and, ≥ One: <ul style="list-style-type: none"> ▶Anti-infective ▶PO anti-infective and, ≥ One: <ul style="list-style-type: none"> ◆O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ◆Respiratory intervention ≥ 4x/24h ◆Aspiration pneumonitis, actual or suspected and, ≥ One: <ul style="list-style-type: none"> ▶Assessment of swallowing function ≤ 24h ▶Feeding tube insertion within last 24h ◆Chest tube and, ≥ One: <ul style="list-style-type: none"> ▶Suction, continuous ▶Drainage ≥ 250 mL/24h ▶Repositioning within last 24h ▶Water seal within last 24h ◆COPD and, All: <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ◆O₂ sat ≤ 91%(0.91) and < baseline ◆Arterial Po₂ ≤ 60 mmHg(8.0 kPa) ◆Arterial or venous Pco₂ > baseline ▶Bronchodilator ≥ 4x/24h ▶Corticosteroid (includes PO or inhaled) ▶Oxygenation, One: <ul style="list-style-type: none"> ◆O₂ sat ≥ 92%(0.92) or baseline ◆O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ◆Functional impairment (new) and rehabilitation initiation ≤ 24h ◆Heart failure and, Both: <ul style="list-style-type: none"> ▶Dyspnea > baseline and, ≥ One: | <p>impairment requiring continued stay for rehabilitation initiation</p> |

| Infection: Pneumonia | | |
|---|--|----------------------|
| Episode Day 4 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> Intervention, ≥ One: <ul style="list-style-type: none"> Diuretic, One: <ul style="list-style-type: none"> ≥ 2x/24h (includes PO) At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: <ul style="list-style-type: none"> Blood urea nitrogen (BUN) increased from baseline Creatinine increased from baseline Dialysis Inadequate oral intake and IV fluid and, One: <ul style="list-style-type: none"> ≥ 75 mL/h and, ≥ One: <ul style="list-style-type: none"> Weight < 60 kg Age ≥ 65 Renal insufficiency Heart failure, chronic ≥ 100 mL/h and weight ≥ 60 kg Post Critical care ≤ 24h | <ul style="list-style-type: none"> O₂ sat ≤ 91%(0.91) and < baseline Worsening edema Heart failure confirmed by imaging Intervention, ≥ One: <ul style="list-style-type: none"> Diuretic, One: <ul style="list-style-type: none"> ≥ 2x/24h (includes PO) At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: <ul style="list-style-type: none"> Blood urea nitrogen (BUN) increased from baseline Creatinine increased from baseline Dialysis Inadequate oral intake and IV fluid Post Critical care ≤ 24h | |

| Infection: Pneumonia | | |
|--|--|---|
| Episode Day 5 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ACUTE, One: <ul style="list-style-type: none"> Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> Anti-infective, One: <ul style="list-style-type: none"> Arranged Completed Not indicated Laboratory values normal or within acceptable limits O₂ sat within acceptable limits Tolerating PO or nutritional route established Complication or comorbidity, One: <ul style="list-style-type: none"> No complication or active comorbidity relevant to this episode of care Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> Aspiration pneumonia and, One: <ul style="list-style-type: none"> Able to swallow without aspirating Feeding tube insertion Chest tube and, One: <ul style="list-style-type: none"> Heimlich valve Removed and no reaccumulation of air or fluid confirmed by chest x-ray (CXR) | <ul style="list-style-type: none"> ACUTE, One: <ul style="list-style-type: none"> Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> Anti-infective, One: <ul style="list-style-type: none"> Arranged Completed Not indicated Laboratory values normal or within acceptable limits O₂ sat within acceptable limits Tolerating PO or nutritional route established Complication or comorbidity, One: <ul style="list-style-type: none"> No complication or active comorbidity relevant to this episode of care Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> Aspiration pneumonia and, One: <ul style="list-style-type: none"> Able to swallow without aspirating Feeding tube insertion Chest tube and, One: <ul style="list-style-type: none"> Heimlich valve Removed and no reaccumulation of air or fluid confirmed by chest x-ray (CXR) | <p>Rationale:</p> <ul style="list-style-type: none"> “Functional evaluation complete” criterion was added to Responder criteria to identify patients with a new functional impairment who are appropriate for discharge |

Infection: Pneumonia

Episode Day 5 (ACUTE)

| Previously Available Content | 2024 March Release | Revision Description |
|--|---|----------------------|
| <ul style="list-style-type: none">♦COPD and, ≥ One:<ul style="list-style-type: none">►O₂ sat within acceptable limits►Arterial Po₂ > 60 mmHg(8.0 kPa) or within acceptable limits►Arterial or venous Pco₂ within acceptable limits♦Heart failure resolving and, Both:<ul style="list-style-type: none">►Dyspnea resolving or at baseline►O₂ sat within acceptable limits►Non-responder, not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, One:<ul style="list-style-type: none">♦Use Extended Stay subset♦Refer for secondary review | <ul style="list-style-type: none">♦COPD and, ≥ One:<ul style="list-style-type: none">►O₂ sat within acceptable limits►Arterial Po₂ > 60 mmHg(8.0 kPa) or within acceptable limits►Arterial or venous Pco₂ within acceptable limits♦Functional evaluation complete♦Heart failure resolving and, Both:<ul style="list-style-type: none">►Dyspnea resolving or at baseline►O₂ sat within acceptable limits►Non-responder, not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, One:<ul style="list-style-type: none">♦Use Extended Stay subset♦Refer for secondary review | |

Infection: Pyelonephritis or Complex UTI



Revision Summary

SIRS and multi-system dysfunction criteria were removed from the Critical level of care. "Multidrug-resistant bacteria (excludes colonization)" criterion was added to the Acute level of care on Episode Day 3.

Infection: Pyelonephritis or Complex UTI

Initial Review (CRITICAL)

Previously Available Content

•CRITICAL, One:

►Sepsis, actual or suspected, **All:**

♦SIRS, **≥ Two:**

►Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ or $< 97.0^{\circ}\text{F}(36.1^{\circ}\text{C})$

►WBC, **≥ One:**

♦ $> 12,000/\text{cu.mm}(12 \times 10^9/\text{L})$ or $< 4,000/\text{cu.mm}(4 \times 10^9/\text{L})$

♦Bands $> 10\%(0.10)$

►Heart rate $> 90/\text{min}$, sustained

►Respiratory rate $> 20/\text{min}$, sustained

♦Organ dysfunction or severe hypoperfusion, **≥ One:**

►ARDS

► $\text{PaO}_2/\text{FiO}_2 < 200 \text{ mmHg}(27.0 \text{ kPa})$

►Coma, stupor, obtundation, or GCS ≤ 9

►DIC

►Hemodynamic instability unresponsive to ≥ 2 liter fluid bolus, requiring ongoing fluid resuscitation $\leq 24\text{h}$ and, **Both:**

♦Hypotension, **≥ One:**

►MAP $< 65 \text{ mmHg}$

►Systolic blood pressure $< 90 \text{ mmHg}$ and $< \text{baseline}$

♦Finding, **≥ One:**

►Lactic acid $> 2 \text{ mmol/L}(18.0 \text{ mg/dL})$

►Urine output $< 0.5 \text{ mL/kg/h}$

►Multi-system dysfunction, **≥ Two:**

♦Serum creatinine $\geq 2 \times$ baseline or estimated baseline

♦Mental status changes (excludes coma, stupor, or obtundation) and GCS 10-12

♦INR > 1.5 without anticoagulation and $> \text{baseline}$

♦Platelet count $< 100,000/\text{cu.mm}(100 \times 10^9/\text{L})$ and $< \text{baseline}$

♦Total bilirubin $\geq 2 \text{ mg/dL}(34.2 \mu\text{mol/L})$ and $> \text{baseline}$

♦Intervention, **≥ One:**

►IV fluid resuscitation $\leq 2\text{d}$

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•CRITICAL, One:

►Sepsis, actual or suspected, **≥ One:**

♦ARDS

♦Coma, stupor, obtundation, or GCS ≤ 9

♦CRRT

♦DIC and blood product transfusion

♦ECMO or ECLS

♦Hemodynamic instability unresponsive to ≥ 2 liter fluid bolus, requiring ongoing fluid resuscitation $\leq 24\text{h}$ and, **Both:**

►Hypotension, **≥ One:**

♦MAP $< 65 \text{ mmHg}$

♦Systolic blood pressure $< 90 \text{ mmHg}$ and $< \text{baseline}$

►Finding, **≥ One:**

♦Lactic acid $> 2 \text{ mmol/L}(18.0 \text{ mg/dL})$

♦Urine output $< 0.5 \text{ mL/kg/h}$

♦Hemodynamic monitoring, invasive

♦HFNC and respiratory monitoring every 1-2h

♦Mechanical ventilation

♦NIPPV and, **≥ One:**

►Arterial $\text{Po}_2 \leq 39 \text{ mmHg}(5.2 \text{ kPa})$

►Arterial or venous $\text{Pco}_2 \geq 60 \text{ mmHg}(8.0 \text{ kPa})$ and, **One:**

♦COPD and arterial or venous pH ≤ 7.24

♦No hx of COPD

♦Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure)

Rationale:

- SIRS criteria were removed to allow patients with SIRS-negative sepsis to be reviewed at the Critical level of care
- " $\text{PaO}_2/\text{FiO}_2 < 200 \text{ mmHg}(27.0 \text{ kPa})$ " criterion was removed as patients can meet using ARDS criteria
- Multi-system dysfunction criteria were removed to focus on the interventions as the drivers for the Critical level of care
- "IV fluid resuscitation $\leq 2\text{d}$ " criterion was removed as patients can meet under hemodynamic instability criteria

Infection: Pyelonephritis or Complex UTI

Initial Review (CRITICAL)

| Previously Available Content | 2024 March Release | Revision Description |
|---|--------------------|----------------------|
| <ul style="list-style-type: none"> ▶Blood product transfusion ▶CRRT ▶ECMO or ECLS ▶Hemodynamic monitoring, invasive ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦Arterial $P_{O_2} \leq 39$ mmHg(5.2 kPa) ♦Arterial or venous $P_{CO_2} \geq 60$ mmHg(8.0 kPa) and, One: <ul style="list-style-type: none"> ▶COPD and arterial or venous pH ≤ 7.24 ▶No hx of COPD ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | | |

Infection: Pyelonephritis or Complex UTI

Episode Day 1 (CRITICAL)

| Previously Available Content | 2024 March Release | Revision Description |
|--|--|---|
| <p>•CRITICAL, One:</p> <ul style="list-style-type: none"> ▶Sepsis, actual or suspected, All: <ul style="list-style-type: none"> ♦SIRS, ≥ Two: <ul style="list-style-type: none"> ▶Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ or $< 97.0^{\circ}\text{F}(36.1^{\circ}\text{C})$ ▶WBC, ≥ One: <ul style="list-style-type: none"> ♦$> 12,000/\text{cu.mm}(12 \times 10^9/\text{L})$ or $< 4,000/\text{cu.mm}(4 \times 10^9/\text{L})$ ♦Bands $> 10\%(0.10)$ ▶Heart rate $> 90/\text{min}$, sustained ▶Respiratory rate $> 20/\text{min}$, sustained ♦Organ dysfunction or severe hypoperfusion, ≥ One: <ul style="list-style-type: none"> ▶ARDS ▶$\text{PaO}_2/\text{FiO}_2 < 200$ mmHg(27.0 kPa) ▶Coma, stupor, obtundation, or GCS ≤ 9 ▶DIC ▶Hemodynamic instability unresponsive to ≥ 2 liter fluid bolus, requiring ongoing fluid resuscitation $\leq 24\text{h}$ and, Both: <ul style="list-style-type: none"> ♦Hypotension, ≥ One: <ul style="list-style-type: none"> ▶MAP < 65 mmHg ▶Systolic blood pressure < 90 mmHg and $< \text{baseline}$ ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Lactic acid > 2 mmol/L(18.0 mg/dL) ▶Urine output < 0.5 mL/kg/h | <p>•CRITICAL, One:</p> <ul style="list-style-type: none"> ▶Sepsis, actual or suspected, ≥ One: <ul style="list-style-type: none"> ♦ARDS ♦Coma, stupor, obtundation, or GCS ≤ 9 ♦CRRT ♦DIC and blood product transfusion ♦ECMO or ECLS ♦Hemodynamic instability unresponsive to ≥ 2 liter fluid bolus, requiring ongoing fluid resuscitation $\leq 24\text{h}$ and, Both: <ul style="list-style-type: none"> ▶Hypotension, ≥ One: <ul style="list-style-type: none"> ♦MAP < 65 mmHg ♦Systolic blood pressure < 90 mmHg and $< \text{baseline}$ ♦Finding, ≥ One: <ul style="list-style-type: none"> ♦Lactic acid > 2 mmol/L(18.0 mg/dL) ♦Urine output < 0.5 mL/kg/h ♦Hemodynamic monitoring, invasive ♦HFNC and respiratory monitoring every 1-2h ♦Mechanical ventilation ♦NIPPV and, ≥ One: <ul style="list-style-type: none"> ▶Arterial $P_{O_2} \leq 39$ mmHg(5.2 kPa) ▶Arterial or venous $P_{CO_2} \geq 60$ mmHg(8.0 kPa) and, One: <ul style="list-style-type: none"> ♦COPD and arterial or venous pH ≤ 7.24 ♦No hx of COPD ♦Vasopressor or inotrope, continuous (excludes low | <p>Rationale:</p> <ul style="list-style-type: none"> • SIRS criteria were removed to allow patients with SIRS-negative sepsis to be reviewed at the Critical level of care • “$\text{PaO}_2/\text{FiO}_2 < 200$ mmHg(27.0 kPa)” criterion was removed as patients can meet using ARDS criteria • Multi-system dysfunction criteria were removed to focus on the interventions as the drivers for the Critical level of care • “IV fluid resuscitation $\leq 2\text{d}$” criterion was removed as patients can meet under hemodynamic instability criteria |

Infection: Pyelonephritis or Complex UTI

Episode Day 1 (CRITICAL)

| Previously Available Content | 2024 March Release | Revision Description |
|---|--|----------------------|
| <ul style="list-style-type: none">▶Multi-system dysfunction, ≥ Two:<ul style="list-style-type: none">♦Serum creatinine $\geq 2\times$ baseline or estimated baseline♦Mental status changes (excludes coma, stupor, or obtundation) and GCS 10-12♦INR > 1.5 without anticoagulation and $>$ baseline♦Platelet count $< 100,000/\text{cu. mm}(100\times 10^9/\text{L})$ and $<$ baseline♦Total bilirubin $\geq 2 \text{ mg/dL}(34.2 \text{ }\mu\text{mol/L})$ and $>$ baseline♦Intervention, ≥ One:<ul style="list-style-type: none">▶IV fluid resuscitation $\leq 2\text{d}$▶Blood product transfusion▶CRRT▶ECMO or ECLS▶Hemodynamic monitoring, invasive▶HFNC and respiratory monitoring every 1-2h▶Mechanical ventilation▶NIPPV and, ≥ One:<ul style="list-style-type: none">♦Arterial $\text{Po}_2 \leq 39 \text{ mmHg}(5.2 \text{ kPa})$♦Arterial or venous $\text{Pco}_2 \geq 60 \text{ mmHg}(8.0 \text{ kPa})$ and, One:<ul style="list-style-type: none">▶COPD and arterial or venous $\text{pH} \leq 7.24$▶No hx of COPD▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | or fixed dose for end stage heart failure) | |

| Infection: Pyelonephritis or Complex UTI | | |
|--|--|---|
| Episode Day 3 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• ACUTE, One:</p> <ul style="list-style-type: none"> ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦ Pyelonephritis or complex UTI unresolved and, All: <ul style="list-style-type: none"> ▶ Urine culture positive ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ Blood cultures positive ♦ Severe pain requiring analgesic ≥ 2 doses ♦ Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ or $< 97.0^{\circ}\text{F}(36.1^{\circ}\text{C})$ ♦ WBC, ≥ One: <ul style="list-style-type: none"> ▶ $> 12,000/\text{cu.mm}(12 \times 10^9/\text{L})$ or $< 4,000/\text{cu.mm}(4 \times 10^9/\text{L})$ ▶ Bands $> 10\%(0.10)$ ▶ ANC $< 500/\text{cu.mm}(500 \times 10^6/\text{L})$ ▶ Anti-infective ♦ AKI, Both: <ul style="list-style-type: none"> ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ Oliguria ♦ Creatinine, One: <ul style="list-style-type: none"> ▶ $\geq 1.5\times$ baseline and chronic kidney disease ▶ $\geq 1.5\times$ ULN ♦ GFR $> 25\%(0.25)$ decrease from baseline ▶ Volume expander $\leq 2\text{d}$ ♦ Adverse drug reaction requiring change in medication $\leq 24\text{h}$ ♦ Renal abscess confirmed by imaging and, All: <ul style="list-style-type: none"> ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ ♦ WBC $\geq 12,000/\text{cu.mm}(12 \times 10^9/\text{L})$ ♦ Bands $> 10\%(0.10)$ ▶ Anti-infective ▶ Drainage, ≥ One: <ul style="list-style-type: none"> ♦ Performed within last 24h ♦ Persistent drainage ♦ Requiring drain repositioning within last 24h ♦ Post Critical care $\leq 24\text{h}$ | <p>• ACUTE, One:</p> <ul style="list-style-type: none"> ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦ Pyelonephritis or complex UTI unresolved and, All: <ul style="list-style-type: none"> ▶ Urine culture positive ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ Blood cultures positive ♦ Positive multidrug-resistant bacteria (excludes colonization) ♦ Severe pain requiring analgesic ≥ 2 doses ♦ Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ or $< 97.0^{\circ}\text{F}(36.1^{\circ}\text{C})$ ♦ WBC, ≥ One: <ul style="list-style-type: none"> ▶ $> 12,000/\text{cu.mm}(12 \times 10^9/\text{L})$ or $< 4,000/\text{cu.mm}(4 \times 10^9/\text{L})$ ▶ Bands $> 10\%(0.10)$ ▶ ANC $< 500/\text{cu.mm}(500 \times 10^6/\text{L})$ ▶ Anti-infective ♦ AKI, Both: <ul style="list-style-type: none"> ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ Oliguria ♦ Creatinine, One: <ul style="list-style-type: none"> ▶ $\geq 1.5\times$ baseline and chronic kidney disease ▶ $\geq 1.5\times$ ULN ♦ GFR $> 25\%(0.25)$ decrease from baseline ▶ Volume expander $\leq 2\text{d}$ ♦ Adverse drug reaction requiring change in medication $\leq 24\text{h}$ ♦ Renal abscess confirmed by imaging and, All: <ul style="list-style-type: none"> ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ ♦ WBC $\geq 12,000/\text{cu.mm}(12 \times 10^9/\text{L})$ ♦ Bands $> 10\%(0.10)$ ▶ Anti-infective ▶ Drainage, ≥ One: <ul style="list-style-type: none"> ♦ Performed within last 24h ♦ Persistent drainage ♦ Requiring drain repositioning within last 24h ♦ Post Critical care $\leq 24\text{h}$ | <p>Rationale:</p> <ul style="list-style-type: none"> • “Positive multidrug-resistant bacteria (excludes colonization)” criterion was added to create a pathway for the patient who has a positive urine culture for multidrug-resistant bacteria and requires a new or a change in anti-infective with the extension to their hospitalization |

Infection: Sepsis



Revision Summary

Organ dysfunction or hypoperfusion criteria rule was changed to clarify the intent of the criteria. SIRS and multi-system dysfunction criteria were removed from the Critical level of care.

Infection: Sepsis

Initial Review (ACUTE)

Previously Available Content

2024 March Release

Revision Description

•ACUTE, One:

- ▶Systemic infection (excludes viral), **Both:**
 - ♦Finding, **≥ One:**
 - ▶Blood cultures positive
 - ▶Organ dysfunction or hypoperfusion, **One:**
 - ♦Moderate risk finding, **≥ Two:**
 - ▶Creatinine $\geq 1.5\times$ and $< 2\times$ baseline or estimated baseline
 - ▶Mental status changes (excludes coma, stupor, or obtundation) and GCS 13-14
 - ▶Platelet count $100,000/\text{cu.mm}(100\times 10^9/\text{L})$ to $150,000/\text{cu.mm}(150\times 10^9/\text{L})$ and $<$ baseline
 - ▶Total bilirubin $1.2\text{ mg/dL}(20.52\text{ }\mu\text{mol/L})$ to $1.9\text{ mg/dL}(32.49\text{ }\mu\text{mol/L})$ and $>$ baseline
 - ▶O₂ sat $\leq 91\%(0.91)$ and $<$ baseline
 - ♦High risk finding, **≥ One:**
 - ▶Creatinine $\geq 2\times$ baseline or estimated baseline
 - ▶Mental status changes (excludes coma, stupor, or obtundation) and GCS 10-12
 - ▶Platelet count $<100,000/\text{cu.mm}(100\times 10^9/\text{L})$ and $<$ baseline
 - ▶Total bilirubin $\geq 2\text{ mg/dL}(34.2\text{ }\mu\text{mol/L})$ and $>$ baseline
 - ▶High risk for disease progression, **Both:**
 - ▶Risk factor, **≥ One:**
 - ♦Age > 75 years
 - ♦Chronic ventilator patient
 - ♦Coexisting disease, **≥ One:**
 - ▶COPD, Grade 3 or 4
 - ▶Chronic kidney disease
 - ▶Diabetes mellitus
 - ▶Heart failure, NYHA Class III or IV
 - ▶Liver disease
 - ▶Malignancy requiring active treatment with chemotherapy or radiation
 - ♦Immunocompromised
 - ♦Indwelling venous catheter
 - ♦Malnutrition

•ACUTE, One:

- ▶Systemic infection (excludes viral), **Both:**
 - ♦Finding, **≥ One:**
 - ▶Blood cultures positive
 - ▶Organ dysfunction or hypoperfusion, **≥ One:**
 - ♦Moderate risk finding, **≥ Two:**
 - ▶Creatinine $\geq 1.5\times$ and $< 2\times$ baseline or estimated baseline
 - ▶Mental status changes (excludes coma, stupor, or obtundation) and GCS 13-14
 - ▶Platelet count $100,000/\text{cu.mm}(100\times 10^9/\text{L})$ to $150,000/\text{cu.mm}(150\times 10^9/\text{L})$ and $<$ baseline
 - ▶Total bilirubin $1.2\text{ mg/dL}(20.52\text{ }\mu\text{mol/L})$ to $1.9\text{ mg/dL}(32.49\text{ }\mu\text{mol/L})$ and $>$ baseline
 - ▶O₂ sat $\leq 91\%(0.91)$ and $<$ baseline
 - ♦High risk finding, **≥ One:**
 - ▶Creatinine $\geq 2\times$ baseline or estimated baseline
 - ▶Mental status changes (excludes coma, stupor, or obtundation) and GCS 10-12
 - ▶Platelet count $<100,000/\text{cu.mm}(100\times 10^9/\text{L})$ and $<$ baseline
 - ▶Total bilirubin $\geq 2\text{ mg/dL}(34.2\text{ }\mu\text{mol/L})$ and $>$ baseline
 - ▶High risk for disease progression, **Both:**
 - ▶Risk factor, **≥ One:**
 - ♦Age > 75 years
 - ♦Chronic ventilator patient
 - ♦Coexisting disease, **≥ One:**
 - ▶COPD, Grade 3 or 4
 - ▶Chronic kidney disease
 - ▶Diabetes mellitus
 - ▶Heart failure, NYHA Class III or IV
 - ▶Liver disease
 - ▶Malignancy requiring active treatment with chemotherapy or radiation
 - ♦Immunocompromised
 - ♦Indwelling venous catheter
 - ♦Malnutrition

Rationale:

- Organ dysfunction or hypoperfusion criteria rule was changed to $\geq$ One to clarify the intent of the criteria and to allow lactic acid to be selected as well as the risk findings

| Infection: Sepsis | | |
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| Initial Review (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦Creatinine $\geq 1.5\times$ and $< 2\times$ baseline or estimated baseline ♦Mental status changes (excludes coma, stupor, or obtundation) and GCS 13-14 ♦Platelet count 100,000/cu.mm (100x10⁹/L) to 150,000/cu.mm (150x10⁹/L) and $<$ baseline ♦Total bilirubin 1.2 mg/dL(20.52 μmol/L) to 1.9 mg/dL(32.49 μmol/L) and $>$ baseline ♦O₂ sat $\leq 91\%(0.91)$ and $<$ baseline ♦Lactic acid $>$ ULN and < 4 mmol/L(36.0 mg/dL) ♦SIRS, ≥ Two: <ul style="list-style-type: none"> ▶Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ or $< 97.0^{\circ}\text{F}(36.1^{\circ}\text{C})$ ▶WBC, ≥ One: <ul style="list-style-type: none"> ♦$> 12,000/\text{cu.mm}(12\times 10^9/\text{L})$ or $< 4,000/\text{cu.mm}(4\times 10^9/\text{L})$ ♦Bands $> 10\%(0.10)$ ▶Heart rate $> 90/\text{min}$, sustained ▶Respiratory rate $> 20/\text{min}$, sustained | <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦Creatinine $\geq 1.5\times$ and $< 2\times$ baseline or estimated baseline ♦Mental status changes (excludes coma, stupor, or obtundation) and GCS 13-14 ♦Platelet count 100,000/cu.mm (100x10⁹/L) to 150,000/cu.mm (150x10⁹/L) and $<$ baseline ♦Total bilirubin 1.2 mg/dL(20.52 μmol/L) to 1.9 mg/dL(32.49 μmol/L) and $>$ baseline ♦O₂ sat $\leq 91\%(0.91)$ and $<$ baseline ♦Lactic acid $>$ ULN and < 4 mmol/L(36.0 mg/dL) ♦SIRS, ≥ Two: <ul style="list-style-type: none"> ▶Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ or $< 97.0^{\circ}\text{F}(36.1^{\circ}\text{C})$ ▶WBC, ≥ One: <ul style="list-style-type: none"> ♦$> 12,000/\text{cu.mm}(12\times 10^9/\text{L})$ or $< 4,000/\text{cu.mm}(4\times 10^9/\text{L})$ ♦Bands $> 10\%(0.10)$ ▶Heart rate $> 90/\text{min}$, sustained ▶Respiratory rate $> 20/\text{min}$, sustained | |

| Infection: Sepsis | | |
|---|---|---|
| Initial Review (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> •CRITICAL, One: <ul style="list-style-type: none"> ▶Sepsis, actual or suspected, All: <ul style="list-style-type: none"> ♦SIRS, ≥ Two: <ul style="list-style-type: none"> ▶Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ or $< 97.0^{\circ}\text{F}(36.1^{\circ}\text{C})$ ▶WBC, ≥ One: <ul style="list-style-type: none"> ♦$> 12,000/\text{cu.mm}(12\times 10^9/\text{L})$ or $< 4,000/\text{cu.mm}(4\times 10^9/\text{L})$ ♦Bands $> 10\%(0.10)$ ▶Heart rate $> 90/\text{min}$, sustained ▶Respiratory rate $> 20/\text{min}$, sustained ♦Organ dysfunction or severe hypoperfusion, ≥ One: <ul style="list-style-type: none"> ▶ARDS ▶PaO₂/FiO₂ < 200 mmHg(27.0 kPa) ▶Coma, stupor, obtundation, or GCS ≤ 9 ▶DIC | <ul style="list-style-type: none"> •CRITICAL, One: <ul style="list-style-type: none"> ▶Sepsis, actual or suspected, ≥ One: <ul style="list-style-type: none"> ♦ARDS ♦Coma, stupor, obtundation, or GCS ≤ 9 ♦CRRT ♦DIC and blood product transfusion ♦ECMO or ECLS ♦Hemodynamic instability unresponsive to ≥ 2 liter fluid bolus, requiring ongoing fluid resuscitation $\leq 24\text{h}$ and, Both: <ul style="list-style-type: none"> ▶Hypotension, ≥ One: <ul style="list-style-type: none"> ♦MAP < 65 mmHg ♦Systolic blood pressure < 90 mmHg and $<$ baseline ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦Lactic acid > 2 mmol/L(18.0 mg/dL) ♦Urine output < 0.5 mL/kg/h | <p>Rationale:</p> <ul style="list-style-type: none"> SIRS criteria were removed to allow patients with SIRS-negative sepsis to be reviewed at the Critical level of care “PaO₂/FiO₂ < 200 mmHg(27.0 kPa)” criterion was removed as patients can meet using ARDS criteria Multi-system dysfunction criteria were removed to focus on the interventions as the drivers for the Critical level of care “IV fluid resuscitation $\leq 2\text{d}$” criterion was removed as patients can meet under hemodynamic instability criteria |

| Infection: Sepsis | | |
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| Initial Review (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ▶ Hemodynamic instability unresponsive to ≥ 2 liter fluid bolus, requiring ongoing fluid resuscitation ≤ 24h and, Both: <ul style="list-style-type: none"> ♦ Hypotension, ≥ One: <ul style="list-style-type: none"> ▶ MAP < 65 mmHg ▶ Systolic blood pressure < 90 mmHg and < baseline ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Lactic acid > 2 mmol/L (18.0 mg/dL) ▶ Urine output < 0.5 mL/kg/h ▶ Multi-system dysfunction, ≥ Two: <ul style="list-style-type: none"> ♦ Serum creatinine ≥ 2x baseline or estimated baseline ♦ Mental status changes (excludes coma, stupor, or obtundation) and GCS 10-12 ♦ INR > 1.5 without anticoagulation and > baseline ♦ Platelet count < 100,000/cu.mm (100x10⁹/L) and < baseline ♦ Total bilirubin ≥ 2 mg/dL (34.2 μmol/L) and > baseline ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ IV fluid resuscitation ≤ 2d ▶ Blood product transfusion ▶ CRRT ▶ ECMO or ECLS ▶ Hemodynamic monitoring, invasive ▶ HFNC and respiratory monitoring every 1-2h ▶ Mechanical ventilation ▶ NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦ Arterial Po₂ ≤ 39 mmHg (5.2 kPa) ♦ Arterial or venous Pco₂ ≥ 60 mmHg (8.0 kPa) and, One: <ul style="list-style-type: none"> ▶ COPD and arterial or venous pH ≤ 7.24 ▶ No hx of COPD ▶ Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <ul style="list-style-type: none"> ♦ Hemodynamic monitoring, invasive ♦ HFNC and respiratory monitoring every 1-2h ♦ Mechanical ventilation ♦ NIPPV and, ≥ One: <ul style="list-style-type: none"> ▶ Arterial Po₂ ≤ 39 mmHg (5.2 kPa) ▶ Arterial or venous Pco₂ ≥ 60 mmHg (8.0 kPa) and, One: <ul style="list-style-type: none"> ♦ COPD and arterial or venous pH ≤ 7.24 ♦ No hx of COPD ♦ Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | |

| Infection: Sepsis | | |
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| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> • ACUTE, One: <ul style="list-style-type: none"> ▶ Systemic infection (excludes viral), All: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Blood cultures positive | <ul style="list-style-type: none"> • ACUTE, One: <ul style="list-style-type: none"> ▶ Systemic infection (excludes viral), All: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Blood cultures positive | <p>Rationale:</p> <ul style="list-style-type: none"> • Organ dysfunction or hypoperfusion criteria rule was changed to ≥ One to clarify the intent of the |

| Infection: Sepsis | | |
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| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>►Organ dysfunction or hypoperfusion, One:</p> <ul style="list-style-type: none"> ♦Moderate risk finding, ≥ Two: <ul style="list-style-type: none"> ►Creatinine $\geq 1.5\times$ and $< 2\times$ baseline or estimated baseline ►Mental status changes (excludes coma, stupor, or obtundation) and GCS 13-14 ►Platelet count 100,000/cu.mm($100\times 10^9/L$) to 150,000/cu.mm($150\times 10^9/L$) and $<$ baseline ►Total bilirubin 1.2 mg/dL($20.52\text{ }\mu\text{mol/L}$) to 1.9 mg/dL($32.49\text{ }\mu\text{mol/L}$) and $>$ baseline ►O₂ sat $\leq 91\%$(0.91) and $<$ baseline ♦High risk finding, ≥ One: <ul style="list-style-type: none"> ►Creatinine $\geq 2\times$ baseline or estimated baseline ►Mental status changes (excludes coma, stupor, or obtundation) and GCS 10-12 ►Platelet count $<100,000/\text{cu.mm}$($100\times 10^9/L$) and $<$ baseline ►Total bilirubin $\geq 2\text{ mg/dL}$($34.2\text{ }\mu\text{mol/L}$) and $>$ baseline ♦High risk for disease progression, Both: <ul style="list-style-type: none"> ►Risk factor, ≥ One: <ul style="list-style-type: none"> ♦Age > 75 years ♦Chronic ventilator patient ♦Coexisting disease, ≥ One: <ul style="list-style-type: none"> ►COPD, Grade 3 or 4 ►Chronic kidney disease ►Diabetes mellitus ►Heart failure, NYHA Class III or IV ►Liver disease ►Malignancy requiring active treatment with chemotherapy or radiation ♦Immunocompromised ♦Indwelling venous catheter ♦Malnutrition ►Finding, ≥ One: <ul style="list-style-type: none"> ♦Creatinine $\geq 1.5\times$ and $< 2\times$ baseline or estimated baseline ♦Mental status changes (excludes coma, stupor, or obtundation) and GCS 13-14 ♦Platelet count 100,000/cu.mm ($100\times 10^9/L$) to 150,000/cu.mm ($150\times 10^9/L$) and $<$ baseline ♦Total bilirubin 1.2 mg/dL($20.52\text{ }\mu\text{mol/L}$) to 1.9 mg/dL($32.49\text{ }\mu\text{mol/L}$) and $>$ baseline ♦O₂ sat $\leq 91\%$(0.91) and $<$ baseline ♦Lactic acid $> \text{ULN}$ and $< 4\text{ mmol/L}$(36.0 mg/dL) ♦SIRS, ≥ Two: <ul style="list-style-type: none"> ►Temperature $\geq 100.4^\circ\text{F}$(38.0°C) or $< 97.0^\circ\text{F}$(36.1°C) ►WBC, ≥ One: | <p>►Organ dysfunction or hypoperfusion, ≥ One:</p> <ul style="list-style-type: none"> ♦Moderate risk finding, ≥ Two: <ul style="list-style-type: none"> ►Creatinine $\geq 1.5\times$ and $< 2\times$ baseline or estimated baseline ►Mental status changes (excludes coma, stupor, or obtundation) and GCS 13-14 ►Platelet count 100,000/cu.mm($100\times 10^9/L$) to 150,000/cu.mm($150\times 10^9/L$) and $<$ baseline ►Total bilirubin 1.2 mg/dL($20.52\text{ }\mu\text{mol/L}$) to 1.9 mg/dL($32.49\text{ }\mu\text{mol/L}$) and $>$ baseline ►O₂ sat $\leq 91\%$(0.91) and $<$ baseline ♦High risk finding, ≥ One: <ul style="list-style-type: none"> ►Creatinine $\geq 2\times$ baseline or estimated baseline ►Mental status changes (excludes coma, stupor, or obtundation) and GCS 10-12 ►Platelet count $<100,000/\text{cu.mm}$($100\times 10^9/L$) and $<$ baseline ►Total bilirubin $\geq 2\text{ mg/dL}$($34.2\text{ }\mu\text{mol/L}$) and $>$ baseline ♦High risk for disease progression, Both: <ul style="list-style-type: none"> ►Risk factor, ≥ One: <ul style="list-style-type: none"> ♦Age > 75 years ♦Chronic ventilator patient ♦Coexisting disease, ≥ One: <ul style="list-style-type: none"> ►COPD, Grade 3 or 4 ►Chronic kidney disease ►Diabetes mellitus ►Heart failure, NYHA Class III or IV ►Liver disease ►Malignancy requiring active treatment with chemotherapy or radiation ♦Immunocompromised ♦Indwelling venous catheter ♦Malnutrition ►Finding, ≥ One: <ul style="list-style-type: none"> ♦Creatinine $\geq 1.5\times$ and $< 2\times$ baseline or estimated baseline ♦Mental status changes (excludes coma, stupor, or obtundation) and GCS 13-14 ♦Platelet count 100,000/cu.mm ($100\times 10^9/L$) to 150,000/cu.mm ($150\times 10^9/L$) and $<$ baseline ♦Total bilirubin 1.2 mg/dL($20.52\text{ }\mu\text{mol/L}$) to 1.9 mg/dL($32.49\text{ }\mu\text{mol/L}$) and $>$ baseline ♦O₂ sat $\leq 91\%$(0.91) and $<$ baseline ♦Lactic acid $> \text{ULN}$ and $< 4\text{ mmol/L}$(36.0 mg/dL) ♦SIRS, ≥ Two: <ul style="list-style-type: none"> ►Temperature $\geq 100.4^\circ\text{F}$(38.0°C) or $< 97.0^\circ\text{F}$(36.1°C) ►WBC, ≥ One: | <p>criteria and to allow lactic acid to be selected as well as the risk findings</p> |

| Infection: Sepsis | | |
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| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦> 12,000/cu.mm(12×10^9/L) or < 4,000/cu.mm(4×10^9/L) ♦Bands > 10%(0.10) ▶Heart rate > 90/min, sustained ▶Respiratory rate > 20/min, sustained ♦Intervention, Both: <ul style="list-style-type: none"> ▶Anti-infective ▶IV fluid | <ul style="list-style-type: none"> ♦> 12,000/cu.mm(12×10^9/L) or < 4,000/cu.mm(4×10^9/L) ♦Bands > 10%(0.10) ▶Heart rate > 90/min, sustained ▶Respiratory rate > 20/min, sustained ♦Intervention, Both: <ul style="list-style-type: none"> ▶Anti-infective ▶IV fluid | |

| Infection: Sepsis | | |
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| Episode Day 1 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•CRITICAL, One:</p> <ul style="list-style-type: none"> ▶Sepsis, actual or suspected, All: <ul style="list-style-type: none"> ♦SIRS, ≥ Two: <ul style="list-style-type: none"> ▶Temperature ≥ 100.4°F(38.0°C) or < 97.0°F(36.1°C) ▶WBC, ≥ One: <ul style="list-style-type: none"> ♦> 12,000/cu.mm(12×10^9/L) or < 4,000/cu.mm(4×10^9/L) ♦Bands > 10%(0.10) ▶Heart rate > 90/min, sustained ▶Respiratory rate > 20/min, sustained ♦Organ dysfunction or severe hypoperfusion, ≥ One: <ul style="list-style-type: none"> ▶ARDS ▶$\text{PaO}_2/\text{FiO}_2 < 200$ mmHg(27.0 kPa) ▶Coma, stupor, obtundation, or GCS ≤ 9 ▶DIC ▶Hemodynamic instability unresponsive to ≥ 2 liter fluid bolus, requiring ongoing fluid resuscitation ≤ 24h and, Both: <ul style="list-style-type: none"> ♦Hypotension, ≥ One: <ul style="list-style-type: none"> ▶MAP < 65 mmHg ▶Systolic blood pressure < 90 mmHg and < baseline ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Lactic acid > 2 mmol/L(18.0 mg/dL) ▶Urine output < 0.5 mL/kg/h ▶Multi-system dysfunction, ≥ Two: <ul style="list-style-type: none"> ♦Serum creatinine ≥ 2x baseline or estimated baseline ♦Mental status changes (excludes coma, stupor, or obtundation) and GCS 10-12 ♦INR > 1.5 without anticoagulation and | <p>•CRITICAL, One:</p> <ul style="list-style-type: none"> ▶Sepsis, actual or suspected, ≥ One: <ul style="list-style-type: none"> ♦ARDS ♦Coma, stupor, obtundation, or GCS ≤ 9 ♦CRRT ♦DIC and blood product transfusion ♦ECMO or ECLS ♦Hemodynamic instability unresponsive to ≥ 2 liter fluid bolus, requiring ongoing fluid resuscitation ≤ 24h and, Both: <ul style="list-style-type: none"> ▶Hypotension, ≥ One: <ul style="list-style-type: none"> ♦MAP < 65 mmHg ♦Systolic blood pressure < 90 mmHg and < baseline ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦Lactic acid > 2 mmol/L(18.0 mg/dL) ♦Urine output < 0.5 mL/kg/h ♦Hemodynamic monitoring, invasive ♦HFNC and respiratory monitoring every 1-2h ♦Mechanical ventilation ♦NIPPV and, ≥ One: <ul style="list-style-type: none"> ▶Arterial $\text{Po}_2 \leq 39$ mmHg(5.2 kPa) ▶Arterial or venous $\text{Pco}_2 \geq 60$ mmHg(8.0 kPa) and, One: <ul style="list-style-type: none"> ♦COPD and arterial or venous pH ≤ 7.24 ♦No hx of COPD ♦Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>Rationale:</p> <ul style="list-style-type: none"> • SIRS criteria were removed to allow patients with SIRS-negative sepsis to be reviewed at the Critical level of care • “$\text{PaO}_2/\text{FiO}_2 < 200$ mmHg(27.0 kPa)” criterion was removed as patients can meet using ARDS criteria • Multi-system dysfunction criteria were removed to focus on the interventions as the drivers for the Critical level of care • “IV fluid resuscitation ≤ 2d” criterion was removed as patients can meet under hemodynamic instability criteria |

| Infection: Sepsis | | |
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| Episode Day 1 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none">> baseline♦Platelet count < 100,000/cu.mm(100x10⁹/L) and < baseline♦Total bilirubin ≥ 2 mg/dL(34.2 μmol/L) and > baseline♦Intervention, ≥ One:<ul style="list-style-type: none">▶IV fluid resuscitation ≤ 2d▶Blood product transfusion▶CRRT▶ECMO or ECLS▶Hemodynamic monitoring, invasive▶HFNC and respiratory monitoring every 1-2h▶Mechanical ventilation▶NIPPV and, ≥ One:<ul style="list-style-type: none">♦Arterial Po₂ ≤ 39 mmHg(5.2 kPa)♦Arterial or venous Pco₂ ≥ 60 mmHg(8.0 kPa)and, One:<ul style="list-style-type: none">▶COPD and arterial or venous pH ≤ 7.24▶No hx of COPD▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | | |

Infection: Skin



Revision Summary

Based upon current guidelines AKI, Immunocompromised, lymphedema, and presence of foreign body were added to the findings under the diabetic foot ulcer criteria at the Acute level of care.

| Infection: Skin | | |
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| Initial Review (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• ACUTE, ≥ One:</p> <ul style="list-style-type: none"> ▶ Diabetic foot ulcer and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Chronic limb-threatening ischemia (CLTI) or impaired vascularity suspected ▶ Deep tissue abscess or infection ▶ Failed outpatient anti-infective (includes PO), ≥ One: <ul style="list-style-type: none"> ♦ Continued deterioration despite ≥ 24h treatment (includes PO) ♦ Unable to tolerate or absorb oral anti-infective ♦ Unresponsive to outpatient anti-infective (includes PO), ≥ One: <ul style="list-style-type: none"> ▶ ≥ 3d ▶ ≥ 5 doses ▶ Gangrene ▶ Involvement of muscle, tendon, joint, or bone ▶ Necrosis ♦ Anti-infective ▶ Herpes zoster and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Disseminated ▶ Immunocompromised ▶ Ophthalmicus ♦ Anti-infective ▶ Wound or surgical site infection and, ≥ One: <ul style="list-style-type: none"> ♦ Temperature ≥ 101.3°F(38.5°C) ♦ Erythema and induration > 5 cm from wound edge ♦ Heart rate > 110/min, sustained ♦ Located over a prosthesis or implanted device ♦ WBC, ≥ One: <ul style="list-style-type: none"> ▶ > 12,000/cu.mm(12x10⁹/L) or < 4,000/cu.mm(4x10⁹/L) ▶ Bands > 10%(0.10) | <p>• ACUTE, ≥ One:</p> <ul style="list-style-type: none"> ▶ Diabetic foot ulcer and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ AKI ▶ Chronic limb-threatening ischemia (CLTI) or impaired vascularity suspected ▶ Deep tissue abscess or infection ▶ Failed outpatient anti-infective (includes PO), ≥ One: <ul style="list-style-type: none"> ♦ Continued deterioration despite ≥ 24h treatment (includes PO) ♦ Unable to tolerate or absorb oral anti-infective ♦ Unresponsive to outpatient anti-infective (includes PO), ≥ One: <ul style="list-style-type: none"> ▶ ≥ 3d ▶ ≥ 5 doses ▶ Gangrene ▶ Immunocompromised ▶ Involvement of muscle, tendon, joint, or bone ▶ Lymphedema ▶ Necrosis ▶ Presence of foreign body ♦ Anti-infective ▶ Herpes zoster and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Disseminated ▶ Immunocompromised ▶ Ophthalmicus ♦ Anti-infective ▶ Wound or surgical site infection and, ≥ One: <ul style="list-style-type: none"> ♦ Temperature ≥ 101.3°F(38.5°C) ♦ Erythema and induration > 5 cm from wound edge ♦ Heart rate > 110/min, sustained ♦ Located over a prosthesis or implanted device ♦ WBC, ≥ One: <ul style="list-style-type: none"> ▶ > 12,000/cu.mm(12x10⁹/L) or < 4,000/cu.mm(4x10⁹/L) ▶ Bands > 10%(0.10) | <p>Rationale:</p> <ul style="list-style-type: none"> • Additional risk factors were added to the Diabetic foot ulcer criteria under finding based upon current guidelines |

| Infection: Skin | | |
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| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• ACUTE, ≥ One:</p> <ul style="list-style-type: none"> ▶ Diabetic foot ulcer and, All: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Chronic limb-threatening ischemia (CLTI) or impaired vascularity suspected ▶ Deep tissue abscess or infection ▶ Failed outpatient anti-infective (includes PO), ≥ One: <ul style="list-style-type: none"> ♦ Continued deterioration despite ≥ 24h treatment (includes PO) ♦ Unable to tolerate or absorb oral anti-infective ♦ Unresponsive to outpatient anti-infective (includes PO), ≥ One: <ul style="list-style-type: none"> ▶ ≥ 3d ▶ ≥ 5 doses ▶ Gangrene ▶ Involvement of muscle, tendon, joint, or bone ▶ Necrosis ♦ Assessment of vascular supply, ≥ One: <ul style="list-style-type: none"> ▶ Vascular imaging ▶ Noninvasive evaluation ▶ Performed within the last 3 mos ♦ Imaging to rule out osteomyelitis ♦ Anti-infective ♦ Surgical evaluation for debridement ▶ Herpes zoster and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Disseminated ▶ Immunocompromised ▶ Ophthalmicus ♦ Anti-infective ▶ Wound or surgical site infection and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Temperature ≥ 101.3°F(38.5°C) ▶ Erythema and induration > 5 cm from wound edge ▶ Heart rate > 110/min, sustained ▶ Located over a prosthesis or implanted device ♦ WBC, ≥ One: <ul style="list-style-type: none"> ♦ > 12,000/cu.mm(12x10⁹/L) or < 4,000/cu.mm(4x10⁹/L) ♦ Bands > 10%(0.10) ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Anti-infective ▶ Incision and drainage or suture removal | <p>• ACUTE, ≥ One:</p> <ul style="list-style-type: none"> ▶ Diabetic foot ulcer and, All: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ AKI ▶ Chronic limb-threatening ischemia (CLTI) or impaired vascularity suspected ▶ Deep tissue abscess or infection ▶ Failed outpatient anti-infective (includes PO), ≥ One: <ul style="list-style-type: none"> ♦ Continued deterioration despite ≥ 24h treatment (includes PO) ♦ Unable to tolerate or absorb oral anti-infective ♦ Unresponsive to outpatient anti-infective (includes PO), ≥ One: <ul style="list-style-type: none"> ▶ ≥ 3d ▶ ≥ 5 doses ▶ Gangrene ▶ Immunocompromised ▶ Involvement of muscle, tendon, joint, or bone ▶ Lymphedema ▶ Necrosis ▶ Presence of foreign body ♦ Assessment of vascular supply, ≥ One: <ul style="list-style-type: none"> ▶ Vascular imaging ▶ Noninvasive evaluation ▶ Performed within the last 3 mos ♦ Imaging to rule out osteomyelitis ♦ Anti-infective ♦ Surgical evaluation for debridement ▶ Herpes zoster and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Disseminated ▶ Immunocompromised ▶ Ophthalmicus ♦ Anti-infective ▶ Wound or surgical site infection and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Temperature ≥ 101.3°F(38.5°C) ▶ Erythema and induration > 5 cm from wound edge ▶ Heart rate > 110/min, sustained ▶ Located over a prosthesis or implanted device ♦ WBC, ≥ One: <ul style="list-style-type: none"> ♦ > 12,000/cu.mm(12x10⁹/L) or < 4,000/cu.mm(4x10⁹/L) ♦ Bands > 10%(0.10) ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Anti-infective ▶ Incision and drainage or suture removal | <p>Rationale:</p> <ul style="list-style-type: none"> • Additional risk factors were added to the Diabetic foot ulcer criteria under finding based upon current guidelines |

Labor and Delivery



Revision Summary

Labor and Delivery content was revised to align with current guidelines. Criteria changes were made to streamline the review process and clarify the intent. Continuous antihypertensive criteria were added to the Critical level of care to be more inclusive of treatment options for pregnant individuals with severe hypertension refractory to treatment.

| Labor and Delivery | | |
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| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• ACUTE, One:</p> <ul style="list-style-type: none"> ▶ Labor, One: <ul style="list-style-type: none"> ♦ Induction of labor $\leq 2d$, $\geq$ One: <ul style="list-style-type: none"> ▶ Artificial rupture of membranes ▶ Mechanical cervical ripening ▶ Oxytocin ▶ Prostaglandin E analogue ♦ Term labor with contractions $\leq 2d$ and, Both: <ul style="list-style-type: none"> ▶ $\leq$ every 5 min apart ▶ Lasting 30-45 sec ▶ Delivery, One: <ul style="list-style-type: none"> ♦ Cesarean section $\leq 4d$ ♦ Vaginal $\leq 2d$ | <p>• ACUTE, One:</p> <ul style="list-style-type: none"> ▶ Labor, One: <ul style="list-style-type: none"> ♦ Induction of labor, $\geq$ One: <ul style="list-style-type: none"> ▶ Mechanical methods ▶ Pharmaceutical methods (includes PO, buccal, intracervical, intravaginal, sublingual) ♦ Prelabor rupture of membranes (PROM) confirmed and, All: <ul style="list-style-type: none"> ▶ ≥ 37 wks gestation ▶ Fetal heart rate monitoring ▶ Uterine monitoring ♦ Term labor and, $\geq$ One: <ul style="list-style-type: none"> ▶ Contractions $\leq$ every 5 min for ≥ 30 sec ▶ Cervical dilation ≥ 4 cm ▶ Cervical effacement $\geq 50\%(0.50)$ ▶ Delivery, One: <ul style="list-style-type: none"> ♦ Cesarean section ♦ Vaginal | <p>Rationale:</p> <ul style="list-style-type: none"> • The time-limited endpoint was removed from "Induction of labor" to be more inclusive of those patients who require more than 48h of labor induction • Induction of labor methods are now included in "Mechanical methods" and "Pharmaceutical methods" hover notes to streamline the review process • Routes have been added to the "Pharmaceutical methods" criteria to clarify the intent of the criteria and to be inclusive of administration routes utilized for induction of labor • "Prelabor rupture of membranes (PROM)" criteria were added as literature notes that delivery is recommended and/or imminent within 12 to 24 hours • "Term labor" criterion has been updated with the removal of the time-limited endpoint to accurately describe the timeframe associated with labor and reflects current best practices • Literature notes that cervical changes and regular contractions are indicative of labor; therefore, cervical change criteria (i.e., cervical dilation, cervical effacement) were added to be inclusive of cervical changes that warrant evaluation and management at the Acute LOC, and contraction criteria have been streamlined to clarify the intent of the criteria |

| Labor and Delivery | | |
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| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| | | <ul style="list-style-type: none"> Time-limited endpoints associated with "Caesarean section" and "Vaginal" delivery were removed to accommodate predetermined length of stay |

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| Episode Day 1 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> Acute kidney injury (AKI) and, Both: <ul style="list-style-type: none"> Finding, ≥ One: <ul style="list-style-type: none"> Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> Loss of P wave Multifocal PVCs Neuromuscular deficit Widening QRS Sodium, One: <ul style="list-style-type: none"> < 120 mEq/L(120 mmol/L) and volume overload > 160 mEq/L(160 mmol/L) Intervention, ≥ One: <ul style="list-style-type: none"> Calcium chloride Calcium gluconate Glucose 50%(0.50) with insulin Diuretic, continuous IV fluid resuscitation ≤ 2d Arrhythmia, ≥ One: <ul style="list-style-type: none"> Urgent electrical cardioversion performed within last 24h Defibrillation performed within last 24h Temporary pacemaker insertion performed within last 2d Symptomatic recurrent or refractory arrhythmia requiring antiarrhythmic and, ≥ One: <ul style="list-style-type: none"> Chest pain Heart failure and, ≥ One: <ul style="list-style-type: none"> Arterial Po₂ < 56 mmHg(7.4 kPa) O₂ sat < 89%(0.89) and < baseline Hemodynamic instability, ≥ One: <ul style="list-style-type: none"> Heart rate > 120/min, sustained Systolic blood pressure, ≥ One: <ul style="list-style-type: none"> < 90 mmHg < 110 mmHg with chronic hypertension > 30 mmHg decrease from baseline Labile | <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> Antihypertensive, continuous DIC and blood product transfusion Eclampsia and neurological assessment every 1-2h ECMO or ECLS Heart failure and, Both: <ul style="list-style-type: none"> Finding, ≥ One: <ul style="list-style-type: none"> Arterial Po₂ < 56 mmHg(7.4 kPa) O₂ sat < 89%(0.89) and < baseline Diuretic every 1-2h or continuous Hemodynamic instability, All: <ul style="list-style-type: none"> ED treatment, ≥ One: <ul style="list-style-type: none"> ≥ 2 units blood product transfusion ≥ 2 liters IV fluid ≥ 1 unit blood product transfusion and ≥ 1 liter IV fluid Findings after ED treatment, ≥ One: <ul style="list-style-type: none"> Heart rate > 120/min Systolic blood pressure < 100 mmHg and < baseline MAP < 65 mmHg Requiring ongoing resuscitation Hemodynamic monitoring, invasive HELLP syndrome, Both: <ul style="list-style-type: none"> Finding, ≥ One: <ul style="list-style-type: none"> Lab values, All: <ul style="list-style-type: none"> Platelet count < 50,000/cu.mm(50x10⁹/L) AST or ALT > 2x ULN LDH ≥ 600 IU/L(10.02 μkat/L) Increased risk of bleeding, ≥ One: <ul style="list-style-type: none"> PT ≥ 1.5x ULN or INR ≥ 2.0 PTT ≥ 1.5x ULN Intervention, ≥ One: <ul style="list-style-type: none"> Blood product transfusion Laboratory monitoring, All: <ul style="list-style-type: none"> CBC LFTs Platelet count | <p>Rationale:</p> <ul style="list-style-type: none"> Acute Kidney Injury (AKI) and CRRT criteria were removed as an individual with AKI or requiring CRRT can be reviewed in the AKI subset "Arrhythmia" content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) Criterion for continuous antihypertensive infusion was added to the Critical level of care to align with guidelines and provide a pathway for pregnant individuals with hypertensive disorders of pregnancy and severe hypertension refractory to treatment The time-limited endpoint was removed from "Eclampsia and neurological assessment every 1-2h" to clarify the intent of the criteria, as the condition and associated monitoring are the drivers that are appropriate for evaluation and management at the Critical Level of Care HELLP criteria were streamlined, and a specified number for AST or ALT was changed to greater than 2x ULN to account for institutional assay variations and to align with the diagnostic criteria noted in current guidelines Based on current guidelines, HELLP criteria were revised by aligning with primary interventions and more appropriate diagnostic benchmarks, removal |

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| Episode Day 1 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>♦MAP < 65 mmHg</p> <p>►CRRT</p> <p>►DIC and blood product transfusion</p> <p>►Eclampsia and neurological assessment every 1-2h, ≤ 2d</p> <p>►ECMO or ECLS</p> <p>►Heart failure and, Both:</p> <p>♦Finding, ≥ One:</p> <p>►Arterial Po₂ < 56 mmHg(7.4 kPa)</p> <p>►O₂ sat < 89%(0.89) and < baseline</p> <p>♦Diuretic every 1-2h or continuous</p> <p>►Hemodynamic instability, All:</p> <p>♦ED treatment, ≥ One:</p> <p>►≥ 2 units blood product transfusion</p> <p>►≥ 2 liters IV fluid</p> <p>►≥ 1 unit blood product transfusion and ≥ 1 liter IV fluid</p> <p>♦Findings after ED treatment, ≥ One:</p> <p>►Heart rate > 120/min</p> <p>►Systolic blood pressure < 100 mmHg and < baseline</p> <p>►MAP < 65 mmHg</p> <p>♦Requiring ongoing resuscitation</p> <p>►Hemodynamic monitoring, invasive</p> <p>►HELLP syndrome ≤ 2d, Both:</p> <p>♦Finding, ≥ One:</p> <p>►Lab values, All:</p> <p>♦Platelet count < 50,000/cu.mm(50x10⁹/L)</p> <p>♦Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 70 U/L(1.17 µkat/L)</p> <p>♦Finding, ≥ One:</p> <p>►Fragmented or contracted red cells on blood smear</p> <p>►Total serum lactate dehydrogenase (LDH) ≥ 600 IU/L(10.02 µkat/L)</p> <p>►Total bilirubin > 1.20 mg/dL(20.52 µmol/L)</p> <p>►Increased risk of bleeding, ≥ One:</p> <p>♦PT ≥ 1.5x upper limit of normal (ULN) or INR ≥ 2.0</p> <p>♦PTT ≥ 1.5x upper limit of normal (ULN)</p> <p>♦Intervention, ≥ One:</p> <p>►Blood product transfusion</p> <p>►Laboratory monitoring, All:</p> <p>♦Complete blood count (CBC)</p> <p>♦Liver function tests (LFT)</p> <p>♦Platelet count</p> <p>►HFNC and respiratory monitoring every 1-2h</p> <p>►Mechanical ventilation</p> <p>►NIPPV and, ≥ One:</p> <p>♦Arterial Po₂ ≤ 39 mmHg(5.2 kPa)</p> <p>♦Arterial or venous Pco₂ ≥ 60 mmHg(8.0 kPa) and, One:</p> <p>►COPD and arterial or venous pH ≤ 7.24</p> <p>►No hx of COPD</p> | <p>►HFNC</p> <p>►Mechanical ventilation</p> <p>►NIPPV</p> <p>►Oxygen ≥ 40%(0.40)</p> <p>►Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure)</p> | <p>of total bilirubin and fragmented or contracted red blood cells criteria, as these are not universally utilized. Time-limited endpoint was removed as the condition requires close monitoring for progression at the Critical Level of Care</p> <ul style="list-style-type: none"> • "HFNC" criterion was added to reflect the patient who is appropriate at the Critical level of care on Episode Day 1 • NIPPV subcriteria were removed because any patient who is newly requiring noninvasive ventilator support is appropriate for the Critical level of care on Episode Day 1 • Oxygen ≥ 40%" criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Critical level of care • "Seizure, recurrent or refractory requiring anticonvulsant" criterion was removed as this subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology |

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| Episode Day 1 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ▶Oxygen $\geq 40\%(0.40)$, $\leq 2d$ ▶Seizure, recurrent or refractory requiring anticonvulsant ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | | |

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| Episode Day 2 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>●ACUTE, $\geq$ One:</p> <ul style="list-style-type: none"> ▶Labor, One: <ul style="list-style-type: none"> ◆Induction of labor $\leq 2d$, $\geq$ One: <ul style="list-style-type: none"> ▶Artificial rupture of membranes ▶Mechanical cervical ripening ▶Oxytocin ▶Prostaglandin E analogue ◆Term labor with contractions $\leq 2d$ ▶Delivery, One: <ul style="list-style-type: none"> ◆Caesarean section $\leq 4d$ ◆Vaginal $\leq 2d$ ▶Bleeding uncontrolled and, Both: <ul style="list-style-type: none"> ◆Finding, $\geq$ One: <ul style="list-style-type: none"> ▶Heart rate 100-120/min, sustained ▶Hct or Hb decreased within last 24h ▶Orthostatic hypotension, Both: <ul style="list-style-type: none"> ◆Sustained drop in blood pressure within 3 min of sitting or standing ◆Blood pressure, $\geq$ One: <ul style="list-style-type: none"> ▶Systolic drop ≥ 20 mmHg ▶Diastolic drop ≥ 10 mmHg ◆Intervention, $\geq$ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶Hct or Hb monitoring at least 2x/24h, $\leq 2d$ ▶Fever and, Both: <ul style="list-style-type: none"> ◆Temperature $\geq 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ ◆Intervention, One: <ul style="list-style-type: none"> ▶Known source and anti-infective ▶New onset and, $\geq$ One: <ul style="list-style-type: none"> ◆Culture pending $\leq 2d$ ◆Drug-induced fever suspected and precipitating drug discontinued $\leq 3d$ ◆Imaging study $\leq 24h$ ▶HELLP syndrome $\leq 5d$, All: <ul style="list-style-type: none"> ◆Aspartate aminotransferase (AST) or alanine | <p>●ACUTE, $\geq$ One:</p> <ul style="list-style-type: none"> ▶Labor, One: <ul style="list-style-type: none"> ◆Induction of labor, $\geq$ One: <ul style="list-style-type: none"> ▶Mechanical methods ▶Pharmaceutical methods (includes PO, buccal, intracervical, intravaginal, sublingual) ◆Term labor with contractions and progressive cervical changes $\leq 2d$ ▶Delivery, One: <ul style="list-style-type: none"> ◆Caesarean section $\leq 4d$ ◆Vaginal $\leq 2d$ ▶Post Critical care $\leq 24h$ | <p>Rationale:</p> <ul style="list-style-type: none"> • The time-limited endpoint was removed from "Induction of labor" to be more inclusive of those patients who require more than 48h of labor induction • Induction of labor methods are now included in "Mechanical methods" and "Pharmaceutical methods" hover notes to streamline the review process • Routes have been added to the "Pharmaceutical methods" criteria to clarify the intent of the criteria and to be inclusive of administration routes utilized for induction of labor • "Term labor criteria" has been updated to include "progressive cervical changes" to clarify the intent of the criteria, as literature notes that cervical changes and regular contractions are indicative of labor • Episode Day 2 Acute level of care complications (i.e., bleeding uncontrolled, fever, HELLP syndrome, Hypertension, Preeclampsia or eclampsia, spinal headache, wound dehiscence) were removed to streamline the review process as individuals will meet for labor and/or delivery for a minimum of 2 days even if complications should arise |

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| Episode Day 2 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>aminotransferase (ALT) > 70 U/L(1.17 µkat/L)</p> <p>♦Platelet count 50,000-100,000/cu.mm(50-100x10⁹/L)</p> <p>♦Finding, ≥ One:</p> <ul style="list-style-type: none"> ►Fragmented or contracted red cells on blood smear ►Total serum lactate dehydrogenase (LDH) ≥ 600 IU/L(10.02 µkat/L) ►Total bilirubin > 1.20 mg/dL(20.52 µmol/L) <p>♦Blood pressure management, ≥ One:</p> <ul style="list-style-type: none"> ►Antihypertensive (includes PO) ►Blood pressure monitoring at least 3x/24h <p>♦Laboratory monitoring, All:</p> <ul style="list-style-type: none"> ►Complete blood count (CBC) ►Liver function tests (LFT) ►Platelet count <p>►Hypertension and, ≥ One:</p> <ul style="list-style-type: none"> ♦≤ 3d since delivery and blood pressure monitoring at least 3x/24h ♦Uncontrolled ≤ 2d, All: <ul style="list-style-type: none"> ►Systolic blood pressure ≥ 150 mmHg or diastolic blood pressure ≥ 100 mmHg last 24h ►Antihypertensive (includes PO) ►Blood pressure monitoring at least 3x/24h <p>►Preeclampsia or eclampsia ≤ 5d and, Both:</p> <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ►≤ 3d since delivery ►Systolic blood pressure ≥ 150 mmHg or diastolic blood pressure ≥ 100 mmHg last 24h ►Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) elevated ►Creatinine > baseline ►Pain, severe persistent and unresponsive to medication and, ≥ One: <ul style="list-style-type: none"> ♦Epigastric ♦Headache, new onset ♦Right upper quadrant ►Platelet count < 100,000/cu.mm(100x10⁹/L) ►Visual disturbances, persistent ♦Intervention, ≥ One: <ul style="list-style-type: none"> ►Antihypertensive (includes PO) ►Magnesium sulfate ►Monitoring, Both: <ul style="list-style-type: none"> ♦Blood pressure monitoring at least 3x/24h ♦Laboratory monitoring, All: <ul style="list-style-type: none"> ►Complete blood count (CBC) ►Creatinine ►Liver function tests (LFT) ►Platelet count <p>►Spinal headache ≤ 2d and, ≥ One:</p> <ul style="list-style-type: none"> ♦Epidural blood patch performed within last 24h ♦IV caffeine and fluid <p>►Wound dehiscence and repair performed within last 2d</p> | | |

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| Episode Day 2 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| ►Post Critical care ≤ 24h | | |

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| Episode Day 2 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>●INTERMEDIATE, ≥ One:</p> <ul style="list-style-type: none"> ►Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦IV antiarrhythmic and hemodynamically stable ♦Antiarrhythmic conversion from IV to PO, One: <ul style="list-style-type: none"> ►No proarrhythmic risk ≤ 24h ►Proarrhythmic risk and continuous cardiac monitoring (excludes Holter) ≤ 3d ►Eclampsia and neurological assessment every 3-4h, ≤ 2d ►NIPPV ►Seizure and, Both: <ul style="list-style-type: none"> ♦Activity resolved ♦Anticonvulsant continuous | <p>●INTERMEDIATE, ≥ One:</p> <ul style="list-style-type: none"> ►Eclampsia and neurological assessment every 3-4h ►HFNC and respiratory monitoring every 3-4h ►NIPPV | <p>Rationale:</p> <ul style="list-style-type: none"> • "Arrhythmia" content was removed as individuals with Arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • The time-limited endpoint was removed from "Eclampsia and neurological assessment every 3-4h" to clarify the intent of the criteria, as the condition and associated monitoring are the drivers that are appropriate for evaluation and management at the Intermediate Level of Care • "HFNC and respiratory monitoring every 3-4h" criterion was added to reflect the patient who is appropriate at the at the Intermediate level of care • "Seizure" criteria were removed as this subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology |

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| Episode Day 2 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶Acute kidney injury (AKI) and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> ♦Loss of P wave ♦Multifocal PVCs ♦Neuromuscular deficit ♦Widening QRS ▶Sodium, One: <ul style="list-style-type: none"> ♦< 120 mEq/L(120 mmol/L) and volume overload ♦> 160 mEq/L(160 mmol/L) ▶Intervention, ≥ One: <ul style="list-style-type: none"> ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶IV fluid resuscitation ≤ 2d ▶Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦Urgent electrical cardioversion performed within last 24h ♦Defibrillation performed within last 24h ♦Temporary pacemaker insertion performed within last 2d ♦Symptomatic recurrent or refractory arrhythmia requiring antiarrhythmic and, ≥ One: <ul style="list-style-type: none"> ▶Chest pain ▶Heart failure and, ≥ One: <ul style="list-style-type: none"> ♦Arterial Po₂ < 56 mmHg(7.4 kPa) ♦O₂ sat < 89%(0.89) and < baseline ▶Hemodynamic instability, ≥ One: <ul style="list-style-type: none"> ♦Heart rate > 120/min, sustained ♦Systolic blood pressure, ≥ One: <ul style="list-style-type: none"> ▶< 90 mmHg ▶< 110 mmHg with chronic hypertension ▶> 30 mmHg decrease from baseline ▶Labile ♦MAP < 65 mmHg ▶CRRT <ul style="list-style-type: none"> ▶DIC and blood product transfusion ▶Eclampsia and neurological assessment every 1-2h, ≤ 2d ▶ECMO or ECLS ▶Heart failure and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Arterial Po₂ < 56 mmHg(7.4 kPa) ▶O₂ sat < 89%(0.89) and < baseline ♦Diuretic every 1-2h or continuous ▶Hemodynamic instability persisting after ≥1L of IVF resuscitation or ≥1 unit blood product transfusion and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: | <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶Antihypertensive, continuous ▶DIC and blood product transfusion ▶Eclampsia and neurological assessment every 1-2h ▶ECMO or ECLS ▶Heart failure and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Arterial Po₂ < 56 mmHg(7.4 kPa) ▶O₂ sat < 89%(0.89) and < baseline ♦Diuretic every 1-2h or continuous ▶Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶IV fluid resuscitation ≥ 1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶HELLP syndrome, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Lab values, All: <ul style="list-style-type: none"> ♦Platelet count < 50,000/cu.mm(50x10⁹/L) ♦AST or ALT > 2x ULN ♦LDH ≥ 600 IU/L(10.02 µkat/L) ▶Increased risk of bleeding, ≥ One: <ul style="list-style-type: none"> ♦PT ≥ 1.5x ULN or INR ≥ 2.0 ♦PTT ≥ 1.5x ULN ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶Laboratory monitoring, All: <ul style="list-style-type: none"> ♦CBC ♦LFTs ♦Platelet count ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Oxygen ≥ 40%(0.40) ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>Rationale:</p> <ul style="list-style-type: none"> • Acute Kidney Injury (AKI) and CRRT criteria were removed as an individual with AKI or requiring CRRT can be reviewed in the AKI subset • "Arrhythmia" content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • Criterion for continuous antihypertensive infusion was added to the Critical level of care to align with guidelines and provide a pathway for pregnant individuals with hypertensive disorders of pregnancy and severe hypertension refractory to treatment • The time-limited endpoint was removed from "Eclampsia and neurological assessment every 1-2h" to clarify the intent of the criteria, as the condition and associated monitoring are the drivers that are appropriate for evaluation and management at the Critical Level of Care • HELLP criteria were streamlined, and a specified number for AST or ALT was changed to greater than 2x ULN to account for institutional assay variations and to align with the diagnostic criteria noted in current guidelines • Based on current guidelines, HELLP criteria were revised by aligning with primary interventions and more appropriate diagnostic benchmarks, removal of total bilirubin and fragmented or contracted red blood cells criteria, as these are not universally utilized. Time-limited endpoint was removed as the condition requires close monitoring for progression at the Critical Level of Care • Oxygen ≥ 40%" criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Critical level of care • "Seizure, recurrent or refractory requiring anticonvulsant" criterion was removed as this |

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| Episode Day 2 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶IV fluid resuscitation ≥1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶HELLP syndrome ≤ 2d, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Lab values, All: <ul style="list-style-type: none"> ♦Platelet count < 50,000/cu.mm(50x10⁹/L) ♦Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 70 U/L(1.17 µkat/L) ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Fragmented or contracted red cells on blood smear ▶Total serum lactate dehydrogenase (LDH) ≥ 600 IU/L(10.02 µkat/L) ▶Total bilirubin > 1.20 mg/dL(20.52 µmol/L) ▶Increased risk of bleeding, ≥ One: <ul style="list-style-type: none"> ♦PT ≥ 1.5x upper limit of normal (ULN) or INR ≥ 2.0 ♦PTT ≥ 1.5x upper limit of normal (ULN) ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶Laboratory monitoring, All: <ul style="list-style-type: none"> ♦Complete blood count (CBC) ♦Liver function tests (LFT) ♦Platelet count ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Oxygen ≥ 40%(0.40), ≤ 2d ▶Seizure, recurrent or refractory requiring anticonvulsant ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | | subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology |

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| Episode Day 3 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• ACUTE, One:</p> <ul style="list-style-type: none"> ▶ Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦ Afebrile ♦ Post delivery, One: <ul style="list-style-type: none"> ▶ Post vaginal delivery > 2d ▶ Post Cesarean section > 4d ♦ Complication or comorbidity, One: <ul style="list-style-type: none"> ▶ No complication or active comorbidity relevant to this episode of care ▶ Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> ♦ Bleeding controlled or resolved ♦ HELLP syndrome and, Both: <ul style="list-style-type: none"> ▶ Blood pressure controlled last 24h and within acceptable limits ▶ Lab values within acceptable limits ♦ Hypertension controlled last 24h and within acceptable limits ♦ Preeclampsia or eclampsia resolved ♦ Spinal headache controlled ♦ Wound with adequate healing and established wound care regimen ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦ Labor, One: <ul style="list-style-type: none"> ▶ Induction of labor ≤ 2d, ≥ One: <ul style="list-style-type: none"> ♦ Artificial rupture of membranes ♦ Mechanical cervical ripening ♦ Oxytocin ♦ Prostaglandin E analogue ▶ Term labor with contractions ≤ 2d ♦ Delivery, One: <ul style="list-style-type: none"> ▶ Cesarean section ≤ 4d ▶ Vaginal ≤ 2d ♦ Bleeding uncontrolled and, Both: <ul style="list-style-type: none"> ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ Heart rate 100-120/min, sustained ♦ Hct or Hb decreased within last 24h ♦ Orthostatic hypotension, Both: <ul style="list-style-type: none"> ▶ Sustained drop in blood pressure within 3 min of sitting or standing ▶ Blood pressure, ≥ One: <ul style="list-style-type: none"> ♦ Systolic drop ≥ 20 mmHg ♦ Diastolic drop ≥ 10 mmHg ▶ Intervention, ≥ One: <ul style="list-style-type: none"> ♦ Blood product transfusion ♦ Hct or Hb monitoring at least 2x/24h, ≤ 2d ♦ Fever and, Both: <ul style="list-style-type: none"> ▶ Temperature ≥ 100.4°F (38.0°C) ▶ Intervention, One: <ul style="list-style-type: none"> ♦ Known source and anti-infective ♦ New onset and, ≥ One: | <p>• ACUTE, One:</p> <ul style="list-style-type: none"> ▶ Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦ Afebrile ♦ Post delivery (vaginal delivery or cesarean section) ♦ Complication or comorbidity, One: <ul style="list-style-type: none"> ▶ No complication or active comorbidity relevant to this episode of care ▶ Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> ♦ Bleeding controlled or resolved ♦ HELLP syndrome and, Both: <ul style="list-style-type: none"> ▶ Blood pressure controlled last 24h and within acceptable limits ▶ Lab values within acceptable limits ♦ Hypertension controlled last 24h and within acceptable limits ♦ Preeclampsia or postpartum hypertension resolved ♦ Post-dural puncture headache controlled ♦ Wound with adequate healing and established wound care regimen ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦ Labor, One: <ul style="list-style-type: none"> ▶ Induction of labor, ≥ One: <ul style="list-style-type: none"> ♦ Mechanical methods ♦ Pharmaceutical methods (includes PO, buccal, intracervical, intravaginal, sublingual) ▶ Term labor with contractions and progressive cervical changes ≤ 2d ♦ Delivery, One: <ul style="list-style-type: none"> ▶ Cesarean section ≤ 4d ▶ Vaginal ≤ 2d ♦ Vaginal bleeding > 1 pad/h and, ≥ One: <ul style="list-style-type: none"> ▶ Hct or Hb monitoring ▶ IV fluid or blood product transfusion ▶ Ultrasound assessment ♦ Fever and, Both: <ul style="list-style-type: none"> ▶ Temperature ≥ 100.4°F (38.0°C) ▶ Intervention, One: <ul style="list-style-type: none"> ♦ Known source and anti-infective ♦ New onset and, ≥ One: <ul style="list-style-type: none"> ▶ Culture pending ≤ 2d ▶ Imaging study ≤ 24h ♦ HELLP syndrome, All: <ul style="list-style-type: none"> ▶ AST or ALT > 2x ULN ▶ Platelet count 50,000-100,000/cu.mm (50-100x10⁹/L) ▶ LDH ≥ 600 IU/L (10.02 µkat/L) ▶ Blood pressure management, ≥ One: <ul style="list-style-type: none"> ♦ Antihypertensive (includes PO) | <p>Rationale:</p> <ul style="list-style-type: none"> • Responder criteria were updated to reflect the patient appropriate for discharge • The time-limited endpoint was removed from "Induction of labor" to be more inclusive of those patients who require more than 48h of labor induction • Induction of labor methods are now included in "Mechanical methods" and "Pharmaceutical methods" hover notes to streamline the review process • "Bleeding" criteria updated and streamlined to focus on the drivers of evaluation and management for the hemodynamically stable postpartum patients with vaginal bleeding that are appropriate for the Acute level of care • "Fever" criteria were streamlined to focus on the drivers of care for individuals who develop fever during hospitalization • HELLP criteria were revised to streamline, clarify criteria intent, account for institutional assay variations, and align with primary drivers and diagnostic benchmarks as noted in current guidelines with the addition of AST or ALT, the removal of total bilirubin and fragmented or contracted red cells criteria, and the removal of the time-limited endpoint • "Preeclampsia" and "hypertension" criteria were revised to streamline the review process • "Spinal headache" was updated to "post-dural puncture headache" to align with current guidelines, pharmacological and conservative management criteria are included in a hover note to streamline the review process and to be inclusive of evaluation and management appropriate for the Acute level of care in the immediate postpartum period • Wound dehiscence criteria was revised to be inclusive of the individuals that require 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| Labor and Delivery | | |
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| Episode Day 3 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ▶ Culture pending ≤ 2d ▶ Drug-induced fever suspected and precipitating drug discontinued ≤ 3d ▶ Imaging study ≤ 24h ♦ HELLP syndrome ≤ 5d, All: <ul style="list-style-type: none"> ▶ Platelet count 50,000-100,000/cu.mm(50-100x10⁹/L) ▶ Blood pressure management, ≥ One: <ul style="list-style-type: none"> ♦ Antihypertensive (includes PO) ♦ Blood pressure monitoring at least 3x/24h ▶ Laboratory monitoring, All: <ul style="list-style-type: none"> ♦ Complete blood count (CBC) ♦ Liver function tests (LFT) ♦ Platelet count ♦ Hypertension and, ≥ One: <ul style="list-style-type: none"> ▶ ≤ 3d since delivery and blood pressure monitoring at least 3x/24h ▶ Uncontrolled ≤ 2d, All: <ul style="list-style-type: none"> ♦ Systolic blood pressure ≥ 150 mmHg or diastolic blood pressure ≥ 100 mmHg last 24h ♦ Antihypertensive (includes PO) ♦ Blood pressure monitoring at least 3x/24h ♦ Preeclampsia or eclampsia ≤ 5d and, Both: <ul style="list-style-type: none"> ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ ≤ 3d since delivery ♦ Systolic blood pressure ≥ 150 mmHg or diastolic blood pressure ≥ 100 mmHg last 24h ♦ Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) elevated ♦ Creatinine > baseline ♦ Pain, severe persistent and unresponsive to medication and, ≥ One: <ul style="list-style-type: none"> ▶ Epigastric ▶ Headache, new onset ▶ Right upper quadrant ♦ Platelet count < 100,000/cu.mm(100x10⁹/L) ♦ Visual disturbances, persistent ▶ Intervention, ≥ One: <ul style="list-style-type: none"> ♦ Antihypertensive (includes PO) ♦ Magnesium sulfate ♦ Monitoring, Both: <ul style="list-style-type: none"> ▶ Blood pressure monitoring at least 3x/24h ▶ Laboratory monitoring, All: <ul style="list-style-type: none"> ♦ CBC ♦ Creatinine ♦ LFTs ♦ Platelet count ♦ Spinal headache ≤ 2d and, ≥ One: <ul style="list-style-type: none"> ▶ Epidural blood patch performed within last 24h ▶ IV caffeine and fluid ♦ Wound dehiscence and repair performed within last 2d ♦ Post Critical care ≤ 24h | <ul style="list-style-type: none"> ♦ Blood pressure monitoring at least 3x/24h ▶ Laboratory monitoring, All: <ul style="list-style-type: none"> ♦ CBC ♦ LFTs ♦ Platelet count ♦ Postpartum hypertension or preeclampsia and, Both: <ul style="list-style-type: none"> ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ Systolic blood pressure ≥ 140 mmHg or diastolic blood pressure ≥ 90 mmHg ≥ 2 measurements ♦ AST or ALT ≥ 2x ULN ♦ Creatinine, ≥ One: <ul style="list-style-type: none"> ▶ > 1.1 mg/dL(97.2 μmol/L) ▶ > 2x baseline ♦ Headache, new onset, severe, persistent, and unresponsive to medication ♦ Platelet count < 100,000/cu.mm(100x10⁹/L) ♦ Visual disturbances, persistent ▶ Intervention, ≥ One: <ul style="list-style-type: none"> ♦ Antihypertensive (includes PO) ♦ Magnesium sulfate ♦ Monitoring, Both: <ul style="list-style-type: none"> ▶ Blood pressure monitoring at least 3x/24h ▶ Laboratory monitoring, All: <ul style="list-style-type: none"> ♦ CBC ♦ Creatinine ♦ LFTs ♦ Platelet count ♦ Post-dural puncture headache and, ≥ One: <ul style="list-style-type: none"> ▶ Epidural blood patch performed within last 24h ▶ Conservative management ≤ 24h ▶ Pharmacological management ♦ Wound dehiscence and, ≥ One: <ul style="list-style-type: none"> ▶ Repair performed within last 2d ▶ Wound care and wound management teaching ≤ 24h ♦ Post Critical care ≤ 24h | <p>stay for 24h for wound care and wound management teaching</p> |

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| Episode Day 3 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>●INTERMEDIATE, ≥ One:</p> <ul style="list-style-type: none"> ▶Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦IV antiarrhythmic and hemodynamically stable ♦Antiarrhythmic conversion from IV to PO, One: <ul style="list-style-type: none"> ▶No proarrhythmic risk ≤ 24h ▶Proarrhythmic risk and continuous cardiac monitoring (excludes Holter) ≤ 3d ▶Eclampsia and neurological assessment every 3-4h, ≤ 2d ▶NIPPV ▶Seizure and, Both: <ul style="list-style-type: none"> ♦Activity resolved ♦Anticonvulsant continuous | <p>●INTERMEDIATE, ≥ One:</p> <ul style="list-style-type: none"> ▶Eclampsia and neurological assessment every 3-4h ▶HFNC and respiratory monitoring every 3-4h ▶NIPPV | <p>Rationale:</p> <ul style="list-style-type: none"> • "Arrhythmia" content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • The time-limited endpoint was removed from "Eclampsia and neurological assessment every 3-4h" to clarify the intent of the criteria, as the condition and associated monitoring are the drivers that are appropriate for evaluation and management at the Intermediate Level of Care • "HFNC and respiratory monitoring every 3-4h" criterion was added to reflect the patient who is appropriate at the at the Intermediate level of care • "Seizure" criteria were removed as this subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology |

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| Episode Day 3 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>●CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶Acute kidney injury (AKI) and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> ♦Loss of P wave ♦Multifocal PVCs ♦Neuromuscular deficit ♦Widening QRS ▶Sodium, One: <ul style="list-style-type: none"> ♦< 120 mEq/L(120 mmol/L) and volume overload | <p>●CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶Antihypertensive, continuous ▶DIC and blood product transfusion ▶Eclampsia and neurological assessment every 1-2h ▶ECMO or ECLS ▶Heart failure and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Arterial Po₂ < 56 mmHg(7.4 kPa) ▶O₂ sat < 89%(0.89) and < baseline ♦Diuretic every 1-2h or continuous ▶Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion | <p>Rationale:</p> <ul style="list-style-type: none"> • Acute Kidney Injury (AKI) and CRRT criteria were removed as an individual with AKI or requiring CRRT can be reviewed in the AKI subset • "Arrhythmia" content was removed as individuals with Arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) |

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| Episode Day 3 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>♦> 160 mEq/L(160 mmol/L)</p> <p>♦Intervention, ≥ One:</p> <ul style="list-style-type: none"> ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶IV fluid resuscitation ≤ 2d <p>▶Arrhythmia, ≥ One:</p> <ul style="list-style-type: none"> ♦Urgent electrical cardioversion performed within last 24h ♦Defibrillation performed within last 24h ♦Temporary pacemaker insertion performed within last 2d ♦Symptomatic recurrent or refractory arrhythmia requiring antiarrhythmic and, ≥ One: <ul style="list-style-type: none"> ▶Chest pain ▶Heart failure and, ≥ One: <ul style="list-style-type: none"> ♦Arterial Po₂ < 56 mmHg(7.4 kPa) ♦O₂ sat < 89%(0.89) and < baseline ▶Hemodynamic instability, ≥ One: <ul style="list-style-type: none"> ♦Heart rate > 120/min, sustained ♦Systolic blood pressure, ≥ One: <ul style="list-style-type: none"> ▶< 90 mmHg ▶< 110 mmHg with chronic hypertension ▶> 30 mmHg decrease from baseline ▶Labile ♦MAP < 65 mmHg <p>▶CRRT</p> <p>▶DIC and blood product transfusion</p> <p>▶Eclampsia and neurological assessment every 1-2h, ≤ 2d</p> <p>▶ECMO or ECLS</p> <p>▶Heart failure and, Both:</p> <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Arterial Po₂ < 56 mmHg(7.4 kPa) ▶O₂ sat < 89%(0.89) and < baseline ♦Diuretic every 1-2h or continuous <p>▶Hemodynamic instability persisting after ≥1L of IVF resuscitation or ≥1 unit blood product transfusion and, Both:</p> <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶IV fluid resuscitation ≥1L ≤ 24h <p>▶Hemodynamic monitoring, invasive</p> <p>▶HELLP syndrome ≤ 2d, Both:</p> <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Lab values, All: <ul style="list-style-type: none"> ♦Platelet count < 50,000/cu.mm(50x10⁹/L) ♦Aspartate aminotransferase (AST) or alanine | <p>and, Both:</p> <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶IV fluid resuscitation ≥ 1L ≤ 24h <p>▶Hemodynamic monitoring, invasive</p> <p>▶HELLP syndrome, Both:</p> <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Lab values, All: <ul style="list-style-type: none"> ♦Platelet count < 50,000/cu.mm(50x10⁹/L) ♦AST or ALT > 2x ULN ♦LDH ≥ 600 IU/L(10.02 µkat/L) ▶Increased risk of bleeding, ≥ One: <ul style="list-style-type: none"> ♦PT ≥ 1.5x ULN or INR ≥ 2.0 ♦PTT ≥ 1.5x ULN ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶Laboratory monitoring, All: <ul style="list-style-type: none"> ♦CBC ♦LFTs ♦Platelet count <p>▶HFNC and respiratory monitoring every 1-2h</p> <p>▶Mechanical ventilation</p> <p>▶NIPPV and, ≥ One:</p> <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h <p>▶Oxygen ≥ 40%(0.40)</p> <p>▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure)</p> | <ul style="list-style-type: none"> • Criterion for continuous antihypertensive infusion was added to the Critical level of care to align with guidelines and provide a pathway for pregnant individuals with hypertensive disorders of pregnancy and severe hypertension refractory to treatment • The time-limited endpoint was removed from "Eclampsia and neurological assessment every 1-2h" to clarify the intent of the criteria, as the condition and associated monitoring are the drivers that are appropriate for evaluation and management at the Critical Level of Care • HELLP criteria were streamlined, and a specified number for AST or ALT was changed to greater than 2x ULN to account for institutional assay variations and to align with the diagnostic criteria noted in current guidelines • Based on current guidelines, HELLP criteria were revised by aligning with primary interventions and more appropriate diagnostic benchmarks, removal of total bilirubin and fragmented or contracted red blood cells criteria, as these are not universally utilized. Time-limited endpoint was removed as the condition requires close monitoring for progression at the Critical Level of Care • Oxygen ≥ 40%" criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Critical level of care • "Seizure, recurrent or refractory requiring anticonvulsant" criterion was removed as this subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology |

| Labor and Delivery | | |
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| Episode Day 3 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>aminotransferase (ALT) > 70 U/L(1.17 µkat/L)</p> <p>♦Finding, ≥ One:</p> <ul style="list-style-type: none"> ►Fragmented or contracted red cells on blood smear ►Total serum lactate dehydrogenase (LDH) ≥ 600 IU/L(10.02 µkat/L) ►Total bilirubin > 1.20 mg/dL(20.52 µmol/L) <p>►Increased risk of bleeding, ≥ One:</p> <ul style="list-style-type: none"> ♦PT ≥ 1.5x upper limit of normal (ULN) or INR ≥ 2.0 ♦PTT ≥ 1.5x upper limit of normal (ULN) <p>♦Intervention, ≥ One:</p> <ul style="list-style-type: none"> ►Blood product transfusion ►Laboratory monitoring, All: <ul style="list-style-type: none"> ♦Complete blood count (CBC) ♦Liver function tests (LFT) ♦Platelet count ►HFNC and respiratory monitoring every 1-2h ►Mechanical ventilation ►NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ►Oxygen ≥ 40%(0.40), ≤ 2d ►Seizure, recurrent or refractory requiring anticonvulsant ►Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | | |

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| Episode Day 4-5 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>♦ACUTE, One:</p> <ul style="list-style-type: none"> ►Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦Afebrile ♦Post delivery, One: <ul style="list-style-type: none"> ►Post vaginal delivery > 2d ►Post Caesarean section > 4d ♦Complication or comorbidity, One: <ul style="list-style-type: none"> ►No complication or active comorbidity relevant to this episode of care ►Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> ♦Bleeding controlled or resolved | <p>♦ACUTE, One:</p> <ul style="list-style-type: none"> ►Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦Afebrile ♦Post delivery (vaginal delivery or cesarean section) ♦Complication or comorbidity, One: <ul style="list-style-type: none"> ►No complication or active comorbidity relevant to this episode of care ►Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> ♦Bleeding controlled or resolved ♦HELLP syndrome and, Both: | <p>Rationale:</p> <ul style="list-style-type: none"> • Responder criteria were updated to reflect the patient appropriate for discharge • "Bleeding" criteria updated and streamlined to focus on the drivers of evaluation and management for the hemodynamically stable postpartum patients with vaginal bleeding that are appropriate for the Acute level of care |

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| Episode Day 4-5 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦HELLP syndrome and, Both: <ul style="list-style-type: none"> ►Blood pressure controlled last 24h and within acceptable limits ►Lab values within acceptable limits ♦Hypertension controlled last 24h and within acceptable limits ♦Preeclampsia or eclampsia resolved ♦Spinal headache controlled ♦Wound with adequate healing and established wound care regimen ►Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦Delivery, One: <ul style="list-style-type: none"> ►Cesarean section ≤ 4d ►Vaginal ≤ 2d ♦Bleeding uncontrolled and, Both: <ul style="list-style-type: none"> ►Finding, ≥ One: <ul style="list-style-type: none"> ♦Heart rate 100-120/min, sustained ♦Hct or Hb decreased within last 24h ♦Orthostatic hypotension, Both: <ul style="list-style-type: none"> ►Sustained drop in blood pressure within 3 min of sitting or standing ►Blood pressure, ≥ One: <ul style="list-style-type: none"> ♦Systolic drop ≥ 20 mmHg ♦Diastolic drop ≥ 10 mmHg ►Intervention, ≥ One: <ul style="list-style-type: none"> ♦Blood product transfusion ♦Hct or Hb monitoring at least 2x/24h, ≤ 2d ♦Fever and, Both: <ul style="list-style-type: none"> ►Temperature ≥ 100.4°F(38.0°C) ►Intervention, One: <ul style="list-style-type: none"> ♦Known source and anti-infective ♦New onset and, ≥ One: <ul style="list-style-type: none"> ►Culture pending ≤ 2d ►Drug-induced fever suspected and precipitating drug discontinued ≤ 3d ►Imaging study ≤ 24h ♦HELLP syndrome ≤ 5d, All: <ul style="list-style-type: none"> ►Platelet count 50,000-100,000/cu.mm(50-100x10⁹/L) ►Blood pressure management, ≥ One: <ul style="list-style-type: none"> ♦Antihypertensive (includes PO) ♦Blood pressure monitoring at least 3x/24h ►Laboratory monitoring, All: <ul style="list-style-type: none"> ♦Complete blood count (CBC) ♦Liver function tests (LFT) ♦Platelet count ♦Hypertension and, ≥ One: <ul style="list-style-type: none"> ►≤ 3d since delivery and blood pressure monitoring at least 3x/24h ►Uncontrolled ≤ 2d, All: <ul style="list-style-type: none"> ♦Systolic blood pressure ≥ 150 mmHg or diastolic blood pressure ≥ 100 mmHg last 24h | <ul style="list-style-type: none"> ►Blood pressure controlled last 24h and within acceptable limits ►Lab values within acceptable limits ♦Hypertension controlled last 24h and within acceptable limits ♦Preeclampsia or postpartum hypertension resolved ♦Post-dural puncture headache controlled ♦Wound with adequate healing and established wound care regimen ►Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦Delivery, One: <ul style="list-style-type: none"> ►Cesarean section ≤ 4d ►Vaginal ≤ 2d ♦Vaginal bleeding > 1 pad/h and, ≥ One: <ul style="list-style-type: none"> ►Hct or Hb monitoring ►IV fluid or blood product transfusion ►Ultrasound assessment ♦Fever and, Both: <ul style="list-style-type: none"> ►Temperature ≥ 100.4°F(38.0°C) ►Intervention, One: <ul style="list-style-type: none"> ♦Known source and anti-infective ♦New onset and, ≥ One: <ul style="list-style-type: none"> ►Culture pending ≤ 2d ►Imaging study ≤ 24h ♦HELLP syndrome and, All: <ul style="list-style-type: none"> ►Platelet count 50,000-100,000/cu.mm(50-100x10⁹/L) ►Blood pressure management, ≥ One: <ul style="list-style-type: none"> ♦Antihypertensive (includes PO) ♦Blood pressure monitoring at least 3x/24h ►Laboratory monitoring, All: <ul style="list-style-type: none"> ♦CBC ♦LFTs ♦Platelet count ♦Postpartum hypertension or preeclampsia and, ≥ One: <ul style="list-style-type: none"> ►Antihypertensive (includes PO) ►Magnesium sulfate ►Blood pressure monitoring at least 3x/24h ♦Post-dural puncture headache and, ≥ One: <ul style="list-style-type: none"> ►Epidural blood patch performed within last 24h ►Conservative management ≤ 24h ►Pharmacological management ♦Wound dehiscence and, ≥ One: <ul style="list-style-type: none"> ►Repair performed within last 2d ►Wound care and wound management teaching ≤ 24h ♦Post Critical care ≤ 24h | <ul style="list-style-type: none"> • "Fever" criteria were streamlined to focus on the drivers of care for individuals who develop fever during hospitalization • HELLP criteria were revised and streamlined to clarify criteria intent with the removal of the time-limited endpoint • "Preeclampsia" and "hypertension" criteria were revised to streamline the review process • "Spinal headache" was updated to "post-dural puncture headache" to align with current guidelines, pharmacological and conservative management criteria are included in a hover note to streamline the review process and to be inclusive of evaluation and management appropriate for the Acute level of care in the immediate postpartum period • Wound dehiscence criteria revised to be inclusive of the individuals that require continued stay for 24h for wound care and wound management teaching |

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| Episode Day 4-5 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦Antihypertensive (includes PO) ♦Blood pressure monitoring at least 3x/24h ♦Preeclampsia or eclampsia $\leq 5d$ and, Both: <ul style="list-style-type: none"> ▶Finding, $\geq$ One: <ul style="list-style-type: none"> ♦$\leq 3d$ since delivery ♦Systolic blood pressure ≥ 150 mmHg or diastolic blood pressure ≥ 100 mmHg last 24h ♦Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) elevated ♦Creatinine $>$ baseline ♦Pain, severe persistent and unresponsive to medication and, $\geq$ One: <ul style="list-style-type: none"> ▶Epigastric ▶Headache, new onset ▶Right upper quadrant ♦Platelet count $< 100,000/cu.mm(100 \times 10^9/L)$ ♦Visual disturbances, persistent ▶Intervention, $\geq$ One: <ul style="list-style-type: none"> ♦Antihypertensive (includes PO) ♦Magnesium sulfate ♦Monitoring, Both: <ul style="list-style-type: none"> ▶Blood pressure monitoring at least 3x/24h ▶Laboratory monitoring, All: <ul style="list-style-type: none"> ♦Complete blood count (CBC) ♦Creatinine ♦Liver function tests (LFT) ♦Platelet count ♦Spinal headache $\leq 2d$ and, $\geq$ One: <ul style="list-style-type: none"> ▶Epidural blood patch performed within last 24h ▶IV caffeine and fluid ♦Wound dehiscence and repair performed within last 2d ♦Post Critical care $\leq 24h$ | | |

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| Episode Day 4-5 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦INTERMEDIATE, $\geq$ One: <ul style="list-style-type: none"> ▶Arrhythmia, $\geq$ One: <ul style="list-style-type: none"> ♦IV antiarrhythmic and hemodynamically stable ♦Antiarrhythmic conversion from IV to PO, One: <ul style="list-style-type: none"> ▶No proarrhythmic risk $\leq 24h$ ▶Proarrhythmic risk and continuous cardiac monitoring (excludes Holter) $\leq 3d$ | <ul style="list-style-type: none"> ♦INTERMEDIATE, $\geq$ One: <ul style="list-style-type: none"> ▶Eclampsia and neurological assessment every 3-4h ▶HFNC and respiratory monitoring every 3-4h ▶NIPPV | <p>Rationale:</p> <ul style="list-style-type: none"> • "Arrhythmia" content was removed as individuals with Arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) |

| Labor and Delivery | | |
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| Episode Day 4-5 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ▶Eclampsia and neurological assessment every 3-4h, ≤ 2d ▶NIPPV ▶Seizure and, Both: <ul style="list-style-type: none"> ♦Activity resolved ♦Anticonvulsant continuous | | <ul style="list-style-type: none"> • The time-limited endpoint was removed from "Eclampsia and neurological assessment every 3-4h" to clarify the intent of the criteria, as the condition and associated monitoring are the drivers that are appropriate for evaluation and management at the Intermediate Level of Care • "HFNC and respiratory monitoring every 3-4h" criterion was added to reflect the patient who is appropriate at the at the Intermediate level of care • "Seizure" criteria were removed as this subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology |

| Labor and Delivery | | |
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| Episode Day 4-5 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶Acute kidney injury (AKI) and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> ♦Loss of P wave ♦Multifocal PVCs ♦Neuromuscular deficit ♦Widening QRS ▶Sodium, One: <ul style="list-style-type: none"> ♦< 120 mEq/L(120 mmol/L) and volume overload ♦> 160 mEq/L(160 mmol/L) ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶IV fluid resuscitation ≤ 2d ▶Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦Urgent electrical cardioversion performed within last 24h | <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶Antihypertensive, continuous ▶DIC and blood product transfusion ▶Eclampsia and neurological assessment every 1-2h ▶ECMO or ECLS ▶Heart failure and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Arterial Po₂ < 56 mmHg(7.4 kPa) ▶O₂ sat < 89%(0.89) and < baseline ♦Diuretic every 1-2h or continuous ▶Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶IV fluid resuscitation ≥ 1L ≤ 24h ▶Hemodynamic monitoring, invasive | <p>Rationale:</p> <ul style="list-style-type: none"> • Acute Kidney Injury (AKI) and CRRT criteria were removed as an individual with AKI or requiring CRRT can be reviewed in the AKI subset • "Arrhythmia" content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • Criterion for continuous antihypertensive infusion was added to the Critical level of care to align with guidelines and provide a pathway for pregnant individuals with hypertensive disorders of pregnancy and severe hypertension refractory to treatment • The time-limited endpoint was removed from "Eclampsia and neurological assessment every 1- |

| Labor and Delivery | | |
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| Episode Day 4-5 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦Defibrillation performed within last 24h ♦Temporary pacemaker insertion performed within last 2d ♦Symptomatic recurrent or refractory arrhythmia requiring antiarrhythmic and, ≥ One: <ul style="list-style-type: none"> ▶Chest pain ▶Heart failure and, ≥ One: <ul style="list-style-type: none"> ♦Arterial Po₂ < 56 mmHg(7.4 kPa) ♦O₂ sat < 89%(0.89) and < baseline ▶Hemodynamic instability, ≥ One: <ul style="list-style-type: none"> ♦Heart rate > 120/min, sustained ♦Systolic blood pressure, ≥ One: <ul style="list-style-type: none"> ▶< 90 mmHg ▶< 110 mmHg with chronic hypertension ▶> 30 mmHg decrease from baseline ▶Labile ♦MAP < 65 mmHg ▶CRRT ▶DIC and blood product transfusion ▶Eclampsia and neurological assessment every 1-2h, ≤ 2d ▶ECMO or ECLS ▶Heart failure and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Arterial Po₂ < 56 mmHg(7.4 kPa) ▶O₂ sat < 89%(0.89) and < baseline ♦Diuretic every 1-2h or continuous ▶Hemodynamic instability persisting after ≥1L of IVF resuscitation or ≥1 unit blood product transfusion and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶IV fluid resuscitation ≥1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶HELLP syndrome ≤ 2d, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Lab values, All: <ul style="list-style-type: none"> ♦Platelet count < 50,000/cu.mm(50x10⁹/L) ♦Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 70 U/L(1.17 μkat/L) ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Fragmented or contracted red cells on blood smear ▶Total serum lactate dehydrogenase (LDH) ≥ 600 IU/L(10.02 μkat/L) ▶Total bilirubin > 1.20 mg/dL(20.52 μmol/L) ▶Increased risk of bleeding, ≥ One: <ul style="list-style-type: none"> ♦PT ≥ 1.5x upper limit of normal (ULN) or INR | <ul style="list-style-type: none"> ▶HELLP syndrome, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Lab values, All: <ul style="list-style-type: none"> ♦Platelet count < 50,000/cu.mm(50x10⁹/L) ♦AST or ALT > 2x ULN ♦LDH ≥ 600 IU/L(10.02 μkat/L) ▶Increased risk of bleeding, ≥ One: <ul style="list-style-type: none"> ♦PT ≥ 1.5x ULN or INR ≥ 2.0 ♦PTT ≥ 1.5x ULN ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶Laboratory monitoring, All: <ul style="list-style-type: none"> ♦CBC ♦LFTs ♦Platelet count ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Oxygen ≥ 40%(0.40) ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>2h" to clarify the intent of the criteria, as the condition and associated monitoring are the drivers that are appropriate for evaluation and management at the Critical Level of Care</p> <ul style="list-style-type: none"> • HELLP criteria were streamlined, and a specified number for AST or ALT was changed to greater than 2x ULN to account for institutional assay variations and to align with the diagnostic criteria noted in current guidelines • Based on current guidelines, HELLP criteria were revised by aligning with primary interventions and more appropriate diagnostic benchmarks, removal of total bilirubin and fragmented or contracted red blood cells criteria, as these are not universally utilized. Time-limited endpoint was removed as the condition requires close monitoring for progression at the Critical Level of Care • Oxygen ≥ 40%" criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Critical level of care • "Seizure, recurrent or refractory requiring anticonvulsant" criterion was removed as this subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology |

| Labor and Delivery | | |
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| Episode Day 4-5 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ≥ 2.0 ♦PTT ≥ 1.5x upper limit of normal (ULN) ♦Intervention, ≥ One: <ul style="list-style-type: none"> ►Blood product transfusion ►Laboratory monitoring, All: <ul style="list-style-type: none"> ♦Complete blood count (CBC) ♦Liver function tests (LFT) ♦Platelet count ►HFNC and respiratory monitoring every 1-2h ►Mechanical ventilation ►NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ►Oxygen ≥ 40%(0.40), ≤ 2d ►Seizure, recurrent or refractory requiring anticonvulsant ►Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | | |

| Labor and Delivery | | |
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| Episode Day 6 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦ACUTE, One: <ul style="list-style-type: none"> ►Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦Afebrile ♦Post delivery, One: <ul style="list-style-type: none"> ►Post vaginal delivery > 2d ►Post Caesarean section > 4d ♦Complication or comorbidity, One: <ul style="list-style-type: none"> ►No complication or active comorbidity relevant to this episode of care ►Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> ♦Bleeding controlled or resolved ♦HELLP syndrome and, Both: <ul style="list-style-type: none"> ►Blood pressure controlled last 24h and within acceptable limits ►Lab values within acceptable limits ♦Hypertension controlled last 24h and within acceptable limits ♦Preeclampsia or eclampsia resolved ♦Spinal headache controlled ♦Wound with adequate healing and established | <ul style="list-style-type: none"> ♦ACUTE, One: <ul style="list-style-type: none"> ►Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦Afebrile ♦Post delivery (vaginal delivery or cesarean section) ♦Complication or comorbidity, One: <ul style="list-style-type: none"> ►No complication or active comorbidity relevant to this episode of care ►Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> ♦Bleeding controlled or resolved ♦HELLP syndrome and, Both: <ul style="list-style-type: none"> ►Blood pressure controlled last 24h and within acceptable limits ►Lab values within acceptable limits ♦Hypertension controlled last 24h and within acceptable limits ♦Preeclampsia or postpartum hypertension resolved ♦Post-dural puncture headache controlled ♦Wound with adequate healing and established | <p>Rationale:</p> <ul style="list-style-type: none"> • Responder criteria were updated to reflect the patient appropriate for discharge |

Labor and Delivery

Episode Day 6 (ACUTE)

| Previously Available Content | 2024 March Release | Revision Description |
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| wound care regimen
► Non-responder , not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, One:
♦ Use Extended Stay subset
♦Refer for secondary review | wound care regimen
► Non-responder , not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, One:
♦ Use Extended Stay subset
♦Refer for secondary review | |

Non-Traumatic Bleeding



Revision Summary

Appropriate findings and interventions were added to the Non-Traumatic Bleeding content to reflect the patient who requires management at each level of care. New criteria were added to accurately describe the patient with spontaneous hematoma (e.g., retroperitoneal, intraperitoneal, rectus sheath hemorrhage) who is hemodynamically stable and requires the Acute level of care. New embolization criterion was added to include the patient with non-traumatic bleeding who requires admission to the Critical level of care for the procedure and post procedure monitoring for up to 24h. The Observation level of care was extended out to Episode Day 3 to account for the patient who requires an additional day of management. Criteria were added to the Acute level of care on Episode Day 3 to include the patient with abnormal uterine or vaginal bleeding who failed management at the Observation level of care have continued finding and requires further management.

| Non-Traumatic Bleeding | | |
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| Initial Review (OBSERVATION) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• OBSERVATION, ≥ One:</p> <ul style="list-style-type: none"> ▶ Epistaxis, anterior and, All: <ul style="list-style-type: none"> ♦ Bleeding through packing ♦ Hct or Hb monitoring at least 2x/24h ♦ Oximetry ▶ Hematuria and, ≥ One: <ul style="list-style-type: none"> ♦ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦ Exertional dyspnea ♦ Foley catheter obstructed due to blood clot ♦ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦ Orthostatic hypotension, Both: <ul style="list-style-type: none"> ▶ Sustained drop in blood pressure within 3 min of sitting or standing ▶ Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ♦ Presyncope or syncope ▶ Uterine or vaginal bleeding, abnormal (excludes pregnancy) and, ≥ One: <ul style="list-style-type: none"> ♦ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦ Exertional dyspnea ♦ ≥ 3 pads/h ♦ INR ≥ 2.0 ♦ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦ Orthostatic hypotension, Both: <ul style="list-style-type: none"> ▶ Sustained drop in blood pressure within 3 min of sitting or standing ▶ Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ♦ Presyncope or syncope | <p>• OBSERVATION, ≥ One:</p> <ul style="list-style-type: none"> ▶ Epistaxis, anterior and, All: <ul style="list-style-type: none"> ♦ Bleeding through packing ♦ Hct or Hb monitoring at least 2x/24h ♦ Oximetry ▶ Hematuria and, ≥ One: <ul style="list-style-type: none"> ♦ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦ Exertional dyspnea worse than baseline ♦ Orthostatic hypotension, Both: <ul style="list-style-type: none"> ▶ Sustained drop in blood pressure within 3 min of sitting or standing ▶ Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ♦ Syncope ♦ Urinary retention due to blood clot ▶ Uterine or vaginal bleeding, abnormal (excludes pregnancy) and, ≥ One: <ul style="list-style-type: none"> ♦ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦ ≥ 3 pads/h ♦ Exertional dyspnea worse than baseline ♦ INR ≥ 2.0 ♦ Orthostatic hypotension, Both: <ul style="list-style-type: none"> ▶ Sustained drop in blood pressure within 3 min of sitting or standing ▶ Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ♦ Syncope | <p>Rationale:</p> <ul style="list-style-type: none"> • "Worse than baseline" was added to "Exertional dyspnea" to clarify that the intent of the criteria point is not appropriate for patients who have chronic dyspnea related to a comorbid condition and describing the patient who has exertional dyspnea over and above their chronic state • "Foley catheter obstructed due to blood clot" was changed to "Urinary retention due to blood clot" to better capture the patient with urinary retention who requires the Observation level of care either with or without a pre-existing foley catheter • "Mental status changes (excludes coma, stupor, somnolence) or GCS 9-14" was removed as this patient would require a higher level of care • "Presyncope" was removed as literature does not speak to presyncope as a finding that requires hospitalization in patients with non-traumatic bleeding |

| Non-Traumatic Bleeding | | |
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| Initial Review (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•ACUTE, ≥ One:</p> <ul style="list-style-type: none"> ▶Epistaxis, anterior with inability to control bleeding in ED, and, Both: <ul style="list-style-type: none"> ◆Embolization or arterial ligation/cautery performed within 24h ◆Hct or Hb monitoring at least 2x/24h and blood product transfusion ▶Epistaxis, posterior with packing ▶Hemophilia or von Willebrand disease with inability to control bleeding in ED and, One: <ul style="list-style-type: none"> ◆Bleeding with hemodynamic stability ▶Hemoptysis and, ≥ One: <ul style="list-style-type: none"> ◆Dyspnea ◆Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ◆O₂ sat < 90%(0.90) and < baseline | <p>•ACUTE, ≥ One:</p> <ul style="list-style-type: none"> ▶Epistaxis, anterior, persistent and, Both: <ul style="list-style-type: none"> ◆Embolization or arterial ligation/cautery performed within 24h ◆Hct or Hb monitoring at least 2x/24h and blood product transfusion ▶Epistaxis, posterior and, ≥ One: <ul style="list-style-type: none"> ◆Embolization or arterial ligation/cautery performed within 24h ◆Posterior packing ▶Hemophilia or von Willebrand disease and, One: <ul style="list-style-type: none"> ◆Persistent bleeding with hemodynamic stability ▶Hemoptysis and, ≥ One: <ul style="list-style-type: none"> ◆Dyspnea ◆Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ◆O₂ sat < 90%(0.90) and < baseline ▶Spontaneous hematoma and, Both: <ul style="list-style-type: none"> ◆Hemodynamic stability ◆Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶Hct or Hb monitoring at least 2x/24h ▶Repeat imaging ▶Reversal of anticoagulant | <p>Rationale:</p> <ul style="list-style-type: none"> • Anterior epistaxis was updated to clarify the intent of the criteria • “Epistaxis, posterior with packing” was restructured to also include other interventions needed to control bleeding that would require management at the Acute level of care • Hemophilia or von Willebrand disease criteria were updated to clarify the intent of the criteria • Criteria were added to account for the patient with a spontaneous hematoma (e.g., retroperitoneal, intraperitoneal, rectus sheath hemorrhage) who is hemodynamically stable and requires admission to the Acute level of care for treatment |

| Non-Traumatic Bleeding | | |
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| Initial Review (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶Hemodynamic instability, All: <ul style="list-style-type: none"> ◆ED treatment, ≥ One: <ul style="list-style-type: none"> ▶≥ 2 units blood product transfusion ▶≥ 2 liters IV fluid ▶≥ 1 unit blood product transfusion and ≥ 1 liter IV fluid ◆Findings after ED treatment, ≥ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ◆Requiring ongoing resuscitation ▶Hemodynamic monitoring, invasive ▶Hemophilia or von Willebrand disease and, One: <ul style="list-style-type: none"> ◆Evidence of hemorrhage on CT or MRI and, ≥ One: | <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶Embolization performed within 24h ▶Hemoptysis and, ≥ One: <ul style="list-style-type: none"> ◆Bronchoscopy and, One: <ul style="list-style-type: none"> ▶Balloon tamponade ▶Cold saline lavage ▶Instillation of vasoconstrictive agent ▶Topical hemostatic tamponade ◆Bronchial artery embolization (BAE) ▶Hemodynamic instability, All: <ul style="list-style-type: none"> ◆ED treatment, ≥ One: <ul style="list-style-type: none"> ▶≥ 2 units blood product transfusion ▶≥ 2 liters IV fluid ▶≥ 1 unit blood product transfusion and ≥ 1 liter IV fluid ◆Findings after ED treatment, ≥ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min | <p>Rationale:</p> <ul style="list-style-type: none"> • “Embolization performed within 24h” criterion was added to reflect the patient with a spontaneous hematoma (e.g., retroperitoneal, intraperitoneal, rectus sheath hemorrhage) who requires emergent embolization at the Critical level of care • “Hemoptysis” and appropriate interventions were added to Initial Review at the Critical level of care to align with Episode Day 1 • HFNC subcriteria were removed because any patient who is newly requiring HFNC is appropriate for the Critical level of care on Initial Review |

| Non-Traumatic Bleeding | | |
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| Initial Review (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ▶ Compartment syndrome ▶ Hemodynamic instability ▶ Mental status change ▶ New onset neurological deficit, ≥ One: <ul style="list-style-type: none"> ♦ Alteration in speech ♦ Gait disturbance ♦ Paresis or paralysis ♦ Sensory deficit ♦ Visual impairment ▶ HFNC and respiratory monitoring every 1-2h ▶ Mechanical ventilation ▶ NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦ Arterial Po₂ ≤ 39 mmHg(5.2 kPa) ♦ Arterial or venous Pco₂ ≥ 60 mmHg(8.0 kPa) and, One: <ul style="list-style-type: none"> ▶ COPD and arterial or venous pH ≤ 7.24 ▶ No history of COPD ▶ Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <ul style="list-style-type: none"> ▶ Systolic blood pressure < 100 mmHg and < baseline ▶ MAP < 65 mmHg ♦ Requiring ongoing resuscitation ▶ Hemodynamic monitoring, invasive ▶ Hemophilia or von Willebrand disease and, One: <ul style="list-style-type: none"> ♦ Evidence of hemorrhage on CT or MRI and, ≥ One: <ul style="list-style-type: none"> ▶ Compartment syndrome ▶ Hemodynamic instability ▶ Mental status change ▶ New onset neurological deficit, ≥ One: <ul style="list-style-type: none"> ♦ Alteration in speech ♦ Gait disturbance ♦ Paresis or paralysis ♦ Sensory deficit ♦ Visual impairment ▶ HFNC ▶ Mechanical ventilation ▶ NIPPV ▶ Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <ul style="list-style-type: none"> • NIPPV subcriteria were removed because any patient who is newly requiring noninvasive ventilator support is appropriate for the Critical level of care on Initial Review |

| Non-Traumatic Bleeding | | |
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| Episode Day 1 (OBSERVATION) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> • OBSERVATION, ≥ One: <ul style="list-style-type: none"> ▶ Epistaxis, anterior and, All: <ul style="list-style-type: none"> ♦ Bleeding through packing ♦ Hct or Hb monitoring at least 2x/24h ♦ Oximetry ▶ Hematuria and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ▶ Exertional dyspnea ▶ Foley catheter obstructed due to blood clot ▶ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ▶ Orthostatic hypotension, Both: <ul style="list-style-type: none"> ♦ Sustained drop in blood pressure within 3 min of sitting or standing ♦ Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ▶ Presyncope or syncope ♦ Hct or Hb monitoring at least 2x/24h and, ≥ | <ul style="list-style-type: none"> • OBSERVATION, ≥ One: <ul style="list-style-type: none"> ▶ Epistaxis, anterior and, All: <ul style="list-style-type: none"> ♦ Bleeding through packing ♦ Hct or Hb monitoring at least 2x/24h ♦ Oximetry ▶ Hematuria and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ▶ Exertional dyspnea worse than baseline ▶ Orthostatic hypotension, Both: <ul style="list-style-type: none"> ♦ Sustained drop in blood pressure within 3 min of sitting or standing ♦ Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ▶ Syncope ▶ Urinary retention due to blood clot ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Bladder irrigation, continuous or cyclic ≤ 2d ▶ Blood product transfusion | <p>Rationale:</p> <ul style="list-style-type: none"> • "Worse than baseline" was added to "Exertional dyspnea" to clarify that the intent of the criteria point is not appropriate for patients who have chronic dyspnea related to a comorbid condition and describing the patient who has exertional dyspnea over and above their chronic state • "Foley catheter obstructed due to blood clot" was changed to "Urinary retention due to blood clot" to better capture the patient with urinary retention who requires the Observation level of care either with or without a pre-existing foley catheter |

| Non-Traumatic Bleeding | | |
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| Episode Day 1 (OBSERVATION) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>One:</p> <ul style="list-style-type: none"> ▶CT urography ≤ 24h ▶Cystoscopy ≤ 24h ▶Bladder irrigation, continuous or cyclic ≤ 2d ▶Blood product transfusion ▶Uterine or vaginal bleeding, abnormal (excludes pregnancy) and, Both: ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ▶Exertional dyspnea ▶≥ 3 pads/h ▶INR ≥ 2.0 ▶Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ▶Orthostatic hypotension, Both: <ul style="list-style-type: none"> ♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ▶Presyncope or syncope ♦Hct or Hb monitoring at least 2x/24h and, All: ▶Hormone therapy administered or contraindicated ▶Blood product transfusion ▶Procedure, ≥ One: <ul style="list-style-type: none"> ♦Transvaginal or pelvic ultrasound performed within 24h ♦Dilation and curettage (D&C) performed within 24h | <ul style="list-style-type: none"> ▶CT urography ≤ 24h ▶Cystoscopy ≤ 24h ▶Uterine or vaginal bleeding, abnormal (excludes pregnancy) and, Both: ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ▶≥ 3 pads/h ▶Exertional dyspnea worse than baseline ▶INR ≥ 2.0 ▶Orthostatic hypotension, Both: <ul style="list-style-type: none"> ♦Sustained drop in blood pressure within 3 min of sitting or standing ♦Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ▶Syncope ♦Hct or Hb monitoring at least 2x/24h and, All: ▶Blood product transfusion or IV fluid ▶Hormone therapy administered or contraindicated ▶Procedure, ≥ One: <ul style="list-style-type: none"> ♦Transvaginal or pelvic ultrasound performed within 24h ♦Dilation and curettage (D&C) performed within 24h | <ul style="list-style-type: none"> • “Mental status changes (excludes coma, stupor, somnolence) or GCS 9-14” was removed as this patient would require a higher level of care • “Presyncope” was removed as literature does not speak to presyncope as a finding that requires hospitalization in patients with non-traumatic bleeding • “Hct or Hb monitoring at least 2x/24h” was changed to “Intervention” to better define the drivers of a patient requiring the Observation level of care due to hematuria • “Blood product transfusion” was changed to “Blood product transfusion or IV fluid” to better reflect the patient who requires the Observation level of care due to abnormal uterine or vaginal bleeding (excludes pregnancy) for hormone therapy and either transvaginal or pelvic ultrasound or dilation and curettage, but not solely for a blood product transfusion |

| Non-Traumatic Bleeding | | |
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| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•ACUTE, ≥ One:</p> <ul style="list-style-type: none"> ▶Epistaxis, anterior with inability to control bleeding in ED, and, Both: <ul style="list-style-type: none"> ♦Embolization or arterial ligation/cautery performed within 24h ♦Hct or Hb monitoring at least 2x/24h and blood product transfusion ▶Epistaxis, posterior with packing and, Both: <ul style="list-style-type: none"> ♦Embolization or arterial ligation/cautery performed within 24h ♦Hct or Hb monitoring at least 2x/24h and blood product transfusion | <p>•ACUTE, ≥ One:</p> <ul style="list-style-type: none"> ▶Epistaxis, anterior, persistent and, Both: <ul style="list-style-type: none"> ♦Embolization or arterial ligation/cautery performed within 24h ♦Hct or Hb monitoring at least 2x/24h and blood product transfusion ▶Epistaxis, posterior and, ≥ One: <ul style="list-style-type: none"> ♦Embolization or arterial ligation/cautery performed within 24h ♦Posterior packing ▶Hemophilia or von Willebrand disease and, Both: <ul style="list-style-type: none"> ♦Persistent bleeding with hemodynamic stability | <p>Rationale:</p> <ul style="list-style-type: none"> • Anterior epistaxis criteria line was updated to clarify the intent of the criteria • “Epistaxis, posterior with packing” was restructured to also include other interventions needed to control bleeding that would require management at the Acute level of care • Hemophilia or von Willebrand disease criteria were updated to clarify the intent of the criteria |

| Non-Traumatic Bleeding | | |
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| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ▶ Hemophilia or von Willebrand disease with inability to control bleeding in ED and, Both: <ul style="list-style-type: none"> ♦ Bleeding with hemodynamic stability ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Cryoprecipitate ▶ Desmopressin ▶ Factor replacement ▶ Fresh frozen plasma (FFP) ▶ Hemoptysis and, All: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Dyspnea ▶ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ▶ O₂ sat < 90%(0.90) and < baseline ♦ Requiring supplemental oxygen, new or > baseline requirement ♦ Procedure, ≥ One: <ul style="list-style-type: none"> ▶ Bronchoscopy performed within 24h ▶ CT or computed tomography angiography (CTA) of chest | <ul style="list-style-type: none"> ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Cryoprecipitate ▶ Desmopressin ▶ Factor replacement ▶ Fresh frozen plasma (FFP) ▶ Hemoptysis and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Dyspnea ▶ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ▶ O₂ sat < 90%(0.90) and < baseline ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Bronchoscopy performed within 24h ▶ CT or computed tomography angiography (CTA) of chest performed within 24h ▶ Requiring supplemental oxygen, new or > baseline requirement ▶ Spontaneous hematoma and, Both: <ul style="list-style-type: none"> ♦ Hemodynamic stability ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Blood product transfusion ▶ Hct or Hb monitoring at least 2x/24h ▶ Repeat imaging ▶ Reversal of anticoagulant | <ul style="list-style-type: none"> • "Hct or Hb monitoring at least 2x/24h and blood product transfusion" was removed as an intervention for "Epistaxis, posterior" to restructure and highlight the intensity of services appropriate for the Acute level of care • "Requiring supplemental oxygen, new or > baseline requirement" was restructured as an option under "Intervention" for the patient who requires the Acute level of care due to hemoptysis • "Performed within 24h" was added to "CT or computed tomography angiography (CTA) of chest" to clarify the intent of the criteria • Criteria were added to account for the patient with a spontaneous hematoma (e.g., retroperitoneal, intraperitoneal, rectus sheath hemorrhage) who is hemodynamically stable and requires admission to the Acute level of care for treatment |

| Non-Traumatic Bleeding | | |
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| Episode Day 1 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> • CRITICAL, ≥ One: <ul style="list-style-type: none"> ▶ Hemoptysis and, ≥ One: <ul style="list-style-type: none"> ♦ Bronchoscopy and, One: <ul style="list-style-type: none"> ▶ Balloon tamponade ▶ Cold saline lavage ▶ Instillation of vasoconstrictive agent ▶ Topical hemostatic tamponade ♦ Bronchial artery embolization (BAE) ▶ Hemodynamic instability, All: <ul style="list-style-type: none"> ♦ ED treatment, ≥ One: <ul style="list-style-type: none"> ▶ ≥ 2 units blood product transfusion ▶ ≥ 2 liters IV fluid ▶ ≥ 1 unit blood product transfusion and ≥ 1 liter IV fluid ♦ Findings after ED treatment, ≥ One: <ul style="list-style-type: none"> ▶ Heart rate > 120/min ▶ Systolic blood pressure < 100 mmHg and < | <ul style="list-style-type: none"> • CRITICAL, ≥ One: <ul style="list-style-type: none"> ▶ Embolization performed within 24h ▶ Hemoptysis and, ≥ One: <ul style="list-style-type: none"> ♦ Bronchoscopy and, One: <ul style="list-style-type: none"> ▶ Balloon tamponade ▶ Cold saline lavage ▶ Instillation of vasoconstrictive agent ▶ Topical hemostatic tamponade ♦ Bronchial artery embolization (BAE) ▶ Hemodynamic instability, All: <ul style="list-style-type: none"> ♦ ED treatment, ≥ One: <ul style="list-style-type: none"> ▶ ≥ 2 units blood product transfusion ▶ ≥ 2 liters IV fluid ▶ ≥ 1 unit blood product transfusion and ≥ 1 liter IV fluid ♦ Findings after ED treatment, ≥ One: <ul style="list-style-type: none"> ▶ Heart rate > 120/min | <p>Rationale:</p> <ul style="list-style-type: none"> • "Embolization performed within 24h" criterion was added to reflect the patient with a spontaneous hematoma (e.g., retroperitoneal, intraperitoneal, rectus sheath hemorrhage) who requires admission to the Critical level of care for emergent embolization • HFNC subcriteria were removed because any patient who is newly requiring HFNC is appropriate for the Critical level of care on Episode Day 1 • NIPPV subcriteria were removed because any patient who is newly requiring noninvasive |

| Non-Traumatic Bleeding | | |
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| Episode Day 1 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> baseline <ul style="list-style-type: none"> ▶MAP < 65 mmHg ♦Requiring ongoing resuscitation ▶Hemodynamic monitoring, invasive ▶Hemophilia or von Willebrand disease and, Both: <ul style="list-style-type: none"> ♦Evidence of hemorrhage on CT or MRI and, ≥ One: <ul style="list-style-type: none"> ▶Compartment syndrome ▶Hemodynamic instability ▶Mental status change ▶New onset neurological deficit, ≥ One: <ul style="list-style-type: none"> ♦Alteration in speech ♦Gait disturbance ♦Paresis or paralysis ♦Sensory deficit ♦Visual impairment ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Cryoprecipitate ▶Factor replacement ▶Fresh frozen plasma (FFP) ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦Arterial $P_{O_2} \leq 39$ mmHg(5.2 kPa) ♦Arterial or venous $P_{CO_2} \geq 60$ mmHg(8.0 kPa) and, One: <ul style="list-style-type: none"> ▶COPD and arterial or venous pH ≤ 7.24 ▶No history of COPD ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <ul style="list-style-type: none"> ▶Systolic blood pressure < 100 mmHg and < baseline <ul style="list-style-type: none"> ▶MAP < 65 mmHg ♦Requiring ongoing resuscitation ▶Hemodynamic monitoring, invasive ▶Hemophilia or von Willebrand disease and, Both: <ul style="list-style-type: none"> ♦Evidence of hemorrhage on CT or MRI and, ≥ One: <ul style="list-style-type: none"> ▶Compartment syndrome ▶Hemodynamic instability ▶Mental status change ▶New onset neurological deficit, ≥ One: <ul style="list-style-type: none"> ♦Alteration in speech ♦Gait disturbance ♦Paresis or paralysis ♦Sensory deficit ♦Visual impairment ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Cryoprecipitate ▶Factor replacement ▶Fresh frozen plasma (FFP) ▶HFNC ▶Mechanical ventilation ▶NIPPV ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>ventilator support is appropriate for the Critical level of care on Episode Day 1</p> |

| Non-Traumatic Bleeding | | |
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| Episode Day 2 (OBSERVATION) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> •OBSERVATION, One: <ul style="list-style-type: none"> ▶Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦Bleeding controlled or resolved ♦Dyspnea or presyncope/syncope improving or resolved (if present during admission) ♦Hct or Hb within acceptable limits ♦Mental status or GCS within normal limits or at baseline ♦Vital signs within acceptable limits ▶Non-responder, not clinically stable for discharge | <ul style="list-style-type: none"> •OBSERVATION, One: <ul style="list-style-type: none"> ▶Epistaxis, anterior and, One: <ul style="list-style-type: none"> ♦Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ▶Bleeding controlled or resolved ▶Hct or Hb within acceptable limits ▶Vital signs within acceptable limits ♦Non-responder, not clinically stable for discharge and exceeds initial 24h Observation for this condition, One: <ul style="list-style-type: none"> ▶For a condition other than epistaxis use | <p>Rationale:</p> <ul style="list-style-type: none"> • “Epistaxis, anterior” Responder and Non-responder criteria were added to accurately capture the patient who is either clinically stable and appropriate for discharge or who is not clinically stable for discharge and requires secondary review or be reviewed in another condition specific subset on Episode Day 1 |

| Non-Traumatic Bleeding | | |
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| Episode Day 2 (OBSERVATION) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>and exceeds initial 24h Observation for this condition, One:</p> <ul style="list-style-type: none"> ♦For bleeding use Episode Day 2 at a higher level of care ♦For a condition other than bleeding use appropriate subset (Episode Day 1) based on clinical findings ♦Refer for secondary review | <p>appropriate subset (Episode Day 1) based on clinical findings</p> <ul style="list-style-type: none"> ►Refer for secondary review <p>►Hematuria and, One:</p> <ul style="list-style-type: none"> ♦Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ►Bleeding controlled or resolved ►Hct or Hb within acceptable limits ►Vital signs within acceptable limits ♦Non-responder, not clinically stable for discharge and exceeds initial 24h Observation for this condition, One: <ul style="list-style-type: none"> ►For hematuria use Episode Day 2 at a higher level of care ►For a condition other than hematuria use appropriate subset (Episode Day 1) based on clinical findings ►Refer for secondary review <p>►Uterine or vaginal bleeding, abnormal (excludes pregnancy) and, One:</p> <ul style="list-style-type: none"> ♦Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ►Bleeding controlled or resolved ►Hct or Hb within acceptable limits ►Vital signs within acceptable limits ♦Partial responder, not clinically stable for discharge and requires continued stay, Both: <ul style="list-style-type: none"> ►Finding, ≥ One: <ul style="list-style-type: none"> ♦Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦≥ 3 pads/h ♦Exertional dyspnea worse than baseline ♦INR ≥ 2.0 ♦Orthostatic hypotension, Both: <ul style="list-style-type: none"> ►Sustained drop in blood pressure within 3 min of sitting or standing ►Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ♦Syncope ►Hct or Hb monitoring at least 2x/24h and, All: <ul style="list-style-type: none"> ♦Blood product transfusion or IV fluid ♦Hormone therapy administered or contraindicated ♦Procedure, ≥ One: <ul style="list-style-type: none"> ►Transvaginal or pelvic ultrasound performed within 24h ►Dilation and curettage (D&C) performed within 24h | <ul style="list-style-type: none"> • “Hematuria” Responder and Non-responder criteria were added to accurately capture the patient who is either clinically stable and appropriate for discharge or is not clinically stable and requires either the Acute level of care or secondary review or be reviewed in another condition specific subset on Episode Day 1 • “Uterine or vaginal bleeding (excludes pregnancy)” Responder criteria were updated to reflect the patient clinically stable and appropriate for discharge • “Uterine or vaginal bleeding (excludes pregnancy)” Partial responder criteria were added to reflect the patient with abnormal uterine or vaginal bleeding who requires an additional day for continued care at the Observation level of care |

| Non-Traumatic Bleeding | | |
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| Episode Day 2 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• ACUTE, One:</p> <p>► Responder, discharge expected today if clinically stable last 12h, All:</p> <ul style="list-style-type: none"> ♦ Bleeding controlled or resolved ♦ Dyspnea or presyncope/syncope improving or resolved (if present during admission) ♦ Hct or Hb within acceptable limits ♦ Mental status or GCS within normal limits or at baseline ♦ O₂ sat within acceptable limits ♦ Vital signs within acceptable limits ♦ No complication or active comorbidity relevant to this episode of care <p>► Partial responder, not clinically stable for discharge and requires continued stay, ≥ One:</p> <ul style="list-style-type: none"> ♦ Epistaxis, posterior and, Both: <ul style="list-style-type: none"> ► Hct or Hb monitoring at least daily ► Posterior packing ♦ Hematuria and, Both: <ul style="list-style-type: none"> ► Finding, ≥ One: <ul style="list-style-type: none"> ♦ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦ Hct or Hb decreased last 24h ♦ Continuous bladder irrigation for persistent hematuria or clot ♦ Exertional dyspnea ♦ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦ Orthostatic hypotension, Both: <ul style="list-style-type: none"> ► Sustained drop in blood pressure within 3 min of sitting or standing ► Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ♦ Presyncope or syncope ► Hct or Hb monitoring at least 2x/24h and, ≥ One: <ul style="list-style-type: none"> ♦ Bladder irrigation, continuous or cyclic ≤ 2d ♦ Blood product transfusion ♦ Hemophilia or von Willebrand disease and, Both: <ul style="list-style-type: none"> ► Bleeding with hemodynamic stability ► Intervention, ≥ One: <ul style="list-style-type: none"> ♦ Cryoprecipitate ♦ Factor replacement ♦ Fresh frozen plasma (FFP) ♦ Hemoptysis and, Both: <ul style="list-style-type: none"> ► Finding, ≥ One: <ul style="list-style-type: none"> ♦ Dyspnea ♦ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦ O₂ sat < 90%(0.90) and < baseline ► Requiring supplemental oxygen, new or > baseline requirement and, ≥ One: <ul style="list-style-type: none"> ♦ Bronchoscopy performed within last 24h | <p>• ACUTE, One:</p> <p>► Responder, discharge expected today if clinically stable last 12h, All:</p> <ul style="list-style-type: none"> ♦ Bleeding controlled or resolved ♦ Hct or Hb within acceptable limits ♦ O₂ sat within acceptable limits ♦ Vital signs within acceptable limits ♦ No complication or active comorbidity relevant to this episode of care <p>► Partial responder, not clinically stable for discharge and requires continued stay, ≥ One:</p> <ul style="list-style-type: none"> ♦ Epistaxis, posterior and, ≥ One: <ul style="list-style-type: none"> ► Embolization or arterial ligation/cautery performed within 24h ► Hct or Hb monitoring at least daily ≤ 24h ► Posterior packing ♦ Hematuria and, Both: <ul style="list-style-type: none"> ► Finding, ≥ One: <ul style="list-style-type: none"> ♦ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦ Hct or Hb decreased last 24h ♦ Exertional dyspnea worse than baseline ♦ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦ Orthostatic hypotension, Both: <ul style="list-style-type: none"> ► Sustained drop in blood pressure within 3 min of sitting or standing ► Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ♦ Persistent hematuria or blood clot ♦ Syncope ► Intervention, ≥ One: <ul style="list-style-type: none"> ♦ Bladder irrigation, continuous or cyclic ≤ 2d ♦ Blood product transfusion ♦ Hemophilia or von Willebrand disease and, Both: <ul style="list-style-type: none"> ► Persistent bleeding with hemodynamic stability ► Intervention, ≥ One: <ul style="list-style-type: none"> ♦ Cryoprecipitate ♦ Factor replacement ♦ Fresh frozen plasma (FFP) ♦ Hemoptysis and, Both: <ul style="list-style-type: none"> ► Finding, ≥ One: <ul style="list-style-type: none"> ♦ Dyspnea ♦ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦ O₂ sat < 90%(0.90) and < baseline ► Intervention, ≥ One: <ul style="list-style-type: none"> ♦ Bronchoscopy performed within last 24h ♦ CT or computed tomography angiography (CTA) of chest performed within last 24h ♦ Requiring supplemental oxygen, new or > baseline requirement ♦ Spontaneous hematoma and, Both: <ul style="list-style-type: none"> ► Hemodynamic stability | <p>Rationale:</p> <ul style="list-style-type: none"> • Responder criteria were updated to reflect the patient appropriate for discharge • “Epistaxis, posterior and” was updated to include other interventions needed to control bleeding that would require management at the Acute level of care • “Worse than baseline” was added to “Exertional dyspnea” to clarify that the intent of the criteria point is not appropriate for patients who have chronic dyspnea related to a comorbid condition and describing the patient who has exertional dyspnea over and above their chronic state • “Continuous bladder irrigation for persistent hematuria or clot” was changed to “Persistent hematuria or blood clot” to clarify the intent of the criteria • “Hct or Hb monitoring at least 2x/24h” was changed to “Intervention” to better highlight the treatments required by the patient with continued and/or unresolved hematuria at the Acute level of care • “Presyncope” was removed as literature does not speak to presyncope as a finding that requires hospitalization in patients with non-traumatic bleeding • “Persistent” was added to the finding of “bleeding with hemodynamic stability” for Hemophilia or von Willebrand disease to clarify the intent of the criteria • “Requiring supplemental oxygen, new or > baseline requirement” was restructured as an option under “Intervention” for the patient who requires the Acute level of care due to hemoptysis • Criteria were added to account for the patient with a spontaneous hematoma (e.g., retroperitoneal, intraperitoneal, rectus sheath hemorrhage) who is hemodynamically stable and requires continued stay at the Acute level of care for blood product |

| Non-Traumatic Bleeding | | |
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| Episode Day 2 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦CT or computed tomography angiography (CTA) of chest performed within last 24h ♦Post Critical care ≤ 24h | <ul style="list-style-type: none"> ▶Intervention, ≥ One: <ul style="list-style-type: none"> ♦Blood product transfusion ♦Hct or Hb monitoring at least 2x/24h ♦Repeat imaging ♦Post Critical care ≤ 24h | transfusion, Hct or Hb monitoring, and/or repeat imaging |

| Non-Traumatic Bleeding | | |
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| Episode Day 2 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> •INTERMEDIATE, ≥ One: <ul style="list-style-type: none"> ▶NIPPV ▶Oxygen ≥ 40%(0.40), ≤ 2d | <ul style="list-style-type: none"> •INTERMEDIATE, ≥ One: <ul style="list-style-type: none"> ▶HFNC and respiratory monitoring every 3-4h ▶NIPPV ▶Oxygen ≥ 40%(0.40), ≤ 2d | Rationale: <ul style="list-style-type: none"> • “HFNC and respiratory monitoring every 3-4h” criterion was added to reflect the patient who is appropriate at the Intermediate level of care |

| Non-Traumatic Bleeding | | |
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| Episode Day 2 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> •CRITICAL, ≥ One: <ul style="list-style-type: none"> ▶AKI and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ♦Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> ♦Loss of P wave ♦Multifocal PVCs ♦Neuromuscular deficit ♦Widening QRS ▶Sodium, One: <ul style="list-style-type: none"> ♦< 120 mEq/L(120 mmol/L) and volume overload ♦> 160 mEq/L(160 mmol/L) ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin | <ul style="list-style-type: none"> •CRITICAL, ≥ One: <ul style="list-style-type: none"> ▶Embolization performed within 24h ▶Post embolization ≤ 24h ▶Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶IV fluid resuscitation ≥ 1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation | Rationale: <ul style="list-style-type: none"> • AKI and CRRT criteria were removed as an individual with AKI or requiring CRRT would be appropriate to be reviewed in the AKI subset • "Embolization performed within 24h" criterion was added to reflect the patient with a spontaneous hematoma who requires the Critical level of care for emergent embolization • “Post embolization ≤ 24h” criterion was added to reflect the patient who underwent embolization in the last 24h and continues to require the Critical level of care for up to 24h of monitoring |

| Non-Traumatic Bleeding | | |
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| Episode Day 2 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ▶Diuretic, continuous ▶Dialysis, One: <ul style="list-style-type: none"> ♦Hemodialysis ≤ 7d since initiation ♦Peritoneal dialysis ≤ 7d since initiation ▶IV fluid resuscitation ≤ 2d ▶CRRT ▶Hemodynamic instability persisting after ≥1L of IVF resuscitation or ≥1 unit blood product transfusion and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶IV fluid resuscitation ≥1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <ul style="list-style-type: none"> ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | |

| Non-Traumatic Bleeding | | |
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| Episode Day 3 (OBSERVATION) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| N/A | <ul style="list-style-type: none"> •OBSERVATION, One: <ul style="list-style-type: none"> ▶Uterine or vaginal bleeding, abnormal (excludes pregnancy) and, One: <ul style="list-style-type: none"> ♦Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ▶Bleeding controlled or resolved ▶Hct or Hb within acceptable limits ▶Vital signs within acceptable limits ♦Non-responder, not clinically stable for discharge and exceeds episode days at the Observation level of care for this condition, One: <ul style="list-style-type: none"> ▶For uterine or vaginal bleeding use Episode Day 3 at a higher level of care ▶For a condition other than uterine or vaginal bleeding use appropriate subset (Episode Day 1) based on clinical findings | <p>Rationale:</p> <ul style="list-style-type: none"> • Responder criteria were added to reflect the patient who is clinically stable for discharge • Non-responder criteria were added to reflect the patient who is not clinically stable, the patient with continued uterine or vaginal bleeding who can step up to Episode 3 at a higher level of care, requires secondary review, or be reviewed in another condition specific subset on Episode Day 1 |

| Non-Traumatic Bleeding | | |
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| Episode Day 3 (OBSERVATION) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| ▶Refer for secondary review | | |

| Non-Traumatic Bleeding | | |
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| Episode Day 3 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•ACUTE, One:</p> <p>▶Responder, discharge expected today if clinically stable last 12h, All:</p> <ul style="list-style-type: none"> ♦Bleeding controlled or resolved ♦Dyspnea or presyncope/syncope improving or resolved (if present during admission) ♦Hct or Hb within acceptable limits ♦Mental status or GCS within normal limits or at baseline ♦O₂ sat within acceptable limits ♦Vital signs within acceptable limits ♦Complication or comorbidity, One: <ul style="list-style-type: none"> ▶No complication or active comorbidity relevant to this episode of care ▶Complication or comorbidity and clinically stable for discharge, One: <ul style="list-style-type: none"> ♦AKI, ≥ One: <ul style="list-style-type: none"> ▶Kidney function improving or within acceptable limits ▶Outpatient dialysis regimen established <p>▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One:</p> <ul style="list-style-type: none"> ♦Epistaxis, posterior and, Both: <ul style="list-style-type: none"> ▶Hct or Hb monitoring at least daily ♦Posterior packing ♦Hematuria and, Both: <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦Hct or Hb decreased last 24h ♦Exertional dyspnea ♦Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦Orthostatic hypotension, Both: <ul style="list-style-type: none"> ▶Sustained drop in blood pressure within 3 min of sitting or standing ▶Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ♦Presyncope or syncope ▶Hct or Hb monitoring at least 2x/24h and, ≥ One: | <p>•ACUTE, One:</p> <p>▶Responder, discharge expected today if clinically stable last 12h, All:</p> <ul style="list-style-type: none"> ♦Bleeding controlled or resolved ♦Hct or Hb within acceptable limits ♦O₂ sat within acceptable limits ♦Vital signs within acceptable limits ♦No complication or active comorbidity relevant to this episode of care <p>▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One:</p> <ul style="list-style-type: none"> ♦Epistaxis, posterior and, ≥ One: <ul style="list-style-type: none"> ▶Embolization or arterial ligation/cautery performed within 24h ▶Hct or Hb monitoring at least daily ≤ 24h ▶Posterior packing ♦Hematuria and, Both: <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦Hct or Hb decreased last 24h ♦Exertional dyspnea worse than baseline ♦Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦Orthostatic hypotension, Both: <ul style="list-style-type: none"> ▶Sustained drop in blood pressure within 3 min of sitting or standing ▶Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ♦Persistent hematuria or blood clot ♦Syncope ▶Intervention, ≥ One: <ul style="list-style-type: none"> ♦Bladder irrigation, continuous or cyclic ≤ 2d ♦Blood product transfusion ♦Hemophilia or von Willebrand disease and, Both: <ul style="list-style-type: none"> ▶Persistent bleeding with hemodynamic stability ▶Intervention, ≥ One: <ul style="list-style-type: none"> ♦Cryoprecipitate ♦Factor replacement ♦Fresh frozen plasma (FFP) ♦Hemoptysis and, Both: | <p>Rationale:</p> <ul style="list-style-type: none"> • Responder criteria were updated to reflect the patient appropriate for discharge • “Epistaxis, posterior and” was updated to include other interventions needed to control bleeding that would require management at the Acute level of care • “Worse than baseline” was added to “Exertional dyspnea” to clarify that the intent of the criteria point is not appropriate for patients who have chronic dyspnea related to a comorbid condition and describing the patient who has exertional dyspnea over and above their chronic state • “Presyncope” was removed as literature does not speak to presyncope as a finding that requires hospitalization in patients with non-traumatic bleeding • “Hct or Hb monitoring at least 2x/24h” was changed to “Intervention” to better highlight the treatments required by the patient with continued and/or unresolved hematuria at the Acute level of care • “Persistent” was added to the finding of “bleeding with hemodynamic stability” for Hemophilia or von Willebrand disease to clarify the intent of the criteria • AKI criteria were removed as a patient who develops AKI would be appropriate to be reviewed in the AKI subset |

| Non-Traumatic Bleeding | | |
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| Episode Day 3 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦Bladder irrigation, continuous or cyclic ≤ 2d ♦Blood product transfusion ♦Hemophilia or von Willebrand disease and, Both: <ul style="list-style-type: none"> ▶Bleeding with hemodynamic stability ▶Intervention, ≥ One: <ul style="list-style-type: none"> ♦Cryoprecipitate ♦Factor replacement ♦Fresh frozen plasma (FFP) ♦Hemoptysis and, Both: <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦Dyspnea ♦Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦O₂ sat < 90%(0.90) and < baseline ▶Requiring supplemental oxygen, new or > baseline requirement ♦AKI, Both: <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦Oliguria ♦Creatinine, One: <ul style="list-style-type: none"> ▶≥ 1.5x baseline and chronic kidney disease ▶≥ 1.5x ULN ♦GFR > 25%(0.25) decrease from baseline ♦Dialysis initiated ▶Intervention, ≥ One: <ul style="list-style-type: none"> ♦Volume expander ≤ 2d ♦Diuretic ≥ 2x/24h, ≤ 3d ♦Dialysis (initial) ≤ 7d since initiation ♦Dialysis discontinued requiring lab monitoring ≤ 2d since discontinuation ♦Immunosuppressant therapy ≤ 3d ♦Post Critical care ≤ 24h | <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦Dyspnea ♦Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦O₂ sat < 90%(0.90) and < baseline ▶Requiring supplemental oxygen, new or > baseline requirement ♦Spontaneous hematoma and, Both: <ul style="list-style-type: none"> ▶Hemodynamic stability ▶Intervention, ≥ One: <ul style="list-style-type: none"> ♦Blood product transfusion ♦Hct or Hb monitoring at least 2x/24h ♦Repeat imaging ♦Uterine or vaginal bleeding, abnormal (excludes pregnancy) and, Both: <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦Exertional dyspnea worse than baseline ♦≥ 3 pads/h ♦Orthostatic hypotension, Both: <ul style="list-style-type: none"> ▶Sustained drop in blood pressure within 3 min of sitting or standing ▶Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ♦Syncope ▶Intervention, Both: <ul style="list-style-type: none"> ♦Blood product transfusion or IV fluid ♦Hct or Hb monitoring at least 2x/24h ♦Post Critical care ≤ 24h | <ul style="list-style-type: none"> • Criteria were added to account for the patient with a spontaneous hematoma (e.g., retroperitoneal, intraperitoneal, rectus sheath hemorrhage) who is hemodynamically stable and requires continued stay for Hct or Hb monitoring and/or repeat imaging • Criteria were added to account for the patient with continued uterine or vaginal bleeding who has failed management at the Observation level of care and requires continued stay at a higher level of care on Episode Day 3 |

| Non-Traumatic Bleeding | | |
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| Episode Day 3 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦INTERMEDIATE, ≥ One: <ul style="list-style-type: none"> ▶NIPPV ▶Oxygen ≥ 40%(0.40), ≤ 2d | <ul style="list-style-type: none"> ♦INTERMEDIATE, ≥ One: <ul style="list-style-type: none"> ▶HFNC and respiratory monitoring every 3-4h ▶NIPPV ▶Oxygen ≥ 40%(0.40), ≤ 2d | <p>Rationale:</p> <ul style="list-style-type: none"> • HFNC and respiratory monitoring every 3-4h” criterion was added to reflect the patient who is appropriate at the Intermediate level of care |

| Non-Traumatic Bleeding | | |
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| Episode Day 3 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶ AKI and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> ♦ Loss of P wave ♦ Multifocal PVCs ♦ Neuromuscular deficit ♦ Widening QRS ▶ Sodium, One: <ul style="list-style-type: none"> ♦ < 120 mEq/L(120 mmol/L) and volume overload ♦ > 160 mEq/L(160 mmol/L) ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Calcium chloride ▶ Calcium gluconate ▶ Glucose 50%(0.50) with insulin ▶ Diuretic, continuous ▶ Dialysis, One: <ul style="list-style-type: none"> ♦ Hemodialysis ≤ 7d since initiation ♦ Peritoneal dialysis ≤ 7d since initiation ▶ IV fluid resuscitation ≤ 2d ▶ CRRT <ul style="list-style-type: none"> ▶ Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Heart rate > 120/min ▶ Systolic blood pressure < 100 mmHg and < baseline ▶ MAP < 65 mmHg ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Blood product transfusion ▶ IV fluid resuscitation ≥ 1L ≤ 24h ▶ Hemodynamic monitoring, invasive ▶ HFNC and respiratory monitoring every 1-2h ▶ Mechanical ventilation ▶ NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦ FiO₂ > 50%(0.50) ♦ Respiratory intervention every 1-2h ▶ Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>• CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶ Embolization performed within 24h ▶ Post embolization ≤ 24h ▶ Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Heart rate > 120/min ▶ Systolic blood pressure < 100 mmHg and < baseline ▶ MAP < 65 mmHg ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Blood product transfusion ▶ IV fluid resuscitation ≥ 1L ≤ 24h ▶ Hemodynamic monitoring, invasive ▶ HFNC and respiratory monitoring every 1-2h ▶ Mechanical ventilation ▶ NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦ FiO₂ > 50%(0.50) ♦ Respiratory intervention every 1-2h ▶ Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>Rationale:</p> <ul style="list-style-type: none"> • AKI and CRRT criteria were removed as an individual with AKI or requiring CRRT would be appropriate to be reviewed in the AKI subset • "Embolization performed within 24h" criterion was added to reflect the patient with a spontaneous hematoma who requires the Critical level of care for emergent embolization • "Post embolization ≤ 24h" criterion was added to reflect the patient who underwent embolization in the last 24h and continues to require the Critical level of care for up to 24h of monitoring |

| Non-Traumatic Bleeding | | |
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| Episode Day 4 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• ACUTE, One:</p> <p>► Responder, discharge expected today if clinically stable last 12h, All:</p> <ul style="list-style-type: none"> ♦ Bleeding controlled or resolved ♦ Dyspnea or presyncope/syncope improving or resolved (if present during admission) ♦ Hct or Hb within acceptable limits ♦ Mental status or GCS within normal limits or at baseline ♦ O₂ sat within acceptable limits ♦ Vital signs within acceptable limits ♦ Complication or comorbidity, One: <ul style="list-style-type: none"> ► No complication or active comorbidity relevant to this episode of care ► Complication or comorbidity and clinically stable for discharge, One: <ul style="list-style-type: none"> ♦ AKI, ≥ One: <ul style="list-style-type: none"> ► Kidney function improving or within acceptable limits ► Outpatient dialysis regimen established <p>► Partial responder, not clinically stable for discharge and requires continued stay, ≥ One:</p> <ul style="list-style-type: none"> ♦ Epistaxis, posterior and, Both: <ul style="list-style-type: none"> ► Hct or Hb monitoring at least daily ► Posterior packing ♦ Hematuria and, Both: <ul style="list-style-type: none"> ► Finding, ≥ One: <ul style="list-style-type: none"> ♦ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦ Hct or Hb decreased last 24h ♦ Exertional dyspnea ♦ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦ Orthostatic hypotension, Both: <ul style="list-style-type: none"> ► Sustained drop in blood pressure within 3 min of sitting or standing ► Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ♦ Presyncope or syncope ► Hct or Hb monitoring at least 2x/24h and, ≥ One: <ul style="list-style-type: none"> ♦ Bladder irrigation, continuous or cyclic ≤ 2d ♦ Blood product transfusion ♦ Hemophilia or von Willebrand disease and, Both: <ul style="list-style-type: none"> ► Bleeding with hemodynamic stability ► Intervention, ≥ One: <ul style="list-style-type: none"> ♦ Cryoprecipitate ♦ Factor replacement ♦ Fresh frozen plasma (FFP) ♦ Hemoptysis and, Both: <ul style="list-style-type: none"> ► Finding, ≥ One: <ul style="list-style-type: none"> ♦ Dyspnea ♦ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦ O₂ sat < 90%(0.90) and < baseline ► Requiring supplemental oxygen, new or > baseline requirement | <p>• ACUTE, One:</p> <p>► Responder, discharge expected today if clinically stable last 12h, All:</p> <ul style="list-style-type: none"> ♦ Bleeding controlled or resolved ♦ Hct or Hb within acceptable limits ♦ O₂ sat within acceptable limits ♦ Vital signs within acceptable limits ♦ No complication or active comorbidity relevant to this episode of care <p>► Partial responder, not clinically stable for discharge and requires continued stay, ≥ One:</p> <ul style="list-style-type: none"> ♦ Epistaxis, posterior and, ≥ One: <ul style="list-style-type: none"> ► Embolization or arterial ligation/cautery performed within 24h ► Hct or Hb monitoring at least daily ≤ 24h ► Posterior packing ♦ Hematuria and, Both: <ul style="list-style-type: none"> ► Finding, ≥ One: <ul style="list-style-type: none"> ♦ Hct < 21%(0.21) or Hb < 7.0 g/dL(70 g/L) ♦ Hct or Hb decreased last 24h ♦ Exertional dyspnea worse than baseline ♦ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦ Orthostatic hypotension, Both: <ul style="list-style-type: none"> ► Sustained drop in blood pressure within 3 min of sitting or standing ► Systolic drop ≥ 20 mmHg or diastolic drop ≥ 10 mmHg ♦ Persistent hematuria or blood clot ♦ Syncope ► Intervention, ≥ One: <ul style="list-style-type: none"> ♦ Bladder irrigation, continuous or cyclic ≤ 2d ♦ Blood product transfusion ♦ Hemophilia or von Willebrand disease and, Both: <ul style="list-style-type: none"> ► Persistent bleeding with hemodynamic stability ► Intervention, ≥ One: <ul style="list-style-type: none"> ♦ Cryoprecipitate ♦ Factor replacement ♦ Fresh frozen plasma (FFP) ♦ Hemoptysis and, Both: <ul style="list-style-type: none"> ► Finding, ≥ One: <ul style="list-style-type: none"> ♦ Dyspnea ♦ Mental status changes (excludes coma, stupor, or obtundation) or GCS 9-14 ♦ O₂ sat < 90%(0.90) and < baseline ► Requiring supplemental oxygen, new or > baseline requirement ♦ Post Critical care ≤ 24h | <p>Rationale:</p> <ul style="list-style-type: none"> • Responder criteria were updated to reflect the patient appropriate for discharge • “Epistaxis, posterior and” was updated to include other interventions needed to control bleeding that would require management at the Acute level of care • “Worse than baseline” was added to “Exertional dyspnea” to clarify that the intent of the criteria point is not appropriate for patients who have chronic dyspnea related to a comorbid condition and describing the patient who has exertional dyspnea over and above their chronic state • “Presyncope” was removed as literature does not speak to presyncope as a finding that requires hospitalization in patients with non-traumatic bleeding • “Persistent hematuria or blood clot” was added as a finding on Episode Day 4 to better capture the patient who requires continued stay for “Bladder irrigation, continuous or cyclic ≤ 2d” or “Blood product transfusion” • “Hct or Hb monitoring at least 2x/24h” was changed to “Intervention” to better highlight the treatments required by the patient with continued and/or unresolved hematuria at the Acute level of care • “Persistent” was added to the finding of “bleeding with hemodynamic stability” for Hemophilia or von Willebrand disease to clarify the intent of the criteria • AKI criteria were removed as a patient who develops AKI would be appropriate to be reviewed in the AKI subset |

| Non-Traumatic Bleeding | | |
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| Episode Day 4 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>stupor, or obtundation) or GCS 9-14</p> <ul style="list-style-type: none"> ♦O₂ sat < 90%(0.90) and < baseline ▶Requiring supplemental oxygen, new or > baseline requirement <p>♦AKI, Both:</p> <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦Oliguria ♦Creatinine, One: <ul style="list-style-type: none"> ▶≥ 1.5x baseline and chronic kidney disease ▶≥ 1.5x ULN ♦GFR > 25%(0.25) decrease from baseline ♦Dialysis initiated ▶Intervention, ≥ One: <ul style="list-style-type: none"> ♦Volume expander ≤ 2d ♦Diuretic ≥ 2x/24h, ≤ 3d ♦Dialysis (initial) ≤ 7d since initiation ♦Dialysis discontinued requiring lab monitoring ≤ 2d since discontinuation ♦Immunosuppressant therapy ≤ 3d <p>♦Post Critical care ≤ 24h</p> | | |
| Non-Traumatic Bleeding | | |
| Episode Day 4 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•INTERMEDIATE, ≥ One:</p> <ul style="list-style-type: none"> ▶NIPPV ▶Oxygen ≥ 40%(0.40), ≤ 2d | <p>•INTERMEDIATE, ≥ One:</p> <ul style="list-style-type: none"> ▶HFNC and respiratory monitoring every 3-4h ▶NIPPV ▶Oxygen ≥ 40%(0.40), ≤ 2d | <p>Rationale:</p> <ul style="list-style-type: none"> • “HFNC and respiratory monitoring every 3-4h” criterion was added to reflect the patient who is appropriate at the Intermediate level of care |

| Non-Traumatic Bleeding | | |
|---|---|--|
| Episode Day 4 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶AKI and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> ♦Loss of P wave ♦Multifocal PVCs ♦Neuromuscular deficit ♦Widening QRS ▶Sodium, One: <ul style="list-style-type: none"> ♦< 120 mEq/L(120 mmol/L) and volume overload ♦> 160 mEq/L(160 mmol/L) ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶Dialysis, One: <ul style="list-style-type: none"> ♦Hemodialysis ≤ 7d since initiation ♦Peritoneal dialysis ≤ 7d since initiation ▶IV fluid resuscitation ≤ 2d ▶CRRT <ul style="list-style-type: none"> ▶Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶IV fluid resuscitation ≥ 1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶Embolization performed within 24h ▶Post embolization ≤ 24h ▶Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶IV fluid resuscitation ≥ 1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>Rationale:</p> <ul style="list-style-type: none"> • AKI and CRRT criteria were removed as an individual with AKI or requiring CRRT would be appropriate to be reviewed in the AKI subset • "Embolization performed within 24h" criterion was added to reflect the patient with a spontaneous hematoma who requires the Critical level of care for emergent embolization • "Post embolization ≤ 24h" criterion was added to reflect the patient who underwent embolization in the last 24h and continues to require the Critical level of care for up to 24h of monitoring |

| Non-Traumatic Bleeding | | |
|---|---|---|
| Episode Day 5 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•ACUTE, One:</p> <p>►Responder, discharge expected today if clinically stable last 12h, All:</p> <ul style="list-style-type: none"> ♦Bleeding controlled or resolved ♦Dyspnea or presyncope/syncope improving or resolved (if present during admission) ♦Hct or Hb within acceptable limits ♦Mental status or GCS within normal limits or at baseline ♦O₂ sat within acceptable limits ♦Vital signs within acceptable limits ♦Complication or comorbidity, One: <ul style="list-style-type: none"> ►No complication or active comorbidity relevant to this episode of care ►Complication or comorbidity and clinically stable for discharge, One: <ul style="list-style-type: none"> ♦AKI, ≥ One: <ul style="list-style-type: none"> ►Kidney function improving or within acceptable limits ►Outpatient dialysis regimen established <p>►Non-responder, not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, One:</p> <ul style="list-style-type: none"> ♦Use Extended Stay subset ♦Refer for secondary review | <p>•ACUTE, One:</p> <p>►Responder, discharge expected today if clinically stable last 12h, All:</p> <ul style="list-style-type: none"> ♦Bleeding controlled or resolved ♦Hct or Hb within acceptable limits ♦O₂ sat within acceptable limits ♦Vital signs within acceptable limits ♦No complication or active comorbidity relevant to this episode of care <p>►Non-responder, not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, One:</p> <ul style="list-style-type: none"> ♦Use Extended Stay subset ♦Refer for secondary review | <p>Rationale:</p> <ul style="list-style-type: none"> • Responder criteria were updated to reflect the patient appropriate for discharge |

Postpartum Complication After Discharge



Revision Summary

Postpartum Complication After Discharge content was updated and restructured to streamline the review process, clarify the intent, and align with current guidelines. An Observation level of care has been added that includes chronic hypertension, rule out preeclampsia, and vaginal bleeding criteria. Preeclampsia without severe features and postpartum endometritis content have been added to the Acute level of care. Continuous antihypertensive criteria were added to the Critical level of care to be more inclusive of treatment options for the postpartum individual with severe hypertension refractory to treatment that requires readmission.

Postpartum Complication After Discharge

Episode Day 1 (OBSERVATION)

Previously Available Content

N/A

2024 March Release

- **OBSERVATION, ≥ One:**
 - ▶ Hypertension, chronic and, **Both:**
 - ♦ Systolic blood pressure ≥ 160 mmHg or diastolic blood pressure ≥ 110 mmHg lasting at least 15 min
 - ♦ Intervention, **Both:**
 - ▶ Antihypertensive (includes PO)
 - ▶ Blood pressure monitoring at least 3x/24h
- ▶ Rule out preeclampsia, **All:**
 - ♦ Finding, **≥ One:**
 - ▶ Systolic blood pressure 140-159 mmHg or diastolic blood pressure 90-109 mmHg
 - ▶ Headache, new onset, severe, persistent, and unresponsive to medication
 - ▶ Visual disturbances, persistent
 - ♦ Laboratory evaluation, **All:**
 - ▶ Urine protein analysis
 - ▶ CBC
 - ▶ Creatinine
 - ▶ LFTs
 - ▶ Platelet count
 - ♦ Blood pressure monitoring at least 3x/24h
- ▶ Vaginal bleeding and, **Both:**
 - ♦ Finding, **≥ One:**
 - ▶ Heart rate 100-120/min, sustained
 - ▶ Hct or Hb decreased since delivery
 - ♦ Intervention, **≥ One:**
 - ▶ IV fluid or blood product transfusion
 - ▶ Hct or Hb monitoring at least 2x/24h, $\leq 2d$

Revision Description

Rationale:

- “Hypertension, chronic,” “Rule out preeclampsia,” and “Vaginal bleeding” criteria were added to be more inclusive of the patients who are appropriate for evaluation and monitoring at the Observation level of care
- “Hypertension, chronic” criteria align with current guidelines and allow for initial surveillance, medication titration, and evaluation for superimposed preeclampsia for up to 48 hours in cases of diagnostic uncertainty
- “Rule out preeclampsia” criteria were added to the Observation level of care to allow for initial surveillance and work-up of preeclampsia before a definitive diagnosis of preeclampsia without severe features is made
- “Bleeding actual or suspected” criteria were moved from the Acute level of care to the Observation level of care, with verbiage changed to “Vaginal bleeding;” and criteria were streamlined and updated to clarify the intent and focus on the primary drivers for evaluation and management of hemodynamically stable individuals

| Postpartum Complication After Discharge | | |
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| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• ACUTE, ≥ One:</p> <ul style="list-style-type: none"> ▶ Bleeding actual or suspected and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Heart rate 100-120/min, sustained ▶ Hct or Hb decreased within last 24h ▶ Orthostatic hypotension, Both: <ul style="list-style-type: none"> ♦ Sustained drop in blood pressure within 3 min of sitting or standing ▶ Blood pressure, ≥ One: <ul style="list-style-type: none"> ♦ Systolic drop ≥ 20 mmHg ♦ Diastolic drop ≥ 10 mmHg ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Blood product transfusion ▶ Hct or Hb monitoring at least 2x/24h, ≤ 2d <p>▶ HELLP syndrome ≤ 5d and, All:</p> <ul style="list-style-type: none"> ♦ Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 70 U/L(1.17 µkat/L) ♦ Platelet count 50,000-100,000/cu.mm(50-100x10⁹/L) ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Fragmented or contracted red cells on blood smear ▶ Total serum lactate dehydrogenase (LDH) ≥ 600 IU/L(10.02 µkat/L) ▶ Total bilirubin > 1.20 mg/dL(20.52 µmol/L) ▶ Hypertension, ≥ One: <ul style="list-style-type: none"> ♦ Systolic blood pressure 140-159 mmHg ♦ Diastolic blood pressure 90-109 mmHg ▶ Proteinuria, ≥ One: <ul style="list-style-type: none"> ♦ 24 hour urine collection and protein ≥ 0.3 g/dL(30.0 g/L) ♦ Random urine sample and protein ≥ 30 mg/mmol(265.5 mg/g) ♦ Blood pressure management, ≥ One: <ul style="list-style-type: none"> ▶ Antihypertensive (includes PO) ▶ Blood pressure monitoring at least 3x/24h ♦ Laboratory monitoring, All: <ul style="list-style-type: none"> ▶ Complete blood count (CBC) ▶ Liver function tests (LFT) ▶ Platelet count ▶ Hypertension, chronic or gestational and, Both: <ul style="list-style-type: none"> ♦ Systolic blood pressure ≥ 160 mmHg or diastolic blood pressure ≥ 110 mmHg lasting at least 15 min ♦ Intervention, Both: <ul style="list-style-type: none"> ▶ Antihypertensive (includes PO) ▶ Blood pressure monitoring at least 3x/24h <p>▶ Preeclampsia with severe features ≤ 5d and, All:</p> <ul style="list-style-type: none"> ♦ Finding, One: <ul style="list-style-type: none"> ▶ Systolic blood pressure ≥ 160 mmHg or diastolic blood pressure ≥ 110 mmHg on ≥ 2 measurements | <p>• ACUTE, ≥ One:</p> <ul style="list-style-type: none"> ▶ HELLP syndrome, All: <ul style="list-style-type: none"> ♦ AST or ALT > 2x ULN ♦ Platelet count 50,000-100,000/cu.mm(50-100x10⁹/L) ♦ LDH ≥ 600 IU/L(10.02 µkat/L) ♦ Blood pressure management, ≥ One: <ul style="list-style-type: none"> ▶ Antihypertensive (includes PO) ▶ Blood pressure monitoring at least 3x/24h ♦ Laboratory monitoring, All: <ul style="list-style-type: none"> ▶ CBC ▶ LFTs ▶ Platelet count ▶ Postpartum endometritis and, Both: <ul style="list-style-type: none"> ♦ Temperature ≥ 100.4°F(38.0°C) and, ≥ One: <ul style="list-style-type: none"> ▶ Purulent vaginal discharge ▶ Abdominal pain or uterine tenderness ▶ Vaginal bleeding ♦ Anti-infective ▶ Preeclampsia with severe features and, All: <ul style="list-style-type: none"> ♦ Finding, One: <ul style="list-style-type: none"> ▶ Systolic blood pressure ≥ 160 mmHg or diastolic blood pressure ≥ 110 mmHg on ≥ 2 measurements ▶ Systolic blood pressure 140-159 mmHg or diastolic blood pressure 90-109 mmHg on ≥ 2 measurements and, ≥ One: <ul style="list-style-type: none"> ♦ AST or ALT > 2x ULN ♦ Creatinine, ≥ One: <ul style="list-style-type: none"> ▶ > 1.1 mg/dL(97.2 µmol/L) ▶ 2x baseline ♦ Headache, new onset, severe, persistent, and unresponsive to medication ♦ Platelet count < 100,000/cu.mm(100x10⁹/L) ♦ Visual disturbances, persistent ♦ Monitoring, Both: <ul style="list-style-type: none"> ▶ Blood pressure monitoring at least 3x/24h ▶ Laboratory monitoring, All: <ul style="list-style-type: none"> ♦ CBC ♦ Creatinine ♦ LFTs ♦ Platelet count ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Antihypertensive (includes PO) ▶ Magnesium sulfate | <p>Rationale:</p> <ul style="list-style-type: none"> • “Bleeding actual or suspected” criteria were removed from the Acute level of care and added to the Observation level of care as hemodynamically stable individuals with vaginal bleeding are appropriate for evaluation and management at the Observation level of care • Time-limited endpoint was removed from HELLP syndrome as this condition requires close monitoring and may require a longer length of stay • HELLP criteria were streamlined, and a specified number for AST or ALT was changed to greater than 2x ULN to account for institutional assay variations and to align with the diagnostic criteria noted in current guidelines • Based on current guidelines, HELLP criteria were revised by aligning with primary interventions and more appropriate diagnostic benchmarks, removal of total bilirubin and fragmented or contracted red blood cells criteria, as these are not universally utilized; and hypertension and proteinuria were also removed to align with guidelines and focus on the primary drivers and diagnostic benchmarks for this condition • “Hypertension, chronic” criteria were moved from the Acute level of care to the Observation level of care to align with current guidelines and allow for initial surveillance, medication titration, and evaluation for superimposed preeclampsia for up to 48 hours in cases of diagnostic uncertainty • Postpartum endometritis has been added to be inclusive of this postpartum maternal infection that is appropriate for evaluation and management at the Acute level of care for IV anti-infectives • Preeclampsia with severe features criteria were revised to streamline the review process, focus on primary findings and drivers for care in the postpartum period, and align with current |

| Postpartum Complication After Discharge | | |
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| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ►Systolic blood pressure 140-159 mmHg or diastolic blood pressure 90-109 mmHg on ≥ 2 measurements and, $\geq$ One: <ul style="list-style-type: none"> ♦Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) $> 2\times$ upper limit of normal (ULN) ♦Creatinine, $\geq$ One: <ul style="list-style-type: none"> ►> 1.1 mg/dL($97.2 \mu\text{mol/L}$) ►$> 2\times$ baseline ♦Pain, severe persistent and unresponsive to medication and, $\geq$ One: <ul style="list-style-type: none"> ►Epigastric ►Headache, new onset ►Right upper quadrant ♦Platelet count $< 100,000/\text{cu.mm}(100\times 10^9/\text{L})$ ♦Visual disturbances, persistent ♦Monitoring, Both: <ul style="list-style-type: none"> ►Blood pressure monitoring at least $3\times/24\text{h}$ ►Laboratory monitoring, All: <ul style="list-style-type: none"> ♦Complete blood count (CBC) ♦Creatinine ♦Liver function tests (LFT) ♦Platelet count ♦Intervention, $\geq$ One: <ul style="list-style-type: none"> ►Antihypertensive (includes PO) ►Magnesium sulfate | | <p>guidelines, and the time-limited endpoint was removed to allow for a longer length of stay</p> |

| Postpartum Complication After Discharge | | |
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| Episode Day 1 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦CRITICAL, $\geq$ One: <ul style="list-style-type: none"> ►Acute kidney injury (AKI) and, Both: <ul style="list-style-type: none"> ♦Finding, $\geq$ One: <ul style="list-style-type: none"> ►Potassium > 6.0 mEq/L(6.0 mmol/L) and, $\geq$ One: <ul style="list-style-type: none"> ♦Loss of P wave ♦Multifocal PVCs ♦Neuromuscular deficit ♦Widening QRS ►Sodium, One: <ul style="list-style-type: none"> ♦< 120 mEq/L(120 mmol/L) and volume overload | <ul style="list-style-type: none"> ♦CRITICAL, $\geq$ One: <ul style="list-style-type: none"> ►Antihypertensive, continuous ►DIC and blood product transfusion ►Eclampsia and neurological assessment every 1-2h ►ECMO or ECLS ►Heart failure and, Both: <ul style="list-style-type: none"> ♦Finding, $\geq$ One: <ul style="list-style-type: none"> ►Arterial $\text{Po}_2 < 56$ mmHg(7.4 kPa) ►O_2 sat $< 89\%(0.89)$ and $<$ baseline ♦Diuretic every 1-2h or continuous ►Hemodynamic instability, All: <ul style="list-style-type: none"> ♦ED treatment, $\geq$ One: | <p>Rationale:</p> <ul style="list-style-type: none"> • Acute Kidney Injury (AKI) and CRRT criteria were removed as an individual with AKI or requiring CRRT can be reviewed in the AKI subset • Criteria for continuous antihypertensive infusion were added to the Critical level of care to align with guidelines and provide a pathway for postpartum individuals with severe hypertension refractory to |

Postpartum Complication After Discharge

Episode Day 1 (CRITICAL)

| Previously Available Content | 2024 March Release | Revision Description |
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| <ul style="list-style-type: none"> ♦ > 160 mEq/L (160 mmol/L) ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Calcium chloride ▶ Calcium gluconate ▶ Glucose 50% (0.50) with insulin ▶ Diuretic, continuous ▶ Hemodialysis ≤ 7d since initiation ▶ IV fluid resuscitation ≤ 2d ▶ CRRT ▶ DIC and blood product transfusion ▶ Eclampsia and neurological assessment every 1-2h, ≤ 2d ▶ ECMO or ECLS ▶ Heart failure and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Arterial Po₂ < 56 mmHg (7.4 kPa) ▶ O₂ sat < 89% (0.89) and < baseline ♦ Diuretic every 1-2h or continuous ▶ Hemodynamic instability, All: <ul style="list-style-type: none"> ♦ ED treatment, ≥ One: <ul style="list-style-type: none"> ▶ ≥ 2 units blood product transfusion ▶ ≥ 2 liters IV fluid ▶ ≥ 1 unit blood product transfusion and ≥ 1 liter IV fluid ♦ Findings after ED treatment, ≥ One: <ul style="list-style-type: none"> ▶ Heart rate > 120/min ▶ Systolic blood pressure < 100 mmHg and < baseline ▶ MAP < 65 mmHg ♦ Requiring ongoing resuscitation ▶ Hemodynamic monitoring, invasive ▶ HELLP syndrome ≤ 2d, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Lab values, All: <ul style="list-style-type: none"> ♦ Platelet count < 50,000/cu.mm (50x10⁹/L) ♦ AST or ALT > 2x ULN ♦ LDH ≥ 600 IU/L (10.02 μkat/L) ▶ Increased risk of bleeding, ≥ One: <ul style="list-style-type: none"> ♦ PT ≥ 1.5x ULN or INR ≥ 2.0 ♦ PTT ≥ 1.5x ULN ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Blood product transfusion ▶ Laboratory monitoring, All: <ul style="list-style-type: none"> ♦ CBC ♦ LFTs ♦ Platelet count ▶ HFNC ▶ Mechanical ventilation ▶ NIPPV ▶ Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <ul style="list-style-type: none"> ▶ ≥ 2 units blood product transfusion ▶ ≥ 2 liters IV fluid ▶ ≥ 1 unit blood product transfusion and ≥ 1 liter IV fluid ♦ Findings after ED treatment, ≥ One: <ul style="list-style-type: none"> ▶ Heart rate > 120/min ▶ Systolic blood pressure < 100 mmHg and < baseline ▶ MAP < 65 mmHg ♦ Requiring ongoing resuscitation ▶ Hemodynamic monitoring, invasive ▶ HELLP syndrome, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Lab values, All: <ul style="list-style-type: none"> ♦ Platelet count < 50,000/cu.mm (50x10⁹/L) ♦ AST or ALT > 2x ULN ♦ LDH ≥ 600 IU/L (10.02 μkat/L) ▶ Increased risk of bleeding, ≥ One: <ul style="list-style-type: none"> ♦ PT ≥ 1.5x ULN or INR ≥ 2.0 ♦ PTT ≥ 1.5x ULN ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Blood product transfusion ▶ Laboratory monitoring, All: <ul style="list-style-type: none"> ♦ CBC ♦ LFTs ♦ Platelet count ▶ HFNC ▶ Mechanical ventilation ▶ NIPPV ▶ Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>treatment who require continuous antihypertensive infusion</p> <ul style="list-style-type: none"> • The time-limited endpoint was removed from "Eclampsia and neurological assessment every 1-2h" to clarify the intent of the criteria, as the condition and associated monitoring are the drivers that are appropriate for evaluation and management at the Critical Level of Care • Based on current guidelines, HELLP criteria were revised by aligning with primary interventions and more appropriate diagnostic benchmarks, removal of total bilirubin and fragmented or contracted red blood cells criteria, as these are not universally utilized. Time-limited endpoint was removed as the condition requires close monitoring for progression at the Critical Level of Care • HFNC subcriteria were removed because any patient who is newly requiring HFNC is appropriate for the Critical level of care on Episode Day 1 • NIPPV subcriteria were removed because any patient who is newly requiring noninvasive ventilator support is appropriate for the Critical level of care on Episode Day 1 • "Seizure, recurrent or refractory requiring anticonvulsant criteria" were removed as this subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology |

| Postpartum Complication After Discharge | | |
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| Episode Day 1 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ▶ HFNC and respiratory monitoring every 1-2h ▶ Mechanical ventilation ▶ NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦ Arterial Po₂ ≤ 39 mmHg(5.2 kPa) ♦ Arterial or venous Pco₂ ≥ 60 mmHg(8.0 kPa) and, One: <ul style="list-style-type: none"> ▶ COPD and arterial or venous pH ≤ 7.24 ▶ No hx of COPD ▶ Seizure (excludes epilepsy), recurrent or refractory requiring anticonvulsant ▶ Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | | |
| Postpartum Complication After Discharge | | |
| Episode Day 2 (OBSERVATION) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| N/A | <ul style="list-style-type: none"> • OBSERVATION, One: <ul style="list-style-type: none"> ▶ Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦ Blood pressure within acceptable limits ♦ No evidence of severe preeclampsia or disease progression ♦ Bleeding controlled or resolved ▶ Non-responder, not clinically stable for discharge and exceeds episode days at the Observation level of care for this condition, One: <ul style="list-style-type: none"> ♦ For preeclampsia use Episode Day 2 at a higher level of care ♦ For a condition not included in the Postpartum Complication After Discharge subset use appropriate subset (Episode Day 1) based on clinical findings ♦ Refer for secondary review | <p>Rationale:</p> <ul style="list-style-type: none"> Responder criteria were added to reflect the patient appropriate for discharge Non-responder criteria were added to reflect the patient who is not clinically stable, the patient with preeclampsia who can step up to Episode 2 at a higher level of care, be reviewed in another condition specific subset on Episode Day 1, or requires secondary review |

| Postpartum Complication After Discharge | | |
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| Episode Day 2 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• ACUTE, ≥ One:</p> <ul style="list-style-type: none"> ▶ Bleeding actual or suspected and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Heart rate 100-120/min, sustained ▶ Hct or Hb decreased within last 24h ▶ Orthostatic hypotension, Both: <ul style="list-style-type: none"> ♦ Sustained drop in blood pressure within 3 min of sitting or standing ▶ Blood pressure, ≥ One: <ul style="list-style-type: none"> ♦ Systolic drop ≥ 20 mmHg ♦ Diastolic drop ≥ 10 mmHg ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Blood product transfusion ▶ Hct or Hb monitoring at least 2x/24h, ≤ 2d ▶ Fever and, Both: <ul style="list-style-type: none"> ♦ Temperature, ≥ One: <ul style="list-style-type: none"> ▶ > 99.4°F(37.4°C) PO ▶ > 100.4°F(38.0°C) PR ♦ Intervention, One: <ul style="list-style-type: none"> ▶ Known source and anti-infective ▶ New onset and, ≥ One: <ul style="list-style-type: none"> ♦ Culture pending ≤ 2d ♦ Drug-induced fever suspected and precipitating drug discontinued ≤ 3d ♦ Imaging study ≤ 24h ▶ HELLP syndrome ≤ 5d and, Both: <ul style="list-style-type: none"> ♦ Lab values, ≥ One: <ul style="list-style-type: none"> ▶ Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 70 U/L(1.17 µkat/L) ▶ Platelet count 50,000-100,000/cu.mm(50-100x10⁹/L) ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Fragmented or contracted red cells on blood smear ▶ Total serum lactate dehydrogenase (LDH) ≥ 600 IU/L(10.02 µkat/L) ▶ Total bilirubin > 1.20 mg/dL(20.52 µmol/L) ♦ Intervention, All: <ul style="list-style-type: none"> ▶ Blood pressure management, ≥ One: <ul style="list-style-type: none"> ♦ Antihypertensive (includes PO) ▶ Blood pressure monitoring at least 3x/24h ▶ Laboratory monitoring, All: <ul style="list-style-type: none"> ♦ Complete blood count (CBC) ♦ Liver function tests (LFT) ♦ Platelet count ▶ Magnesium sulfate ▶ Hypertension uncontrolled ≤ 2d, All: <ul style="list-style-type: none"> ♦ Systolic blood pressure ≥ 150 mmHg or diastolic blood pressure ≥ 100 mmHg last 24h ♦ Antihypertensive (includes PO) ♦ Blood pressure monitoring at least 3x/24h ▶ Preeclampsia or eclampsia ≤ 5d and, Both: | <p>• ACUTE, One:</p> <ul style="list-style-type: none"> ▶ Responder, discharge expected today if clinically stable last 12h, Both: <ul style="list-style-type: none"> ♦ Afebrile ♦ Clinical stability, ≥ One: <ul style="list-style-type: none"> ▶ HELLP syndrome and, Both: <ul style="list-style-type: none"> ♦ Blood pressure controlled last 24h ♦ Lab values within acceptable limits ▶ Preeclampsia resolved ▶ Endometritis resolving and, ≥ One: <ul style="list-style-type: none"> ♦ Anti-infective regimen established and tolerated ♦ Symptoms resolving ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦ HELLP syndrome and, Both: <ul style="list-style-type: none"> ▶ Blood pressure management, ≥ One: <ul style="list-style-type: none"> ♦ Antihypertensive (includes PO) ▶ Blood pressure monitoring at least 3x/24h ▶ Laboratory monitoring, All: <ul style="list-style-type: none"> ♦ CBC ♦ LFTs ♦ Platelet count ♦ Postpartum endometritis and anti-infective ♦ Preeclampsia with severe features and, ≥ One: <ul style="list-style-type: none"> ▶ Blood pressure management, ≥ One: <ul style="list-style-type: none"> ♦ Antihypertensive (includes PO) ▶ Blood pressure monitoring at least 3x/24h ▶ Magnesium sulfate ♦ Preeclampsia without severe features and, Both: <ul style="list-style-type: none"> ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ Limited access to prompt medical care and arrangements for outpatient services pending ≤ 24h ♦ Outpatient services unavailable ♦ Patient or caregiver unreliable and unable to adhere to outpatient instructions or treatment ♦ Severe and persistent mental illness, cognitive impairment, or substance use disorder ▶ Blood pressure management, ≥ One: <ul style="list-style-type: none"> ♦ Antihypertensive (includes PO) ▶ Blood pressure monitoring at least 3x/24h ♦ Post Critical care ≤ 24h | <p>Rationale:</p> <ul style="list-style-type: none"> • Responder criteria were added to reflect the patient appropriate for discharge • “Bleeding actual or suspected” criteria were removed from the Acute level of care and added to the Observation level of care to allow for evaluation and management for up to 48 hours for hemodynamically stable individuals with postpartum vaginal bleeding • “Fever” criteria were removed as an individual with postpartum fever and infection can be reviewed utilizing the appropriate relevant criteria within this subset (i.e., postpartum endometritis) or the relevant condition-specific subset (e.g., Infection: General, Infection: GI/GYN, Infection: Pyelonephritis or Complex UTI, Infection: Sepsis, Infection: Skin) • “Postpartum endometritis” criteria were added to reflect the patient who is appropriate for continued stay at the Acute Level of Care in alignment with guidelines indicating that postpartum endometritis should be treated with IV anti-infectives for a minimum of 24-48 hours after symptoms have resolved and the patient is afebrile • Time-limited endpoint was removed from HELLP syndrome as this condition requires close monitoring and may require a longer length of stay • HELLP syndrome criteria were revised and streamlined to focus on the primary drivers appropriate for continued stay evaluation and management • “Preeclampsia” and “hypertension” criteria were revised to streamline the review process and focus on primary findings and drivers of care that are appropriate for continued stay evaluation and management • Preeclampsia without severe features criteria have been added to align with guidelines and provide a |

| Postpartum Complication After Discharge | | |
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| Episode Day 2 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>♦Finding, ≥ One:</p> <ul style="list-style-type: none"> ▶Systolic blood pressure ≥ 150 mmHg or diastolic blood pressure ≥ 100 mmHg last 24h ▶Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) elevated ▶Creatinine $>$ baseline ▶Pain, severe persistent and unresponsive to medication and, ≥ One: <ul style="list-style-type: none"> ♦Epigastric ♦Headache, new onset ♦Right upper quadrant ▶Platelet count $< 100,000/\text{cu.mm}(100 \times 10^9/\text{L})$ ▶Visual disturbances, persistent <p>♦Intervention, ≥ One:</p> <ul style="list-style-type: none"> ▶Antihypertensive (includes PO) ▶Magnesium sulfate <p>♦Monitoring, Both:</p> <ul style="list-style-type: none"> ♦Blood pressure monitoring at least 3x/24h ♦Laboratory monitoring, All: <ul style="list-style-type: none"> ▶Complete blood count (CBC) ▶Creatinine ▶Liver function tests (LFT) ▶Platelet count | | <p>pathway for evaluation and management at the Acute level of care for individuals without severe features that have coexisting comorbidities, risk factors, social determinants of health (SDOH), or barriers to discharge that preclude treatment in the outpatient setting</p> <ul style="list-style-type: none"> • “Post critical care $\leq 24\text{h}$” criterion was added to provide a pathway for the patient who is stepping down from the Critical to the Acute level of care |

| Postpartum Complication After Discharge | | |
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| Episode Day 2 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>♦INTERMEDIATE, ≥ One:</p> <ul style="list-style-type: none"> ▶Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦IV antiarrhythmic and hemodynamically stable ♦Antiarrhythmic conversion from IV to PO, One: <ul style="list-style-type: none"> ▶No proarrhythmic risk $\leq 24\text{h}$ ▶Proarrhythmic risk and continuous cardiac monitoring (excludes Holter) $\leq 3\text{d}$ ▶Eclampsia and neurological assessment every 3-4h $\leq 2\text{d}$ ▶NIPPV ▶Oxygen, ≥ One: <ul style="list-style-type: none"> ♦HFNC and respiratory monitoring every 3-4h ♦Oxygen $\geq 40\%(0.40) \leq 2\text{d}$ ▶Seizure and, Both: <ul style="list-style-type: none"> ♦Activity resolved | <p>♦INTERMEDIATE, ≥ One:</p> <ul style="list-style-type: none"> ▶Eclampsia and neurological assessment every 3-4h ▶NIPPV ▶HFNC and respiratory monitoring every 3-4h ▶Oxygen $\geq 40\%(0.40)$ | <p>Rationale:</p> <ul style="list-style-type: none"> • "Arrhythmia" content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • The time-limited endpoint was removed from "Eclampsia and neurological assessment every 3-4h" to clarify the intent of the criteria, as the condition and associated monitoring are the drivers that are appropriate for evaluation and management at the Intermediate Level of Care |

| Postpartum Complication After Discharge | | |
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| Episode Day 2 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| ♦Anticonvulsant continuous | | <ul style="list-style-type: none"> Oxygen $\geq 40\%$” criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Intermediate level of care “Seizure” criteria were removed as this subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology |

| Postpartum Complication After Discharge | | |
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| Episode Day 2 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•CRITICAL, $\geq$ One:</p> <ul style="list-style-type: none"> ▶Acute kidney injury (AKI) and, Both: <ul style="list-style-type: none"> ♦Finding, $\geq$ One: <ul style="list-style-type: none"> ▶Potassium > 6.0 mEq/L(6.0 mmol/L) and, $\geq$ One: <ul style="list-style-type: none"> ♦Loss of P wave ♦Multifocal PVCs ♦Neuromuscular deficit ♦Widening QRS ▶Sodium, One: <ul style="list-style-type: none"> ♦< 120 mEq/L(120 mmol/L) and volume overload ♦> 160 mEq/L(160 mmol/L) ♦Intervention, $\geq$ One: <ul style="list-style-type: none"> ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶Hemodialysis ≤ 7d since initiation ▶IV fluid resuscitation ≤ 2d ▶CRRT ▶DIC and blood product transfusion ▶Eclampsia and neurological assessment every 1-2h, ≤ 2d ▶ECMO or ECLS | <p>•CRITICAL, $\geq$ One:</p> <ul style="list-style-type: none"> ▶Antihypertensive, continuous ▶DIC and blood product transfusion ▶Eclampsia and neurological assessment every 1-2h ▶ECMO or ECLS ▶Heart failure and, Both: <ul style="list-style-type: none"> ♦Finding, $\geq$ One: <ul style="list-style-type: none"> ▶Arterial $P_{O_2} < 56$ mmHg(7.4 kPa) ▶O_2 sat $< 89\%$(0.89) and $<$ baseline ♦Diuretic every 1-2h or continuous ▶Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion and, Both: <ul style="list-style-type: none"> ♦Finding, $\geq$ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and $<$ baseline ▶MAP < 65 mmHg ♦Intervention, $\geq$ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶IV fluid resuscitation ≥ 1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶HELLP syndrome, Both: <ul style="list-style-type: none"> ♦Finding, $\geq$ One: <ul style="list-style-type: none"> ▶Lab values, All: | <p>Rationale:</p> <ul style="list-style-type: none"> Acute Kidney Injury (AKI) and CRRT criteria were removed as an individual with AKI or requiring CRRT can be reviewed in the AKI subset Criteria for continuous antihypertensive infusion were added to the Critical level of care to align with guidelines and provide a pathway for postpartum individuals with severe hypertension refractory to treatment who require continuous antihypertensive infusion The time-limited endpoint was removed from "Eclampsia and neurological assessment every 1-2h" to clarify the intent of the criteria, as the condition and associated monitoring are the drivers that are appropriate for evaluation and management at the Critical Level of Care Based on current guidelines, HELLP criteria were revised by aligning with primary interventions and |

Postpartum Complication After Discharge

Episode Day 2 (CRITICAL)

| Previously Available Content | 2024 March Release | Revision Description |
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| <ul style="list-style-type: none"> ▶Heart failure and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Arterial Po₂ < 56 mmHg(7.4 kPa) ▶O₂ sat < 89%(0.89) and < baseline ♦Diuretic every 1-2h or continuous ▶Hemodynamic instability persisting after ≥1L of IVF resuscitation or ≥1 unit blood product transfusion and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶IV fluid resuscitation ≥1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶HELLP syndrome ≤ 2d, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Lab values, All: <ul style="list-style-type: none"> ♦Platelet count < 50,000/cu.mm(50x10⁹/L) ♦Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 70 U/L(1.17 μkat/L) ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Fragmented or contracted red cells on blood smear ▶Total serum lactate dehydrogenase (LDH) ≥ 600 IU/L(10.02 μkat/L) ▶Total bilirubin > 1.20 mg/dL(20.52 μmol/L) ▶Increased risk of bleeding, ≥ One: <ul style="list-style-type: none"> ♦PT ≥ 1.5x upper limit of normal (ULN) or INR ≥ 2.0 ♦PTT ≥ 1.5x upper limit of normal (ULN) ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶Laboratory monitoring, All: <ul style="list-style-type: none"> ♦Complete blood count (CBC) ♦Liver function tests (LFT) ♦Platelet count ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Seizure (excludes epilepsy), recurrent or refractory requiring anticonvulsant ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <ul style="list-style-type: none"> ♦Platelet count < 50,000/cu.mm(50x10⁹/L) ♦AST or ALT > 2x ULN ♦LDH ≥ 600 IU/L(10.02 μkat/L) ▶Increased risk of bleeding, ≥ One: <ul style="list-style-type: none"> ♦PT ≥ 1.5x ULN or INR ≥ 2.0 ♦PTT ≥ 1.5x ULN ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶Laboratory monitoring, All: <ul style="list-style-type: none"> ♦CBC ♦LFTs ♦Platelet count ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>more appropriate diagnostic benchmarks, removal of total bilirubin and fragmented or contracted red blood cells criteria, as these are not universally utilized. Time-limited endpoint was removed as the condition requires close monitoring for progression at the Critical Level of Care</p> <ul style="list-style-type: none"> • “Seizure, recurrent or refractory requiring anticonvulsant criteria” were removed as this subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology |

| Postpartum Complication After Discharge | | |
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| Episode Day 3 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• ACUTE, One:</p> <ul style="list-style-type: none"> ▶ Early Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦ Afebrile ♦ Clinical stability, ≥ One: <ul style="list-style-type: none"> ▶ Bleeding controlled or resolved ▶ HELLP syndrome and, Both: <ul style="list-style-type: none"> ♦ Blood pressure controlled last 24h ♦ Lab values within acceptable limits ▶ Hypertension controlled or within acceptable limits last 24h ▶ Preeclampsia or eclampsia resolved ♦ No complication or active comorbidity relevant to this episode of care ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦ Bleeding actual or suspected and, Both: <ul style="list-style-type: none"> ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ Heart rate 100-120/min, sustained ♦ Hct or Hb decreased within last 24h ♦ Orthostatic hypotension, Both: <ul style="list-style-type: none"> ▶ Sustained drop in blood pressure within 3 min of sitting or standing ♦ Blood pressure, ≥ One: <ul style="list-style-type: none"> ♦ Systolic drop ≥ 20 mmHg ♦ Diastolic drop ≥ 10 mmHg ▶ Intervention, ≥ One: <ul style="list-style-type: none"> ♦ Blood product transfusion ♦ Hct or Hb monitoring at least 2x/24h, $\leq 2d$ ♦ Fever and, Both: <ul style="list-style-type: none"> ▶ Temperature, ≥ One: <ul style="list-style-type: none"> ♦ $> 99.4^{\circ}\text{F}(37.4^{\circ}\text{C})$ PO ♦ $> 100.4^{\circ}\text{F}(38.0^{\circ}\text{C})$ PR ▶ Intervention, One: <ul style="list-style-type: none"> ♦ Known source and anti-infective ♦ New onset and, ≥ One: <ul style="list-style-type: none"> ▶ Culture pending $\leq 2d$ ▶ Drug-induced fever suspected and precipitating drug discontinued $\leq 3d$ ▶ Imaging study $\leq 24h$ ♦ HELLP syndrome $\leq 5d$ and, Both: <ul style="list-style-type: none"> ▶ Lab values, ≥ One: <ul style="list-style-type: none"> ♦ Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 70 U/L(1.17 $\mu\text{kat/L}$) ♦ Platelet count 50,000-100,000/cu.mm(50-100$\times 10^9$/L) ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Fragmented or contracted red cells on blood smear ▶ Total serum lactate dehydrogenase (LDH) ≥ 600 IU/L(10.02 $\mu\text{kat/L}$) ▶ Total bilirubin > 1.20 mg/dL(20.52 $\mu\text{mol/L}$) | <p>• ACUTE, One:</p> <ul style="list-style-type: none"> ▶ Responder, discharge expected today if clinically stable last 12h, Both: <ul style="list-style-type: none"> ♦ Afebrile ♦ Clinical stability, ≥ One: <ul style="list-style-type: none"> ▶ HELLP syndrome and, Both: <ul style="list-style-type: none"> ♦ Blood pressure controlled last 24h ♦ Lab values within acceptable limits ▶ Preeclampsia resolved ▶ Endometritis resolving and, ≥ One: <ul style="list-style-type: none"> ♦ Anti-infective regimen established and tolerated ♦ Symptoms resolving ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦ HELLP syndrome and, Both: <ul style="list-style-type: none"> ▶ Blood pressure management, ≥ One: <ul style="list-style-type: none"> ♦ Antihypertensive (includes PO) ♦ Blood pressure monitoring at least 3x/24h ▶ Laboratory monitoring, All: <ul style="list-style-type: none"> ♦ CBC ♦ LFTs ♦ Platelet count ♦ Postpartum endometritis and anti-infective ♦ Preeclampsia with severe features and, ≥ One: <ul style="list-style-type: none"> ▶ Blood pressure management, ≥ One: <ul style="list-style-type: none"> ♦ Antihypertensive (includes PO) ♦ Blood pressure monitoring at least 3x/24h ▶ Magnesium sulfate ♦ Preeclampsia without severe features and, Both: <ul style="list-style-type: none"> ▶ Finding, ≥ One: <ul style="list-style-type: none"> ♦ Limited access to prompt medical care and arrangements for outpatient services pending $\leq 24h$ ♦ Outpatient services unavailable ♦ Patient or caregiver unreliable and unable to adhere to outpatient instructions or treatment ♦ Severe and persistent mental illness, cognitive impairment, or substance use disorder ▶ Blood pressure management, ≥ One: <ul style="list-style-type: none"> ♦ Antihypertensive (includes PO) ♦ Blood pressure monitoring at least 3x/24h ♦ Post Critical care $\leq 24h$ | <p>Rationale:</p> <ul style="list-style-type: none"> • Responder criteria were updated to reflect the patient appropriate for discharge • “Bleeding actual or suspected” criteria were removed from the Acute level of care, and added to the Observation level of care to allow for evaluation and management for up to 48 hours for hemodynamically stable individuals with postpartum vaginal bleeding • “Fever” criteria were removed as an individual with postpartum fever and infection can be reviewed utilizing the appropriate relevant criteria within this subset (i.e., postpartum endometritis) or the relevant condition-specific subset (e.g., Infection: General, Infection: GI/GYN, Infection: Pyelonephritis or Complex UTI, Infection: Sepsis, Infection: Skin) • “Postpartum endometritis” criteria were added to reflect the patient who is appropriate for continued stay at the Acute Level of Care in alignment with guidelines indicating that postpartum endometritis should be treated with IV anti-infectives for a minimum of 24-48 hours after symptoms have resolved and the patient is afebrile • Time-limited endpoint was removed from HELLP syndrome as this condition requires close monitoring and may require a longer length of stay • HELLP syndrome criteria were revised and streamlined to focus on the primary drivers appropriate for continued stay evaluation and management • “Preeclampsia” and “hypertension” criteria were revised to streamline the review process and focus on primary findings and drivers of care that are appropriate for continued stay evaluation and management • Preeclampsia without severe features criteria have been added to align with guidelines and provide a |

| Postpartum Complication After Discharge | | |
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| Episode Day 3 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p> Intervention, All:
 ♦ Blood pressure management, ≥ One:
 ▶ Antihypertensive (includes PO)
 ▶ Blood pressure monitoring at least 3x/24h
 ♦ Laboratory monitoring, All:
 ▶ Complete blood count (CBC)
 ▶ Liver function tests (LFT)
 ▶ Platelet count
 ♦ Magnesium sulfate
 ♦ Hypertension uncontrolled $\leq 2d$ and, All:
 ▶ Systolic blood pressure ≥ 150 mmHg or diastolic blood pressure ≥ 100 mmHg last 24h
 ▶ Antihypertensive (includes PO)
 ▶ Blood pressure monitoring at least 3x/24h
 ♦ Preeclampsia or eclampsia $\leq 5d$ and, Both:
 ▶ Finding, ≥ One:
 ♦ Systolic blood pressure ≥ 150 mmHg or diastolic blood pressure ≥ 100 mmHg last 24h
 ♦ Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) elevated
 ♦ Creatinine $>$ baseline
 ♦ Pain, severe persistent and unresponsive to medication and, ≥ One:
 ▶ Epigastric
 ▶ Headache, new onset
 ▶ Right upper quadrant
 ♦ Platelet count $< 100,000/cu.mm(100 \times 10^9/L)$
 ♦ Visual disturbances, persistent
 ▶ Intervention, ≥ One:
 ♦ Antihypertensive (includes PO)
 ♦ Magnesium sulfate
 ♦ Monitoring, Both:
 ▶ Blood pressure monitoring at least 3x/24h
 ▶ Laboratory monitoring, All:
 ♦ Complete blood count (CBC)
 ♦ Creatinine
 ♦ Liver function tests (LFT)
 ♦ Platelet count
 ♦ Post Critical care $\leq 24h$ </p> | | <p>pathway for evaluation and management at the Acute level of care for individuals without severe features that have coexisting comorbidities, risk factors, SDOH, or barriers to discharge that preclude treatment in the outpatient setting</p> |

| Postpartum Complication After Discharge | | |
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| Episode Day 3 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• INTERMEDIATE, ≥ One:</p> <ul style="list-style-type: none"> ▶ Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦ IV antiarrhythmic and hemodynamically stable ♦ Antiarrhythmic conversion from IV to PO, One: <ul style="list-style-type: none"> ▶ No proarrhythmic risk ≤ 24h ▶ Proarrhythmic risk and continuous cardiac monitoring (excludes Holter) ≤ 3d ▶ Eclampsia and neurological assessment every 3-4h ≤ 2d ▶ NIPPV ▶ Oxygen, ≥ One: <ul style="list-style-type: none"> ♦ HFNC and respiratory monitoring every 3-4h ♦ Oxygen ≥ 40%(0.40) ≤ 2d ▶ Seizure and, Both: <ul style="list-style-type: none"> ♦ Activity resolved ♦ Anticonvulsant continuous | <p>• INTERMEDIATE, ≥ One:</p> <ul style="list-style-type: none"> ▶ Eclampsia and neurological assessment every 3-4h ▶ NIPPV ▶ HFNC and respiratory monitoring every 3-4h ▶ Oxygen ≥ 40%(0.40) | <p>Rationale:</p> <ul style="list-style-type: none"> • "Arrhythmia" content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • The time-limited endpoint was removed from "Eclampsia and neurological assessment every 3-4h" to clarify the intent of the criteria, as the condition and associated monitoring are the drivers that are appropriate for evaluation and management at the Intermediate Level of Care • Oxygen ≥ 40%" criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Intermediate level of care • "Seizure" criteria were removed as this subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology |

| Postpartum Complication After Discharge | | |
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| Episode Day 3 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶ Acute kidney injury (AKI) and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> ♦ Loss of P wave ♦ Multifocal PVCs ♦ Neuromuscular deficit ♦ Widening QRS | <p>• CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶ Antihypertensive, continuous ▶ DIC and blood product transfusion ▶ Eclampsia and neurological assessment every 1-2h ▶ ECMO or ECLS ▶ Heart failure and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Arterial Po₂ < 56 mmHg(7.4 kPa) ▶ O₂ sat < 89%(0.89) and < baseline | <p>Rationale:</p> <ul style="list-style-type: none"> • Acute Kidney Injury (AKI) and CRRT criteria were removed as an individual with AKI or requiring CRRT can be reviewed in the AKI subset • Criteria for continuous antihypertensive infusion were added to the Critical level of care to align with guidelines and provide a pathway for postpartum |

Postpartum Complication After Discharge

Episode Day 3 (CRITICAL)

| Previously Available Content | 2024 March Release | Revision Description |
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| <ul style="list-style-type: none"> ▶Sodium, One: <ul style="list-style-type: none"> ♦< 120 mEq/L(120 mmol/L) and volume overload ♦> 160 mEq/L(160 mmol/L) ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Calcium chloride ▶Calcium gluconate ▶Glucose 50%(0.50) with insulin ▶Diuretic, continuous ▶Hemodialysis ≤ 7d since initiation ▶IV fluid resuscitation ≤ 2d ▶CRRT ▶DIC and blood product transfusion ▶Eclampsia and neurological assessment every 1-2h, ≤ 2d ▶ECMO or ECLS ▶Heart failure and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Arterial Po₂ ≤ 56 mmHg(7.4 kPa) ▶O₂ sat < 89%(0.89) and < baseline ♦Diuretic every 1-2h or continuous ▶Hemodynamic instability persisting after ≥1L of IVF resuscitation or ≥1 unit blood product transfusion and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶IV fluid resuscitation ≥1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶HELLP syndrome ≤ 2d, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Lab values, All: <ul style="list-style-type: none"> ♦Platelet count < 50,000/cu.mm(50x10⁹/L) ♦Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 70 U/L(1.17 µkat/L) ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Fragmented or contracted red cells on blood smear ▶Total serum lactate dehydrogenase (LDH) ≥ 600 IU/L(10.02 µkat/L) ▶Total bilirubin > 1.20 mg/dL(20.52 µmol/L) ▶Increased risk of bleeding, ≥ One: <ul style="list-style-type: none"> ♦PT ≥ 1.5x upper limit of normal (ULN) or INR ≥ 2.0 ♦PTT ≥ 1.5x upper limit of normal (ULN) ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶Laboratory monitoring, All: <ul style="list-style-type: none"> ♦Complete blood count (CBC) | <ul style="list-style-type: none"> ♦Diuretic every 1-2h or continuous ▶Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion and, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Heart rate > 120/min ▶Systolic blood pressure < 100 mmHg and < baseline ▶MAP < 65 mmHg ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶IV fluid resuscitation ≥ 1L ≤ 24h ▶Hemodynamic monitoring, invasive ▶HELLP syndrome, Both: <ul style="list-style-type: none"> ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Lab values, All: <ul style="list-style-type: none"> ♦Platelet count < 50,000/cu.mm(50x10⁹/L) ♦AST or ALT > 2x ULN ♦LDH ≥ 600 IU/L(10.02 µkat/L) ▶Increased risk of bleeding, ≥ One: <ul style="list-style-type: none"> ♦PT ≥ 1.5x ULN or INR ≥ 2.0 ♦PTT ≥ 1.5x ULN ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶Laboratory monitoring, All: <ul style="list-style-type: none"> ♦CBC ♦LFTs ♦Platelet count ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>individuals with severe hypertension refractory to treatment who require continuous antihypertensive infusion</p> <ul style="list-style-type: none"> • The time-limited endpoint was removed from "Eclampsia and neurological assessment every 1-2h" to clarify the intent of the criteria, as the condition and associated monitoring are the drivers that are appropriate for evaluation and management at the Critical Level of Care • Based on current guidelines, HELLP criteria were revised by aligning with primary interventions and more appropriate diagnostic benchmarks, removal of total bilirubin and fragmented or contracted red blood cells criteria, as these are not universally utilized. Time-limited endpoint was removed as the condition requires close monitoring for progression at the Critical Level of Care • "Seizure, recurrent or refractory requiring anticonvulsant criteria" were removed as this subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology |

| Postpartum Complication After Discharge | | |
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| Episode Day 3 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦Liver function tests (LFT) ♦Platelet count ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Seizure (excludes epilepsy), recurrent or refractory requiring anticonvulsant ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | | |

| Postpartum Complication After Discharge | | |
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| Episode Day 4 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•ACUTE, One:</p> <ul style="list-style-type: none"> ▶Responder, discharge expected today if clinically stable last 12h, Both: <ul style="list-style-type: none"> ♦Afebrile ♦Clinical stability, ≥ One: <ul style="list-style-type: none"> ▶Bleeding controlled or resolved ▶HELLP syndrome and, Both: <ul style="list-style-type: none"> ♦Blood pressure controlled last 24h ♦Lab values within acceptable limits ▶Hypertension controlled or within acceptable limits last 24h ▶Preeclampsia or eclampsia resolved ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦Bleeding actual or suspected and, Both: <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦Heart rate 100-120/min, sustained ♦Hct or Hb decreased within last 24h ♦Orthostatic hypotension, Both: <ul style="list-style-type: none"> ▶Sustained drop in blood pressure within 3 min of sitting or standing ▶Blood pressure, ≥ One: <ul style="list-style-type: none"> ♦Systolic drop ≥ 20 mmHg ♦Diastolic drop ≥ 10 mmHg ▶Intervention, ≥ One: <ul style="list-style-type: none"> ♦Blood product transfusion | <p>•ACUTE, One:</p> <ul style="list-style-type: none"> ▶Responder, discharge expected today if clinically stable last 12h, Both: <ul style="list-style-type: none"> ♦Afebrile ♦Clinical stability, ≥ One: <ul style="list-style-type: none"> ▶HELLP syndrome and, Both: <ul style="list-style-type: none"> ♦Blood pressure controlled last 24h ♦Lab values within acceptable limits ▶Preeclampsia resolved ▶Endometritis resolving and, ≥ One: <ul style="list-style-type: none"> ♦Anti-infective regimen established and tolerated ♦Symptoms resolving ▶Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦HELLP syndrome and, Both: <ul style="list-style-type: none"> ▶Blood pressure management, ≥ One: <ul style="list-style-type: none"> ♦Antihypertensive (includes PO) ♦Blood pressure monitoring at least 3x/24h ▶Laboratory monitoring, All: <ul style="list-style-type: none"> ♦CBC ♦LFTs ♦Platelet count ♦Postpartum endometritis and anti-infective ♦Preeclampsia with severe features and, ≥ One: <ul style="list-style-type: none"> ▶Blood pressure management, ≥ One: | <p>Rationale:</p> <ul style="list-style-type: none"> • Responder criteria were updated to reflect the patient appropriate for discharge • “Bleeding actual or suspected” criteria were removed from the Acute level of care, and added to the Observation level of care to allow for evaluation and management for up to 48 hours for hemodynamically stable individuals with postpartum vaginal bleeding • “Fever” criteria were removed as an individual with postpartum fever and infection can be reviewed utilizing the appropriate relevant criteria within this subset (i.e., postpartum endometritis) or the relevant condition-specific subset (e.g., Infection: General, Infection: GI/GYN, Infection: Pyelonephritis or Complex UTI, Infection: Sepsis, Infection: Skin) • “Postpartum endometritis” criteria were added to reflect the patient who is appropriate for continued |

| Postpartum Complication After Discharge | | |
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| Episode Day 4 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦Hct or Hb monitoring at least 2x/24h, ≤ 2d ♦Fever and, Both: <ul style="list-style-type: none"> ►Temperature, ≥ One: <ul style="list-style-type: none"> ♦> 99.4°F(37.4°C) PO ♦> 100.4°F(38.0°C) PR ►Intervention, One: <ul style="list-style-type: none"> ♦Known source and anti-infective ♦New onset and, ≥ One: <ul style="list-style-type: none"> ►Culture pending ≤ 2d ►Drug-induced fever suspected and precipitating drug discontinued ≤ 3d ►Imaging study ≤ 24h ♦HELLP syndrome ≤ 5d and, Both: <ul style="list-style-type: none"> ►Lab values, ≥ One: <ul style="list-style-type: none"> ♦Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) > 70 U/L(1.17 µkat/L) ♦Platelet count 50,000-100,000/cu.mm(50-100x10⁹/L) ♦Finding, ≥ One: <ul style="list-style-type: none"> ►Fragmented or contracted red cells on blood smear ►Total serum lactate dehydrogenase (LDH) ≥ 600 IU/L(10.02 µkat/L) ►Total bilirubin > 1.20 mg/dL(20.52 µmol/L) ►Intervention, All: <ul style="list-style-type: none"> ♦Blood pressure management, ≥ One: <ul style="list-style-type: none"> ►Antihypertensive (includes PO) ►Blood pressure monitoring at least 3x/24h ♦Laboratory monitoring, All: <ul style="list-style-type: none"> ►Complete blood count (CBC) ►Liver function tests (LFT) ►Platelet count ♦Magnesium sulfate ♦Hypertension uncontrolled ≤ 2d and, All: <ul style="list-style-type: none"> ►Systolic blood pressure ≥ 150 mmHg or diastolic blood pressure ≥ 100 mmHg last 24h ►Antihypertensive (includes PO) ►Blood pressure monitoring at least 3x/24h ♦Preeclampsia or eclampsia ≤ 5d and, Both: <ul style="list-style-type: none"> ►Finding, ≥ One: <ul style="list-style-type: none"> ♦Systolic blood pressure ≥ 150 mmHg or diastolic blood pressure ≥ 100 mmHg last 24h ♦Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) elevated ♦Creatinine > baseline ♦Pain, severe persistent and unresponsive to medication and, ≥ One: <ul style="list-style-type: none"> ►Epigastric ►Headache, new onset ►Right upper quadrant ♦Platelet count < 100,000/cu.mm(100x10⁹/L) ♦Visual disturbances, persistent ►Intervention, ≥ One: | <ul style="list-style-type: none"> ♦Antihypertensive (includes PO) ♦Blood pressure monitoring at least 3x/24h ►Magnesium sulfate ♦Post Critical care ≤ 24h | <p>stay at the Acute Level of Care in alignment with guidelines indicating that postpartum endometritis should be treated with IV anti-infectives for a minimum of 24-48 hours after symptoms have resolved and the patient is afebrile</p> <ul style="list-style-type: none"> • Time-limited endpoint was removed from HELLP syndrome as this condition requires close monitoring and may require a longer length of stay • HELLP syndrome criteria were revised and streamlined to focus on the primary drivers appropriate for continued stay evaluation and management • "Preeclampsia" and "hypertension" criteria were revised to streamline the review process and focus on primary findings and drivers of care that are appropriate for continued stay evaluation and management |

Postpartum Complication After Discharge

Episode Day 4 (ACUTE)

| Previously Available Content | 2024 March Release | Revision Description |
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| <ul style="list-style-type: none"> ♦Antihypertensive (includes PO) ♦Magnesium sulfate ♦Monitoring, Both: <ul style="list-style-type: none"> ▶Blood pressure monitoring at least 3x/24h ▶Laboratory monitoring, All: <ul style="list-style-type: none"> ♦Complete blood count (CBC) ♦Creatinine ♦Liver function tests (LFT) ♦Platelet count ♦Post Critical care ≤ 24h | | |

Postpartum Complication After Discharge

Episode Day 4 (INTERMEDIATE)

| Previously Available Content | 2024 March Release | Revision Description |
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| <ul style="list-style-type: none"> •INTERMEDIATE, ≥ One: <ul style="list-style-type: none"> ▶Arrhythmia, ≥ One: <ul style="list-style-type: none"> ♦IV antiarrhythmic and hemodynamically stable ♦Antiarrhythmic conversion from IV to PO, One: <ul style="list-style-type: none"> ▶No proarrhythmic risk ≤ 24h ▶Proarrhythmic risk and continuous cardiac monitoring (excludes Holter) ≤ 3d ▶Eclampsia and neurological assessment every 3-4h ≤ 2d ▶NIPPV ▶Oxygen, ≥ One: <ul style="list-style-type: none"> ♦HFNC and respiratory monitoring every 3-4h ♦Oxygen ≥ 40%(0.40) ≤ 2d ▶Seizure and, Both: <ul style="list-style-type: none"> ♦Activity resolved ♦Anticonvulsant continuous | <ul style="list-style-type: none"> •INTERMEDIATE, ≥ One: <ul style="list-style-type: none"> ▶Eclampsia and neurological assessment every 3-4h ▶NIPPV ▶HFNC and respiratory monitoring every 3-4h ▶Oxygen ≥ 40%(0.40) | <p>Rationale:</p> <ul style="list-style-type: none"> • "Arrhythmia" content was removed as individuals with arrhythmia can be reviewed in the relevant Arrhythmia subset (i.e., Blocks, Atrial, or Ventricular or Abnormal ECG Finding) • The time-limited endpoint was removed from "Eclampsia and neurological assessment every 3-4h" to clarify the intent of the criteria, as the condition and associated monitoring are the drivers that are appropriate for evaluation and management at the Intermediate Level of Care • Oxygen ≥ 40%" criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Intermediate level of care • "Seizure" criteria were removed as this subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology |

| Postpartum Complication After Discharge | | |
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| Episode Day 4 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶ Acute kidney injury (AKI) and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Potassium > 6.0 mEq/L(6.0 mmol/L) and, ≥ One: <ul style="list-style-type: none"> ♦ Loss of P wave ♦ Multifocal PVCs ♦ Neuromuscular deficit ♦ Widening QRS ▶ Sodium, One: <ul style="list-style-type: none"> ♦ < 120 mEq/L(120 mmol/L) and volume overload ♦ > 160 mEq/L(160 mmol/L) ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Calcium chloride ▶ Calcium gluconate ▶ Glucose 50%(0.50) with insulin ▶ Diuretic, continuous ▶ Hemodialysis ≤ 7d since initiation ▶ IV fluid resuscitation ≤ 2d ▶ CRRT ▶ DIC and blood product transfusion ▶ Eclampsia and neurological assessment every 1-2h, ≤ 2d ▶ ECMO or ECLS ▶ Heart failure and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Arterial Po₂ < 56 mmHg(7.4 kPa) ▶ O₂ sat < 89%(0.89) and < baseline ♦ Diuretic every 1-2h or continuous ▶ Hemodynamic instability persisting after ≥1L of IVF resuscitation or ≥1 unit blood product transfusion and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Heart rate > 120/min ▶ Systolic blood pressure < 100 mmHg and < baseline ▶ MAP < 65 mmHg ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Blood product transfusion ▶ IV fluid resuscitation ≥1L ≤ 24h ▶ Hemodynamic monitoring, invasive ▶ HELLP syndrome ≤ 2d, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Lab values, All: <ul style="list-style-type: none"> ♦ Platelet count < 50,000/cu.mm(50x10⁹/L) ♦ AST or ALT > 2x ULN ♦ LDH ≥ 600 IU/L(10.02 µkat/L) ▶ Increased risk of bleeding, ≥ One: <ul style="list-style-type: none"> ♦ PT ≥ 1.5x ULN or INR ≥ 2.0 ♦ PTT ≥ 1.5x ULN ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Blood product transfusion ▶ Laboratory monitoring, All: <ul style="list-style-type: none"> ♦ CBC ♦ LFTs ♦ Platelet count ▶ HFNC and respiratory monitoring every 1-2h ▶ Mechanical ventilation ▶ NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦ FiO₂ > 50%(0.50) ♦ Respiratory intervention every 1-2h ▶ Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>• CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶ Antihypertensive, continuous ▶ DIC and blood product transfusion ▶ Eclampsia and neurological assessment every 1-2h ▶ ECMO or ECLS ▶ Heart failure and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Arterial Po₂ < 56 mmHg(7.4 kPa) ▶ O₂ sat < 89%(0.89) and < baseline ♦ Diuretic every 1-2h or continuous ▶ Hemodynamic instability persisting after ≥ 1L of IVF resuscitation or ≥ 1 unit blood product transfusion and, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Heart rate > 120/min ▶ Systolic blood pressure < 100 mmHg and < baseline ▶ MAP < 65 mmHg ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Blood product transfusion ▶ IV fluid resuscitation ≥ 1L ≤ 24h ▶ Hemodynamic monitoring, invasive ▶ HELLP syndrome, Both: <ul style="list-style-type: none"> ♦ Finding, ≥ One: <ul style="list-style-type: none"> ▶ Lab values, All: <ul style="list-style-type: none"> ♦ Platelet count < 50,000/cu.mm(50x10⁹/L) ♦ AST or ALT > 2x ULN ♦ LDH ≥ 600 IU/L(10.02 µkat/L) ▶ Increased risk of bleeding, ≥ One: <ul style="list-style-type: none"> ♦ PT ≥ 1.5x ULN or INR ≥ 2.0 ♦ PTT ≥ 1.5x ULN ♦ Intervention, ≥ One: <ul style="list-style-type: none"> ▶ Blood product transfusion ▶ Laboratory monitoring, All: <ul style="list-style-type: none"> ♦ CBC ♦ LFTs ♦ Platelet count ▶ HFNC and respiratory monitoring every 1-2h ▶ Mechanical ventilation ▶ NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦ FiO₂ > 50%(0.50) ♦ Respiratory intervention every 1-2h ▶ Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>Rationale:</p> <ul style="list-style-type: none"> • Acute Kidney Injury (AKI) and CRRT criteria were removed as an individual with AKI or requiring CRRT can be reviewed in the AKI subset • Criteria for continuous antihypertensive infusion were added to the Critical level of care to align with guidelines and provide a pathway for postpartum individuals with severe hypertension refractory to treatment who require continuous antihypertensive infusion • The time-limited endpoint was removed from "Eclampsia and neurological assessment every 1-2h" to clarify the intent of the criteria, as the condition and associated monitoring are the drivers that are appropriate for evaluation and management at the Critical Level of Care • Based on current guidelines, HELLP criteria were revised by aligning with primary interventions and more appropriate diagnostic benchmarks, removal of total bilirubin and fragmented or contracted red blood cells criteria, as these are not universally utilized. Time-limited endpoint was removed as the condition requires close monitoring for progression at the Critical Level of Care • "Seizure, recurrent or refractory requiring anticonvulsant criteria" were removed as this subset has eclampsia seizure criteria, and seizures that are not eclampsia-related can be reviewed in the Epilepsy or General Medical subset depending on the etiology |

| Postpartum Complication After Discharge | | |
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| Episode Day 4 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> smear ▶Total serum lactate dehydrogenase (LDH) ≥ 600 IU/L(10.02 µkat/L) ▶Total bilirubin > 1.20 mg/dL(20.52 µmol/L) ▶Increased risk of bleeding, ≥ One: <ul style="list-style-type: none"> ♦PT ≥ 1.5x upper limit of normal (ULN) or INR ≥ 2.0 ♦PTT ≥ 1.5x upper limit of normal (ULN) ♦Intervention, ≥ One: <ul style="list-style-type: none"> ▶Blood product transfusion ▶Laboratory monitoring, All: <ul style="list-style-type: none"> ♦Complete blood count (CBC) ♦Liver function tests (LFT) ♦Platelet count ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Seizure (excludes epilepsy), recurrent or refractory requiring anticonvulsant ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | | |
| Postpartum Complication After Discharge | | |
| Episode Day 5 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> •ACUTE, One: <ul style="list-style-type: none"> ▶Responder, discharge expected today if clinically stable last 12h, Both: <ul style="list-style-type: none"> ♦Afebrile ♦Clinical stability, ≥ One: <ul style="list-style-type: none"> ▶Bleeding controlled or resolved ▶HELLP syndrome and, Both: <ul style="list-style-type: none"> ♦Blood pressure controlled last 24h, ♦Lab values within acceptable limits ▶Hypertension controlled or within acceptable limits last 24h ▶Preeclampsia or eclampsia resolved ▶Non-responder, not clinically stable for discharge and exceeds episode days at the Acute level of care | <ul style="list-style-type: none"> •ACUTE, One: <ul style="list-style-type: none"> ▶Responder, discharge expected today if clinically stable last 12h, Both: <ul style="list-style-type: none"> ♦Afebrile ♦Clinical stability, ≥ One: <ul style="list-style-type: none"> ▶HELLP syndrome and, Both: <ul style="list-style-type: none"> ♦Blood pressure controlled last 24h, ♦Lab values within acceptable limits ▶Preeclampsia resolved ▶Endometritis resolving and, ≥ One: <ul style="list-style-type: none"> ♦Anti-infective regimen established and tolerated ♦Symptoms resolving ▶Non-responder, not clinically stable for discharge | <p>Rationale:</p> <ul style="list-style-type: none"> • Responder criteria were updated to reflect the patient appropriate for discharge |

Postpartum Complication After Discharge

Episode Day 5 (ACUTE)

| Previously Available Content | 2024 March Release | Revision Description |
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| for this condition, One:
♦Use Extended Stay subset
♦Refer for secondary review | and exceeds episode days at the Acute level of care
for this condition, One:
♦Use Extended Stay subset
♦Refer for secondary review | |

Pulmonary Embolism

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|  Revision Summary | <p>“Confirmed by imaging” criterion was removed from Initial Review and Episode Day 1 at all levels of care and the subset introduction note now indicates that patients must have a confirmed pulmonary embolism on imaging to be reviewed in this subset. “Bridging anticoagulation” criteria were renamed to “Pretreatment or bridging anticoagulation” at the Acute level of care and were updated to be more inclusive of patients who require pretreatment with parenteral anticoagulation prior to starting dabigatran or edoxaban and to better define patients who require hospitalization due to the inability to administer parenteral anticoagulation at home. Thrombolytic therapy criteria were added on Episode Day 2 at the Critical level of care to capture patients who have worsened and require thrombolytic therapy on continued stay.</p> |
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| Pulmonary Embolism | | |
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| Initial Review (OBSERVATION) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| ●OBSERVATION, One:
▶Pulmonary embolism confirmed by imaging | ●OBSERVATION, One:
▶Pulmonary embolism and anticoagulation (includes PO) | Rationale: <ul style="list-style-type: none"> • “Confirmed by imaging” was removed from the criteria and is now located in the introduction note, indicating that patients must have a confirmed pulmonary embolism on imaging to be reviewed in this subset • The intervention “anticoagulation (includes PO)” was added to the Initial Review to better define the patients appropriate for the Observation level of care |

| Pulmonary Embolism | | |
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| Initial Review (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| ●ACUTE, One:
▶Pulmonary embolism confirmed by imaging and, ≥ One:
♦Anticoagulation initiation (includes PO) and, ≥ One:
▶Heart failure requiring IV diuretic therapy or titration of oral diuretic therapy | ●ACUTE, One:
▶Anticoagulation (includes PO) and, ≥ One:
♦Heart failure requiring IV diuretic therapy or titration of oral diuretic therapy
♦O ₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen
▶UFH, continuous and, ≥ One: | Rationale: <ul style="list-style-type: none"> • “Pulmonary embolism confirmed by imaging” was removed from the criteria and is now located in the introduction note, indicating that patients must |

| Pulmonary Embolism | | |
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| Initial Review (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ▶HIT, suspected or confirmed ▶O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ♦Bridging anticoagulation requiring monitoring and, ≥ One: <ul style="list-style-type: none"> ▶Compliance risk, Both: <ul style="list-style-type: none"> ♦Patient unreliable and unable to adhere to outpatient instructions or treatment ♦Caregiver unavailable or unable to manage care ▶Creatinine clearance ≤ 30 mL/min ♦IVC filter placement performed ≤ 24h ♦Unfractionated heparin, continuous ≤ 5d and, ≥ One: <ul style="list-style-type: none"> ▶Antiphospholipid antibody syndrome ▶Bleeding, active or increased risk ▶Creatinine clearance ≤ 30 mL/min ▶Morbid obesity (BMI ≥ 40 kg/m² or weight > 120kg) | <ul style="list-style-type: none"> ♦Antiphospholipid antibody syndrome ♦Bleeding, active or increased risk ♦BMI ≥ 40 kg/m² or weight > 120kg ♦Creatinine clearance ≤ 30 mL/min ▶HIT, suspected or by history and anticoagulation ▶IVC filter placement performed ≤ 24h ▶Pretreatment or bridging anticoagulation, All: <ul style="list-style-type: none"> ♦Patient or caregiver unable to administer ♦Outpatient services unavailable ♦Anticoagulation | <p>have a confirmed pulmonary embolism on imaging to be reviewed in this subset</p> <ul style="list-style-type: none"> • “Confirmed” was removed and “by history” was added to the HIT criterion since it may take several days to confirm a diagnosis of HIT; therefore, patients with suspected or a history of HIT are appropriate for the use of this criterion • PO anticoagulation was removed as an intervention for HIT as there is not enough evidence to support the use of oral agents for the treatment of HIT • “Bridging anticoagulation” was changed to “Pretreatment or bridging anticoagulation,” and the criteria were updated to capture patients who require pretreatment with parenteral anticoagulation prior to starting dabigatran or edoxaban, and to better define the patients who require hospitalization due to the inability to administer parenteral anticoagulation therapy at home • “Creatinine clearance ≤ 30mL/min” was removed under bridging criteria as these patients should be reviewed using the unfractionated heparin criteria and can continue to be reviewed there while receiving bridging or pretreatment • The end point of ≤ 5d previously attached to “unfractionated heparin, continuous” criterion was removed to allow patients to continue to meet due to a risk factor or until a therapeutic INR is achieved when transitioning to warfarin |

| Pulmonary Embolism | | |
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| Initial Review (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•INTERMEDIATE, One:</p> <ul style="list-style-type: none"> ▶Right ventricular dysfunction, Both: <ul style="list-style-type: none"> ♦Pulmonary embolism confirmed by imaging ♦Finding, ≥ One: <ul style="list-style-type: none"> ▶Suspected by biomarkers, ≥ One: <ul style="list-style-type: none"> ♦Abnormal troponin ♦Abnormal brain natriuretic peptide (BNP) ♦Abnormal N-terminal pro-brain natriuretic peptide (NT-proBNP) ▶Confirmed by imaging | <p>•INTERMEDIATE, One:</p> <ul style="list-style-type: none"> ▶Right ventricular dysfunction, ≥ One: <ul style="list-style-type: none"> ♦Suspected by biomarkers, ≥ One: <ul style="list-style-type: none"> ▶BNP or NT-proBNP > ULN and > baseline ▶Troponin > ULN and > baseline ♦Confirmed by imaging | <p>Rationale:</p> <ul style="list-style-type: none"> • “Pulmonary embolism confirmed by imaging” was removed from the criteria and is now located in the introduction note, indicating that patients must have a confirmed pulmonary embolism on imaging to be reviewed in this subset • “Abnormal” was changed to “> ULN and > baseline” to clarify the intent of the criteria |

| Pulmonary Embolism | | |
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| Initial Review (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶Thrombolytic therapy, ≥ One: <ul style="list-style-type: none"> ♦Catheter directed thrombolysis ♦Systemic thrombolysis ▶ECMO or extracorporeal life support (ECLS) ▶Hemodynamic monitoring, invasive ▶Mechanical ventilation ▶NIPPV ▶Oxygen ≥ 40%(0.40) ▶HFNC and respiratory monitoring every 1-2h ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶Thrombolytic therapy, ≥ One: <ul style="list-style-type: none"> ♦Catheter directed thrombolysis scheduled or performed ≤ 24h ♦Systemic thrombolysis ≤ 24h ▶ECMO or extracorporeal life support (ECLS) ▶Hemodynamic monitoring, invasive ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV ▶Oxygen ≥ 40%(0.40) ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>Rationale:</p> <ul style="list-style-type: none"> • “Schedule or performed ≤ 24h” was added to catheter directed thrombolysis to clarify the intent of the criteria • “≤ 24h” was added to “Systemic thrombolysis” criterion to clarify the intent of the criterion that patients can be reviewed at the Critical level of care for one day for this procedure |

| Pulmonary Embolism | | |
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| Episode Day 1 (OBSERVATION) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•OBSERVATION, Both:</p> <ul style="list-style-type: none"> ▶Pulmonary embolism confirmed by imaging | <p>•OBSERVATION, One:</p> <ul style="list-style-type: none"> ▶Pulmonary embolism and anticoagulation (includes | <p>Rationale:</p> |

| Pulmonary Embolism | | |
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| Episode Day 1 (OBSERVATION) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ▶Anticoagulation initiation (includes PO) | <ul style="list-style-type: none"> PO) | <ul style="list-style-type: none"> • “Confirmed by imaging” was removed from the criteria and is now located in the introduction note, indicating that patients must have a confirmed pulmonary embolism on imaging to be reviewed in this subset |

| Pulmonary Embolism | | |
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| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>●ACUTE, One:</p> <ul style="list-style-type: none"> ▶Pulmonary embolism confirmed by imaging and, ≥ One: <ul style="list-style-type: none"> ♦Anticoagulation initiation (includes PO) and, ≥ One: <ul style="list-style-type: none"> ▶Heart failure requiring IV diuretic therapy or titration of oral diuretic therapy ▶HIT, suspected or confirmed ▶O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ♦Bridging anticoagulation requiring monitoring and, Both: <ul style="list-style-type: none"> ▶Finding, ≥ One: <ul style="list-style-type: none"> ♦Compliance risk, Both: <ul style="list-style-type: none"> ▶Patient unreliable and unable to adhere to outpatient instructions or treatment ▶Caregiver unavailable or unable to manage care ♦Creatinine clearance ≤ 30 mL/min ▶Intervention, ≥ One: <ul style="list-style-type: none"> ♦Fondaparinux ♦LMWH ♦Unfractionated heparin ♦Hospital acquired and initiation of anticoagulation ≤ 24h ♦IVC filter placement performed ≤ 24h ♦Unfractionated heparin, continuous ≤ 5d and, ≥ One: <ul style="list-style-type: none"> ▶Antiphospholipid antibody syndrome ▶Bleeding, active or increased risk ▶Creatinine clearance ≤ 30 mL/min ▶Morbid obesity (BMI ≥ 40 kg/m2 or weight > 120kg) | <p>●ACUTE, One:</p> <ul style="list-style-type: none"> ▶Anticoagulation (includes PO) and, ≥ One: <ul style="list-style-type: none"> ♦Heart failure requiring IV diuretic therapy or titration of oral diuretic therapy ♦O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ▶UFH, continuous and, ≥ One: <ul style="list-style-type: none"> ♦Antiphospholipid antibody syndrome ♦Bleeding, active or increased risk ♦ BMI ≥ 40 kg/m2 or weight > 120kg ♦Creatinine clearance ≤ 30 mL/min ▶HIT, suspected or by history and anticoagulation ▶Hospital acquired and initiation of anticoagulation (includes PO) ≤ 24h ▶IVC filter placement performed ≤ 24h ▶Pretreatment or bridging anticoagulation, All: <ul style="list-style-type: none"> ♦Patient or caregiver unable to administer ♦Outpatient services unavailable ♦Anticoagulation | <p>Rationale:</p> <ul style="list-style-type: none"> • “Pulmonary embolism confirmed by imaging” was removed from the criteria and is now located in the introduction note, indicating that patients must have a confirmed pulmonary embolism on imaging to be reviewed in this subset • “Confirmed” was removed and “by history” was added to the HIT criterion since it may take several days to confirm a diagnosis of HIT; therefore, patients with suspected or a history of HIT are appropriate for the use of this criterion • PO anticoagulation was removed as an intervention for HIT as there is not enough evidence to support the use of oral agents for the treatment of HIT • “Includes PO” was added to the criterion “Hospital acquired and initiation of anticoagulation” to clarify the intent of the criterion as oral anticoagulants were previously identified as meeting this criterion in the attached note • “Bridging anticoagulation” was changed to “Pretreatment or bridging anticoagulation,” and the criteria were updated to capture patients who require pretreatment with parenteral anticoagulation prior to starting dabigatran or |

| Pulmonary Embolism | | |
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| Episode Day 1 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| | | <p>edoxaban and to better define patients who require hospitalization due to the inability to administer parenteral anticoagulation therapy at home</p> <ul style="list-style-type: none"> • “Creatinine clearance $\leq 30\text{mL/min}$” was removed under bridging criteria as these patients should be reviewed using the unfractionated heparin criteria and can continue to be reviewed there while receiving bridging or pretreatment • “Fondaparinux” and “LMWH” were removed and are now located in a hover over note attached to “anticoagulation” • “Unfractionated heparin” was removed as an intervention for bridging criteria as these patients should be reviewed using the standalone unfractionated heparin criteria • The end point of $\leq 5\text{d}$ previously attached to “unfractionated heparin, continuous” criterion was removed to allow patients to continue to meet due to a risk factor or until a therapeutic INR is achieved when transitioning to warfarin |

| Pulmonary Embolism | | |
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| Episode Day 1 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• INTERMEDIATE, One:</p> <ul style="list-style-type: none"> ▶ Right ventricular dysfunction, Both: ♦ Pulmonary embolism confirmed by imaging ♦ Right ventricular dysfunction, $\geq$ One: <ul style="list-style-type: none"> ▶ Suspected by biomarkers, $\geq$ One: <ul style="list-style-type: none"> ♦ Abnormal troponin ♦ Abnormal brain natriuretic peptide (BNP) ♦ Abnormal N-terminal pro-brain natriuretic peptide (NT-proBNP) ▶ Confirmed by imaging | <p>• INTERMEDIATE, One:</p> <ul style="list-style-type: none"> ▶ Right ventricular dysfunction, $\geq$ One: <ul style="list-style-type: none"> ♦ Suspected by biomarkers, $\geq$ One: <ul style="list-style-type: none"> ▶ BNP or NT-proBNP $>$ ULN and $>$ baseline ▶ Troponin $>$ ULN and $>$ baseline ♦ Confirmed by imaging | <p>Rationale:</p> <ul style="list-style-type: none"> • “Pulmonary embolism confirmed by imaging” was removed from the criteria and is now located in the introduction note, indicating that patients must have a confirmed pulmonary embolism on imaging to be reviewed in this subset • “Abnormal” was changed to “$>$ ULN and $>$ baseline” to clarify the intent of the criteria |

| Pulmonary Embolism | | |
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| Episode Day 1 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•CRITICAL, One:</p> <p>▶Pulmonary embolism confirmed by imaging and, ≥ One:</p> <ul style="list-style-type: none"> ♦Thrombolytic therapy, ≥ One: <ul style="list-style-type: none"> ▶Catheter directed thrombolysis scheduled or performed ≤ 24h ▶Systemic thrombolysis ♦ECMO or extracorporeal life support (ECLS) ♦Hemodynamic monitoring, invasive ♦Mechanical ventilation ♦NIPPV ♦Oxygen ≥ 40%(0.40) ≤ 2d ♦HFNC and respiratory monitoring every 1-2h ♦Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶Thrombolytic therapy, ≥ One: <ul style="list-style-type: none"> ♦Catheter directed thrombolysis scheduled or performed ≤ 24h ♦Systemic thrombolysis ≤ 24h ▶ECMO or extracorporeal life support (ECLS) ▶Hemodynamic monitoring, invasive ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV ▶Oxygen ≥ 40%(0.40) ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>Rationale:</p> <ul style="list-style-type: none"> • “Pulmonary embolism confirmed by imaging” was removed from the criteria and is now located in the introduction note, indicating that patients must have a confirmed pulmonary embolism on imaging to be reviewed in this subset • “≤ 24h” was added to “Systemic thrombolysis” criterion to clarify the intent of the criterion that patients can be reviewed at the Critical level of care for one day for this procedure • Oxygen ≥ 40%” criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Intermediate level of care |

| Pulmonary Embolism | | |
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| Episode Day 2 (OBSERVATION) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•OBSERVATION, One:</p> <p>▶Responder, discharge expected today if clinically stable last 12h, Both:</p> <ul style="list-style-type: none"> ♦Anticoagulation regimen established with no evidence of bleeding ♦Patient or caregiver demonstrates ability to manage care <p>▶Non-responder, not clinically stable for discharge and exceeds initial 24h of Observation for this condition, One:</p> <ul style="list-style-type: none"> ♦For pulmonary embolism use Episode Day 2 at a higher level of care ♦For a condition other than pulmonary embolism use appropriate subset (Episode Day 1) based on clinical findings ♦Refer for secondary review | <p>•OBSERVATION, One:</p> <p>▶Responder, discharge expected today if clinically stable last 12h, One:</p> <ul style="list-style-type: none"> ♦Anticoagulation regimen established <p>▶Non-responder, not clinically stable for discharge and exceeds initial 24h of Observation for this condition, One:</p> <ul style="list-style-type: none"> ♦For pulmonary embolism use Episode Day 2 at a higher level of care ♦For a condition other than pulmonary embolism use appropriate subset (Episode Day 1) based on clinical findings ♦Refer for secondary review | <p>Rationale:</p> <ul style="list-style-type: none"> • Responder criteria were updated to better describe patients appropriate for discharge |

| Pulmonary Embolism | | |
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| Episode Day 2 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• ACUTE, One:</p> <ul style="list-style-type: none"> ▶ Early responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦ Anticoagulation regimen established with no evidence of bleeding or IVC filter ♦ O₂ sat within acceptable limits or home oxygen arranged ♦ No complication or active comorbidity relevant to this episode of care ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦ Anticoagulation (includes PO) and, ≥ One: <ul style="list-style-type: none"> ▶ O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ▶ Biomarkers, ≥ One: <ul style="list-style-type: none"> ♦ Abnormal troponin ♦ Abnormal brain natriuretic peptide (BNP) ♦ Abnormal N-terminal pro-brain natriuretic peptide (NT-proBNP) ♦ Bridging anticoagulation and INR non-therapeutic and, ≥ One: <ul style="list-style-type: none"> ▶ Compliance risk, Both: <ul style="list-style-type: none"> ♦ Patient unreliable and unable to adhere to outpatient instructions or treatment ♦ Caregiver unavailable or unable to manage care ▶ Creatinine clearance ≤ 30 mL/min ♦ Hct or Hb decreased within last 24h and blood product transfusion ♦ HIT, actual or suspected, and anticoagulation (includes PO) ♦ IVC filter placement performed ≤ 24h ♦ Pain consistent with pulmonary embolism requiring analgesic ≤ 2d ♦ Unfractionated heparin, continuous ≤ 5d and, ≥ One: <ul style="list-style-type: none"> ▶ Antiphospholipid antibody syndrome ▶ Bleeding, active or increased risk ▶ Creatinine clearance ≤ 30 mL/min ▶ Morbid obesity (BMI ≥ 40 kg/m² or weight > 120kg) ♦ COPD and, Both: <ul style="list-style-type: none"> ▶ Persistent dyspnea, ≥ One: <ul style="list-style-type: none"> ♦ Arterial Po₂ < baseline ♦ Arterial or venous Pco₂ > baseline ♦ O₂ sat ≤ 91%(0.91) and < baseline ♦ Increased work of breathing, ≥ One: <ul style="list-style-type: none"> ▶ Difficulty taking PO ▶ Prefers sitting ▶ Talks in phrases ▶ Use of accessory muscles ▶ Intervention, Both: | <p>• ACUTE, One:</p> <ul style="list-style-type: none"> ▶ Early responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦ Anticoagulation regimen, One: <ul style="list-style-type: none"> ▶ Established ▶ Contraindicated ♦ O₂ sat within acceptable limits or home oxygen arranged ♦ No complication or active comorbidity relevant to this episode of care ▶ Partial responder, not clinically stable for discharge and requires continued stay, ≥ One: <ul style="list-style-type: none"> ♦ Anticoagulation (includes PO) and, ≥ One: <ul style="list-style-type: none"> ▶ O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ▶ Biomarkers, ≥ One: <ul style="list-style-type: none"> ♦ BNP or NT-proBNP > ULN and > baseline ♦ Troponin > ULN and > baseline ♦ UFH, continuous and, ≥ One: <ul style="list-style-type: none"> ▶ Antiphospholipid antibody syndrome ▶ Bleeding, active or increased risk ▶ BMI ≥ 40 kg/m² or weight > 120kg ▶ Creatinine clearance ≤ 30 mL/min ♦ HIT, suspected or by history and anticoagulation ♦ IVC filter placement performed ≤ 24h ♦ Pain consistent with pulmonary embolism requiring analgesic ≥ 3x/24h, ≤ 2d ♦ Pretreatment or bridging anticoagulation, All: <ul style="list-style-type: none"> ▶ Patient or caregiver unable to administer ▶ Outpatient services unavailable ▶ Intervention, One: <ul style="list-style-type: none"> ♦ INR non-therapeutic and anticoagulation (includes PO) ♦ Anticoagulation ≤ 5d since initiation ♦ Bleeding, actual or suspected and, ≥ One: <ul style="list-style-type: none"> ▶ Blood product transfusion ▶ Hct or Hb monitoring at least 2x/24h, ≤ 2d ♦ COPD and, Both: <ul style="list-style-type: none"> ▶ Persistent dyspnea and, ≥ One: <ul style="list-style-type: none"> ♦ Arterial Po₂ < baseline ♦ Arterial or venous Pco₂ > baseline ♦ O₂ sat ≤ 91%(0.91) and < baseline ♦ Increased work of breathing, ≥ One: <ul style="list-style-type: none"> ▶ Difficulty taking PO ▶ Prefers sitting ▶ Talks in phrases ▶ Use of accessory muscles ▶ Intervention, Both: <ul style="list-style-type: none"> ♦ Bronchodilator ≥ 4x/24h ♦ Corticosteroid (includes inhaled) ♦ Heart failure and, Both: | <p>Rationale:</p> <ul style="list-style-type: none"> • Early responder criteria were updated to better describe patients appropriate for discharge • “Abnormal” was changed to “> ULN and > baseline” to clarify the intent of the criteria • Analgesic frequency of “≥ 3x/24h” was added to clarify the intent of the criteria • “Bridging anticoagulation” was changed to “Pretreatment or bridging anticoagulation” and criteria were updated to capture patients who require pretreatment with parenteral anticoagulation prior to starting dabigatran or edoxaban, and to better define patients who require hospitalization due to the inability to administer parenteral anticoagulation therapy at home • “Includes PO” was added to the “INR non-therapeutic and anticoagulation” line to clarify the intent of the criteria as it is intended for patients on oral anticoagulation who have yet to achieve a therapeutic INR • “Anticoagulation ≤ 5d since initiation” was added to capture the patients who require at least 5 days of anticoagulation prior to starting dabigatran or edoxaban and are unable to administer parenteral anticoagulation at home • “Creatinine clearance ≤ 30mL/min” was removed under bridging criteria as these patients should be reviewed using the unfractionated heparin criteria and can continue to be reviewed there while receiving bridging or pretreatment • Bleeding criteria were updated to capture patients who have suspected bleeding • “Actual” was removed and “by history” was added to the HIT criteria since it often takes several days to confirm a diagnosis of HIT; therefore, patients with suspected or a history of HIT is appropriate for the use of this criterion |

| Pulmonary Embolism | | |
|--|---|---|
| Episode Day 2 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ♦Bronchodilator ≥ 4x/24h ♦Corticosteroid (includes inhaled) ♦Heart failure and, Both: <ul style="list-style-type: none"> ▶Dyspnea > baseline and, ≥ One: <ul style="list-style-type: none"> ♦O₂ sat ≤ 91%(0.91) and < baseline ♦Worsening peripheral edema ♦Heart failure confirmed by imaging ▶Intervention, ≥ One: <ul style="list-style-type: none"> ♦Diuretic, One: <ul style="list-style-type: none"> ▶≥ 2x/24h (includes PO) ▶At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: <ul style="list-style-type: none"> ♦Blood urea nitrogen (BUN) increased from baseline ♦Creatinine increased from baseline ♦Dialysis ♦Post Critical care ≤ 24h | <ul style="list-style-type: none"> ▶Dyspnea > baseline and, ≥ One: <ul style="list-style-type: none"> ♦O₂ sat ≤ 91%(0.91) and < baseline ♦Worsening peripheral edema ♦Heart failure confirmed by imaging ▶Intervention, ≥ One: <ul style="list-style-type: none"> ♦Diuretic, One: <ul style="list-style-type: none"> ▶≥ 2x/24h (includes PO) ▶At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: <ul style="list-style-type: none"> ♦Blood urea nitrogen (BUN) increased from baseline ♦Creatinine increased from baseline ♦Dialysis ♦Post Critical care ≤ 24h | <ul style="list-style-type: none"> • PO anticoagulation was removed as an intervention for HIT as there is not enough evidence to support the use of oral agents for the treatment of HIT • The end point of ≤ 5d previously attached to “unfractionated heparin, continuous” criterion was removed to allow patients to continue to meet due to a risk factor or until a therapeutic INR is achieved when transitioning to warfarin • “and” was added to “Persistent dyspnea” to clarify the intent of the criteria |

| Pulmonary Embolism | | |
|---|--|---|
| Episode Day 2 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> •CRITICAL, ≥ One: <ul style="list-style-type: none"> ▶Post thrombolytic therapy ≤ 24h ▶ECMO or extracorporeal life support (ECLS) ▶Hemodynamic monitoring, invasive ▶Mechanical ventilation ▶NIPPV ▶Oxygen ≥ 40%(0.40) ≤ 2d ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <ul style="list-style-type: none"> •CRITICAL, ≥ One: <ul style="list-style-type: none"> ▶Thrombolytic therapy, ≥ One: <ul style="list-style-type: none"> ♦Catheter directed thrombolysis scheduled or performed ≤ 24h ▶Systemic thrombolysis ≤ 24h ▶Post thrombolytic therapy ≤ 24h ▶ECMO or extracorporeal life support (ECLS) ▶Hemodynamic monitoring, invasive ▶HFNC and respiratory monitoring every 1-2h ▶Mechanical ventilation ▶NIPPV ▶Oxygen ≥ 40%(0.40) ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>Rationale:</p> <ul style="list-style-type: none"> • Thrombolytic therapy criteria were added to capture patients who have worsened and may require thrombolytic therapy on Episode Day 2 • “HFNC and respiratory monitoring every 1-2h” criterion was added to reflect the patients who are appropriate at the Critical level of care • Oxygen ≥ 40%” criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Critical level of care |

| Pulmonary Embolism | | |
|---|--|--|
| Episode Day 3-4 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>• ACUTE, One:</p> <p>► Responder, discharge expected today if clinically stable last 12h, All:</p> <ul style="list-style-type: none"> ♦ Anticoagulation regimen established with no evidence of bleeding or IVC filter ♦ Laboratory values within acceptable limits ♦ O₂ sat within acceptable limits or home oxygen arranged ♦ Complication or comorbidity, One: <ul style="list-style-type: none"> ► No complication or active comorbidity relevant to this episode of care ► Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> ♦ Bleeding controlled or resolved ♦ COPD exacerbation resolving ♦ Heart failure resolving ♦ HIT stable ► Pain controlled or manageable <p>► Partial responder, not clinically stable for discharge and requires continued stay, ≥ One:</p> <ul style="list-style-type: none"> ♦ O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ♦ Bridging anticoagulation and INR non-therapeutic and, ≥ One: <ul style="list-style-type: none"> ► Compliance risk, Both: <ul style="list-style-type: none"> ♦ Patient unreliable and unable to adhere to outpatient instructions or treatment ♦ Caregiver unavailable or unable to manage care ► Creatinine clearance ≤ 30 mL/min ♦ Hct or Hb decreased within last 24h and blood product transfusion ♦ HIT and anticoagulation (includes PO) ♦ IVC filter placement performed ≤ 24h ♦ Pain consistent with pulmonary embolism requiring analgesic ≤ 2d ♦ Unfractionated heparin, continuous ≤ 5d and, ≥ One: <ul style="list-style-type: none"> ► Antiphospholipid antibody syndrome ► Bleeding, active or increased risk ► Creatinine clearance ≤ 30 mL/min ► Morbid obesity (BMI ≥ 40 kg/m2 or weight > 120kg) ♦ Unfractionated heparin, continuous discontinued ≤ 24h ♦ COPD and, Both: <ul style="list-style-type: none"> ► Persistent dyspnea, ≥ One: <ul style="list-style-type: none"> ♦ Arterial Po₂ < baseline ♦ Arterial or venous Pco₂ > baseline ♦ O₂ sat ≤ 91%(0.91) and < baseline ♦ Increased work of breathing, ≥ One: <ul style="list-style-type: none"> ► Difficulty taking PO ► Prefers sitting | <p>• ACUTE, One:</p> <p>► Responder, discharge expected today if clinically stable last 12h, All:</p> <ul style="list-style-type: none"> ♦ Anticoagulation regimen, One: <ul style="list-style-type: none"> ► Established ► Contraindicated ♦ Laboratory values within acceptable limits ♦ O₂ sat within acceptable limits or home oxygen arranged ♦ Complication or comorbidity, One: <ul style="list-style-type: none"> ► No complication or active comorbidity relevant to this episode of care ► Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> ♦ Bleeding controlled or resolved ♦ COPD exacerbation resolving ♦ Heart failure resolving ♦ Pain controlled or manageable <p>► Partial responder, not clinically stable for discharge and requires continued stay, ≥ One:</p> <ul style="list-style-type: none"> ♦ UFH, continuous and, ≥ One: <ul style="list-style-type: none"> ► Antiphospholipid antibody syndrome ► Bleeding, active or increased risk ► BMI ≥ 40 kg/m2 or weight > 120kg ► Creatinine clearance ≤ 30 mL/min ► Pretreatment or bridging ♦ O₂ sat ≤ 91%(0.91) and < baseline requiring supplemental oxygen ♦ HIT and anticoagulation ♦ IVC filter placement performed ≤ 24h ♦ Pain consistent with pulmonary embolism and, One <ul style="list-style-type: none"> ► Analgesic ≥ 3x/24h, ≤ 2d ► Transition to PO analgesic ≤ 24h ♦ Pretreatment or bridging anticoagulation, All: <ul style="list-style-type: none"> ► Patient or caregiver unable to administer ► Outpatient services unavailable ► Intervention, One: <ul style="list-style-type: none"> ♦ INR non-therapeutic and anticoagulation (includes PO) ♦ Anticoagulation ≤ 5d since initiation ♦ Bleeding, actual or suspected and, ≥ One: <ul style="list-style-type: none"> ► Blood product transfusion ► Hct or Hb monitoring at least 2x/24h, ≤ 2d ♦ COPD and, Both: <ul style="list-style-type: none"> ► Persistent dyspnea and, ≥ One: <ul style="list-style-type: none"> ♦ Arterial Po₂ < baseline ♦ Arterial or venous Pco₂ > baseline ♦ O₂ sat ≤ 91%(0.91) and < baseline ♦ Increased work of breathing, ≥ One: <ul style="list-style-type: none"> ► Difficulty taking PO ► Prefers sitting | <p>Rationale:</p> <ul style="list-style-type: none"> • Responder criteria were updated to better describe the patients appropriate for discharge • Analgesic frequency of “≥ 3x/24h” was added to clarify the intent of the criteria • “Transition to PO analgesic ≤ 24h” was added to allow patients to meet for 24 hours while their oral pain regimen is established • “Bridging anticoagulation” was changed to “Pretreatment or bridging anticoagulation,” and criteria were updated to capture patients who require pretreatment with parenteral anticoagulation prior to starting dabigatran or edoxaban and to better define the patients who require hospitalization due to the inability to administer parenteral anticoagulation therapy at home • “Includes PO” was added to the “INR non-therapeutic and anticoagulation” line to clarify the intent of the criteria as it is intended for patients on an oral anticoagulation who have yet to achieve a therapeutic INR • “Anticoagulation ≤ 5d since initiation” was added to capture the patients who requires at least 5 days of anticoagulation prior to starting dabigatran or edoxaban and are unable to administer parenteral anticoagulation at home • “Creatinine clearance ≤ 30mL/min” was removed under bridging criteria as these patients should be reviewed using the unfractionated heparin criteria and can continue to be reviewed there while receiving bridging or pretreatment • “Pretreatment or bridging” criteria were added under “UFH, continuous” to capture patients who were started on a UFH infusion on a previous episode day so they can continue to be reviewed while they are receiving pretreatment or bridging to an oral anticoagulation agent |

| Pulmonary Embolism | | |
|---|---|--|
| Episode Day 3-4 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> ►Talks in phrases ►Use of accessory muscles ►Intervention, Both: <ul style="list-style-type: none"> ◆Bronchodilator ≥ 4x/24h ◆Corticosteroid (includes inhaled) ◆Heart failure and, Both: <ul style="list-style-type: none"> ►Dyspnea > baseline and, ≥ One: <ul style="list-style-type: none"> ◆O₂ sat ≤ 91%(0.91) and < baseline ◆Worsening peripheral edema ◆Heart failure confirmed by imaging ►Intervention, ≥ One: <ul style="list-style-type: none"> ◆Diuretic, One: <ul style="list-style-type: none"> ►≥ 2x/24h (includes PO) ►At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: <ul style="list-style-type: none"> ◆Blood urea nitrogen (BUN) increased from baseline ◆Creatinine increased from baseline ◆Dialysis ◆Post Critical care ≤ 24h | <ul style="list-style-type: none"> ►Talks in phrases ►Use of accessory muscles ►Intervention, Both: <ul style="list-style-type: none"> ◆Bronchodilator ≥ 4x/24h ◆Corticosteroid (includes inhaled) ◆Heart failure and, Both: <ul style="list-style-type: none"> ►Dyspnea > baseline and, ≥ One: <ul style="list-style-type: none"> ◆O₂ sat ≤ 91%(0.91) and < baseline ◆Worsening peripheral edema ◆Heart failure confirmed by imaging ►Intervention, ≥ One: <ul style="list-style-type: none"> ◆Diuretic, One: <ul style="list-style-type: none"> ►≥ 2x/24h (includes PO) ►At least 1x/24h and requires daily medication adjustment (includes PO) and, ≥ One: <ul style="list-style-type: none"> ◆Blood urea nitrogen (BUN) increased from baseline ◆Creatinine increased from baseline ◆Dialysis ◆Post Critical care ≤ 24h | <ul style="list-style-type: none"> • PO anticoagulation was removed as an intervention for HIT as there is not enough evidence to support the use of oral agents for the treatment of HIT • The end point of ≤ 5d previously attached to “unfractionated heparin, continuous” criterion was removed to allow patients to continue to meet due to a risk factor or until a therapeutic INR is achieved when transitioning to warfarin • Bleeding criteria were updated to capture the patients who have suspected bleeding • “Unfractionated heparin, continuous discontinued ≤ 24h” criterion was removed due to no clinical indication for the patients to remain hospitalized for an additional 24 hours after the infusion is discontinued. If the unfractionated heparin infusion was discontinued due to bleeding, the patient can be reviewed using the bleeding criteria • “and” was added to “Persistent dyspnea” to clarify the intent of the criteria |

| Pulmonary Embolism | | |
|---|---|---|
| Episode Day 3-4 (INTERMEDIATE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <ul style="list-style-type: none"> •INTERMEDIATE, ≥ One: <ul style="list-style-type: none"> ►Right ventricular dysfunction confirmed by echocardiogram ≤ 3d ►NIPPV ►Oxygen ≥ 40%(0.40) ≤ 2d | <ul style="list-style-type: none"> •INTERMEDIATE, ≥ One: <ul style="list-style-type: none"> ►Right ventricular dysfunction confirmed by echocardiogram ≤ 3d ►HFNC and respiratory monitoring every 3-4h ►NIPPV ►Oxygen ≥ 40%(0.40) | <p>Rationale:</p> <ul style="list-style-type: none"> • “HFNC and respiratory monitoring every 3-4h” criterion was added to reflect the patients who are appropriate at the Intermediate level of care • Oxygen ≥ 40%” criterion was updated with removal of the endpoint to clarify the intent of the criteria point, as the oxygen requirement is the driver that is appropriate for the Intermediate level of care |

| Pulmonary Embolism | | |
|--|--|---|
| Episode Day 3-4 (CRITICAL) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶Post thrombolytic therapy ≤ 24h ▶ECMO or extracorporeal life support (ECLS) ▶Hemodynamic monitoring, invasive ▶Mechanical ventilation ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>•CRITICAL, ≥ One:</p> <ul style="list-style-type: none"> ▶Post thrombolytic therapy ≤ 24h ▶ECMO or extracorporeal life support (ECLS) ▶Hemodynamic monitoring, invasive ▶Mechanical ventilation ▶HFNC and respiratory monitoring every 1-2h ▶NIPPV and, ≥ One: <ul style="list-style-type: none"> ♦FiO₂ > 50%(0.50) ♦Respiratory intervention every 1-2h ▶Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure) | <p>Rationale:</p> <ul style="list-style-type: none"> • “HFNC” criterion was added to reflect the patients who are appropriate at the Critical level of care |

| Pulmonary Embolism | | |
|---|--|--|
| Episode Day 5 (ACUTE) | | |
| Previously Available Content | 2024 March Release | Revision Description |
| <p>•ACUTE, One:</p> <ul style="list-style-type: none"> ▶Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦Anticoagulation regimen established or IVC filter ♦O₂ sat within acceptable limits or home oxygen arranged ♦Complication or comorbidity, One: <ul style="list-style-type: none"> ▶No complication or active comorbidity relevant to this episode of care ▶Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> ♦Bleeding controlled or resolved ♦COPD exacerbation resolving ♦Heart failure resolving ♦HIT stable ♦Pain controlled or manageable ▶Non-responder, not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, One: <ul style="list-style-type: none"> ♦Use Extended Stay subset ♦Refer for secondary review | <p>•ACUTE, One:</p> <ul style="list-style-type: none"> ▶Responder, discharge expected today if clinically stable last 12h, All: <ul style="list-style-type: none"> ♦Anticoagulation regimen, One: <ul style="list-style-type: none"> ▶Established ▶Contraindicated ♦Laboratory values within acceptable limits ♦O₂ sat within acceptable limits or home oxygen arranged ♦Complication or comorbidity, One: <ul style="list-style-type: none"> ▶No complication or active comorbidity relevant to this episode of care ▶Complication or comorbidity and clinically stable for discharge, ≥ One: <ul style="list-style-type: none"> ♦Bleeding controlled or resolved ♦COPD exacerbation resolving ♦Heart failure resolving ♦Pain controlled or manageable ▶Non-responder, not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, One: <ul style="list-style-type: none"> ♦Use Extended Stay subset ♦Refer for secondary review | <p>Rationale:</p> <ul style="list-style-type: none"> • Responder criteria were updated to better describe the patients appropriate for discharge |

Rhabdomyolysis or Crush Syndrome



Revision Summary

Calcium chloride or gluconate criterion was added as an intervention to the Acute kidney injury criteria at the Critical level of care.

Rhabdomyolysis or Crush Syndrome

Global (CRITICAL)

Previously Available Content

- ▶ Acute kidney injury (AKI) and, **Both:**
 - ♦ Finding, **≥ One:**
 - ▶ Potassium > 6.0 mEq/L (6.0 mmol/L) and, **≥ One:**
 - ♦ Loss of P wave
 - ♦ Multifocal PVCs
 - ♦ Neuromuscular deficit
 - ♦ Widening QRS
 - ▶ Sodium, **One:**
 - ♦ < 120 mEq/L (120 mmol/L) and volume overload
 - ♦ > 160 mEq/L (160 mmol/L)
 - ♦ Intervention, **≥ One:**
 - ▶ IV fluid resuscitation ≤ 2d
 - ▶ Diuretic, continuous
 - ▶ Dialysis, **One:**
 - ♦ Hemodialysis ≤ 7d since initiation
 - ♦ Peritoneal dialysis ≤ 7d since initiation

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- ▶ Acute kidney injury (AKI) and, **Both:**
 - ♦ Finding, **≥ One:**
 - ▶ Potassium > 6.0 mEq/L (6.0 mmol/L) and, **≥ One:**
 - ♦ Loss of P wave
 - ♦ Multifocal PVCs
 - ♦ Neuromuscular deficit
 - ♦ Widening QRS
 - ▶ Sodium, **One:**
 - ♦ < 120 mEq/L (120 mmol/L) and volume overload
 - ♦ > 160 mEq/L (160 mmol/L)
 - ♦ Intervention, **≥ One:**
 - ▶ **Calcium chloride or gluconate**
 - ▶ IV fluid resuscitation ≤ 2d
 - ▶ Diuretic, continuous
 - ▶ Dialysis, **One:**
 - ♦ Hemodialysis ≤ 7d since initiation
 - ♦ Peritoneal dialysis ≤ 7d since initiation

Rationale:

- Calcium chloride or gluconate criterion was added as an intervention to the Acute kidney injury (AKI) criteria to better describe the patient appropriate at the Critical level of care

Revisions to medical codes

| Subset Description (Guideline Name) | CPT/HCPCs | Revision |
|-------------------------------------|-----------|----------|
| General Surgical | 22836 | Add |
| General Surgical | 22837 | Add |
| General Surgical | 22838 | Add |
| General Surgical | 27278 | Add |
| General Surgical | 31242 | Add |
| General Surgical | 31243 | Add |
| General Surgical | 33276 | Add |
| General Surgical | 33277 | Add |
| General Surgical | 33278 | Add |
| General Surgical | 33279 | Add |
| General Surgical | 33280 | Add |
| General Surgical | 33281 | Add |
| General Surgical | 33287 | Add |
| General Surgical | 33288 | Add |
| General Surgical | 52284 | Add |
| General Surgical | 58580 | Add |
| General Surgical | 61889 | Add |
| General Surgical | 61891 | Add |
| General Surgical | 61892 | Add |
| General Surgical | 64596 | Add |
| General Surgical | 64597 | Add |
| General Surgical | 64598 | Add |

| Subset Description (Guideline Name) | CPT/HCPCs | Revision |
|-------------------------------------|-----------|----------|
| General Surgical | 0404T | Delete |
| General Surgical | 0424T | Delete |
| General Surgical | 0425T | Delete |
| General Surgical | 0426T | Delete |
| General Surgical | 0427T | Delete |
| General Surgical | 0428T | Delete |
| General Surgical | 0429T | Delete |
| General Surgical | 0430T | Delete |
| General Surgical | 0431T | Delete |
| General Surgical | 0432T | Delete |
| General Surgical | 0433T | Delete |
| General Surgical | 0434T | Delete |
| General Surgical | 0465T | Delete |
| General Surgical | 0499T | Delete |
| General Surgical | 0784T | Add |
| General Surgical | 0785T | Add |
| General Surgical | 0786T | Add |
| General Surgical | 0787T | Add |
| General Surgical | 0790T | Add |
| General Surgical | 0813T | Add |
| General Surgical | 0816T | Add |
| General Surgical | 0817T | Add |
| General Surgical | 0818T | Add |
| General Surgical | 0819T | Add |

| Subset Description (Guideline Name) | CPT/HCPCs | Revision |
|-------------------------------------|-----------|----------|
| General Surgical | 0823T | Add |
| General Surgical | 0824T | Add |
| General Surgical | 0825T | Add |
| General Surgical | 0861T | Add |
| General Surgical | 0862T | Add |
| General Surgical | 0863T | Add |
| General Surgical | 0864T | Add |

Review and Incorporation of Recent Medical Literature

Optum, Inc. is committed to keeping the InterQual product suite current and accurate. Criteria are continually reviewed and updated, with new editions of every product released at least annually. Optum's staff of physicians, nurses, other licensed healthcare professionals, and its extensive array of primary care and specialty consultants participate in ongoing criteria revision as new medical information emerges. Each release of the criteria reflects a thorough review of new medical literature, society guidelines, current practice standards, and incorporation of expert clinical consultant and user feedback.

Additional Resources

InterQual® Resource Center

The [InterQual Resource Center](#) is a central location for the most up-to-date InterQual clinical documentation and resources.

The Resource Center provides access to:

- What's New—Includes release highlights for each of the InterQual® Criteria modules included in a content release.
- Clinical Revisions—Includes the clinical revisions for the current InterQual Criteria year and two years prior.
- Clinical Resources—Includes information such as the Knowledge Articles, the Known Issues List, and more.
- Webinars—Includes the Increase Your IQ educational webinar recordings.
- Additional Resources—Includes access to resources such as Customer Care Hub, the InterQual® Learning Source, and Download Connect.

Accessing Customer Care Hub

[Customer Care Hub](#) is a web portal that enables you to submit, update, and view support case details and status.

To obtain a user ID and password, from the Customer Care Hub Welcome page, select Request Account.

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|--|---|
| Merck Manual - MD Calc | https://www.merckmanuals.com/medical-calculators/CommunityAcqPneumonia-ja.htm |
| Social Security Actuarial Life Table (I) | https://www.ssa.gov/oact/STATS/table4c6.html |
| Fracture Risk Assessment Tool (FRAX) | https://www.sheffield.ac.uk/TBS/ |
| Breast Cancer Risk Assessment Tool | https://bcrisktool.cancer.gov/ |
| BRCAPRO | https://projects.iq.harvard.edu/bayesmendel/brcapro |
| CanRisk (BOADICEA) | https://canrisk.org/about/ |
| UNC | https://www.med.unc.edu/mfm/nips-calc |
| Marfan Foundation | http://www.marfan.org/dx/zscore |

| Third Party | Link to Calculator |
|-------------------|---|
| Marfan Foundation | https://marfan.org/dx/score/ |